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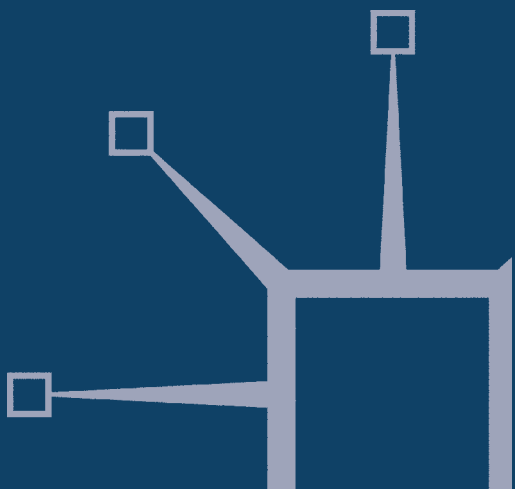
Medicine, Madness and Social History

Essays in Honour of Roy Porter

Edited by

Roberta Bivins and John V. Pickstone



Medicine, Madness and Social History

Also by Roberta Bivins

ACUPUNCTURE, EXPERTISE AND CROSSCULTURAL MEDICINE

ALTERNATIVE MEDICINE?: A History

Also by John V. Pickstone

WAYS OF KNOWING: A New History of Science, Technology and Medicine

COMPANION TO MEDICINE IN THE TWENTIETH CENTURY (*editor with Roger Cooter*)

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and

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To Roy Porter
'for he wrote books *de omni scribili* …'

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Acknowledgements

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Introduction: '*De Omni Scribili*'

John V. Pickstone and Roberta Bivins

This book is a tribute to a historian of rare ability, fertility and generosity: a grammar school boy from working-class London who became Britain's authority on the histories of the Enlightenment and of madness; a Stakhanovite scholar, teacher and editor who became a media star; a Bohemian of astonishing efficiency who freely gave encouragement and assistance to students and colleagues, and to amateurs and the public at large. It was not hard to find willing contributors to this volume, for many historians grieved at his early death as we also treasure his example and are grateful for his rich gifts. This volume's authors, like its editors, were Roy's colleagues, collaborators and students; the collection represents some of his interests and approaches, and it illustrates the impact of his work on the fields which he enlivened. It is, therefore, also an exploration of what it means to be a 'Porterian' social historian.

As many of our contributors note, Roy had no ambitions to found a historical school or system. His was an embracing and flexible intellect, always willing to support the work of others, even when its subjects or approaches were outside his own wide range. So what does characterize his work and that of his collaborators and students? In part, it is an enthusiasm for the individual, the experiential and the local – for the people, places and things of an historical moment, especially for those which had been submerged or disregarded as tangential to the sweep of history. He wrote about the mad when few had done so, and about the Enlightenment of England rather than of the Continent. When medical history was mostly about doctors, he wrote about patients and their views. But although Roy took a magpie's delight in the stories turned up by this approach, they were not just adornments of the text. In his history, the striking stories did the heavy lifting and made the theoretical load of social history seem light. They exemplified the historical impact of individual (though never isolated) choices and lives – not just those of 'great men' but of ordinary men and women. In turn, the appeal of the stories made the histories accessible to all.

The chapters in this volume cover topics ranging from Spanish anarchism to Hungarian hypnotism. The casts and periods are similarly diverse: begrimed gardeners jostle with grinning dentists in settings from the seventeenth to the mid-twentieth centuries. Some of our authors ask big questions in small places: can the suburbs of south London, Porter's home ground, tell us why blood itself did not become a commodity in Britain? Or what did Spanish anarchists and British working-class utopians have in common (the answer is sex), and why? Other essays take key present-day issues – the applicability of the insanity defence; the shaping of access to science, and social and individual responses to disability – and place them in rich and revealing historical contexts. One way or another, they all combine characteristics from Roy's work. We hope they will appeal to the varied audiences that he reached so well.

We begin with an essay by Hal Cook, who came from Wisconsin to follow Roy as Director at the Wellcome. Examining the themes and style of the Porter corpus, Cook traces a central concern with the ideas of the Enlightenment as they were embodied in the practices of individuals – great and small, high and low. He notes that Roy was not much interested in the theory of historical writing or in models of society; instead, he valued the history of medicine, and especially the history of madness, as a tool for exploring the variety of lives and the richness of their interactions, both social and commercial. Roy loved eighteenth-century England for its variety and openness: the ranks could mix and merit could rise. He was not blind to power and class, and recognized the enormous contributions of E. P. Thompson and colleagues on the histories of oppression and resistance; but Porter's world was the haunts of London rather than the factories and farms, trading houses or government offices. Cook is right that beneath Porter's capacious bed there was no ideological Red. But if Porter did not write politics explicitly, his sympathies were well known. He had grown up in post-war, inclusive Britain, with its model welfare state and an educational system that encouraged and supported him to the highest level. He sought to repay that good fortune; egalitarian generosity was built into him; and by his standards, the Thatcherite (and Blairite) reaction was both controlling and mean.

In the next part of the volume, we move outward from Roy himself to address his impact as a historian and his approaches to the sciences of history and the politics of science. We turn first to Michel Foucault – another historian of madness, medicine and much more – who was in curious ways Roy's opposite number in France. Foucault was a formalist and an ideologue of the left, who became known for his rereading of 'moral' psychiatry as extending professional control from the body to the mind. Roy was not unsympathetic to the anti-psychiatry movement, but his realism tempered his suspicion of professional medicine and he worked hard to develop a history of psychiatry that would satisfy doctors as well as social historians. For him, Foucault's stimulating history could do neither – partly because the

notion of a grand confinement from the seventeenth century simply did not fit the British case. But what of the other great work of Foucault on medicine – *The Birth of the Clinic* – a book which has remained foundational for the social history of medicine because it analysed the foundations of modern medicine, again in post-revolutionary Paris? In our second essay, Adrian Wilson contrasts their two approaches to the histories of clinical examination and pathological anatomy, which together constituted the clinical gaze. Foucault found a massive discontinuity in the hospitals of Paris; for him, Xavier Bichat's concept of 'tissues' gave a new geography of the body by which clinical signs could be linked with postmortem findings, so that both achieved a previously unknown significance. But Porter stressed continuity. For him, pathological anatomy was an old idea, well realized by Morgagni; nor were the French the first to use percussion to examine the chest. Porter, one might say, was bound to see continuity because in his rather traditional model of scientific medicine, pioneers created novelties which by accumulation and interaction changed practice over time. This is the Porter who wrote general surveys of medicine, and especially *The Greatest Benefit of Mankind*, a work which is indeed much more 'progressivist' and far less social than admirers of Porter's other work had expected. Wilson resolves the tension between these two views by focusing on the practical difficulty of the programme which Morgagni and others may have envisaged, but which they were not in a position to realize.

Peter Burke has worked on Italy and on the principles of historical writing, and here he helps explain the history of history, more specifically the history of social and cultural history in eighteenth-century Europe. That history should not be about battles and kings, but should focus on common people, working lives and material culture, is not just a recent cry; it was common in the mid-eighteenth century. It seems to have stemmed from Naples, an exuberantly ungovernable city and the birthplace of Vico's pioneering work on the history of cultures. Resounding northwards, the call for a history of the social was answered by the *philosophes* in France, and by the teachers and writers of the Scottish universities. Burke links it with antiquarianism and other forms of natural history, and with delight in the collecting of objects and tales, exotic or merely quaint. But it had wider intellectual ambitions that separate it from more recent cultural history. For the *philosophes*, and especially the Scots, these histories of ordinary life were part of attempts to analyse the workings of civil society, and especially its stadial development from hunter-gatherers, through settled agriculture, to towns and trade. This enterprise linked easily with contemporary 'anthropology' and, of course, it was progressive: traditions and superstitions were interesting in part because they were being left behind. In mapping these connections, Burke deepens our understandings of the self-consciousness of the Enlightenment; but he also explores the reasons for the emergence of these ideas and finds part of the explanation in the widening of reading publics, and especially the advent of women as consumers of social, as well as natural, history.

So what enabled radicals in the 1970s and 1980s to feel that they were creating history anew? Here Burke points to Ranke and the nineteenth-century Germans: what really happened to history was the nation and its archive. Historiographical hegemony passed from social studies to constitutional history and philological studies, to systems of politics and languages rather than civil society in all its generality. Around the end of the empire of German scholarship, before and after the Great War, cultural history was reinvented by Karl Lamprecht and others. And after the Second World War, Britain picked it up from the United States and the *Annales* School in France. In the 1960s and 1970s, such history still had a directionality – celebrating the common people, and the fact that common people (like Porter) could now do history with the best of the rest. Cultural history is stronger than ever now, thanks especially to English literature studies, but that sense of liberation, where it exists, seems to have been privatized or restricted to gendered or sexual relations.

In Ian Burney's essay, we again circle round the differences between the eighteenth and nineteenth centuries, and the way that Porter saw them. Why, Burney asks, did Porter seem unsympathetic to Thomas Wakley – political radical and medical reformer in Regency and early Victorian London? Was Wakley not wonderfully outspoken and witty as a journalist and a critic of the *ancien régime* in medicine? Did he not address a host of Porterian issues, from coroners and graveyards to medical science? Does he thus not offer as remarkable a window on early nineteenth-century medicine and society as, for example, did Beddoes at the end of the eighteenth? Yes; but if Wakley was a reformer, heart and soul, he was also a man of systems and party. The medicine he championed was that of Revolutionary France, which sought universals in corpses and disregarded the individuality of the patient and the doctors. Something was being lost, something in which Porter specialized and revelled – the idiosyncrasies of bodies and minds; the particularities of constitutions, whether of man, place or state; the coffee-tables of medicine, which the autopsy tables seemed to threaten. On the surface, then, Wakley's medical world was not Porter's – systems, whether in politics or in medicine, did not attract Roy's sympathy; and his sensitivities as a historian help demarcate for us the structural changes that he did not care to analyse. Yet as Burney goes on to argue, a closer examination of Wakley's medico-political life as a whole not only renders Wakley's world more accurately, but also reveals the power of Porter's individualistic approach as a tool for nineteenth- as well as eighteenth-century studies.

Even for radicals of Wakley's stamp, democratic openness had its limits; these are further illuminated by Dorothy Porter's essay on Brougham and Birkbeck. A historian of public health, Dorothy Porter joined with Roy in explorations of patients' histories and medical markets. Later, she worked at Birkbeck College, London, named for the chief originator of the Mechanics Movement, which from the 1820s sought to educate artisans in the principles

of science. Her essay reflects on that radical tradition, from its inception through to the mid-twentieth-century popular writings of J. D. Bernal, the Irish crystallographer and historian (and Birkbeck's best-known scientist). She contrasts two traditions from the Age of Reform to the present: one which sought to reform and modernize the social elite by scientific education and propaganda; and the other which was embodied and advanced by practitioners of scientific occupations who saw their own struggles for power as part of a more general democratization of society. Babbage represented the first tendency, though he was perhaps rather to the left of most of his Cambridge allies in the nascent British Association for the Advancement of Science. By contrast, Birkbeck, like Wakley, was a doctor determined to end the *ancien régime* in his profession. Medicine was to be meritocratic and democratic; and as Porter shows, democratic practitioners celebrated naturalistic accounts of man and society based on skills and technical improvement. But in education, as in medicine, the spectrum of reform extended leftwards far beyond the professional classes; popular medical movements extended the new empiricism, ranking the experiences of the patient above the supposed knowledge of professions, while guilds of craftsmen tried to protect their secrets from the new professionals. The new medics wanted to open up the profession – but only to the educated. The founders of Mechanics Institutes wanted education for the workers, but they worried when workers organized their own schools.

Dorothy Porter's concluding pages follow the contestations of science and democracy to the later twentieth century. For Bernal and his friends, science and technology were the keys to ending the interwar depression, to winning the Second World War and the development of a new world order. That vision remains powerful, and the 'public understanding of science' is now central to the agenda of British scientific organizations. Babbage would be pleased, but what of Birkbeck's wider claim that a truly democratic science should start with the wishes and skills of the people? Here Porter invokes the counter-culture of the Frankfurt School and of Habermas – rooted in the phenomenology of the everyday, and in communicative rather than instrumental rationality. This is the culture which sees 'Science' as an establishment, and which looks now to non-governmental organizations to represent the public interest world-wide.

But when we look for examples of communicative societies, the coffee houses of the eighteenth century reappear from behind the disciplines of the nineteenth century, and we are back again among the forms of knowledge of which Roy Porter, rather than Foucault or Bernal, was the historian extraordinary. So in this volume's second part, we turn to the Enlightenment, starting with an essay by one of Roy's close contemporaries.

Colin Jones is best known for his work on the political, social and medical history of France. Here he offers a bite of his work on French dentists, and

how they bridged the Channel. It is Porterian in its exploration of markets and material culture, of self-creations and presentations. The differences between Britain and France in technology, luxury trades and medical regulation are here encapsulated in porcelain teeth. French dentists had broken away from the barber-surgeons to become a sub-profession that prided itself on technical advance and preventive measures. Their (rich) French patients could afford broad smiles; so too those in Britain who patronized the French dentists who had arrived as Huguenots or Revolutionary émigrés. But the native British dentists remained as tooth drawers, and their patients' teeth continued to rot, even as the cartoonists poked fun at French mouths. But then, soon after the Revolution, the supply of French dentists seems to have dried up when new medical regulations failed to give them a place in the educational system for French professions. By the mid-nineteenth century, Europeans were beginning to look to America – to a consumer society which valued technical fixes (including ether – that Yankee dentist's dodge which became surgical anaesthesia).

With Anne Secord we visit another aspect of this commodity society: gardeners and the (forced) fruits of their expertise. The English were known for their natural, irregular gardens, which contrasted with the gardens – and the teeth, it would seem – of the French. Here, however, we explore another way of bending nature: the production of cucumbers out of season. In summer, they were the food of the poor, cold fruits for hot times; but in spring or winter, when they did not suit (or naturally survive) the season, they were an occasion for boasting about the skills of one's gardener and the resources one could expend on unseasonal luxuries. Early cucumbers were a rich man's vicarious quest and as such offer an insight into the dynamics of luxury cultures. (A century later, among the artisans whom Secord has also explored, growing extraordinary produce had become a sign of the assiduity and skill which the respectable working classes could bring to bear on their own hobbies.) But the forced cucumber of the Enlightenment roused more than the competitive urges of its aristocratic patrons and artisanal producers. It roused the ire of medical practitioners who knew that one should only eat cold fruits when the weather was hot; they despaired of a clientele determined to exult in delicacies which damaged both the physical and economic health of the nation. Thus, the commodified cucumber reveals the interplays between understandings of the body, shared by professionals and laity alike, and of the social order which emerged from such conspicuous consumption.

From commodities we turn to men, and to two studies which depend on that characteristic tool of social history, the prosopographical analysis of institutional records. Neither, however, allows analysis or interpretation of the group to drown out the recovered voices or erase the idiosyncrasies of individuals. Geoff Hudson has worked on the books of the Chelsea Hospital and on the records of the tribunals which judged the claims of would-be

military pensioners. He reveals a body of men who were keen to secure what they took to be their rights, earned by service and by suffering. Unlike the subservient patients portrayed in many medical histories, they were not above importuning and exaggeration. They would work when they could, but not as if their dignity depended on it. In this respect, Hudson highlights a tradition of 'bolshie' patients, which is recurrent across the centuries. But the records also reveal secular changes that cast light on medicine more generally. In the late seventeenth century, the records are patients' tales, where illness appears as humoral disturbance; by the end of the eighteenth century, military doctors are being quoted, and their language is the shorthand of disease categories. The arguments here link nicely with the work of Hal Cook and of Mary Fissell, and with the arguments over Foucault and the birth of the clinic. As Cook suggests, the employment of doctors as agents of power may tend to introduce a technical vocabulary which does not necessarily correspond to the patients' own claims. This lays the groundwork for more analytical and anatomical accounts by surgeons who were getting used to looking into innards.

Illustrating the power of a history incorporating both attention to individuals and prosopography 'from below', Akhito Suzuki gives a nice answer to a problem made evident by the quantity of writings on women and psychiatry. If women suffered from – or at least were diagnosed with – hysteria as a characteristic form of madness, what did men have, and why? Or are we, too, assuming that men were the ground against which the peculiarities of women were portrayed by male doctors? We already know that such questions cannot be understood independently of class. In the Great War, for example, while officers' shell shock required talking cures from fellow officers, the ranks were diagnosed with simpler diseases, treatable by shock therapy. Suzuki has looked at working men in Britain's Hanwell asylum in the mid-nineteenth century. He has drawn on a unique set of records compiled by doctors who interviewed the families of patients, eliciting accounts of how and why the patients became ill enough to require confinement. In many cases, the causes were attributed to peculiarities of the patient, but the majority were thought to be suffering from the effects of their environment. Suzuki nicely connects his account with histories of public health in that period: the confined men were worn out by work or worry; they could no longer take the strain of battling for a living for their families, when incomes were meagre and insecure. They paid the price of economic development, perhaps of urbanization, and of new expectations about gender and respectability. That some doctors denied these readings, preferring organic causes or an aetiology rooted in assumptions of working-class vice brings us to a central question for the next two essays: the relations between the biomedical sciences and political critiques of existing society.

Lesley Hall, surveying British writings on sexual utopias, notes how Edward Carpenter, Havelock Ellis and Stella Browne distanced themselves from

biomedical norms as well as the social arrangements which those norms underpinned and justified. Carpenter, having withdrawn from the Church, advocated the simple life and the possible role of homosexuals as 'the advance guard of that great movement which will one day transform the common life by substituting the bond of personal affection and compassion for the monetary, legal and other external ties which now control and confine society'. For Carpenter, as for his friend Havelock Ellis (who gave up medicine to study sexology), the sexual varieties of mankind were to be cherished. Like her older comrades, Browne was suspicious of contemporary medicine, not least the eugenics which she attacked as class-bound and scientifically simplistic. For all these reformers, and for many others whom Hall introduces, a new age of health demanded the reordering of society, including the sexual roles. These 'sandal-wearers', demonized by George Orwell, undoubtedly fed the public politics of state reform; but Hall notes too how the contemplatives also provided the texts from which individuals of several twentieth-century generations, and of several orientations, gained both sexual enlightenment and new sensibilities.

In Spain, the battle-lines were somewhat different because the Catholic Church was so much more powerful there than the divided establishments of British religion. As Katharina Rowold shows, the anarchists on whom she concentrates certainly sought the authority of science, but only if it was cleansed of the gender prejudices brought about by centuries of Catholicism. Anarchist feminists settled for a biology of sexual equality, of difference and complementarity; but for some, these true relations would only become clear in a new society, where women could 'freely satisfy their wishes and needs as beings who feel and think'. Perhaps this would be a slow process, for they, like their less radical fellow travellers, feared that evolution under existing condition might have produced women who were, in fact, inferior. In this chapter, as in Hall's, one sees only too well that social struggle necessitated an engagement with biology and medicine as authoritative systems. Scientific texts had now to be debated in a way that could be compared with older contests over biblical texts. Crucially and correctively, Rowold and Hall's close focus on individuals shows that neither the debates nor the debaters presented 'science' as a monolith or juggernaut; and we can learn from that as we debate similar issues now.

The last chapter in this part, by Kim Pelis, is temporally somewhat removed from the rest. But geographically and thematically, this is Porter territory. It is a story of social organization and charity in south London between the wars, and in the Millwall of his post-war childhood Roy would have known similar philanthropists. It concerns the origins of the national Blood Donor service, which is commonly attributed to the Second World War and the social solidarity that extended to the new National Health Service. But Pelis has looked closer and discovered, in true Porterian fashion, the work of a local *lay* organizer, Percy Lane Oliver, whose idiosyncratic but highly

effective service served as a model, first for the Red Cross and then for the state. The powerful patient strikes again, but this time in the middle of authoritative medicine's 'golden age'.

Blood donation was first organized at the end of the Great War, when hospitals began to look for civilian ways of obtaining blood for the transfusions which were becoming technically possible and medically attractive. Oliver, a local government official who ran the Camberwell Red Cross organization in his spare time, was also responding to the war's end. He was looking for new civilian roles to maintain his group, and he settled on assistance to his local hospital. Despite – or perhaps because of – a falling-out with centralizing bosses in the London division of the Red Cross, Oliver created an unpaid donor service that contrasted sharply with those taking shape in the US. So when the Medical Research Council contacted the London Red Cross to help organize a service, it was to the estranged Oliver that they turned. There is much else here about national differences and elites, not least about the Keynes brothers – economist and surgeon – but in essence, this is social innovation from below. Titmuss may have got the history wrong when he wrote of a state blood transfusion, but the spirit he depicted in *The Gift Relationship* was surely correct and not far removed from that 'great' and 'compassionate' movement to which Edward Carpenter had looked.

In this volume's last part we turn to the branch of medical and social history that Roy made especially his own: the history of madness and psychiatry. The first three chapters all portray events, but not as turning points in history (though they were certainly crucial for the individuals involved). Rather, these accounts illustrate the ways in which certain striking happenings have been construed – and reconstrued – in terms of the medical theories, and the common sense, of particular times and places.

Mary Lindemann is best known as a historian of Hamburg; here, she explores several sensational legal cases in that city, each of which hinged on claims of madness. The murders attracted wide attention; they thus illuminated many aspects of a civic life that in Hamburg was peculiar for its republicanism, the formal equality of the (male) burghers and their sturdy refusal to give precedence to expert opinion. Lindemann details the processes, precedents and qualities through which madness was assessed, and its legal shelter granted or denied, both before and after the medical and social changes of the eighteenth century. She finds an intertwined moral, municipal and medical tale: paradoxically, in an age of enlightened medicine and of Enlightenment confidence in the perfectibility of man, mad acts and mental disturbance could not be taken as proof of *sufficient*, disabling madness. Civic responsibility extended to self-control; so civic virtue would be threatened if a moral failure of mind was to excuse murder. Moreover, in Hamburg, claims to the increased authority which was elsewhere granted to medical experts provoked both popular and magisterial suspicion: was not the doctors' newly complex and nuanced picture of madness a threat to lay

understanding of free will itself? Thus Hamburg's battles over the insanity defence reveal broader tensions over how to balance expertise with equality and over who should assess conditions situated at the boundaries of mental responsibility, where the leading edge of medical power encountered the solid and particular substance of legislative and civic order.

The 'event' that engages Chandak Sengoopta's attention had a happier outcome, at least for its principal: his chapter examines the famous 'breakdown' of John Stuart Mill in 1826. At the age of 20, after years of study, Mill was in a 'dull state of nerves'. He had fortuitously, but rashly, wondered whether all the social reforms which he had been brought up to pursue would in fact make him happy. Realizing that the answer was no, he lost confidence and hope, but continued with his work. When he chanced to read a touching tale of the death of a father and the devotion of a son he wept – and learned that he had a sensitive heart as well as a powerful head. It was the beginning of Mill's reorientation towards a less utilitarian, more Romantic view of life. Sengoopta explores changing interpretations of Mill's breakdown, particularly the uses of his case by the nineteenth-century neurophysiologist/psychologist Alexander Bain, and by the recent psychoanalytical historian Bruce Mazlish. The former diagnosed overwork, as he did for most breakdowns; the latter an oedipal complex. That both notions are now generally discredited, according to Sengoopta, underlines the dangers of explaining historical events by psychological (and, by extension, medical) theories – better to stick to the accounts given by the actors and with the common sense that historians can share with their readers.

The third case is one of hypnosis in a Hungarian castle. As Emese Lafferton relates, the hypnotic subject, the daughter of the castle, and famous as a sensitive, first challenged medical authority by diagnosing the illness of a distant patient in wonderfully technical terms – and then she died. The lay hypnotist, a well-known showman, was cleared of her death, largely because the family stood by him and because the much-loved local doctor was a supporter of hypnosis. But such was the tumult and discord among Hungary's leading specialists in the face of this shocking event, and such the worry among doctors and governors about the social impact of widespread abuse of hypnosis, that the government not only banned its use by laymen, but uniquely allowed doctors to use it only therapeutically – so excluding research or demonstration. Lafferton, like Lindemann, uses local records to transport us to a particular place and time, and then traces the networks and influences connecting that specific locale to its wider global context. She illustrates how a deeply provincial setting – almost archetypally 'peripheral' – could powerfully influence the established 'centres' of medical and political power. And like Lindemann's study, this history illuminates the vexed question of responsibility and the will. Who was responsible for the safety of the hypnotic subject and for preserving personal health and virtue? Conversely, who was responsible for the safety of a society endangered by

hypnotic action? In other words, who was responsible for preserving civic order? Of course, the answers to such questions depended heavily on attitudes towards the hypnotic phenomenon itself, and Lafferton pays careful attention to international medical and scientific efforts to establish or to undermine its authenticity. Again, ultimately, Lafferton demonstrates that it is the interplay between civil exigencies and scientific claims to authority, and between local and international cultures that determines particular outcomes. From such fine-grained analysis, she argues, more nuanced models of large-scale historical processes will emerge.

The last two chapters take a different tack, moving from specific events to currents of theory and the cultures of theorists. The theme of responsibility, however, remains central to Daniel Pick's chapter. Best known as a historian of degenerationism, Pick here explores reactions to Freudian psychoanalysis in interwar Britain. Hostility and concern, he shows, again focused on the question of moral responsibility – in G. K. Chesterton's terms, did Freud simply offer the latest set of excuses for bad behaviour? In this respect, paradoxically, Freud stood with the sciences he had renounced: with evolutionary materialism and its neurological models, with degenerationist descriptions of diseases of the will, and with hypnotism. For all their differences, and their variations, these potent accounts of humanity offered *causes* for troubling behaviour; and whenever there was a cause, there was some question as to individual responsibility. We come back to the lessons of the other chapters in this part. To those who sought, for scientific, medical, literary or personal reasons, to classify and dissect behavioural abnormalities, explanatory theories gave insight; they linked analyses of individuals with wide views of culture and history, and with individual and social therapeutic practices. By giving rational insight into the self, they offered a way of extending, rather than diminishing, the reach of reason. And yet hypnosis, eugenics, neurology and Freudian analysis were minority pursuits. They were countered by approaches which derived from religious teachings, and by legal and political traditions which stressed individual autonomy before God and the law. And from these perspectives, causal accounts looked dangerously like escape mechanisms by which individuals could place themselves beyond social or legal sanction.

In the final essay, Mark Micale reflects on the variety of connections between science, medicine, philosophy and the arts. For the eighteenth century, one could take such interplay for granted. Indeed, as Roy showed, we might better think of 'medical-literary' worlds, where individuals were not compartmentalized. But what of later times, including ours, when the 'two cultures' of C. P. Snow seem an acceptable starting point for most discussions of intellectual configurations? Micale, as a leading historian of hysteria, has read across many genres and classes of author, especially for the later nineteenth century, and especially for France. For that period, he is sceptical of the two-culture model; here he exemplifies his argument by a rich

discussion of medical men, philosophers and *litterateurs* who cross-dressed, as it were, and communicated prolifically and enthusiastically. He links this rich *fin-de-siècle* mix to the decline of positivism and to the rising conviction that literature and art, religion and the paranormal were to be explained, but not explained away. More generally, he discounts the standard accounts of culture separation in the nineteenth century, suggesting that we have taken too much notice of Wordsworth and his fellow Romantics. We should recognize the partialness and partiality of their vision.

What, then, of the later twentieth century? Was Snow right, or should we look for signs that the more integrated – or at least interwoven – culture of the *fin de siècle* persisted? In concluding, Micale sides with Snow, pointing to the changes in education, the separation of arts and sciences in schools and universities, and the massive professionalization of science since the Second World War. He exemplifies this pessimistic argument with the case of contemporary psychiatry, seeing its present emphasis on biological theory and chemical treatments as one product of the rise of science. But in all such estimates, what you see depends on where you look, and with what intent. It is true that biological psychiatry has ascended since the 1970s, and that psychoanalysis is at a low ebb. But it is also true that cognitive therapies are now commonplace, and that counselling is a major industry. Genetics is one of the excitements of the present, but its messages are not the preserve of scientists alone; patients, lay observers and many kinds of experts help staff the panels which decide the public consequences. Scientists, at least in Britain, participate in public discussions, and there is some engagement; it is not all propaganda.

That is indeed one reason why Roy Porter was so well known. There is intense public interest in science and medicine, if not in the technicalities. Publics want to know about their diseases and the woes of their societies. They find historical explanation accessible and interesting, partly because history works at the level of common knowledge but includes the esoteric. The history of medicine, it could be argued, is no historiographical ghetto; it is rather central to a culture in which sciences and medicine are integral, not isolated or hermetic. There is a sense in which a Habermasian culture of communication is now ascendant over more technocratic accounts of sciences, and as we noted above, that brings us back to the salons, the coffee houses and the world of Porter.

The volume closes with a tribute to Roy from Bill Bynum, his close colleague for over three decades. They were postgraduates together in Cambridge, and later it was Bynum who appointed Roy to the Wellcome Institute for the History of Medicine to which he remained deeply attached to the end of his life. They were a formidable pair, Bynum the Director and Porter the prolific: many jointly edited volumes stand witness to their collaboration. And after the Wellcome Trust had sought a change of direction, Roy gave a year as Director, helping to launch a new regime. He did it

very well: his honesty, openness, generosity and wisdom were as obvious as his knowledge, imagination and wit. But this was not how Roy wanted to spend his time. He took early retirement in September 2001 and moved to Hastings St Leonards with Natsu Hattori. There he wrote and gardened; and it was there he suddenly died in March 2002.

No book can bring Roy back or fully capture his exuberance and his influence. He was a man who wrote and wrote well (as our epigraph and this Introduction's title suggest) about everything he saw. In his absence, we offer this collection from a variety of his friends. It will, at least, indicate the range and coherence of his interests, and the inspiration he gave – to see history through people, and to allow people to see themselves through history.

1
Roy Porter and the Persons of History

Harold J. Cook

It is notable that two of the books Roy Porter wrote late in his shortened life, *The Enlightenment* and *Flesh in the Age of Reason*,[1] were the ones he cared about most – or so he told friends and colleagues at the time. More clearly than in many of his other works, they show him engaging with age-old problems about the nature of humanity: the relationships between mind and body, reason and flesh, spiritual journeys and material conditions, the conscious self that senses its ability to make free choices and the passionate will that knows it is trapped by nature's laws. From this perspective, it is easy to see why he turned to the study of the history of medicine with such enthusiasm after being persuaded to come to the Wellcome Institute in 1979, for not only were medical people themselves often impelled to consider such problems carefully and to write about them suggestively, but a range of topics within the medical field brought them into relief. Were diseases caused by forces beyond our control or by bad behaviour? Were institutions like the early modern madhouse really meant to care for their inmates, to salve the consciences of the great and the good, or to remove difficult characters from families and communities? Did medical innovations lead to better health or line the pockets of those who sold the treatments? The whole range of life – from comedy to tragedy, from money-grubbing to sincere philanthropy, from material interests to religious persuasion, from biology to philosophy – is at the medical historian's disposal, and for someone with an appetite for historical curiosities as large as Roy's, and for cocking a snoot at the overly self-assured, medical history was a playground. Or at least it was for a while, as long as Roy could make use of medical sources while not getting trapped in explanations of the minutiae of medical developments themselves. What he said about medicine in an interview in 1997 is revealing, for he considered economy and philosophy to be more important:

> More and more money is going into producing fewer and fewer results. At the end of the next century, we won't look back and praise medicine

> for being one of the saviours of humanity. In the future, it will probably be economics that saves the world, or philosophy. I don't know. But not medicine.[2]

That view applied just as well to earlier periods, for he rarely celebrated the development of a new and effective treatment for a disease and often tended (too easily, I think) to consider all early modern medical treatments equally ineffective or even harmful. For a while, documents related to medicine provided grist for his mill. But he never considered medicine to be the centre of the world. It was, rather, an aspect of his concern to explore the human condition in body as well as intellect.

Roy's huge and varied corpus shows him to have been a historian preoccupied by a grand, twofold project: to get to the bottom of some of the most profound expressions of human thought over the course of the 'long eighteenth century' and to puzzle out how the persons who penned such ideas interacted with the rapidly changing world around them. Or, to put it more simply, at the time he began writing history, before the term 'cultural history' had currency, he shared in the attempt of many of his contemporaries to unite a history of thought with socio-economic history. It also seems clear that at heart Roy gave more attention to the history of thought than to socio-economic changes – at least in his early and late work. He was also far too appreciative of the individuality, creativity and intellect of his subjects ever to consider their work to be determined by the socio-economic conditions of their worlds.[3] His writing is most satisfying when it connects self-conscious expression to the world in which it was written down, usually through biography, as in his essay on William Hunter, his book on Edward Gibbon or the countless persons who populate his larger histories.[4] This also brings into view his love of literature, theatre and music. When it came to deciding about the priority of general causes, then, Roy's writings seldom gave firm guidance, for he revelled in the complexities and ambiguities of the mixing of multiple personal lives without the need to reduce his accounts to single causes. This was one of his great strengths as a historian, for Roy was truly impressive in the breadth and depth of his knowledge, and in his eclectic appetite for all kinds of history writing, which also made him a supportive mentor and generous critic. But he was neither the founder of a school of history nor an aspirant for such a role. His analyses were rooted in persons and moments rather than in structures.

In this essay, then, I simply want to show by a few examples why I think this is a correct reading of the range and quality of Roy's work, and so challenge those who pigeonhole him as a 'social historian of medicine'. He consistently shied away from explaining humans as motivated mainly by one set of ambitions or another, whether *homo oeconomicus* or *philosophicus*. As a result, it is best to think of him as one of a generation of historians who were trying to recover a new vision of the past

that emphasized the variety of how things were rather than the making of causal arguments. Because it often told stories about ordinary people without drawing large conclusions, it was sometimes criticized as 'history with the politics left out'.[5] But Roy continued on his course not because of ignorance of other views, much less naiveté, but because of his firm conviction about the complexity of lived experience and his abiding interest in how people in the past explained their worlds to themselves as they got on with life.

Indeed, when Roy joined the Wellcome Institute at the end of the 1970s, it seemed an almost revolutionary turn in the writing of the history of medicine to argue that the most important changes in medicine were not the result of scientific or clinical improvements but due to 'social' causes. Others had already taken up the banner – indeed, a 'Society for the Social History of Medicine' had been founded in 1970 in Britain, while Susan Reverby and David Rosner had just published their manifesto for the study of medicine in the US[6] – and Roy quickly came to recognize and honour the many social historians of medicine who had written before him, from Henri Sigerist to George Rosen, Richard Shryock and Erwin Ackerknecht, as well as his many peers walking similar paths. At the time, 'social history' in general seemed in the vanguard of a new approach to understanding the past. The term stood for a loose collection of views holding that large-scale historical changes could be explained only by collective behaviour, so that all kinds of historical actors needed to be brought into view, not only the 'elite'. Under this rubric was lumped work as different as that of historians of labour, demography, urbanization, race, revolution, even econometricians, the French Annales School and the Italian 'micro-' historians, to say nothing of the first wave of feminist historians. To many of those who disliked such trends, social history smacked too much of leftist political agendas, and indeed, many of those who took up the banner did advocate a Marxist or socialist position.[7] When asked, Roy identified his politics with 'old Labour' and, following Margaret Thatcher's abolition of the GLC, he even inserted a comment in one of his books about how London should have its own government.[8] But he hated ideologues of all kinds and avoided obvious partisanship in his historical writing. His politics appear more by indirection, by the way in which he explored past lives, than by arguing a position paper. Roy's kind of social history was, therefore, an eclectic topic held together at least as much by what it opposed: monocausal arguments, whether about economy, politics, ideas or any other subject. The move towards social history allowed Roy to write about whatever held his attention and, more importantly, to connect it to other things he cared about, without fear of trespass.

Roy's deepest and most abiding concern seems to have been with what might be called the social history of ideas, placing the history of thought foremost among his concerns but seeing it as arising from a variety of

people and processes rather than from intellectuals and the academy. His personal background prepared him to make the point.[9] He was well educated at a London grammar school before being shaped at Cambridge by two eminent historians, Quentin Skinner and Jack Plumb.[10] For his PhD, he went on to investigate what would now be termed the 'social history' of eighteenth-century geology under the direction of Martin Rudwick, being particularly interested in the many 'small' and local figures from whom most of the hard-won knowledge of the day emerged, but who had been largely overlooked by historians concerned only with the famous theoretical treatises. He, like so many historians of the day, was also moved by E. P. Thompson's book *The Making of the English Working Class*, which Roy considered to have 'dynamited' the older view that the eighteenth century was concerned almost entirely with the history of the patrician oligarchy.[11] Thompson had shown that ordinary people thought clearly and took action according to their lights: they were neither unthinking automata nor 'the masses' merely responding to stimuli such as wealth or famine. Roy was also deep in conversation with fellow students of history at Cambridge, most importantly two friends, John Brewer and Simon Schama. Brewer in particular saw how ordinary people could shape political events even in the oligarchic eighteenth century, using some of the new concepts about how patterns of consumption were even more important than producers in changing the economy.[12] Schama was also thinking about the relationships between politics, ideas and economy, while stressing the role of contingency in altering the course of history.[13]

All these themes appear in Roy's *English Society in the Eighteenth Century* of 1982, a path-breaking survey still assigned to many students today, which placed consumption and the pleasure principle in the middle of things. Yet his conclusions largely conformed to the traditional historical consensus about the period. There was the 'fundamental strength and resilience of [England's] social hierarchy' – its governing oligarchy was 'inegalitarian and oozing privilege (some of it hereditary) yet it was not rigid or brittle' – and this oligarchy strove 'to secure consensus within this acquisitive, restless society' so that 'the Georgian battle was mainly for the mind'.[14] In other words, social history allowed Roy to see the eighteenth century from new perspectives, to note the growth of capitalism and consumption, markets and interests, yet also allowed him to place the 'battle for the mind' centre-stage.

Therefore, when Roy came to turn his attention to the 'social history of medicine', which he was doing at the same time he was finishing his book on English society, his goal was to connect changes in medical ideas to other aspects of human life, thereby breaking down the boundaries between medicine and other historical topics. This strategy minimized the importance of medical doctors while bringing all the complications of life and

mentality, and the powerful position of the propertied classes, into the story. Roy's opening manifesto for his history-of-thought-in-society-as-read-through-the-lens-of-medicine made this clear:

> It would be indeed paradoxical if English medicine and Enlightenment thought had not significantly interacted, because numerous philosophes were medically active, and many medical men were men of letters . . . My contention is that Enlightenment currents will explain major changes in the evaluation of life which occurred in the eighteenth century . . . The offspring of the Enlightenment and medicine must be sought in England not in the medical profession nor in the State, but amongst the propertied classes, who were pre-eminently individualistic. The clinical gaze must be lowered onto the laity.[15]

He never wavered in his concern for how people thought about their worlds, while turning literate documents from 'the laity' (even from the middle and upper classes) into a history of medicine 'from below'.[16]

Within a few years, Roy had refined this last comment about the 'laity' into the phrase for which he is perhaps best known among historians of medicine, 'the patient's view', which became the title of an article of 1985 that still appears in footnotes as a call to arms.[17] As he elaborated his aims, Roy focused his attack on the 'physician-centred account of the rise of medicine'. That account 'may involve a major historical distortion' since 'a great deal of healing in the past . . . has involved professional practitioners only marginally, or not at all'. Consequently, 'the initiatives have often come from, and power has frequently rested with, the sufferer, or with lay people in general, rather than with the individual physician or the medical profession at large'.[18] Despite the qualified 'may' this was an implicit attack on the authority of medical doctors, who interested him more as fallible people than as repositories of expert knowledge and technique. But going after them directly was not his aim; indeed, he thought the work of Charles Webster and others, whose historical work lamented the dominance of the medical profession in decision-making of many kinds, only served to 'enhance the fascination of the villain', reinforcing the view 'that the history of medicine is about doctors, what they know, what they do'. It was as a counterweight to such views that 'we also need . . . a patient-centred history, or, to be precise, in the first instance a sick people's or suffers' history'. His aim here was to restore the lives of everyday people along the lines called for in Raphael Samuel's 'people's history', in which everyone is treated as being as fully complex as anyone else.[19] The most important power of social history was, then, in giving ordinary people agency, recalling them from victimization, 'for it is precisely the dynamic interplay between sufferers and practitioners that requires study, the tug-of-war of supply and demand, patient power and doctor power. By starting with the patient we can put medical history back

on its feet'.[20] Throughout the rest of his writing (and of his teaching) he would continue to do his best to treat all his subjects as having complicated inner lives, and to find witty ways of bursting the balloons of his pompous and pretentious contemporaries, especially among well-educated people who should know better, whether medics or academics, often by producing telling counter-examples to their pet theories.

In Roy's historical practice, this call to arms was plainly aimed more at description than causality: it should be possible to 'hack our way into the empirical forests of the past in all their strangeness and diversity'.[21] In other words, there was a strong element of historical purism about his aims, a desire to be true to the past as a foreign country rather than treating it as comfortable and familiar territory. After discussing the very different outlooks of two seventeenth-century diarists – Samuel Pepys and Ralph Josselin – he concluded that 'we lack a historical atlas of sickness experience and response, graduated by age, gender, class, religious faith, and other significant variables. It is terra incognita . . . '[22] The material conditions of life needed mapping, as did the experience and meanings of sick people, the language of pain, the ways in which illnesses were classified, the remedies used and the other actions taken by the sick, including the choices they made in 'the medical market'.[23] In conclusion, Roy repeated, we need to 'lower the historical gaze onto the sufferers' without any 'theoretical models' getting in the way.[24] History was best seen as dynamic rather than structural. Therefore, 'a people's history of suffering might restore to the history of medicine its human face'.[25] Throughout much of the remainder of his voluminous writings on medical history, Roy carried out this agenda by developing often amusing and pointed narratives heavily populated by strange, stubborn and complicated people.

In the midst of his mainly descriptive historical work Roy many times pointed to economy as one of the engines of historical change, but it never over-determined his account. His *English Society* had been especially strong on the chapters on material life – 'Keeping Life Going', 'Getting and Spending', 'Having and Enjoying' and 'Towards Industrial Society' – and in the conclusion to the book he wrote energetically about matters such as the perceived threats to the old moral order from 'this market society'.[26] When it came to medicine, Roy used the term 'medical market' in his 1985 manifesto to indicate that medical 'customers' had many choices to hand, which gave them social power,[27] and he came back to the phrase from time to time in later work. Of course, he was not the only one to consider the power of early modern markets to change social dynamics and ideas. Among medical historians, around the same period Roy published his article, Irvine Loudon detailed a GP's income; Matthew Ramsey considered the dynamics of medical political economies (although neither used the phrase 'medical market'); Katherine Park used the expression as a chapter-title in her book; and I used the term to indicate that the growth of a commercialized medical

milieu promoted a set of intellectual values inimical to the old medical regime.[28] Of course, these were also the days of the Reaganite and Thatcherite reforms, when many public activities were thought to be best reconceived in terms of markets, and there was much conversation in the news about medical markets. Exactly where the phrase came from is therefore unclear. But in using it, Roy was grasping for a concept that would help him to explain change in the medical world of the eighteenth century.

The concept seldom did much analytical work for him, however. The book whose title appears to make most use of it, *Health for Sale*, has a chapter on 'Medical entrepreneurship in the consumer society' and it is typically full of detail and interest, from medical advertising to a few bits of information about medical incomes. Roy mentioned the 'burgeoning bourgeois culture' that fetishized goods, so that 'medicaments assumed greater status in the healing process' than they had before. But rather than dwell on such matters, he quickly moved on to explore one of the implications, the 'quintessentially bourgeois anxiety: hypochondria'. Indeed, the chapter (like the book) is full of concern for matters such as 'the Enlightenment's dream of perfecting health and prolonging life' and 'a rousing, populist, Enlightenment ideology' about 'the diffusion of knowledge'.[29] In short, the work focuses on 'quacks', their self-representations and the public responses to them as an aspect of the Enlightenment, showing both its goals and failures.[30] He made it clear – here as elsewhere – that 'medicine expanded as part of the general growth of the service sector in a thriving consumer economy'.[31] In this sense, the monetization and commercialization of England was a precondition for what happened in medicine. But the ways in which medical buying and selling occurred did not occupy his attention for long, for he found other things to be of equal or greater interest.

If we turn the question round and look at the role of medical economy in madness, another topic that preoccupied Roy in the period, one again sees that it is a precondition for other things, but does not get much attention in itself. As he noted in his 1985 manifesto, 'the mad . . . are among the few groups of sufferers to have attracted much interest' as patients.[32] He himself quickly began to explore the printed primary and secondary sources on the subject.[33] In his *Mind-Forg'd Manacles*, Roy noted the articulation of 'a market' for madhouses because of the 'lucrative' nature of the trade,[34] noted pounds, shillings and pence where they surfaced in documents, and otherwise indicated that economy was a part of the story he told. Perhaps more important was one of the implications of his account: as medicine *per se* became more concerned with the buying and selling of somatic treatment, one of 'physic's' old concerns, the mutual interactions of mind and body, was split off into 'the making of psychiatry', as he titled chapter 4. Yet Roy was after even bigger game, showing among other things how Foucault's views were incorrect, or at least did not apply to England.[35] The same was true with *The History of Bethlem*, to which he contributed: while income and

expenditure figure throughout the book, its ambition was to treat the whole institution and its associations as broadly as possible, making questions of money and markets only one small matter among others.[36]

Even the one article among his huge corpus that dealt with the medicinal trade itself, 'The rise of the English drugs industry' (co-authored with Dorothy Porter), balks at any hint of economic determinism. They showed clearly that the doctor-centred approach had led historians of medicine to overlook the great importance of those who made up medicines, many of whom, like Thomas Corbyn, became quite wealthy as well as influential. Manufacturing druggists and their distributors were 'the very sinews of medicine', to borrow a phrase from John Brewer.[37] They were also early advocates for what might be called the 'cultural' implications of business, especially for the way it depended on 'moral and business codes' that included a good reputation over the long term, reliability and trust, distinguishing between the small shopkeeper and the larger manufacturer, and they picked up the importance of overseas markets for 'the expansion of the pharmaceutical trade'.[38] In their conclusion, they went as far as to state that 'the making and marketing of drugs provided the commodity upon which the modern business of medicine was founded'.[39] Yet, despite a promise in a footnote to publish a more thorough study of Corbyn's business, nothing ever came of it.[40] Nor did they publish any other studies of the trade in medicinals, although Roy did co-edit an anthology on drugs.[41] Once more, while treating the medical market with insight and nuance, the concept did not dominate even this article. Instead, Corbyn himself, rather than the systems governing his life, got most of the attention, and this despite the fact that during this period Roy and Dorothy worked with John Brewer at the Clark Library in Los Angeles on a several years-long project to study the history of consumption.[42]

The most absorbing kind of history for Roy therefore remained trying to recover complex human interactions, including the misunderstandings, sleights of hand and frauds of human intercourse, the bodily and mental pleasures people found, and the careful, honest crafting of words that spoke for the human condition as lived in the moment. Like that of other influential historians, his work relies more on description than argument about causes, and it keeps coming back to profound questions about the nature of humanity, although mainly through the words of those he quoted. In other words, Roy's histories have affinities with serious literature, in which multiple meanings, plots and changing characters are present, an affinity for which he no doubt strove, sharing the ambition with his friend Simon Schama.[43] His work therefore appeals to a vast range of readers, as well as historians, although as a result, trying to assess what he 'really' thought sometimes seems hopeless. To label him as one of the pre-eminent social historians of medicine of his generation, then, captures only a part of his huge historical project, which was always associated with how embodied people of the long eighteenth century thought about the nature of humankind.

Part I

The Science of History and the Politics of Science

2

Porter versus Foucault on the 'Birth of the Clinic'

Adrian Wilson

Texts and contexts

It is well known that Roy Porter's *oeuvre* in the history of psychiatry was partly stimulated by, and in no small measure pitched against, Michel Foucault's *Madness and Civilization* (*Folie et déraison*). But what was Porter's response to Foucault's *Birth of the Clinic* (*Naissance de la clinique*)?[1] By the 'birth of the clinic', let us recall, Foucault referred to the remarkable change in medicine that took place in the Paris École de Santé between 1800, when Xavier Bichat's *Traité des membranes* initiated his new doctrine of tissues, which rapidly invigorated pathological anatomy, and 1817, when René-Théophile-Hyacinthe Laennec (1781–1826), one of the most influential pupils of the new school and himself a master of pathological anatomy, invented the stethoscope. Foucault himself, as we shall see, regarded this as the most important transformation in Western medicine's entire history, and although not all commentators would go this far, none would deny that pathological anatomy made giant strides in the Paris school, or that the stethoscope gave an unprecedented impulse to physical examination, with permanent effects. Thus we are dealing here with a momentous set of events, which means that Porter's view of *Birth of the Clinic* is of considerable interest.

If we concentrate on the directly corresponding passage in *The Greatest Benefit to Mankind* we find considerable harmony, for with respect to physical examination, Porter seems to have been persuaded by Foucault's version of events.[2] But when it comes to the other dimension of *la clinique*, namely the development of pathological anatomy, Porter's account was radically different from Foucault's. The way in which this becomes apparent is in their respective treatments of Bichat's precursors, notably – though not only – Giovanni Battista Morgagni, author of the monumental *De Sedibus et Causis Morborum per Anatomen Indagatis* (1761). This could be brought out using the appropriate passage from *Greatest Benefit* itself;[3] but it is simpler to use for this purpose another Porter work of the 1990s, part of his contribution

to the collectively authored *The Western Medical Tradition*, in which Porter was responsible for the chapter on the eighteenth century.

In this chapter, therefore, I shall juxtapose two texts – Foucault's *Birth of the Clinic* (mainly, though not only, its Preface) and Porter's discussion of eighteenth-century 'Pathology' (by which he meant pathological anatomy) in *The Western Medical Tradition*.[4] We shall see that these display both a systematic divergence and a subtle and surprising agreement, and that in each respect the Foucault/Porter comparison is emblematic of wider historiographical trends.

Foucault's picture

As I have remarked elsewhere, Foucault depicted the 'birth of the clinic' as a step from non-knowledge to knowledge.[5] The corollary of this, or indeed its prerequisite, was that the transition from the old medicine to the new was portrayed as a *discontinuous* development. Yet this fundamental posited discontinuity was never argued; rather, it was installed rhetorically, by the devices of metaphor and selective exemplification.

Metaphor

The very title *Naissance de la clinique* was, of course, a metaphor of discontinuity; further metaphors of this kind proliferated throughout Foucault's Preface, which invoked in turn a 'sharp line', a 'moment', a 'mutation in discourse' and a 'turning-point';[6] and last but not least, the climax of the book – a passage that I shall have occasion to examine later – spoke of 'the great break in the history of Western medicine'.[7] These multiple metaphorical resources (birth, line, moment, mutation, turning-point, break) range across the living and the non-living, the spatial and the temporal, the physical and the mathematical, but they have this in common, that each of them is an image of discontinuity.

Exemplification

The governing structure of *Naissance de la clinique* was set up in the Preface by means of a contrast between two texts, from 1769 and 1825, both concerned with 'membranes' but in very different ways: the first by Pierre Pomme (1735–1812), recounting a treatment for hysteria which caused 'membranous tissues' to be expelled from various bodily orifices; the second by Antoine-Laurent-Jesse Bayle (1799–1858), describing the meninges of the brain. According to Foucault's gloss, Pomme 'speaks to us in the language of fantasy', whereas in the work of Bayle and his contemporaries 'language has turned into rational discourse', making it 'possible . . . for the patient's bed to become a field of scientific investigation'. A little later in the Preface Foucault introduces another contrast, whose purpose is slightly different but whose structure is the same: here J. F. Meckel (1724–74), weighing portions

of the brain in 1764, is counterposed to Bichat and his contemporaries (roughly simultaneous with Bayle), who examined the brain in qualitative terms. The effect of this twofold comparison is to install as representatives of mid-eighteenth-century medicine the forgotten figures of Pomme and Meckel, and conversely to depict 'medicine as a clinical science' as beginning with Bichat, Bayle et al. – that is, in the early nineteenth century.[8] But the force of all this hinges on the choice of the examples, as can be indicated by a thought-experiment: imagine replacing Pomme and Meckel with their contemporaries Morgagni (1682–1771) and Leopold Auenbrugger (1722–1809) and the entire contrast collapses. For as Foucault half-acknowledges elsewhere in the book (I shall touch on this in due course), the grand themes which defined *la clinique* – pathological anatomy and physical examination – had already flourished in the 1760s, when Morgagni published his incomparable *De Sedibus* and Auenbrugger announced his discovery of thoracic percussion. No wonder, therefore, that the Preface (and, for that matter, most of the book) rigorously excludes Morgagni and Auenbrugger, and *a fortiori* Morgagni's teacher Antonio Maria Valsalva (1666–1723) and his seventeenth-century predecessors such as Johann Jacob Wepfer (1620–95, author of the classic work on cerebral haemorrhage) and Théophile Bonet (1620–89, compiler of the 1679 *Sepulchretum*, a massive digest of cases and autopsies): the achievements of these individuals contradict the very premises of Foucault's argument.

Porter's picture

Porter, in sharp contrast, depicts the development with which he is concerned – pathological anatomy – as a *continuous* process with a protracted history. Yet his radically different picture has this in common with Foucault's: that its literary form is perfectly attuned to its content. The simplest way to bring this out is to inspect the key points of the story as Porter tells it:

> Since Vesalius, the idea had grown that the good practitioner must be proficient in gross anatomy. An inevitable consequence was that increased attention began to be paid to the connexions between the sick body and the disease signs afforded by the corpse. Anatomy, in other words, paved the way for morbid anatomy . . .
>
> The trail was blazed by . . . Morgagni . . . . Building on earlier necropsy studies by . . . Wepfer . . . and . . . Bonet . . . Morgagni . . . published in 1761 his *De Sedibus et Causis Morborum* (*On the Sites and Causes of Disease*) . . .
>
> In *De Sedibus*, Morgagni demonstrated that . . . disease symptoms tally with anatomical lesions . . . . It was Morgagni who thus finally clinched the direct relevance of anatomy to clinical medicine . . .

> Others continued his work. In 1793, Matthew Baillie . . . published his *Morbid Anatomy* . . . .
>
> Pathology became more fully systematised with the publication in 1800 of the *Traité des Membranes* (*Treatise on Membranes*) by . . . Bichat . . . As developed by Morgagni, pathology had dealt with organs. Bichat changed the focus [to the tissues] . . . .
>
> Around 1800 . . . thanks to developments in pathology, attention was newly being paid to normal and abnormal structures and functions, a trend that would come to fruition in the nineteenth century.

Again there are two rhetorical elements in play, which it is convenient to treat in the opposite order from that which I used for Foucault.

Exemplification

Porter deploys a very different cast of characters from that used by Foucault. Not only does he begin with Andreas Vesalius (1514–64), thereby giving pathological anatomy a vastly extended lineage; he also makes Morgagni his key eighteenth-century character, with the effect that the medicine of the nineteenth century is linked with that of the eighteenth, rather than separated from it as in Foucault's picture. In addition there are several other individuals who, so to speak, fill up the historical spaces: Wepfer and Bonet stand for the seventeenth century, Matthew Baillie (1761–1823) for the late eighteenth. Finally, at the very end of the story, comes Bichat – the solitary point of overlap with Foucault's picture, an overlap which is no accident, for Bichat was the individual in whom Foucault located the origin of modern medicine, as we shall see.

A vocabulary of continuity

The very presence of Porter's temporally extended cast list – Vesalius, Wepfer, Bonet, Morgagni, Baillie, Bichat – conveys a message of continuity. But more than this, whereas Foucault separated the characters of his non-story, Porter systematically connects them; and the way that he does so is consistently through a rhetoric of continuity. In the opening and closing phases of the story that rhetoric is largely metaphorical – *idea had grown*, *paved the way*, *trail was blazed*, *building on*, *come to fruition*; in the heart of the passage, that is to say when dealing with the eighteenth century, Porter abandons metaphorical language and uses direct representation: *continued his work*, *became more fully systematized*. Yet both forms of writing achieve the same effect, namely, continuity – the precise opposite of what Foucault had constructed with his panoply of metaphors.

In short, where Foucault depicted discontinuity, Porter installed a continuous story. And this disagreement is by no means confined to these two authors, for much the same contrast is to be found between the writings of Russell Maulitz (who treats pathology as effectively beginning with

the Paris school, apart from a nod to Morgagni) and Othmar Keel (who has argued vigorously against Foucault's claims for the novelty of the Paris school).[9]

A hidden consensus

Thus far it appears that the stories told by Foucault and Porter have nothing in common, apart from their unsurprising convergence on the figure of Bichat and of course their considerable rhetorical mastery. Yet I shall now suggest that between those stories there subsists a subtle and interesting agreement, one that resides not so much in what they say as in what they tend to exclude.

The theme that I have in mind is the project of *relating postmortem findings to the prior symptoms of the living patient*. Let us first notice that by the time of *la clinique* that project was well over two centuries old. It had already been attempted quite commonly, albeit occasionally, in the sixteenth century;[10] it was formulated explicitly by Francis Bacon in 1605[11] and was pursued by many seventeenth-century medical men, among them Wepfer and Bonet; in the eighteenth century it was carried forward with particular rigour by Morgagni, whose efforts show, incidentally, how complex and difficult this undertaking was.[12] And in the epoch of *la clinique* this same project was attested, appropriately enough, by both Bichat and Laennec – the one the effective founder of the morbid anatomy which flourished in the Paris school, the other the begetter of that school's crowning invention, the stethoscope. Here is how Bichat, writing in 1801, depicted the future of pathological anatomy (with my emphases):[13]

> It seems to me that we have reached an epoch in which morbid anatomy is bound to experience a new and rapid development. This science is not only that of the organic derangements which arrive slowly, as principals or as sequelae, in chronic illnesses; [rather] it consists of the examination of all the alterations which the parts can experience, at whatever stage their illnesses are examined. Thus apart from certain fevers and nervous affections, almost everything in pathology is within the competence of this science. How slight are the reasonings of famous physicians when we examine them not in their books, but in the dead body! Medicine was long excluded from the circle of the exact sciences; it will have the right to be associated with them, at least in respect of diagnostics, when *rigorous observation* [sc.: of symptoms] *is everywhere united with* the examination of the alterations suffered by the organs. This direction is beginning to be taken by all reasonable minds; it will without doubt soon become general. What is observation, if we are ignorant of the place where the evil is seated? For twenty years from morning to evening you have taken notes at the bedside of patients, on the affections of the heart, the lung, the gastric

> viscera, etc., and all is confusion for you in the symptoms, which, having no common connection, exhibit only a series of incoherent phenomena. Open some cadavers, and you will soon see disappearing the obscurity which observation alone could not have dispelled.

For Bichat, then, morbid anatomy alone was not a sufficient basis for bringing medicine into 'the circle of the exact sciences'; rather, it had to 'be united with' the observation of disease in the living patient. And some ten years later, by which time Bichat's vision had already been realized in Paris by Jean-Nicolas Corvisart (1755–1821, the reviver of Auenbrugger's percussion) and many others, Laennec made much the same point (emphases mine):[14]

> Pathological anatomy is a science whose aim is the knowledge of the visible alterations produced on the organs of the human body by the state of disease. The opening up of corpses is the means of acquiring this knowledge; but in order for it to become of direct use . . . it must be *joined to observation of the symptoms or alterations of functions that coincide with each alteration in the organs*.

In short, both Bichat and Laennec proclaimed that *morbid anatomy was not enough on its own*, but had to be 'united' or 'joined' with the study of symptoms in the living patient. This is all the more striking when it is remembered that these two men were, at least in the judgement of posterity, the most eminent practitioners of morbid anatomy produced by the Paris school. Yet at the same time their formulations betray a curious omission or elision: neither of them made explicit what was in fact the precondition for linking symptomatology with pathological anatomy, namely that *the same individual* be studied first when alive (as a patient) and then when dead (as a corpse).[15] This oversight, I suggest, was no accident, but rather reflected the inherent difficulty of the subject: just as the enterprise of linking postmortem findings with prior symptoms presented an immense challenge both practically and conceptually (as the work of Morgagni vividly attests), so also the theorization of that enterprise – the articulation of what it involved – was a troublesome task in its own right. For instance, this activity had no identifiable locus within the medical disciplinary matrix: it was connected with all three of pathology (the theory of disease), pathological anatomy and the observation of symptoms, but could not be subsumed within any of these.

How has this theme – the integration of postmortem findings with prior symptoms – been figured by Foucault and by Porter? We anticipate, of course, that Foucault will depict it (falsely) as the sudden product of the Paris school, whereas Porter will (correctly) give it an extended lineage, and this expectation is indeed borne out; yet as will emerge, their formulations also display an unexpected harmony.

Naissance de la clinique engages with the matter in chapter 8, which deals with what Foucault calls 'the age of Bichat'[16] and is entitled – significantly, as we shall see – 'Open up a few corpses' (*Ouvrez quelques cadavres*). A measure of the importance of this chapter is that it is precisely here that Foucault announces, in a phrase that I noted earlier, 'the great break in the history of Western medicine'.[17] It is appropriate to consider the chapter as a whole in order to see how my theme (which appears almost halfway through) is introduced.

The opening section of the chapter elaborates two theses in turn, the first historiographic, the second substantive. The *historiographic* argument is pitched against a longstanding medical myth: that before the Paris clinic, pathological anatomy had been held back by 'the opposition of religion, morality, and stubborn prejudice to the opening up of corpses'.[18] In rebutting that myth Foucault has to invoke the very figures that his Preface had so carefully suppressed: Bonet, Valsalva, Morgagni and Auenbrugger.[19] But the discontinuity constructed by the Preface is reinstated through Foucault's next, *substantive* claim: that the pathological anatomy initiated by Bichat was different in kind from Morgagni's. In place of 'the densities of the organs' investigated by Morgagni, Bichat substituted what Foucault calls 'the thinness of the tissue': that is, 'volumes' were replaced by 'surfaces'.[20] And this leads Foucault to the pivotal thesis of his book: that this new focus on 'tissual surfaces' made it possible to transfer the nosographic project of the late eighteenth century, based on Condillac's 'method of analysis', from the universe of symptoms – another 'surface', but a surface of a very different type – into the physical body.[21] This claim is rich in implications, both benign and baleful. On the one hand, it has the great virtue of making intelligible the fact (overlooked by some other commentators, including Porter) that the new, tissue-based pathological anatomy initiated by Bichat was entirely consistent with the continuation of the nosographical project.[22] On the other hand, it permits Foucault to advance a new myth of his own making, one which stands as the precise substitute for the one he had refuted a few pages earlier: that only now, in the 'age of Bichat', was pathological anatomy related to prior symptoms.[23] Accordingly, the remainder of the chapter is devoted to the ways that Bichat and his immediate successors – Corvisart, Gaspard-Laurent Bayle (uncle of Antoine-Laurent-Jesse Bayle, whom Foucault had used in his Preface), Laennec, Petit, Bouillaud – handled this (supposedly novel) task of relating postmortem findings to antecedent symptoms in the living patient.

Foucault depicts that task as presenting two aspects:[24] the integration of the spatial (anatomy) with the temporal (symptoms); and the need to overcome the distorting effect of postmortem changes, which confuse the significance of what is found at autopsy. Notice that this analysis drastically reduces the scope of the problem – for neither of these formulations begins to capture the awful and ineffable gulf that separates the dead

body, which knows no pain and neither feels nor speaks, from the living, breathing, suffering patient. Nevertheless, that problem is initially in focus, as is signalled by the fact that Foucault here includes Laennec's injunction (which I reproduced above) that pathological anatomy has to be 'joined to observation of the symptoms or alterations of functions that coincide with each alteration in the organs'. Yet as Foucault's discussion proceeds (by way of an elegant meditation on death in the writings of Bichat, supplemented by those of Bayle and Laennec), the problem gradually slips out of view,[25] until it is finally erased in the punchline of the chapter – the very passage which announces 'the great break in the history of Western medicine'. For here Foucault quotes from Bichat's view of morbid anatomy, just as he had earlier quoted that of Laennec, but in doing so he *omits* Bichat's requirement that 'the examination of the alterations that our organs suffer' should be 'everywhere united with' the 'rigorous observation' of patients' symptoms:[26]

> the great break in the history of Western medicine dates precisely from the moment clinical experience became the anatomico-clinical gaze. Pinel's *Médicine clinique* dates from 1802 . . . the rules of analysis seem to triumph in the pure decipherment of symptomatic totalities. But a year before, Bichat had relegated them to history:
>
> for twenty years, from morning to night, you have taken notes at patients' bedsides on affections of the heart, the lungs, the gastric viscera, and all is confusion for you in the symptoms which, refusing to yield up their meaning, offer you a succession of incoherent phenomena. Open up a few corpses: you will dissipate at once the darkness that observation alone could not dissipate.
>
> The living night is dissipated in the brightness of death.

The elegance of Foucault's concluding paraphrase should not blind us to the fact that we have been robbed: for the effect of this selective quotation is to obliterate Bichat's recognition of the need to *unite* postmortem findings with the observation of symptoms. And it can now be seen that the chapter's title is faithful to its trajectory: the injunction 'open up a few corpses' conveys precisely half, but only half, of what Bichat had in fact enjoined. Thus the effect of the chapter, ironically enough, is to eliminate from view the very activity which Foucault himself has claimed distinguished *la clinique* from all that went before it, namely the integrating of symptoms with anatomy.[27]

Turning to our Porter text we find the same suppression but in a different form, corresponding to the fact that Porter – quite rightly, and in radical opposition to Foucault – depicts anatomico-symptomatic correlation as a longstanding activity. What happens here is that this theme repeatedly enters and then disappears, submerged within the figure of pathological anatomy. This movement takes place at the very outset:

> Since Vesalius, the idea had grown that the good practitioner must be proficient in gross anatomy. An inevitable consequence was that increased attention began to be paid to *the connexions between the sick body and* the disease signs afforded by the corpse. Anatomy, in other words, paved the way for *morbid anatomy*.

Here the living patient has been introduced in the form of 'the sick body', and Porter rightly points to the 'connexions' between this and 'the disease signs afforded by the corpse'; yet in the very next sentence all of this is lost, for the focus shifts to the mere corpse ('morbid anatomy'). And precisely the same transmutation takes place at the key moment of the passage, that is to say, in the discussion of Morgagni and his impact. Porter's account of Morgagni's *De Sedibus* is admirably explicit as to the importance within that work of anatomico-symptomatic correlation:

> In *De Sedibus*, Morgagni demonstrated that . . . *disease symptoms tally with anatomical lesions* . . . . It was Morgagni who thus finally clinched the direct relevance of anatomy to clinical medicine.

Yet this insight is instantly cancelled in what follows. For as an example of those who 'continued' Morgagni's work, Porter offers Matthew Baillie – whose *Morbid Anatomy* in fact contained no case histories, and in its original edition (1793) did not even mention symptoms.[28] Thus the price of assimilating Morgagni to Baillie is neither more nor less than the elimination of anatomico-symptomatic correlation as a significant dimension of Morgagni's enterprise. And this is the same contradiction (*mutatis mutandis*) which we have seen at the climax of Foucault's *Birth of the Clinic*.

Whence this shared elision? I suggest that it reflects the inherently recalcitrant nature of the matter in question. We saw earlier that anatomico-symptomatic correlation was extremely difficult to carry out; that it cut across medicine's internal boundaries, involving at least three different areas and therefore falling into none of them; and moreover that this activity resisted theorization, for even Bichat and Laennec fell short of articulating fully what it entailed. The fact that Foucault and Porter, from their very different perspectives, have both written out this theme can be regarded as the historiographic echo of this same difficulty: that is, the activity of relating symptoms to post-mortem findings has been as difficult for historians to grasp as it was for early modern medical men to practise and to theorise. Similarly, neither Maulitz nor Keele has brought anatomico-symptomatic correlation into focus; instead, for all their disagreement, they are united in taking 'pathology' as their theme.[29]

Conclusion

The convergence that we have just been noting makes all the more remarkable the contrast with which we began – between Foucault's discontinuity picture and Porter's story of continuity. By way of conclusion, I want to suggest that neither of these accounts is sufficient in itself, and yet each of them captures a very important aspect of the problem. That is to say, there was indeed a sudden shift, as Foucault would have it, yet that shift had profound roots in earlier medical practices, as Porter claims.

We can reconcile the two pictures in this way if we posit that the activity of anatomico-symptomatic correlation, though it had often been *attempted*, attained a new level of *success* in the context of the Paris Ecole de Santé. And there are indeed good grounds for believing that this is the case. The earlier attempts of course require an extended discussion, for which there is not space here; but what can be said in a brief compass is that connecting symptoms with postmortem findings was an extraordinarily *difficult* enterprise, far more demanding than normal anatomy. Even for just a single illness it required at an absolute minimum: (a) observation and recording of the living patient's symptoms; (b) the death of the patient (this is by no means trivial, for even in early modern medicine most patients survived most of their illnesses); (c) the opportunity to dissect the body; (d) not just one such patient-and-corpse but many of them, because both symptoms and post-mortem findings were complex and variable; and finally (e) a rigorous method for collating the results. And it turns out that this combination of conditions was seldom satisfied: for instance, in eighteenth-century England most hospital patients survived (because the hospitals selected patients as curable), while conversely many and perhaps most anatomized bodies came with no record of symptoms (because corpses for teaching were probably secured by grave-robbing). The effect of such constraints was exactly *as if* there had existed either the supposed taboo on dissection (the myth demolished by Foucault) or the failure to attempt anatomico-symptomatic correlation (the alternative myth which Foucault installed). But in the Paris Ecole de Santé, these constraints were broken – not because this had been intended, but simply as a result of the fortuitous triple combination of practices and circumstances that emerged there. In the first place, hospital patients were used as clinical teaching material on an unprecedented scale. Second, it so happened that these hospitals were places of very high mortality. And third, from 1798 the bodies of those same patients were made available to anatomy teachers – initially purely for normal anatomy, which is all that the formal curriculum envisaged. The effect of this conjunction was that the preconditions for anatomico-symptomatic correlation were satisfied in the Paris Ecole to an unprecedented degree: hence the activities and achievements of Bichat, Corvisart, Laennec and their contemporaries.

If in this respect Foucault's picture of discontinuity is accurate, we should also recall that Porter's continuity story has a powerful element of truth; and

it is appropriate, in a volume of this kind, to give Porter the last word. Let us therefore go back once more to the beginning of our little Porter passage:

> Since Vesalius, the idea had grown that the good practitioner must be proficient in gross anatomy. An inevitable consequence was that increased attention began to be paid to the connexions between the sick body and the disease signs afforded by the corpse. Anatomy, in other words, paved the way for morbid anatomy . . .

In thus launching his continuist picture, Porter captured a crucial element of seventeenth-century anatomy, and one which has strangely been marginalized by the specialist historians of anatomy. For the nub of his argument was that pathological anatomy was neither a new development in the eighteenth century nor a separate enterprise from normal anatomy, but rather an integral part of anatomy as such; and there is every reason to believe that this picture is correct. The historians of seventeenth-century anatomy have tended to overlook this by implicitly (and without discussion) defining 'anatomy' as 'normal anatomy';[30] but the anatomists of the seventeenth century (and to some degree at least those of the sixteenth as well) deployed a broader definition of anatomy, one which *included* what would later become the specialism of pathological anatomy. For bringing this out, as for so much else, Roy Porter – a historian of the eighteenth century, telling us something of profound importance about an earlier period – deserves our permanent gratitude.

3

The 'New History' of the Enlightenment: An Essay in the Social History of Social History[1]

Peter Burke

In the last hundred years or so, from the 'New History' of James Harvey Robinson (1863–1936) to the 'New Cultural History' of the last two or three decades, a number of social and cultural historians have been calling for a new approach to history that would find a space for ordinary people as well as 'heroes', for women and children as well as men, and for everyday activities as well as battles and political debates. However, all these demands had already been made in the eighteenth century and some scholars were responding to them in a constructive way, as this chapter will show. My aim in what follows is to discuss the rise of this old 'new history', emphasizing the international character of a movement associated in the English-speaking world with Hume, Robertson and Gibbon, but including distinguished French, Italian and German exponents. I shall first consider what the new historians of the eighteenth century believed themselves to be doing, drawing attention to their concern with 'manners' and the social 'system'. An attempt will then be made to reposition the socio-cultural approach in its own socio-cultural contexts, ranging from the Enlightenment to the rise of the female reader.

Naming the new history

To describe the goal of this movement as 'social history' is to risk anachronism. How did the scholars describe what they were doing? They often emphasized what they were *not* doing, and the rejection of the history of wars and battles in particular became a topos at this time.

Voltaire, for instance, introduced his book on the age of Louis XIV by telling readers that they would not find detailed descriptions of wars. Instead, he offered descriptions of human manners. He was repeating, as he so often did, what had been said before him, notably by the Count of Boulainvilliers, who had complained about historians who wrote about princes, favourites,

mistresses, battles and treaties, neglecting 'the genius of each age', manners, customs, passions and 'dominant ideas'.[2]

By the end of the century this complaint had been echoed again and again so that we might speak of a 'drum and trumpet topos'. In Scotland, for instance, Hugh Blair declared that 'whatever displays the state and life of mankind . . . is more useful and interesting than the detail of sieges and battles'.[3] In Germany, August Ludwig von Schlözer rejected the history of battles, or 'stories of death' (*Mordgeschichten*) as he called them.[4]

More positively, scholars interested in the new history described their own practices as 'civil history', the 'history of manners' or the 'history of society', more rarely the 'history of private life', of 'everyday life', of 'culture' or the history of the 'people'.

Civil history

Of all these terms, 'civil history' is the most ambiguous. Pietro Giannone's *Istoria civile di Napoli* was the most famous example of the genre, beginning with the declaration that this history will be *tutta civile* and therefore *tutta nuova*, 'completely new'.[5] The term derived from the Italian *vita civile*, a phrase that referred to civic life in particular. There was also an association with the civil law, in other words Roman law. Giannone was a lawyer, and law bulks large in his history of Naples.

The term *civile* was also associated with civility and with what was coming to be called 'civilization'. In some contexts, it also had overtones of 'civilian', as opposed to 'military'. Thus Giannone, making his contribution to the drum and trumpet topos, declared his intention not 'to deafen the readers with the noise of battle and the clash of arms'.

Giannone's phrase, like the book itself, was imitated in eighteenth-century Italy. Vico's famous *Scienza nuova*, first published in 1725, described the Homeric poems as 'civil histories of the ancient Greek customs'.[6] One of Giannone's leading imitators was the Venetian lawyer Vettore Sandi, who defined the term *civile* 'in a wide sense', equating it with the 'internal history' of a society, and noting the importance of the history of law, commerce and 'the history of the arts and crafts'.[7]

This Italian model of historical writing soon attracted interest elsewhere. Montesquieu, for instance, thought of writing a *Histoire civile de France*.[8] Giannone's book was translated into English in 1729–31, and found admirers in England and Scotland, including Gibbon (who read it in French).[9] Adam Ferguson produced his *Essay on the History of Civil Society* in 1767. In German, Ludwig Spittler taught what he called *bürgerliche Geschichte* at Göttingen in the late eighteenth century. An even more popular slogan of the new historians was 'the history of manners'.

Manners, customs, mentalities

One of the leading Italian historians of the eighteenth century was Ludovico Muratori. In his collection of essays on 'the antiquities of Italy' (1738), he discussed many medieval customs, including games, festivals, the code of chivalry and the so-called 'judgment of God' by means of the duel or ordeal. An even sharper focus on customs can be found in the work of a lesser-known Italian scholar, Michelangelo Carmeli, whose *Storia di vari costumi sacri e profani* (1750) included essays on the use of water and fire in ritual, on flagellation and on the symbolic importance of the right hand.

In France, Montesquieu's famous study of the 'spirit' of laws was concerned to relate them to the customs of different societies, including once again the ordeals and duels of the Middle Ages. Voltaire's famous *Histoire des moeurs* (1756) placed manners and customs at the centre of his stage. The orientalist Joseph de Guignes described the manners and customs of the Huns in a study on which Gibbon drew for his famous chapter on the 'manners' of pastoral nations.[10]

In Britain, in his *Letters on Chivalry* (1762), Richard Hurd discussed medieval romances as expressions of what he called the 'Gothic system' of 'heroic manners', while Robert Wood approached Homer in similar fashion, comparing the 'heroic manners' or 'patriarchal manners' of the Bedouin (whom he knew at first hand) with those of the ancient Greeks. However, it was Robert Henry, in his *History of Great Britain*, who wrote most fully on the subject, devoting chapter 7 of each book to 'the history of the manners, virtues, vices, remarkable customs, language, dress, diet and diversions'.

The historians associated with the Scottish Enlightenment also had a good deal to say about manners. Hume devoted an appendix to his *History of England* to 'The Feudal and Anglo-Norman Government and Manners'; Adam Smith studied 'the influence of commerce on the manners of a people'; and Adam Ferguson treated chivalry as a 'system of manners'. In Germany, too, Justus Möser and J. C. Nachtigall wrote the history of what they called *Gewohnheiten* or *Sitten*.[11]

The interest in manners extended to the history of ways of thinking or mentalities, to the discovery of what would later be described as the 'primitive'. Vico was particularly interested in this topic, describing it as 'a history of ideas' (*una storia dell'idee umane*) and referring to the 'poetic way of thinking of the heroic ages'. In similar fashion, Montesquieu explained the medieval custom of the ordeal by what he called 'the way of thinking of our ancestors'.[12]

In Britain, Thomas Warton's *History of Poetry* discussed examples of the medieval 'mode of thinking'. In Denmark, Jens Kraft's comparative essay on savage peoples concentrated on their 'way of thinking' (*Taenke-Maade*). The German scholar Johann Christoph Gatterer wrote on the style of thought of the ancient Greeks, and the Swiss Johann Jakob Bodmer on that of medieval Germany.[13]

The history of society

There were yet other ways to describe the new history. One was the 'history of the people', as in the case of the Newcastle clergyman Henry Bourne's *Antiquitates vulgares* (1725), a study in what would be known in the nineteenth century as 'folklore'. The Marquis de Condorcet, a leading figure in the Enlightenment, was concerned to incorporate ordinary people into history.[14]

Another formulation was 'the history of private life', a phrase that goes back long before the revival of this approach to the past in the 1980s. Voltaire declared in his *Essay on Manners* (chapter 81) that he wanted to discover 'how people lived within the family', while another Frenchman, P. J. B. Legrand d'Aussy, published in 1782 what he called the *Histoire de la vie privée des français*. Another formulation with a longer history than is generally thought is the history of the 'everyday'. Thus the French antiquarian Jean-Baptiste Couture studied ancient Rome from this point of view, focusing on 'what an individual leading an ordinary life did in the course of a day'.

William Robertson employed the term 'history of society' sometimes but not always to refer to 'stadial history', in other words the idea of the progress of mankind through hunting and gathering as a 'mode of subsistence', to the pastoral stage, the agricultural stage and finally the commercial stage.[15] The German equivalent of Robertson's phrase was the 'history of culture' (*Geschichte der Cultur*), a term used by Johann Christoph Adelung and Johann Gottfried Eichhorn to refer to the sum of the 'special' histories (*Specialgeschichten*) of different human activities.[16]

Even the idea of what Fernand Braudel famously called 'total history' was not unknown in the Enlightenment. After all, Montesquieu studied history in search of 'l'esprit général', while Johann Christoph Gatterer claimed that 'Everything hangs together' (*Alles hängt zusammen*).[17]

Practising the new history

The obvious question to ask at this point is about the extent to which the new historians practised what they preached. Between the rhetoric of the rejection of drum and trumpet history and eighteenth-century practice a gap is sometimes visible. Three-quarters of Voltaire's *Siècle de Louis XIV*, for instance, is concerned with politics and war. Even his *Essai sur les moeurs* is less what we would call a social history than a general history with a place for the arts alongside events such as the Council of Basel, the capture of Constantinople, the Gunpowder Plot and the Thirty Years' War. On the other hand, a considerable range of social practices was incorporated into written history under the broad umbrellas of civil history and the history of manners.

Some of the new historians extended their interests to the history of demography, material culture and the social structure. In the case of demography, the Italians were the pioneers, as they had been in the practice of census-taking. Marco Lastri's *Researches on the ancient and modern population of Florence* illustrates the trend at the level of the monograph, while Riguccio Galluzzi's study of Tuscany incorporated demographic data into a more general history.[18]

In the case of material culture, the French contribution was probably the greatest. Voltaire's concept of the history of manners – like that of Norbert Elias later – had a place for the dinner table, cutlery and tablecloths.[19] Le Grand d'Aussy's ambitious history of private life declared that it would begin with food, clothes and housing, but never got beyond three volumes devoted to food and drink. If Le Grand pursued these topics in an antiquarian manner for their own sake, August Ludwig von Schlözer, a professor at Göttingen, viewed them as part of broader trends, arguing that the history of the introduction of tobacco into Europe can be 'at least as interesting a subject as the history of the great Tamerlane' if it is linked to 'great changes', which would give it 'world historical dignity'.[20]

In similar fashion to Schlözer, Gibbon had argued in 1776 in the second chapter of his *Decline and Fall* that 'it will not be unworthy of the dignity . . . of an historical work' to mention the plants and animals that reached Europe from the East. The old idea of the dignity of history, which had long helped to block the entry of the social, was declining, or at the least it was 'under pressure' or being redefined.[21] What we might call 'cultural history' was also attracting attention at this time, including the history of art, literature, music, mathematics and medicine.[22]

Historians were also taking an increasing interest in changes in the social structure. In this domain, the Scots made a particularly important contribution. John Millar of Glasgow published a study of the *Origin of the Distinction of Ranks,* and Adam Ferguson (as we have seen) an *Essay on the History of Civil Society*, while Robertson and Hume both included discussions of the changing social structure in their general histories. Robertson noted the rise of 'a distinction of professions' in the Middle Ages, and Hume referred to 'the total want of a middling rank of men' in Anglo-Saxon England.[23]

Linked to this interest in structures was what might be called the sense of system shown by so many eighteenth-century historians. It is true that criticism of scholars for squeezing information into false systems is easy to find, with Montesquieu's critique of Boulainvilliers and Dubos on the history of medieval France a notorious example, but Montesquieu himself looked beneath the surface of history for what he called the 'principles' of government and law. William Robertson, although an empiricist historian rather than a theorist, regularly uses the term 'system' to refer not only to philosophy but also to religion and politics – 'the complex and intricate system of Italian politics' for instance. Adam Ferguson used phrases such as

'the system of chivalry' or 'system of manners', while Samuel Johnson spoke to Boswell of 'the great system of society'.[24]

The essential idea was that of interdependent parts, sometimes imagined as the parts of a machine, as in this passage from Millar:

> In every system of law or government, the different parts have an intimate connexion with each other. As it is dangerous to tamper with this machine, unless we are previously acquainted with the several wheels and springs of which it is composed; so there is reason to fear, that the violent alteration of any single part may . . . produce the utmost disorder and confusion.[25]

First applied to philosophical or scientific systems such as those of Descartes, Newton and Linnaeus, the mechanical metaphor was extended to language and also to society, prompting new approaches to its history.[26]

A well-known example of the consequences for historical research of this systematizing tendency of the eighteenth century is the discovery of the 'feudal system'. 'Feudal system' is a phrase that first appears in the 1750s in the lectures of Adam Smith and in the publications of the Scottish lawyers John Dalrymple and Lord Kames, as well as in Robertson's *History of Scotland*, which referred to 'the feudal system of laws and policy, that stupendous and singular fabric'.[27] The point of the phrase, which most historians of the Middle Ages still employ, was to draw attention to the similarity between the social arrangements of different parts of medieval Europe and to show how the mode of land tenure and the mode of warfare fitted together.

How new was the new history?

This many-sided 'discovery of the social', as we might call it, was not a creation *ex nihilo*. The debt of the new historians to the antiquarian movement was particularly great. The enthusiasm of the humanists of the Renaissance for the revival of classical antiquity went with an awareness of the need to discover what that antiquity had been like: hence the researches of Flavio Biondo, for instance, on Roman marriage, childrearing and whatever 'concerned the way of life of individuals', in other words the history of private life. Biondo in turn followed the precedent of ancient historians from Herodotus onwards, who discussed the customs of foreign peoples.[28] In two domains the legacy of the humanist antiquarians to the new historians of the Enlightenment was particularly rich: the domain of the law and that of material culture.

Many lawyers were also humanists and some famous humanists such as Andrea Alciato and Guillaume Budé, were also lawyers, combining their interests in the study of legal history. In the course of this study lawyer-scholars such as Claude Fauchet and Etienne Pasquier extended

their concern to the history of customs (via their professional interest in customary law) and more widely to changes in a whole 'way of life', a *manière de vivre* as they sometimes called it.[29] The antiquaries of the sixteenth and seventeenth centuries were fascinated by the material culture of past ages as both a source for the reconstruction of past customs and an object of interest in its own right. They filled their studies and cabinets of curiosities with these relics of the past, whether classical (coins and medals, for instance) or 'barbarian' (medieval swords, drinking-horns, seals, and so on), and wrote about these objects in treatises that were often illustrated.[30] What was new in the eighteenth century was the incorporation of the information discovered by the antiquarians into more general accounts and interpretations of the past. This innovation prompts the question why the new history should have emerged at the particular time that it did.

Contextualizing the new history

The new history did not develop by accident in the 'Age of Enlightenment'. On the contrary, it made an essential contribution to the Enlightenment movement. The cosmopolitanism of the Enlightenment is visible in the histories written by Voltaire, Robertson, Hume and Gibbon.[31] The idea of the progress of the human mind (*l'esprit humain*) informed the history of mathematics by Jean Etienne Montucla and the history of humanity by Condorcet, as well as the work of the political and social reformers of the period. The traditional mentalities or 'superstitions' associated with a wide range of practices, from the veneration of relics to trials by ordeal, were objects of concern, as obstacles to reform, at the very time that they were objects of study on the part of Vico, Muratori or Montesquieu.[32]

These points are not coincidences. A number of the scholars cited in these pages were involved with the project of Enlightenment, and the success of their books in their own time stems from the concerns they shared with their public. If they did not tell their readers what they wanted to hear, they did at least offer them the kind of past that suited their interests in the present and their hopes for the future.

The rise of an interest in the history of manners was also linked to increasing knowledge of the world beyond Europe, and the consequent rise of debates among philosophers and others about cultural differences.[33] A still more fundamental condition for the rise of what we have come to describe as social history was the acceleration of social change in the eighteenth century, making both the scholars and their readers aware of the world they had lost, or were in the process of losing, whether they thought the changes were for the better or for the worse. In an age of 'commercial revolution', the medieval past looked increasingly like a foreign country, while the rise of a 'consumer culture' encouraged interest in the simpler material culture

of earlier centuries. As the sub-title of this chapter is intended to suggest, the practice of social history has its own social history.

To these large cultural and social developments should be added some changes in the composition of the reading public. As Roy Porter once remarked, 'It is misleading to examine historical writing without asking about its audience'.[34] Some of the books discussed in the last few pages were among the bestsellers of their day, notably Montesquieu on law and Robertson on the age of Charles V (a work for which the publisher gave him an advance of £3,400, a huge sum by the standards of the time). On occasion their success was international. Giannone's *Civil History*, for instance, was translated into English, French and German, and Gibbon's *Decline and Fall* into French and Italian (and by 1811, German).

The rise of women readers made the public – and the market – more receptive to books on social history. This story should not be told in too simple and dramatic a fashion though. There had been female readers in the Middle Ages, including readers of history. However, there are signs that in the seventeenth and eighteenth centuries more women were reading books, that the books that they read included history as well as fiction, and that some writers of history were courting female readers.[35]

These signs have been studied most carefully in the case of Britain, where Hume famously declared that 'There is nothing which I would recommend more earnestly to my female readers than the study of history'.[36] Individual readers, such as Lady Sarah Cowper and Anna Larpent, have been studied with care. Attention has been drawn to books written specially for this public, such as Charlotte Cowley's *The Ladies History of England* (1780). Some women turned to Lord Clarendon's history of the civil war or to Gilbert Burnet's history of his own time.

I should like to take one further step and argue for a connection between the rise of the female reading public and the rise of social history. There is English evidence, for instance, that women took a particular interest in family history and antiquities, in other words the history of private life.[37] Again, we find Anna Larpent, a lady whose reading habits were recently examined in detail by John Brewer, interested in a number of the scholars who have already appeared in this chapter, among them Hume, Ferguson, Smith and Gibbon.[38]

A similar connection can be seen in France. The seventeenth-century historian Eudes de Mézeray was concerned to attract female readers, and so included more material on manners and customs into successive editions of his history of France.[39] Again, it was to Madame de Rambouillet, who headed a famous intellectual salon, that the writer Guez de Balzac remarked how 'unfortunate' it was to know so much about the battles of the ancient Romans and so little about their private lives. Again, it was in a letter to Mademoiselle Cousinot that Count Boulainvilliers made his famous denunciation of a history confined to wars and high politics, while Voltaire's *Essai*

sur les moeurs was written for the famous bluestocking Emilie du Châtelet. Madame Du Deffand's remarks on the new novel offer a valuable clue to the interests of female readers: 'I love domestic details . . . in novels I prefer them to all the great events and beautiful descriptions.'[40]

The increasing place given to women in the new histories is part of the trend. In the later eighteenth century, three histories of women were published, one in French, one in English and one in German. The 'advertisement' to William Alexander's book declares that the work was 'composed solely for the amusement and instruction of the Fair Sex', while the Preface to the work by Christoph Meiners refers to his readers of both sexes.[41]

In short, the similarities between the new history movement of the eighteenth century and its equivalent in the twentieth go beyond slogans such as 'the history of private life' and 'the history of the everyday'. They extend to the social basis of the two movements, the larger and more varied groups who were both reading and writing history, with female readers more prominent in both cases than in the preceding century, the seventeenth or the nineteenth.

Ranke's reaction

To explain, or at least to contextualize, this European movement it is necessary to look at the places where it flourished most, such as the cities of Paris, Edinburgh and Göttingen, where four of the historians mentioned in this chapter – Meiners, Gatterer, Schlözer and Spittler – were teaching in the later eighteenth century. The next major trend in historical thought and writing, associated with Leopold von Ranke, would have its centre in Berlin, a very different location from the small town of Göttingen, though both towns were the seats of new universities.

There were precedents for Ranke's concern to base written history on official documents rather than unofficial sources, but the change in historical practice that he inaugurated may still be described as a revolution. It was associated with a shift from world history ('universal history', as they called it at the time) to national history, and a return to political events at the expense of social and cultural trends.

The reasons for these changes in the age of the nation-state will be obvious enough. There is a political history of social history as well as a social one.[42] The point I want to make here is that, like most revolutions, Ranke's historiographical revolution had its price, which was to cut off in its prime the practice of social history.[43]

Despite famous survivals such as Burckhardt's essay on the Renaissance, the practice would need to be reinvented at the beginning of the twentieth century, the age of Karl Lamprecht, of the American 'New History' and of a French group that would become the famous Annales School. Marc Bloch,

for instance, placed the feudal system in a broader socio-cultural context than most of his professional colleagues. Fernand Braudel told the story of the impact of capitalism on material culture and everyday life. The third generation of Annales, Jacques Le Goff for instance, turned to the history of modes of thought or 'mentalities'.[44] As for the study of women's history, it was relaunched by number of gifted female historians.

4

The Politics of Particularism: Medicalization and Medical Reform in Nineteenth-century Britain

Ian Burney

Roy Porter loved a good story. His monumental works in medical history consistently focused on individual lives of doctors and patients, the micro-histories of their embodied experiences and their contextually specific and negotiated – and often contested – relations. Stories for Porter served not merely to entertain, though entertainment was, he lamented, an underrated virtue amongst historians. Instead, histories of the individual and the particular were his antidote to the historiographical affliction of abstraction. For the history of medicine, the principal symptom of this chronic malady was the uncritical use of grand analytical categories – modernization, professionalization, medicalization, and the like. These abstractions, Porter insisted, rested on the presumed triumph of what he termed 'the medical model', in which an institutionalized and authoritative medical science and technology progressively expropriated the experience of embodiment and illness, ultimately rendering patients 'paradigmatically passive'.[1] Such accounts, he maintained, tended to be cast in terms of linearity and inevitability, driven by an assumption of the power and efficacy of medical modernizers, of their ability to sweep aside existing structures and relationships and thereby remake the medical world in their own image.

Part of Porter's dissent stemmed from a sympathy with social historians' efforts to restore the voices of those traditionally seen as subjects without agency. Accounts of a triumphant modernization, to him, took for granted the very disablement that they claimed to analyse: medicine's historical objective of silencing the patient was mirrored by a historiographical tendency to deny that those supposedly marginal actors and accounts held anything of interest for medical historians. But Porter's sensitivity to the themes of particularity, politics and agency was also rooted in his unrivalled understanding of the eighteenth-century worlds of medicine. In this fragmented world of theory and practice, there was no place for medicine as a monolithic entity. Patients themselves could exercise powerful control

over their own health and healing experiences, supported by a tradition of self-physicking, a harmony of lay and medical views of the body, health and disease, a biographical approach to medicine that privileged individuals in context over abstract collectives, and a patronage system that supported patient individuation and agency.

Porter's vision of a vital eighteenth-century medical world not only served as a historiographical corrective, but also as a means of questioning that world's demonization by nineteenth-century medical modernizers who (like their twentieth-century successors) saw it as profoundly dysfunctional and ripe for righteous transformation by progressively minded, scientific reformers. Porter's analysis of the Georgian age, in this sense, served as a citadel from which he defended his conception of medical history grounded in the individual and the particular. It offered a base for disrupting key assumptions about medical modernization. Yet Porter was tentative in taking the fight systematically into the historical terrain of the nineteenth century and beyond. Though he often speculated that even in its modern guise medicine was not as modern as usually depicted – that patient power, for example, was never really extinguished, or that medical expertise was always a matter of negotiation – he also tentatively conceded the abstractions beloved of modernization analyses as a possible outcome of nineteenth-century transformations:

> Perhaps such categorical divides have no historical truth whatever (being essentially ideological weapons of war in the contemporary politics of health); perhaps they possess some validity only since some time in the nineteenth century, as medicine has become more specialized and esoteric.[2]

As this rumination suggests, the nineteenth century features in his work largely as a dénouement to the world he relished, and his engagement with it was typically confined to localized skirmishes. In these, one figure occupies a notable and interesting place. Thomas Wakley, provincial surgeon, crusading medical journalist and radical politician, served previous generations of historians as a standard-bearer for the project of liberating the profession from the distortions of tradition. In such accounts, he and his reformist journal, the *Lancet*, exemplified the laudable campaign to sweep away a bankrupt eighteenth-century medical world and replace it with a modern regime grounded in reason and science. Porter's Wakley was notably different. Stripped of the heroic, transhistorical righteousness of his cause, Wakley became a local combatant in the ideological warfare of his time and place, and not an especially attractive one at that. Porter's references to Wakely suggested a degree of distaste: of his crusade against corrupt medical institutions, he counselled historians to 'take Wakley's whingeings with a pinch of salt. A man of passion and prejudice, he habitually dipped his

pen in bile'.[3] Elsewhere Wakley appears as a small-minded Victorian, who 'rested content with performing as the time-honoured iconoclast, sticking the lancet in'.[4]

Such judgements are somewhat puzzling, as there was much about Wakley that might have appealed to Porterian sensibilities. Here was a man of boundless energy, possessed of formidable self-promotional skills, physically and mentally fearless, who used the most colourful language to batter his opponents. Any modern reader who chances on a copy of the *Lancet* from the first several decades of Wakley's editorship cannot help but be struck by the sheer vitality, the vertiginous cascade of invective, the range of topics that seemed to belong to a medical journalist in the first half of the nineteenth century. Wakley does not whinge so much as declaim and denounce. He is less the grim and pious destroyer of icons than the creator of a carnivalesque gallery of grotesques – led by 'Dirty Blind Bats' and the 'Hags of Rhubarb Hall' – parading through the *Lancet*'s pages. Porter also agreed with important aspects of Wakley's agenda: no lover of corporate power or hierarchy, he granted Wakley his critiques of the nepotistic and corrupt practices of an institutionalized elite. But in Wakley, Porter also sensed the demagogue and self-righteous zealot, bent on closing down a plural world of individuals, biographies and difference by erecting a rigid set of professional boundaries derived from the ostensibly universal principles of science.

Porter's unconventional depiction of Wakley, then, derives from the latter's involvement in historical and historiographical excesses: first, through his enthusiastic participation in an historical process of medical modernization, the merits and ultimate efficacy of which Porter remained sceptical; and second, through the place commonly accorded him in an insular and linearly ascendant modernization narrative uncritically constructed by some historians. Yet for all its bluster, Wakley's medical modernity was neither insular, linear nor unproblematically ascendant. Here, I argue that an engagement with the terms of Wakley's reformist crusade – by probing the limits as well as the excesses of medical modernization – can, paradoxically, underscore Porter's insistence on a history of contingency and particularity.

My analysis has three main parts. First, it shows that arguments for medical change – always, for Porter, political and ideological interventions – did not take place in a sealed context of their own choosing. The medical reformism of Wakley and the *Lancet*, most crucially, cannot be separated from the broader framework of early nineteenth-century British politics. 'The abuses under which we groan in all the departments of our profession have but *one* source, and that is, *misgovernment*', the *Lancet* insisted within days of the passing into law of the landmark Reform Act. 'Hence to this one point, and to this one point only', it concluded, 'should the whole of our remedial exertions be directed'.[5] In this identification of misgovernance as the principal impediment to good medical order, medicine and politics

self-consciously merged. Nor was the *Lancet* alone in casting as political the issue of medical reform. Contemporary medical comment – ranging from the orthodox 'ultras' who regarded the threat of medical 'destructives' with apocalyptic foreboding, to reforming 'moderates' who sought to eliminate the worst features of corruption in order to preserve the system as a whole – routinely took an explicitly political cast.

Second, I argue that this politicized framing was not merely rhetorical window-dressing, but that the nature of contemporary political debate had a discernible impact on the way that medical reform was conceptualized. Early nineteenth-century British politics was indelibly marked by the polarizing impact of the French Revolution. In this fraught world, one line of division stands out most starkly. It pitted advocates of a gradualist accommodation of change within the particularistic, individualistic and historical institutions of 'established' English society against champions of change based on universal, abstract, rationalistic principles. In politics, of course, this polarity was most clearly embodied by Edmund Burke and Thomas Paine. In medicine, it can be figured heuristically by Wakley and his *Lancet* allies, representing universalism, and the editors of and contributors to the *London Medical Gazette* (*LMG*), representing a medical establishment anxiously reforming in order to preserve.

The *Lancet* and the *LMG*, then, seemingly represented two distinct and irreconcilable voices of medical reform: on the one hand, unblinking adherents of a French-inspired enthusiasm for a root-and-branch dismantling of historically entrenched medical corruption; on the other, apologists for the peculiarities of English medicine. Yet the third aim of this chapter is to show that this would only be a partial reading of a much richer interchange between medicine, politics and reform. Wakley's medical radicalism, I argue, was capable of embracing both revolutionary universalism *and* ostensibly conservative particularism. In other words, and as Porter suspected but did not concretely demonstrate, the tidy oppositions out of which the modernization analysis are constructed belie, on closer inspection, a more nuanced and contingent historical dynamic. My analysis of the complex political lexicon of early nineteenth-century medical reform in this sense serves as an invitation for further Porterian readings of the nineteenth-century medical world.

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French medicine, like French politics, cast a long shadow across the landscape of British reform. Historians of medicine agree on the broad features of the French medical system that emerged from the crucible of revolution. Enhanced medical control of large public hospitals drew together vast numbers of (poor) patients and enabled their systematic arrangement according to the class of disease suffered. This was facilitated by the elaboration of (often instrumentally assisted) diagnostic techniques designed to

uncover commonalities between ostensibly different symptom complexes, and to elicit information capable of being embodied in comparative statistical representations of 'cases'. Also characteristic was the shift, under the rubric of 'general anatomy', from a view of the body as a large number of discrete and discontinuous organs to one comprised of a more limited set of interconnecting elemental tissues; and the conceptualization of disease as a distinct entity, signalled by lesions on a specific (internal) bodily structure. Finally, autopsy to confirm the suspected correlation of disease symptom and localized bodily lesion was routinized.[6]

Conceived in these ideal terms, French medicine privileged what can be described as a 'universalist' approach to the worlds of health and sickness. At the cognitive level, it emphasized commonality over uniqueness, smoothing over the diagnostic and therapeutic consequences of individual (constitutional, historical, social) difference. At the level of practice, it challenged the traditional hierarchical differentiation between a surgery that treated those localized complaints which did not vary between bodies and an elite physic concerned with the complex symptomologies of highly differentiated and self-knowing patients. It represented, in short, a repudiation of the decentred and patient-driven world of 'bedside' (or 'biographical') medicine. In its place, French medicine posited a reconstructed authoritative medicine grounded institutionally in the hospital, conceptually in the non-dialogic physical examination of patients and politically in the repudiation of historically embedded educational, institutional and indeed corporeal structures.

For Wakley and his sympathizers, French 'scientific medicine' promoted a vision of medicine as a professionally shared, systematically transmissible and meritocratically accessible body of knowledge.[7] A medicine thus unified in theory and practice fitted easily within the ahistorical rationality demanded by enthusiasts of the Revolution: 'the laws of medical science', one declared, 'were universal and republican'.[8] This interplay of political and medical agendas, a commonplace among medical radicals, was most fully articulated by Robert Grant, a leading exponent of French medical science and a key ally of Wakley. Addressing a meeting of the militant British Medical Association in 1841, Grant delivered a sustained attack on hierarchical distinctions – in medicine and in society – that rested on historically specific 'compacts founded on convention'. As products of history rather than reason, Grant argued, these conventions were inevitably contingent and self-interested, and thus tended to degenerate, in the face of ever-advancing knowledge, into 'conservatories of bigotry and ignorance'.[9]

Against this, Grant proposed nature and reason as the proper foundations of a healthy society, grounding this vision in the terms of a modern, universalist medicine. 'Human structure and its derangements', Grant declared, 'do not vary perceptibly with time or place'. From this simple statement – itself a repudiation of the differentiated medicine of the bedside – flowed a critique of the stratified world of corporate medicine. Education 'must be one

and uniform', reflecting the core truth that, as with the unvarying physical body, 'there are no natural grades in the Healing Art'. This principle, Grant argued, extended to the social practice of medicine and entailed sweeping away the system of differently qualified practitioners for the poor and for the rich, a system that was, in his estimation, 'revolting to humanity'.[10] Medical universalism, in Grant's analysis, was part and parcel of the broader reforming spirit of the age: 'the universal race of man is up and stirring', he proclaimed, no longer willing to admire the 'antiquated qualities' of traditional structures.[11] The dead hand of the past, Grant concluded, would only be lifted from medicine and from society by the triumph of radical principles: the popular constitution, popular election, representative government, each predicated on the premise that 'the only legitimate source of [governmental] power, is in the intelligence and will of the entire community'.[12]

Grant's insistence on the unity of physical bodies across 'time and place' and his extension of this principle into the social realm were ultimately grounded in the lessons of modern (French) medicine which, like its politics, demonstrated the error of viewing bodies (physical and political) as highly stratified and historically differentiated entities. Statistics won praise among English radical reformers for its capacity to raise the threshold of medical perception above its distorting emphasis on a case's unique aspects. General anatomy posited this simplified, continuous body at a still deeper level: Xavier Bichat's displacement of an organ-centred mapping of the body (which stressed the structural and functional singularity of the body's component parts) with a tissue-centred vision, presented the body as a network of more fundamental, homologous matter. On a properly scientific analysis of bodies, the surgical firebrand Thomas King declared, the hierarchically differentiated body beloved by the traditional physician was exposed as an illusion: 'what appears complex and difficult when presented in an insulated and immethodical manner, becomes simple and easy in the extreme when viewed scientifically'.[13] With this insight, the University of London's controversial professor of surgery, William Lawrence, informed his students, the task of medicine shifted from the understanding of patient particularity to the recognition of underlying unities within and between bodies: 'The entire structure and functions are universally and intimately connected', Lawrence observed. 'Although individual organs are numerous, the elements of organic structure are few. The various proportions in which they are combined make the difference, as the various combinations of a few letters produce the infinite variety of words'.[14] The analytical interchangeability of the alphabet, its suspension of hierarchy and of history in the pursuit of universal sense, provided for Lawrence a salutary model for a properly scientific approach to the body.

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In the hands of medical radicals, then, the ahistorical and abstract truths of reason and science promoted a vision of the physical body as a template for

reorganizing both the structure of English medicine and the social relations within which that structure was suspended. It was precisely these aspects of medical radicalism that the *London Medical Gazette*, as the voice of conservative medical reform, found most disturbing. In opposing the likes of Grant and Wakley, the *LMG* embraced a similarly political set of arguments, ones grounded not in abstract rationalist principles, however, but in 'native' sensibility. Like their political fellow travellers, medical 'destructives', in seeking to 'annihilate all existing colleges and corporations – all institutions – all grades – all educational regulations', were following the example of French revolutionary excess.[15] Happily, in the *LMG*'s view, this wantonness ran counter to the English predisposition to protect 'the spirit of its ancient and congenial freedoms' and to resist the 'true revolutionary principle of "levelling down" all the old establishments bequeathed by the wisdom of their ancestors, and erecting a wholly new and immaculate system'.[16]

In true Burkean fashion, the *LMG* insisted that abstract propositions ('immaculate systems') obscured more fundamental truths derived from custom. Hierarchical divisions in medicine, for instance, were to be justified less by reference to first principle than by dint of their practical embeddedness in English life:

> these are so firmly established by usage, by courtesy, by chartered rights, and all the provisions of law, that they have become interwoven with the very constitution of society. No enactment, even if characterized by all the fervour which accompanies revolution, could annihilate the existence and practical operation of these divisions in our profession.[17]

Usage, moreover, was grounded in an analysis of the composition and nature of the users who, like practitioners, were embedded in historically sanctioned social relations legitimating the medical system that had developed within them. Stratified medicine in this sense simply reflected social realities. Criticizing the social and medical egalitarianism espoused by Grant and his fellow 'ultras', the *LMG* observed that: 'It is all very fine to tell us that science knows no gradations – that the diseases of the poor require the same skill as those of the rich – which may be true in the abstract, but which practically is mere talk. When the Deity ordains that ranks, gradations, and distinctions in society, shall cease – that there shall be no rich man and no poor – that is to say, when the millennium comes', medical uniformity might be contemplated. In the meantime, defenders of differentiated medical estates ought to maintain or improve the educational and practical conditions within which practitioners could operate according to the distinctive social characteristics of their clients.[18]

In its tendency to value practical results over abstract truths, the content of English medicine, like its form, rightly reflected English sensibilities. The *LMG*'s very first editorial conceded that continental students, compared

with their English counterparts, received 'a more elaborate scientific education . . . and have a more minute knowledge of morbid anatomy, that is, of disease, as an object of natural history'. Yet, when evaluated on its practical results, it was the continental scientists who were found wanting, a result that could easily be explained with reference to national character – that is, by the 'plain practical sense . . . for which the English are remarkable'.[19] The supposed inadequacies of English medicine – its lack of system, its individualist orientation – were in fact the source of its strength, 'partak[ing] of the free character of all the institutions of the country'.[20] This recourse to custom and usage clearly sought to displace the reckless progressivism of the 'republican' system-builders, setting English traditions as a bulwark against the pretences of universalizing science.

Conservative reformers, like their 'levelling' antagonists, made their medicine fit their politics. For them, medicine was a historically sanctioned system responding to the needs of historically situated individuals, with a correspondingly particularist human body as its proper object. The obsession of foreign anatomists with locating and minutely describing morbid lesions, the Barts consultant physician Peter Mere Latham informed his students, threatened to turn clinical instruction into 'a discussion of abstract pathology and therapeutics'.[21] 'Diseases are not abstractions', he continued, 'they are modes of acting . . . modes of disorganizing, modes of suffering, and modes of dying; and there must be a living, moving, sentient body, for all this'.[22] The same could be said for other signal techniques of French medicine. Diagnostic signs that yielded to statistical representation, for example, were of lesser value than those that indicated something unique about the case at hand. Thus, Latham urged, in time-honoured fashion, that his students attend to qualitative diagnostic features (the tonality of a pulse rather than its rate, for instance), since 'these qualities, much more than its mere number, serve to guide us'.[23] General anatomy came under similar criticism. Its tendency to break down bodies into underlying, unifying elementary tissue bypassed attention to the body as a collection of discrete organic units, and artificially reduced the body to an assemblage of homologous matter. The result, the metropolitan physician Leonard Stewart warned, was an overly abstracted schema that failed to provide a realistic representation of the body's structure or function. Such schemata 'may be called scientific', Stewart observed, but 'have no similitude with the tumult and activity of living pathological phenomena'.[24] In its systematizing and abstracting zeal, then, continental science had lost sight of the qualitative aspects of life – its 'tumult and activity' – that were the proper remit of the practical, English medical man.

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Thus far we have considered the porousness of the early nineteenth-century programme of medical modernization, and its substantive inflection by

historically-specific political discourses of reform. Historical abstractions, as Porter insisted, are inevitably filtered through the particularities of time and space. But this excursion into the politics of early nineteenth-century medical reform can illustrate a further point: medical modernizers did not speak an insular, unitary language. Until now we have seen a reformist discourse polarized along an ostensibly stable opposition between medical traditionalists and revolutionaries, with Wakley cast in the role as an arch-modernizer bent on destroying the historically nurtured institutions of medical society. Yet this vision of medical radicalism overstates the case. When one examines Wakley's brand of reform more closely, the distinction between the historical and the particular on the one hand, and the universal and the abstract on the other, begins to lose its clarity. As I have argued at length elsewhere, this is certainly the case for one of the sustained campaigns waged by Wakley and the *Lancet* during this period – the reform of the ancient inquest system. Against what one might expect of a French inspired 'ultra', Wakley framed his demand for a medical coronership resolutely within the discourse of English liberties, blending his claims for an authoritative place for medicine with an insistence on the need for open, public participation at inquest deliberations.[25]

This emphasis on openness, for Wakely, was grounded in a strain of English radicalism that upheld the protean principles of popular constitutionalism as a bulwark against despotism.[26] His immersion in this political tradition resulted in the curious bilingualism that marked his pronouncements on medical reform, as illustrated by his call for the public scrutiny of medical examinations and appointments. On the one hand, these demands had impeccable revolutionary credentials, the French system of *concours* (or public contest) being favourably contrasted to the closed practices of examination and appointment operating in the corrupt world of English medicine. But Wakley also denounced the secrecy of the medical establishment from within the discourse of English popular liberties, describing them as modern-day equivalents of the Star Chamber beloved by Tudor and Stuart despots, and routinely vilified by early ninteenth-century radicalism.[27] This more historically minded critique also served as a generalized framework for conceptualizing medical reform. Urging rank-and-file practitioners to pressure the medical establishment for fundamental reform – in his words, to obtain 'their Magna Charta' – one *Lancet* correspondent insisted that 'there cannot be a better pattern of liberty than is to be found in the constitution of England, which gradually attained its perfection by the people unanimously and repeatedly insisting on their rights'.[28] Radical medical reform, notwithstanding the charges of continental abstractionism levelled against it by its critics, could and often did embrace historical wisdom and cultural particularities as a foundation for progressive renewal and repair.

We can find another example of this dynamic by examining an issue closer to the explicit themes of this essay – Wakley's views on the nature

and constitution of representative institutions. In his explicitly political pronouncements, Wakley typically positioned himself as an exponent of 'direct' democratic representation, in which representatives were chosen and bound by the majority view expressed by an expanded electorate divided into equal franchise districts. But Wakley also articulated the more traditional, 'virtual' view of representation featuring qualitatively distinct electoral constituencies, based not on number but on property and intelligence.[29] Accepting the central tenet of moderate reform – that English representative institutions needed to be adapted in order to eliminate the worst aspects of corruption and to incorporate 'rising' elements into the system – Wakley argued that the rank-and-file general practitioner constituted one such element deserving special inclusion in a renewed constitutional dispensation. In an editorial promoting 'Representation of Medical Men in Parliament', the *Lancet* insisted that, according to constitutional theory, Parliament not only represented society as an abstract collective, but also 'the individual interests of each, even of the smallest departments of the whole common weal'. Yet unlike other categories of recognized particular interest, the rising ranks of progressive medical scientists had no viable representation. True, the 'ancient wisdom' of the constitution provided for representation of science in the form of university seats, but these had fallen prey to corruption, chosen for patronage rather than 'the advancement of the interests of science'. In the face of this degenerate system, the *Lancet* demanded restitution, but in terms far from those of a levelling firebrand:

> Let us, therefore, at least hope, as the original draft of our constitution presumes a representation of the medical, as well as of every other great division of the state, that in any reform of the abuses, or addition made to its present powers, this salutary indication may not be slighted; but that, instead of being a mockery, as at present, it will be improved into a virtual advocacy of the interests of science and its practical cultivators.[30]

This adaptation of the constitutional model of virtual interest to secure representation for progressive medicine, Wakley insisted later, was not retrograde special pleading: 'It is a claim which conciliates every interest, binding together, by the sacred fellowship of knowledge, the extremes of society, awakening in all, by the eloquent voice of science, the purest feeling of brotherhood and the deepest sense of mutual responsibility'.[31] Here again the language of historical particularism supplements the case medical reform. Adding *medical* science to the list of specially constituted interests is not rejected on universalist principle, but is instead embraced as means of promoting medical modernization and at the same time of bringing harmony to the English constitution.

The *Lancet*'s claim that a constitutional adjustment enabling parliamentary representation of its brand of medical science would be welcomed

by the whole of society was, as Porter recognized, a contentious one. This is because its version of science was, as has been argued above, an inextricably politicized intervention in the contested field of medical reform. The meritocratic overtones of the call for science in medicine were, at one level, directed against the patronage-bound world of the medical corporations. But Wakley and his colleagues were not simply fighting a battle against the medical elite. Instead, their calls for progressive reform were aimed at establishing the scientifically trained general practitioner as the cornerstone of a medical world that defined itself against the retrograde forces of the elite *and* what they took to be the growing threat of medical irregulars. 'Never have quacks, quackish doctrines, and quack medicines, exercised a greater influence over the minds and bodies of the people of this country, than they exert at the present epoch', the *Lancet* warned in 1835. The cause of this scourge was clear: it was the necessary consequence of 'the odious, the exclusive conduct of our rotten and contemptible medical corporations'.[32] By excluding the medical rank and file from full membership, the royal colleges demeaned the majority of practitioners before their public, implying that their qualifications were not sufficient to distinguish them from those practising without a licence.

The *Lancet*'s anti-quackery campaign was in this sense an integral component of its ultimate aim of eliminating medical corruption. But the campaign also exposed the limits of its universalist credentials, and led Wakley to a further embrace of traditionalism, this time as a counter to demands for a still more radically egalitarian medical world. In an editorial response to a 'heretical' suggestion that all corporations be eliminated in the interest of medical 'free trade', for instance, he recoiled:

> it is no argument, because corporate and other public medical bodies have behaved wrongfully, and created much injustice . . . that, as a consequence, all medical corporate bodies, present and future, are, and must be, radically bad and mischievous. The present evils are a very good reason for reform, but hardly, as we conceive, for destruction, except in the minds of very rash and sanguine persons.[33]

In its pursuit of meritocratically derived distinction, the *Lancet* also found itself upholding exclusive privilege. Medical sectarians challenged Wakley's demand that public medical posts be restricted to licensed practitioners, dismissing this as a form of retrograde monopolism. Wakley countered this by invoking the language of constitutionalism: the 'possession of special rights', he argued, had an irreproachable pedigree: 'All the ancient charters were conferred on particular bodies of our ancestors with a similar object, and, in more modern times, we have witnessed the practical operation of the theory which sustained that principle'.[34]

As an organ of radical reform, then, Wakley's *Lancet* was anything but straightforward. Resolutely enthusiastic about a French-inspired model of

medical universalism, it simultaneously embraced a version of English particularism; while championing participatory institutions as a check on monopolistic corruption, it argued the cause of exclusionary professional prerogative; a proponent of democratic egalitarianism, it upheld interest-based representation based on the wisdom of England's historic constitution.

These conflicting attributes enable us to reconsider Porter's relationship to Wakley, his brand of nineteenth-century medicine and reform, and to their evaluation in modern historical accounts. For Porter, Wakley is best understood not as the embodiment of a righteous professional progressivism, but as a political animal engaged in local skirmishes and representing historically specific interests. The analysis in the foregoing pages supports the main features of this reading. Yet it also suggests that we might go further than simply recognizing the contextually-specific politics of professionalization. Interrogating the terms of Wakley's contingent and political intervention in the world of early nineteenth-century medical reform shows how the ostensibly stable and oppositional categories that structure the modernization story in which Wakley stars were in and of themselves contingent and political. In Wakley's reformist lexicon, the meanings and uses of abstraction versus particularism, universalism versus historicity, rather than pre-established givens able to be deployed unproblematically by opposing camps to clear effect, were in fact explicit stakes in the debates, formulated and deployed in real, contested time. The main categories of nineteenth-century medical modernization, this suggests, should be reinjected with notions of story and indeed history, not merely as prior, pristine truths that might serve to recover lost voices, but as forces that informed competing contemporary visions of the process itself. Particularism and historicity, the core features of the Porterian account of the eighteenth-century medical world, also belong in the conceptual and analytical toolkit of nineteenth-century medical historians.

5

Charles Babbage and George Birkbeck: Science, Reform and Radicalism

Dorothy Porter

Two strands emerge in the early nineteenth-century debates over the reform of science and democratization of society. Both believed firmly in the moral and practical value of science to society. However, proponents of a professional, elite science faced off against those who believed the broadening of the base of participation would more readily deploy its reforming effects. Here I will explore those arguments, and their implications for science policy today, in relation to Charles Babbage, the inventor of the first analytical calculating engine, and George Birkbeck, the founder of the first Mechanics Institute. Babbage and the nineteenth-century Romantics were radical Whigs who wanted to establish science as a gentlemanly profession which would reform society by creating a meritocracy and supporting technological innovation in manufactures. George Birkbeck was a doctor and an educator who believed that science could become the most powerful democratic social force if all were given access to it through mass education.

Babbage and his associates John Herschel, George Peacock, Alexander D'Arblay, Edward Ryan and Edward Bromhead, who formed the Analytical Society at Cambridge University in 1812, thought that science was in decline in Britain because British education continued to prize the study of classical literature over discovering the secrets of nature.[1] They reasoned that British scientific institutions were corrupted by aristocratic oligarchies who had never attempted a mathematical proof or stepped into a laboratory, and that the government had no interest in changing the status quo. The Analyticals believed that conservativism had held science back throughout the eighteenth century in Britain allowing the continentals, and above all the French, to take the lead in mathematics, chemistry and astronomy. When the Analytical Society was formed, all Cambridge intellectual groupings were suspected of political sedition and broken up by the ruling Dons. Even the Analyticals' aim to reform mathematical notation by 'propagating the gospel of the "d"s and consigning to perdition all supporters of the dots' – that

is replacing Newtonian dot notation with Leibniz's 'd' – was regarded as treason.[2] The Society confounded its reputation for treachery by eventually publishing Herschel and Peacock's translation of Sylvester Lacroix's *Sur le calcul différentiel et integral*.[3] But between 1812 and 1814 the Analytical comrades solidified their radical beliefs about the reform of science, society, government, the economy and education which inspired their subsequent careers. Peacock became the most radical nineteenth-century reformer of Cambridge University, integrating science fully into the curriculum,[4] and Babbage and the others fought oligarchy in the organization of science by setting up rival clubs and associations to the Royal Society, including the Astronomical Society, the British Association for the Advancement of Science and the London Statistical Society. As the historians Morrell and Thackray have argued, the BAAS was formed with the aim of establishing science as a gentlemanly profession based on intellectual talent.[5] Babbage believed that professional scientific associations could become forums for fulfilling the Analyticals' dream of integrating the advancement of science into the growth of industrial technology in Britain.[6]

The Analyticals' zeal for reforming science was matched by their allegiance to radical liberalism, which equally took its cue from French philosophy. The materialist philosophies of the ideologues Condorcet and Cabanis fuelled their Benthamite avocation of government based on the politics of expertise, and social organization based on rationalistic values instituted through a legal code.[7] Through his wife's family Babbage had met the Prince of Canino, Napoleon's brother Lucian Bonaparte, when he was living in exile in Worcestershire. In a fit of enthusiasm Napoleon had appointed the mathematical astronomer and probability theorist Pierre Simon Laplace as Minister of Interior, at which the mathematical astronomer quickly proved to be an unmitigated disaster. Napoleon replaced Laplace with Lucian, but the two brothers quarrelled as one moved closer and closer to dictatorship and the other became an ever more committed republican. To Babbage, Lucian Bonaparte represented the best of French politics: revolution without the Terror, a Bonaparte without Bonapartism, the Republic against the Empire. Equally, he represented the vitality of Paris's intellectual society with his European view that political and social change were inherently bound to a revolutionary approach to science.[8]

When he wrote the pamphlet on the *Decline of Science* in 1830, and his book *On the Economy of Machinery and Manufactures* in 1832, Babbage outlined his vision of how the reform of science could change British society. Furious after failing to get his friend John Herschel elected president, following Humphrey Davy's resignation, Babbage used the *Decline of Science* to attack the Royal Society as a self-perpetuating oligarchy of aristocrats, most of whom knew little about science and those who did insisting on natural philosophy remaining an esoteric sophistry of abstract reflection. He highlighted how the reactionary values of the Society retained a structural stranglehold on scientific research and education. Babbage wanted a

meritocratic system of state funding to be established which would nurture talent and allow an independent class of professional scientists to apply science to the service of economic expansion. He bemoaned the lack of professional inducements to enter science:[9]

> The pursuit of science does not, in Britain, constitute a distinct profession, as it does in many other countries . . . . As there exists with us no peculiar class professedly devoted to science, it frequently happens that when a situation, requiring for the proper fulfilment of its duties considerable scientific attainments, is vacant, it becomes necessary to select from among amateurs.

Among the army of amateurs, he prominently included 'those who have chosen the profession of medicine' because they were required to pick up a little chemistry and natural history to ply what Babbage nevertheless decisively identified as their practical trade in healing.[10] He indicted the government for providing no national support for scientific inquiry even though British society had clearly profited from discovery and invention.

Teaching posts in universities were not enough to foster a progressive scientific culture. Few institutions had the power to create such positions, and when they did the 'emolument is small' and 'the lectures which are required from the professor are not perhaps in all cases the best mode of employing the energies of those who are capable of inventing'.[11] Babbage cited Galileo's view of the conflict between teaching and research:[12]

> My private lectures and domestic pupils are a great hindrance and interruption of my studies, I wish to live entirely exempt from the former, and in great measure from the latter.

Babbage argued that if it was inappropriate for a province like Tuscany to make one of its legendary natural philosophers teach for a living, then surely it was even more reprehensible that a wealthy society like Britain should 'allow a genius like Mr Dalton [the chemist who developed atomic weight theory] to be employed in the drudgery of elementary instruction'.[13] Babbage stridently inquired, 'where would have been the military renown of Britain, if, with an equally improvident waste of mental power, its institutions had forced the Duke of Wellington to employ his life in drilling recruits, instead of planning campaigns?'[14] The comparison with France highlighted the deplorable state of things in Britain: Laplace dedicated *Mechanique Coeleste* and the *Theorie Analytique des Probablilités* to Napoleon because 'During the reign of that extraordinary man, the triumphs of France were as eminent in Science as they were splendid in arms'.[15]

The prospects of a young British man 'impelled by an almost irresistible desire to devote himself to the abstruser sciences'[16] were for Babbage almost

hopeless because '[t]here are no situations in the state; there is no position in society to which hope can point, to cheer him in his laborious path'.[17] And a university career was not an attractive or easily attainable option:[18]

> If, indeed, he belong to one of our universities, there are some few chairs in his *own* Alma Mater to which he may at some distant day pretend; but these are not numerous; and whilst the salaries attached are seldom sufficient for the sole support of the individual, they are very rarely enough for that of a family.

(Babbage knew all about the difficulties of trying to get a job. All his applications for university positions had been turned down by this point, and none of them would have provided him with a sufficient salary to support his family of seven children.)[19] Thus disillusioned, our talented young natural philosopher would quickly come to the realization that he[20]

> must give up that line of life in which his habits of thought and his ambition qualify him to success eminently, and he *must* choose the bar, or some other profession . . . the loss to him is great, but to the country it is greater. We thus, by a destructive misapplication of talent which our institutions create, exchange a profound philosopher for but a tolerable lawyer.

Babbage's remedy was a system of state funding for scientific research plus the payment of adequate academic salaries to allow the support of family life with sufficient time for independent study while serving the needs of students. He argued that the institution of a professional class of scientists engaged in experimental and abstract reasoning would give Britain the innovatory edge over its competitors in manufacturing, and would disseminate the democratic principles on which scientific knowledge was based, thus transforming the values of British society and eliminating the bigotry of tradition and mysticism.[21]

As I have mentioned, their failure to penetrate the robust defences of the Royal Society led Babbage and his fellow reformers to pursue their goals through the foundation of alternative clubs and journals and by the transformation of educational curricula. They used the BAAS as an extra-metropolitan 'parliament of science' with the power to legitimate knowledge-making through a travelling roadshow of provincial associations and meetings. Babbage and his coterie of gentlemen scientists believed dissemination would professionalize science and release 'useful knowledge' from its status as the moribund property of an aristocratic oligarchy.[22] However, while the gentlemen scientists of the 1820s were prosecuting their 'top-down' model of the cultural dissemination and professionalization of

science, a completely different social class of knowledge-makers, of whom Babbage was highly suspicious, was developing a 'bottom-up' model.[23]

Babbage thought that natural philosophical talent could come from any social quarter. No case demonstrated this more clearly than the career of the baker/corn miller mathematician, George Green, who wrote the first essay on the mathematics of electricity and magnetism. Babbage's friend Edmund Bromhead had been critical in supporting Green's career from the bakery to 4th wrangler at Cambridge.[24] Nevertheless Babbage, Herschel and their generation of gentlemen scientists were insistent that universalizing 'scientific knowledge' was ontologically separate from cunning 'craft', which was invested with particular interests and shrouded in secrecy.

Recently, the historian of science, Iwan Morus, has pointed out how the relationship of instrument-makers to the scientific community dramatically changed between the seventeenth and early nineteenth centuries.[25] At the time of its founding, philosopher-demonstrators like Robert Hooke could aspire to a fellowship of the Royal Society despite his never-ending disputes with Isaac Newton over proprietorial knowledge rights. By the early nineteenth century disputes between gentlemen scientists like Babbage and his engineer Joseph Clements and between one of the co-founders of the Astronomical Society, John South and his telescope-maker Edward Troughton drew a much firmer demarcation between science and craft. Babbage and South both tried to marginalize their craftsmen's claims to be knowledge-makers.[26] Clements, whom Babbage acknowledged was critical to the design of his Difference Engine, had threatened to take the specific tools he created for the enterprise with him when he left the project. South had accused Troughton of imprecise engineering to account for faults in a new telescope he commissioned to house a 12-inch glass he had been given by Cauchoix. South refused to pay Troughton's expenses unless all future work was done under his direct supervision. But Troughton refused to compromise his independence and, like Clements, asserted his ownership of the knowledge that produced practical results. Clements and Troughton based their knowledge claims on the traditional system of rights that operated in the workshop rather than the laboratory.[27]

As Morus points out, Babbage's close friend, the astronomer and mathematician Herschel, articulated the natural philosopher's suspicion of the craftsman's protectionist tradition of knowledge ownership in his *Preliminary Discourse on the Study of Natural Philosophy*. Herschel was clear about the hierarchical pedagogic nature of craft, backed up by monopolistic power operated by guilds which he tellingly compared to the oligarchy of the Royal Society. Craft was protected by technical secrecy and rigorous control through the apprenticeship system. It was able to bury knowledge 'in technicalities, and so place pride in particular short cuts and mysteries known only to adepts; to surprise and astonish by results, but conceal processes'.[28] By contrast, scientific knowledge was characterized by transparency and

universal accessibility through experimental evaluation and disinterested, irreducible measurement. Scientific knowledge must be 'Divested, as far as possible, of artificial difficulties and stripped of all such technicalities as tend to place it in the light of a craft and a mystery, inaccessible without a kind of apprenticeship'.[29] (By such a measure, modern scientific practice may well have been identified by Herschel as a craft.)

Morus argues that demarcation disputes over knowledge-making between gentlemen scientists and instrument-makers in the 1820s represented a confrontation between head and hand. Traditional hands-on ownership of practical knowledge was being challenged by abstract notions of expertise. This was a battle over social status, credibility and control of resources.[30] But just at the time that Babbage, South and numerous of their compatriots were scrapping with their engineers and tool-makers, artisan knowledge claims were being radically defended by London journeymen, ex-midshipmen, patent agents and practitioners of what Babbage dismissed as an amateur scientist trade: medicine. Another model of scientific democracy emerged from within the London Mechanics movement which was linked to a profoundly wider view of social change.

In 1823 the founders of the *Mechanics Magazine* – the ex-midshipman whom some historians have claimed taught Karl Marx the theory of surplus value, Thomas Hodgskin, and the journalist and patent agent from Fleet Street, Joseph Clinton Robertson – told their readers that mechanics pressed the power of nature into their service and that artisan science represented a continuum between workshop and laboratory practices.[31] Like Babbage and the Analyticals, they believed in the emancipatory potential of educational reform.[32] After reading about the Glasgow Mechanics Institute which had originated with the mechanics classes taught at the Anderson Institute by George Birkbeck, Robertson wrote a manifesto in his magazine in August 1823 for the creation of a London Mechanics Institute. Birkbeck eagerly responded and teamed up with Roberston, Hodgskin, Francis Place, Henry Brougham and the master engineers John Martineau, Bryan Donkin and Alexander Galloway at a meeting of about 50 at the Crown and Anchor Tavern in the Strand in November to launch an appeal for funds. After a series of further public meetings – eventually attracting 2,000 participants – subscriptions were taken and the first Mechanics Institute opened at the Presbyterian chapel, Monkwell Street in February.[33] Although Robertson had initially welcomed Birkbeck's enthusiasm and supported his election as the first president, within a year he was accusing Birkbeck of betraying the original goals of the institution and making it his personal fiefdom. The dispute was over the role of philanthropy and the model of pedagogy offered.[34]

Morus has pointed out that Robertson and Hodgskin had wanted the Mechanics Institute to be self-sustaining, funded through the mechanics' subscriptions alone so that trhey could retain their independence from the

material influence of masters in the guise of philanthropists.[35] Ignoring the hierarchical organization and protectionist property rights that characterized guild pedagogy, both Robertson and Hodgskin believed that artisan science education would freely exchange knowledge among equals.

Disputes began over the London Mechanics Institute from the outset when Robertson and Hodgskin led a rebellion against William Bayley, an Islington ironmonger, to prevent him from making an address to raise further money and against Francis Place's initial draft of a set of Rules and Orders.[36] But outright war followed once significant funds had to be raised to construct a lecture theatre after the Institute moved to leased accommodation in Southampton Buildings, Chancery Lane.[37] Place successfully raised £1,000 from Francis Burdett and got his fellow radical MP, J. C. Hobhouse, to give £100, as did Jeremy Bentham. Even Lord Byron gave £50, but the majority of the sum for the new lecture theatre was a £3,700 loan to the Institute by Birkbeck himself at an interest rate of 4 per cent. Robertson now launched an all-out attack on the undermining of the principles of independence and self-support and characterized Birkbeck's loan as

> the most objectionable of all the objectionable measures by which the management of the Institution has been distinguished. It is placing the Institution under the foot of one man; it is making a private speculation of what was meant to rest on the broadest basis of public co-operation and utility.[38]

Birkbeck was vigorously defended by Francis Place, but the Institute lost the support of the *Mechanics Magazine* and received damagingly bad press from the *Literary Chronicle*. Things got even worse when the secretary, James Flather, fled London pursued by his creditors and Robertson himself was discovered to have embezzled some of the original subscriptions.[39]

From the earliest days of his support for the venture, Birkbeck had championed the value of artisan intellectual, educational and material independence. In his initial letter to the *Mechanics Magazine*, in response to Robertson's manifesto, he wrote:[40]

> Of that part of your proposal which relates to the contributions of the mechanics, I do entirely approve. Whilst science was to them of doubtful value, it might be offered, as was formerly the case, gratuitously. But its worth, in the most mercantile acceptation of the word, is now indisputably established; and it is right, in order that it may be fully estimated, and in its possession be unaccompanied by any feeling of dependence, that it should become their own by purchase.

Although he acknowledged Robertson's arguments about the necessity of independence, Birkbeck believed that dissemination of the dialectical relationship between practical and theoretical knowledge must take

precedence. When he first conceived of the mechanics' classes at the Anderson Institution, Birkbeck had recognized the link between the workshop and the laboratory. Innovation resulted from 'the observation of practical defects and from the gradual application of suitable means, dictated by practical maxims, to obviate or remove them',[41] but this could not be separated from 'the principles upon which they operate'.[42] His motivation for giving the Anderson mechanics lectures was to provide the means 'for engrafting upon the elementary truths mechanics learn, the extensive researches of the illustrious philosophers, by whom the boundaries of science have been enlarged'. For Birkbeck the priority was that 'the door of Science should be thrown open, and that its mysteries should be revealed to all mankind . . . without reference to age, occupation, or condition', even if you had to raise money from wealthy patrons to provide the accommodation in which to do it.[43] He believed that the rules governing the proportion of working-class representation in the governance of the Institute ensured that it served the mechanics' interests, and that the biggest challenge to existing political structures would be made by the kind of knowledge that would be taught.[44] Birkbeck had already experienced the way that mass education could undermine hierarchy and protectionism through his involvement in another radical educational experiment in the 1820s and 1830s, the expansion of metropolitan private medical schools.[45] His association with those fighting the forces of reaction within medicine was an excellent training ground for strategizing against social inequity and intellectual authoritarianism, as he had discovered through his role as a teacher in the Aldersgate Medical School and physician to the dispensary.

The first three decades of the nineteenth century witnessed a ferocious war within the medical profession between an emergent class of 'proletarian' general practitioners and the traditional ruling elites who ran the medical corporations. The growing army of general practitioners increasingly challenged the oligarchic grip of the Royal Colleges over medical education and access to the profession as their economic domination of the healing market expanded.[46] Qualified general practitioners were largely the product of the integrated medical curriculum offered at Edinburgh from the mid-eighteenth century and many of them who returned to practise in London brought the radical philosophies of the Scottish Enlightenment with them.[47] Birkbeck was himself a product of this system. It was at Edinburgh that he formed close friendships and alliances with philosophical radicals such as Brougham, Sydney Smith and Francis Horner and future scientific and literary luminaries like David Brewster, Thomas Thomson and Walter Scott. Edinburgh also nurtured the growth of French materialism and transmutationist philosophies among its medical students.[48]

Education was a critical battlefield in the early nineteenth-century medical wars. The proliferation of private medical schools in London seriously challenged the oligarchic rule of the Medical Corporations. Throughout the

eighteenth century medical education had become a booming business as population growth and more widely distributed economic expansion increased demand for medical services.[49] The expanded medical marketplace, together with the first measures of state regulation following the Apothecaries Act 1815, stimulated an explosion of private medical schools throughout the metropolis to meet the huge increase in demand for preparation for the LSA. Little schools were set up by what the Royal College of Surgeons called 'destructives'. These were anatomists and general practitioners who were the sons of shopkeepers and industrial manufacturers and whose clients were not wealthy aristocrats, but came from among the trading classes and the working poor. Often proprietors had been goaded into creating their own medical schools by being excluded from teaching or practising in the London teaching hospitals because they were members of what elite hospital surgeons dubbed the 'low-born, cell-bred, selfish, servile crew', who were 'not gentlemen and very dirty'.[50] Proprietors located their schools close to the large hospitals in order to steal students by offering cut-price courses. They also successfully competed with the hospital schools for the precious commodity of cadavers and offered anatomy courses all year round, including in highly successful summer schools. The Royal College of Surgeons attempted to control the proliferation of medical schools with bylaws preventing summer dissections – with the excuse of atmospheric contamination – but until the 1840s they failed. Private schools also became hotbeds of ideological controversy where private teachers taught the seditious transmutationist philosophies of Geoffroy St Hilaire in comparative anatomy classes. Thus the little schools became breeding grounds for medical radicalism which thirsted for a bloody dismemberment of the oligarchy of 'Bats' which ran the medical corporations and tried to control the healing economy. The little schools became another weapon in the armoury of luminary intellectual and political radicals such as the Geoffroy disciple-anatomist Robert Grant, and his early comrade in arms, the pub-fighter general practitioner-turned-radical MP Thomas Wakley (who was identified by his opponents as 'the *honourable* (!) member who represents the Jew clothes-venders of Finsbury') and who was the viciously witty editor of the *Lancet*.[51]

Birkbeck had joined the Aldersgate General Dispensary in 1807 just after the death of his wife. The Aldersgate Dispensary was the first of its kind, founded in 1770 by John Coakley Lettsom to serve the London poor. By the time Birkbeck joined, numerous other dispensaries had been established, which together provided a major voluntary social service in the metropolis. Birkbeck's partner at the dispensary was Henry Clutterbuck, who became the editor of the *Medical and Chirurgical Review* (a forerunner to the *BMJ*) and one of the metropolis's best-known medical figures. After the Apothecaries Act was passed, Clutterbuck persuaded Birkbeck to join him in establishing the Aldersgate Medical School. Birkbeck taught anatomy there and was its

resident physician. By the early 1830s Aldersgate School's council included unionists and members of the democratic London Medical College, set up by Wakley and Grant at a meeting at the Crown and Anchor Tavern – the centre of all radical social intercourse it seems – as a counter-organization to the Royal Corporations.[52] According to the historian Adrian Desmond, Aldersgate School became the most radical medical institution in the metropolis, a centre of agitation and Parisian materialism.[53] Birkbeck continued this tradition at the Mechanics Institute, taking the extraordinarily bold measure of teaching a course of anatomy there in 1827, which included the dissection of a cadaver.[54] Anatomy teaching was at the centre of scientific controversy at this time because it offered the opportunity to introduce the blasphemous transmutationist philosophies of Geoffroy. Teaching anatomy using public demonstration of human dissection to working-class men who were not medical students was even more shocking than Thomas Southwood Smith's dissection of Jeremy Bentham at Edward Grainger's Webb St medical school. Birkbeck's courage in taking this step brought huge praise in the *Lancet* from Wakely, who commended his bravery in 'dangerously' defying the most sacredly protected pedagogy of the Medical Corporations, and from Francis Place, who said that Birkbeck's courageous act made him 'the most excellent, the most sincere and most invaluable friend of the working class'.[55]

Birkbeck's experience of the politics of knowledge, education and hierarchical power within medicine reaffirmed his view of educational access as the greatest democratizing force. He shared Babbage's ideals about the power of scientific knowledge to change values and restructure society, but he had a completely different model of how that could be achieved. Babbage's strategy was to create a gentlemanly class of professional scientists who would offer the world discoveries that could be applied to the development of manufacturing. By contrast Birkbeck believed that science could change society only if all classes participated in it. Like Robertson and Hodgskin, Birkbeck believed that there was a continuous relationship between the mechanics' art and experimental science which could be demonstrated only by allowing mechanics the opportunity to understand the abstract principles on which their practical knowledge was based. Birkbeck uniquely prioritized educational access as the means by which the democratization of social power could be achieved.

Conclusion

What implications did these early nineteenth-century debates about the reform of science and the democratization of society have for developments in the twentieth century?

Babbage has been credited by later generations of British scientists with being responsible for changing the social culture of science which led eventually to the establishment by Parliament of Scientific and Medical Research

Councils which allowed science to be conducted on a professional basis. Equally, science became integrated into university curricula throughout Britain, significantly increasing the number of academic scientific posts.[56]

The professionalization of science did not lead, however, to the democratization of society through an automatic rationalization of belief as Babbage had assumed it would. In the inter-war years of the twentieth century a group of British socialist scientists, who saw themselves as intellectual descendants of Babbage and political descendants of Robertson and Hodgskin, decided that the socialist transformation of society could only be achieved by popularizing science.[57] Mathematician–turned-social biologist Lancelot Hogben wrote books on *Mathematics for the Millions* which he believed would liberate citizens through self-education in science because 'no society is safe in the hands of so few clever people'.[58] Grandson of Thomas and brother of Aldous, the biologist Julian Huxley believed that the dictates of evolution should provide new ethics of social cohesion by creating a brave new world in which eugenics was a religion[59] – a rather ironic view given his brother's dystopic vision of the same thing.[60] J. B. S. Haldane, who with R. A. Fisher created the modern synthesis of evolution, wrote popular allegories to tell the world that despite the great destruction produced by the application of the physical sciences to warfare, the biological sciences could create peaceful societies.[61] The communist crystallographer John Desmond Bernal wrote science fiction to tell the world that scientifically planned societies would translate the inherently communistic universalism of science into truly egalitarian idylls – albeit in off-world airless spheres.[62] This group of interwar scientists who wanted to popularize science in order to bring about a socialist revolution has been characterized by the historian Gary Werskey as a *Visible College*.[63] Within the confines of this chapter it is not possible to tell the full history of the popularization of science in Britain from the Visible College to the creation of contemporary academic chairs in the public understanding of science. One conclusion can be drawn without doing that, however: despite all the efforts to popularize it, the role played by science in the social democratization of society has been fiercely contested.

The popularization of a public understanding of science has not been able to protect it from the accusation that is the author of authoritarianism rather than a democratic social force. At the same time as the Visible College was trying to save an apocalyptic world by popularizing science, a critique of Enlightenment rationalism and its legacies was being offered by continental philosophers, who identified science as the cultural determinant of modernity's authoritarian power.[64] Again, within this chapter, it is not possible to provide a complete history of the counter-cultural critique of science in the twentieth century. But for our purpose, let me outline some of its most significant moments.

Just as Bernal was writing his book on the *Social Function of Science* in 1939,[65] philosophers and sociologists at the University of Frankfurt were

trying to explore the implications of Weber's analysis of rationalism and the rise of bureaucratic society. Theodore Adorno and Max Horkheimer identified positivist epistemology as cultural autocracy, defining authenticity in a way that allowed advanced capitalist societies and liberal democratic states to resist change by repressing opposition through mass integration.[66] The dialectics of Enlightenment rationalism resulted in an end-game that their colleague Herbert Marcuse called *One Dimensional Man*[67] and facilitated what a younger French philosopher, Michel Foucault, identified as the rise of a disciplinary society based on the operation of power through ideological surveillance.[68] The Hegelian-Marxist critique of the dialectics of the Enlightenment and the poststructuralist analysis of intellectual and social order put science and the material rationalism on which it is based in the dock. By the late twentieth century science was no longer seen as a democratic force but an authoritarian power with the potential to create unthinkable totalitarian futures such as the genoist post-human society recently envisaged by one of America's most public intellectuals, Francis Fukuyama.[69]

Contemporary popular conceptions of the relationship between science and society powerfully reflect elements of the counter-culture's critique. Certainly Jürgen Habermas, one of the Frankfurt School's younger sons, has persuasively argued that the 'rational society' has had a legitimation crisis throughout the latter half of the twentieth century.[70] Habermas has suggested that this is the result of the pretentiously futile claims of scientific knowledge to be 'disinterested' and universal since, he notes, all knowledge reflects some human interest or other.[71] It is at this point perhaps that Birkbeck's understanding of the social relations of science has something to offer postmodern society.

Unlike Babbage, Birkbeck acknowledged that any claims that science had to universalism depended on the degree to which all had access to and participated in it. He recognized the significance of practical knowledge in the creation of theory and promoted educational access as effecting scientific praxis. It remains debateable whether he was correct in believing that open access to and participation in the creation of scientific knowledge was the fastest way to establish a tolerant and democratically civil society. But as far as science is concerned, Birkbeck's model of universal educational access at least offers the possibility of making science a democratic form of knowledge. A people's science might be a step in the direction towards realizing Enlightenment rationalism as a socially emancipatory force, or at least one which might resist the rise of fanaticism.

Part II

Bodies, Commodities and Social Difference

6

French Dentists and English Teeth in the Long Eighteenth Century: A Tale of Two Cities and One Dentist

Colin Jones

In 1811, the English satirist Thomas Rowlandson produced a coloured engraving, 'A French Dentist Shewing a Specimen of his Artificial Teeth and False Palates' (Figure 1). Under the heading 'Mineral Teeth' we read: 'Monsieur de Charmant from Paris engages to affix from one tooth to a whole set without pain. Monsieur Dubois can also affix an artificial palate or a glass eye. He also distils.'[1]

As so often with Rowlandson, generalized social satire has specific targets. Behind 'Monsieur de Charmant', whose dentures make such a comic impact on the English physiognomy, we can detect the presence of Nicolas Dubois de Chémant, a Parisian surgeon practising dentistry in London at the time. Dubois de Chémant was the Prince Charming of French dentists for those in English fashionable society wealthy enough to afford his wares, most notably the porcelain dentures whose invention he proudly claimed and which Rowlandson wickedly mocks. The leading edge of Paris fashion and technology had come to London with a new, French smile, and Rowlandson did not like it one bit.

In this chapter, I want to focus on the dentist Dubois de Chémant to tell a story about teeth and smiles: English teeth and French smiles, in fact. But I shall tell that story in a way that not only illuminates the history of teeth, smiles and dentistry, but which also exemplifies important multi-stranded aspects of the relationship between England and France, and between London and Paris in the long eighteenth century. Although England and France were at conflict for much of the so-called 'Second Hundred Years War' between 1688 and 1815, the Revolutionary and Napoleonic Wars between 1793 and 1815 placed relationships under particularly severe strain. Francophobia, which Linda Colley and others have viewed as a formative factor in emergent English national identity, was especially rampant at this time.[2] The perennial flood of Grand Tourists and English visitors to

Figure 1 Thomas Rowlandson, 'A French Dentist Shewing a Speciment of his Artificial Teeth and False Palates' (1811). Wellcome Library, London

France dwindled to nothing, diminishing the formerly abundant and reciprocal cultural exchange between France and England. In addition, economic historians have suggested that the drying up of technology transfers across the Channel during this period contributed to French economic backwardness after 1815.[3] This makes it all the more intriguing, therefore, that Dubois de Chémant spent much of the period of the Revolutionary and Napoleonic Wars flitting back and forth across the Channel between Paris and London, bolstering the influence of French dental science and Parisian fashionable behaviour over a period allegedly recalcitrant to both technological and cultural transfer. This, then, will be a Tale of Two Cities and One Dentist – and much else besides.

If Dubois de Chémant is credited with the invention of 'mineral' (in fact porcelain) teeth, false teeth *tout court* are as old as the pyramids – literally so, as the archaeological remains of Egyptian pharaohs attest.[4] The Renaissance had seen a marked revival of interest in artificial teeth, but it was the century or so before Dubois appeared on the scene in the 1780s which had seen a real spurt of interest in their technology. George Washington famously had wooden dentures, but a range of alternatives was also on offer, with walrus and hippopotamus bone supplying the top of the range. In addition, it was possible to transplant teeth. In 1685, the first author in England to publish on the noble art of tooth-pulling had suggested that dog, sheep, goat and

baboon teeth might all be employed.[5] Understandably perhaps, preference was given to human teeth.

Human teeth for transplantation came from the mouths of both the quick and the dead. Dentists advertised for teeth, and might have collections of literally thousands.[6] Teeth pulled from battlefield corpses constituted a particularly abundant supply: 'Waterloo teeth' and the sequelae of American Civil War battlefields were notorious in this respect. The drawing of teeth from the living poor to provide for the rich was another means of provision – one which Rowlandson again satirized, highlighting ethical problems attached to the practice[7] (Figure 2). Transplanted teeth also had potential medical and aesthetic drawbacks. They did not always take. Even if they did, they tended to discolour, as indeed did walrus and hippopotamus bone. Informed sources also held that artificial teeth became so malodorous and visually repulsive that they had to be changed annually. In addition, transplanted teeth spread disease – or at least that was the view of Dubois de Chémant. The gradual dissolution of such implants, he held, 'occasioned by the fermentation which results from the heat of the mouth, can inoculate the distemper of the person out of whose mouth they were taken', and he cited the celebrated surgeon John Hunter in support of his assertion that artificial teeth 'contained a venereal or scrofulous virulency'. Transplanted

Figure 2 Thomas Rowlandson, 'Transplanting the Teeth' (1787–90). Courtesy of the Wellcome Library, London

syphilis was a threat to all who dared to place the teeth of others in their mouths.[8]

The idea for mineral teeth (Figure 3) had hit Dubois de Chémant powerfully, he later recounted, when he was still reeling from an evening in the presence of a woman with powerful halitosis caused by her hippopotamus dentures.[9] This epiphanic moment came in the 1780s, when we know that experiments to find a mineral substitute for human, animal and vegetable teeth had already started. A Paris surgeon by training, Dubois began to experiment with a technique which had been trialled since the 1770s by Alexis Duchâteau, an apothecary at Saint-Germain-en-Laye near Paris.[10] One of the biggest technical problems Dubois encountered was that the firing of dentures caused uneven shrinkage which made fit-to-mouth problematic and painful. There were also difficulties relating to colour retention, surface cracking and excessive brittleness. Dubois experimented with hard-paste porcelain which was just being developed in France following the discovery

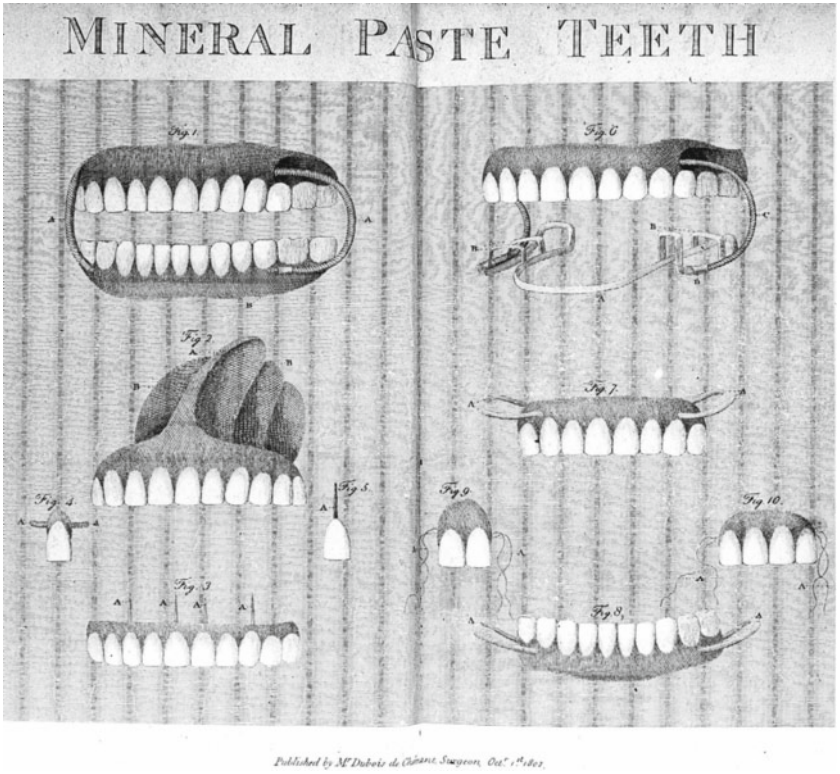


Figure 3 Nicolas Dubois de Chémant, illustration showing dentures, from *Dissertation on Artificial Teeth* (1804). Wellcome Library, London

of kaolin in the Limousin. He decided to go direct to the royal porcelain manufactory at Sèvres to draw on the expertise of workers there. He later explained that Sèvres workmen had proposed to him that he should take wax impressions of the inside of a client's mouth and then make a model in plaster of Paris. They also helped him to create a little furnace within the premises of the manufactory, on which he undertook a series of trials so as to get the best, most lifelike colours, the most resilient paste and the most convincing appearances.

In 1788, Dubois de Chémant was ready to go public with a new product. He approached the Royal Society of Medicine, the Academy of Sciences and the Paris Medical Faculty for approval – which he got. Advertising these endorsements to all and sundry, he set up business in the ultra-fashionable Palais-Royal in central Paris. Owned by the Duke of Orleans, the Palais-Royal had become the acme of Parisian high life and low living and the mecca of Paris's international tourist trade. Potential clients were plentiful. In September 1791, moreover, under the terms of legislation introduced by the Constituent Assembly established in 1789 at the outset of the French Revolution, Dubois obtained government permission for manufacturing and selling, for a period of 15 years, his 'odourless dentures made of incorruptible paste'.

The time was not, however, propitious for the launch of the new product – at least not in France. The institutional context of science, technology and trade were still in flux following the Revolution. In addition, much of the Paris-based noble and monied elite had emigrated, while political and economic problems worsened business conditions. In addition, Dubois was harassed by commercial rivals, notably a lobby of Parisian dentists led by one Dubois-Foucou (no relation). These men claimed that his patent was fraudulent in that he had merely plagiarized a technique which belonged as of right to Duchâteau. The lobby wailed that the generalization of mineral teeth would be 'the overthrow and ruin of their profession'.[11]

Dubois de Chémant saw off these dental Luddites. But the general state of the French economy persuaded him to offset his risks and increase his client base by setting up a sister company in London – whither many potential clients (now *émigrés*) were fleeing. On 11 May 1791, Dubois obtained an English patent for the porcelain paste with which he would create his dentures. As time went on, the London office in Frith Street, Soho became his centre of operations. In a way he had merely pursued his clients. For Soho was among the most populous area of *émigré* concentration: one-fifth of the London-based *émigré* community lived there in a neighbourhood which was already highly Francophone due to the presence of Huguenot exiles since the seventeenth century.[12] Skilled Huguenot artisans helped on the production side of Dubois's business, while the location of French-language bookshops assisted publicity and marketing. The swankiest of *émigrés* preferred the village of Marylebone, it is true. But the upper bourgeois composition

of the Soho grouping constituted a strong client base from which Dubois could extend his ambitions towards the mouths of English elites.

In Paris, meanwhile, Dubois had moved his premises from the Palais-Royal – which was soon to be in eclipse following the disgrace and guillotining of the Duke of Orleans – setting up business on the still fashionable Quai Conti opposite the Île de la Cité on the Left Bank. After France had embarked on war with most of Europe from 1792, however, Paris lurched unsteadily towards the establishment of a republic and the Terror. Times were increasingly fraught. On 22 August 1792 – less than a fortnight after the overthrow of Louis XVI and with Allied troops advancing on the capital – we find Dubois, in a letter addressed to the National Assembly, making an unsolicited gift of one of his horses for the war effort. He cracked shortly afterwards. The Terror was evidently not a good time for teeth, even (maybe especially) incorruptible ones. Someone more important had sewn up the market for Revolutionary incorruptibility. (And we know in fact that Robespierre, *l'Incorruptible*, was a keen man with toothbrush in hand.)[13]

Dubois not only fled his native land (probably some time in 1793). In 1797 he obtained English nationality (like the fathers of Isambard Brunel and August Pugin) one of a tiny minority of the *émigré* community to take this step. This faithful pupil of Sèvres craftsman was by later in the decade turning to the equally famous Wedgwood business to provide him with the finest quality pastes.[14] (The artificial teeth which Rowlandson was to mock so heartlessly in 1811 were thus in essence an English commodity produced by a naturalized Englishman following the technology of one of the greatest of English industrialists. Significantly, only the smile which was the butt of Rowlandson's humour was French.) Also in 1797, Dubois published an information brochure-cum-advertisement.[15] Both the *Dissertation sur les dents artificielles* and the *Dissertation on Artificial Teeth* were published in London (Figure 3), but it seems that Dubois had also put his toe into Parisian waters again under the less frightening regime of the Directory (1795–9). In 1797, he exhibited his wares at the Industrial Exposition which Interior Minister François de Neufchâteau organized on the Champ de Mars as a means of stimulating French arts and crafts. The following year, we find Dubois re-established on the Rue Vivienne, just round the corner from the Palais-Royal. For the rest of his life, he seems to have run a business on both sides of the Channel.[16]

It took considerable ingenuity for Dubois to maintain a dual identity over these difficult years in Anglo-French relations, when individual, cultural and technological exchanges between the two countries were at a low ebb. Although it would perhaps be exaggerating to say that 'the sciences were never at war', science did represent one area in which transfers were possible, especially where medical and humanitarian issues were at stake. The speedy generalization of Jenner's smallpox inoculation techniques across Napoleonic Europe only a matter of years after its invention in 1798 demonstrates

this.[17] Dubois exploited this approach. His advertisements stressed scientific endorsements for his dentures – indeed from 1804 he even displayed a letter of support from Edward Jenner himself.[18] In addition, he played a two-faced game as regards his *émigré* status, milking it for all its worth to achieve tolerance in London (where his naturalization also helped), but also benefiting from the relaxation of anti-*émigré* legislation in Directorial and Napoleonic France to re-establish his business *outre Manche*.

Dubois de Chémant was rewarded for his entrepreneurship by being recognized by his peers on both sides of the Channel for his innovatory porcelain dentures. Even though the Paris-based Italian dentist da Fonzi devised individual porcelain teeth in 1809, which in theory gave greater flexibility and possibility of use than Dubois's denture sets, they were rarely cited in the professional literature. 'Mineral teeth', noted the surgeon-dentist Joseph Murphy in 1811, 'were invented by M. De Chemant who in addition to being the inventor is as yet unrivalled in the art of constructing and adapting them'.[19]

It was not simply mineral teeth that the world owed to French dentistry. Indeed, for nearly a hundred years before the advent of Dubois de Chémant, Parisians had led the world in all aspects of the theory and the practice of dental science.[20] When the Austrian Empress Maria Theresa had wanted the poorly arranged teeth of her daughter, the future queen, Marie-Antoinette, to be straightened, she summoned a French dentist.[21] When the salon conversationalist, the abbé Galiani, was finding that the loss of all his teeth was making it difficult for Neapolitan friends to understand what he was saying, he sent for a Paris-made denture set to restore his articulacy.[22] When George Washington had toothache on campaign, he drew on the expertise of the French expatriate dentist Pierre Le Mayeur, who courageously penetrated British lines to reach the future president.[23]

The French had a European hegemony in medical and surgical care of the mouth at the end of the eighteenth century, a hegemony which formed the context for Dubois de Chémant's bid for fame. The tradition reached back to a century earlier. *Circa* 1700, socially ambitious Parisian surgeons were eschewing tooth-pulling: they said that it made them resemble itinerant and plebeian extractors who attended fairs and markets with a troupe of acrobats and tightrope walkers, mixing play performances and the sale of snake-oil remedies with public tooth-pulling displays.[24] In 1699, the Paris College of Surgery established a category of 'experts for teeth' (sometimes referred to as 'operators for the teeth'), who were certified to practise within the capital, but without undertaking full surgical training. These men largely took tooth-pulling out of the hands of the surgeons. From their ranks emerged Pierre Fauchard, who in 1728 published *Le Chirurgien-dentiste, ou Traité des dents*, a work which historians of dentistry generally regard as foundational in the establishment of modern dental science – and which to boot coined the term *dentiste*.[25] Fauchard set himself against the traditional tooth-puller.

The latter extracted teeth which were causing pain and discomfort, but the dentist concerned himself as much with preventive work. The tooth-puller was a charlatan and showman who tricked the public and cultivated secret tricks. But Fauchard prided himself on communicating his findings with the scientific community through works which circulated in the public sphere. In addition, Fauchard developed a battery of tried-and-tested dental implements as well as supplying details of the best spring-loaded dentures hitherto available (though even these had the unfortunate habit of leaping dramatically from the owner's mouth at unguarded moments, adding to the quotidian dramas of salon life).

Fauchard's volume launched a wave of similar works concerning the care of the health of the teeth and mouth. The fact that French dentistry found a niche within the world of surgery, which itself was brimming with energy and dynamism over the course of the century, helped in the establishment of its European hegemony.[26] Most of the authors of French works on dentistry were either Fauchard's co-workers or rivals who utilized his methods and profited from the new status he had brought to the trade.[27] From the 1750s there were about 30 individuals calling themselves *dentistes* practising in Paris under the aegis of the Paris College of Surgery.[28] Significantly, too, the French royal family placed dentists on their medical retinues. Visiting the dentist was becoming an entrenched social custom, even for kings and queens.[29]

The growth and establishment of the community of dentists in Paris testified to the growing technical expertise of Parisian practitioners – to which, as I have suggested, the rest of Europe deferred. But these developments also highlighted the emergence over these decades of a strong domestic demand for dental services. Study of French newspapers reveals an astonishingly rich collection of advertisements for mouth products of every sort which promoted good teeth and a healthy and preferably beautiful mouth: tooth-files, toothpicks, tongue-scrapers, tooth-powders, tooth-whitening agents, lipsticks, mouth deodorants, and the like. The humble toothbrush also now came into fashion, and all of Europe accepted that the best were Paris-made.[30] Such evidence highlights the entrepreneurial flair of the new *dentistes*, but it also indicates a dynamic demand for what such advertisers bade to provide: a clean, healthy and beautiful mouth.

To some extent we need to view the vibrant level of demand in the context of a likely worsening of the quality of teeth in this period. The massive intake of sugar in the diet of individuals even from humble circumstances throughout western Europe since the late seventeenth century, in the form of bon-bons, sugared delicacies, chocolate and sweetened drinks, including coffee, tea and lemonade, meant that the state of the teeth of Parisians (and Londoners, as well as other Europeans) was probably worse than it had ever been. A functionalist approach, however, takes us only part of the way towards understanding what was more a cultural than a biological

phenomenon. The work of Daniel Roche and others has familiarized us with the idea of a proto-consumer revolution in eighteenth-century Paris, as individuals even well down the social scale dressed and cared for themselves in ways more receptive to fashion and exchange.[31] We have also become comfortable with the cognate notion of the development of a new body – more *soigné* and cared for, more self-aware, more individualistic in appearance and more attuned to emergent codes of politeness and the dictates of fashion.[32] The prizing of a healthy and preferably beautiful mouth seems to have been an offshoot of these overarching developments – certainly in France. The Jesuit writer Joseph Cérutti claimed that laughter was 'the distinguishing characteristic' of the French. He was not alone in making this a national trait particularly observable in the capital. Paris was, in the eyes of the Italian writer Caraccioli, 'the home of laughter' and buzzed with 'a moderated gaiety which consists not of great bursts of laughter but of a smiling countenance'.[33] The celebrated theorist of physiognomy, Lavater, who specialized in mouths (Figure 4), was in general agreement. The French were always smiling: 'I know [the French] chiefly by their teeth and their laugh.'[34] And that French smile showed off white and healthy teeth to best advantage, such as the smile which – in contraventions of all artistic conventions of expression dating back to Antiquity, and to the accompaniment of no little scandal – Madame Vigée-Lebrun displayed in certain of her portraits.[35]

If in France it appears that white teeth were coming to be an integral part of the smiling countenance, this may have been less obviously the case in England. In his *Letters to his Son*, that *vade-mecum* of tight-lipped eighteenth-century *politesse*, the Earl of Chesterfield specifically warns against noisy, open-mouthed laughter which he associated with the French. 'A man of parts and fashion', he held, 'is only seen to smile and never to laugh.'[36] It was precisely that open-mouthed 'French' smile, of course, which Rowlandson would mock in his 1811 engraving. For the English satirist, the toothy French smile looked ridiculous to any free-born Englishman – but it was ridiculous to the power of three if it also highlighted artificial teeth, and in addition English mouths which should have known better.

In matters dental, moreover, the English were way behind the French.[37] Indeed, they were so far behind that English teeth owed a considerable debt of gratitude to immigrant French dentists and dental craftsmen. French Huguenots, for example, who had taken refuge in England at the end of the seventeenth century, were among the first tooth-drawers to call themselves dentists and they were also in the vanguard of the artificial tooth trade. Pierre (Peter) Hemet, for example, arrived in London in 1687, directly after the Revocation of the Edict of Nantes, and by 1701 had become the Queen's Operator for Teeth in Ordinary. His son would hold the same post later in the century and both would work on artificial teeth production.[38]

French and French-trained immigrants – not solely Huguenots – were among the most prominent and most frequented dental practitioners in

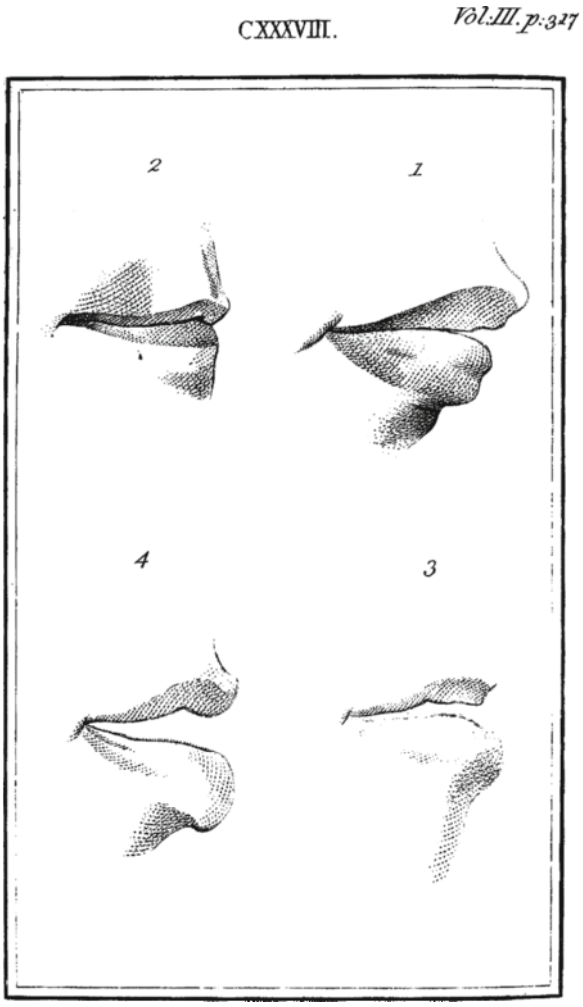


Figure 4 Johann Caspar Lavater, 'Four Depictions of Mouths', from *Essays on Physiognomy* (1804). Courtesy of the Wellcome Library, London

Georgian England. Many were craftsmen who had specialized in trades such as apothecary, jeweller, goldsmith, watchmaker and herniary truss-maker which valorized manual dexterity, artisanal skill and aesthetic sensibility – all seemingly of value in the handling of recalcitrant teeth and the making of false tooth apparatuses. Alongside the Hemets, the Ruspini dynasty was prominent for generations. The dynastic patriarch 'Chevalier' Bartholomeo Ruspini was born in Italy but claimed to be the pupil of the famed Caperon,

Dentiste du Roi in Paris and Versailles. First in Georgian Bath, then from his elegant premises on Pall-Mall, he built up a clientele of *le Tout-Londres* which included personal service to the Prince of Wales. In 1767, he published a *Treatise on the Teeth* which went into at least 15 editions down to 1810. It was precisely he (another foreigner) who was caricatured in Rowlandson's 'Transplanting of the Teeth'. His son trained with the Parisian dentist Catalan and, like his father, became the Prince of Wales' personal dentist. The Talma dynasty was another case. Michel François Talma (the father incidentally of the great French actor François-Joseph Talma) established himself as a dentist in the 1770s, boasting Parisian training. His son Jean-Joseph practised in Paris for six years and became a member of the Academy of Surgery before settling in England in 1792 – another denizen like Dubois de Chémant of the dental French Revolutionary emigration. He was a prominent practitioner in the English provinces well into the nineteenth century.[39]

Technology transfer by diaspora (whether Huguenot or *émigré*) was still insufficient to overcome England's laggardliness in regard to France in this domain. Whereas in France Fauchard's seminal work of 1728 had spawned a whole literature of science-driven mouth care, English publications were feeble. Significantly, Fauchard's foundational work would only be translated into English in 1946! One would like to think that English practitioners were reading it in French, but frankly that may be optimistic. Characteristically, there was even much resistance in England to the adoption of the term 'dentist', whose French analogue Fauchard had coined. The *Edinburgh Chronicle* in 1775 assailed this term as one which 'figures . . . now in our newspapers and may do well enough for a French puffer'. English practitioners, it seemed, still preferred to be thought of tooth-drawers. Presumably under client pressure though, the term 'dentist' eventually became current.[40]

Works on dentistry with even the lightest trappings of science appeared later in England than in France and were far fewer in number. The Parisian dentist Étienne Bourdet noted that as early as 1756 people were saying that there were 'a larger number of writers [on dentistry in France] than there is any other branch of physic'.[41] Yet introducing a (rather feeble) pamphlet on dentistry in 1779, the surgeon William Bennett noted that for England at least 'it should seem strange that among all the volumes written upon Physic and the cure of diseases, so little has been said on the present subject'.[42] If we put aside the work of the Paris-trained Ruspini, then the first major publications on dentistry in English were the *Treatise on the Disorders and Deformities of the Teeth and Gums* by Robert Berdmore in 1768 and the *Practical Observations on the Human Teeth* by his pupil Robert Wooffendale in 1783. These men were academic lightweights compared with Fauchard, whose work was still the most authoritative in Europe when Rowlandson made his 'French Dentist' engraving. True, John Hunter's *The Natural History of the Human Teeth*, published in 1771, was important in its way. Indeed, Robert Blake, in a survey of the field in 1801, regarded it as

'the only treatise of consequence which has appeared in our language'. But even Wooffendale admitted that Hunter's writings were the product of 'a very accurate anatomist' rather than of 'a practical dentist'.[43]

If we read the works of Fauchard and his disciples and followers in France, we note learned citations and footnote practices showing the makings of a solidly based academic tradition with appropriate scholarly protocols. Yet this was far less evident in English writings. There is something decidedly odd about the rudimentary scholarly level of Berdmore and Wooffendale, for example, and the insular sense of these early luminaries of autochthonous English dentistry that they were entering a scientific *terra incognita* in which they would find no help from Ancients, Moderns or (especially) foreigners. In 1768, Thomas Berdmore, claimed that he had kept citations in his work to a minimum so as not to distract his lay audience. He continued:

> without departing from the subject, I could only have quoted a few French authors who have written *to make their name known* and one or two English who have translated very injudiciously.[44]

A similarly snooty lack of grace was exhibited by Joseph Fox, who in 1802 published *The Natural History and Diseases of the Human Teeth*, which would remain a key text in English dentistry for more than half a century. But as the French dentist Joseph Audibran observed in 1821: 'Joseph Fox does not deign to make even the slightest mention of the writings and the work of those who have practised and continue to practice the same art as he.'[45] Fox was held moreover simply to have plagiarized Hunter, and indeed mindless borrowing was a sin that beset a great deal of this early dental literature.[46] It was not simply that the English were ignorant. On the contrary, they gloried in it: it was seemingly better to be ignorant and English than to draw on French science.

Rather than see them as bearers of a superior scientific culture than their own, English dentists reserved little but scorn for foreign practitioners practising in England. In their eyes, such figures were little more than quacks. Berdmore, for example, launched with no little gusto into the 'mountebanks, *valets de chambre* and nostrum-mongers' who were treading on his territory and trying to get their fingers into English mouths. He attacked these

> Gentlemen who quit the Continent for our sakes and walk in a more exalted sphere picquing themselves on the dignity of having served courts and marquises in the station of valets de chambre and of having seen the art of [tooth]-filing practised in twenty provinces.[47]

Such attacks conjoined the xenophobia which was such a significant part of elite as well as demotic English culture in this period with the anti-quack discourse which, as Roy Porter has shown, was a staple of English medical writings.[48] This conflation was not rare. One distrusted the charlatan because he

was not like the regular practitioner: settled, learned, philanthropic, sincere, uncommercial – and English.

French practitioners in England were, moreover, vulnerable to charges of quackery and charlatanism. They had had to create their own market, and for commercial success needed to be itinerant and to clothe their dental skills with a certain amount of razzmatazz, plus the sale of remedies and other dental products (toothbrushes, anodyne necklaces, toothbrush cases, toothpicks, and so on). These traits – mobility and blatant commercialism – were regarded as hallmarks of the quack, making foreign dental practitioners easy targets. One critic proclaimed roundly against 'hocus-pocus operators' who had 'probably . . . appended Monsieur ou Signor to their common name'.[49] Foreign rivals were thus worse than quacks: they were uppity foreign quacks. A dentist like Dubois de Chémant also suffered guilt by association from his *émigré* clients. Generally, the English were remarkably hospitable to the French political exiles. But it did not take much provocation to slip into angry, Francophobic contempt for foppish *petits-maîtres* and their ilk, who were crudely portrayed as snivelling frog-eating monkeys with their French smiles.[50] The French influence on women was even more reprehensible. Rowlandson surpassed himself for misogyny in a cartoon in which we see an ululating old harpy converted – thanks to all kinds of artificial paraphernalia, including a full set of dentures – into the sweetest, smiliest French miss[51] (Figure 5).

Figure 5 Thomas Rowlandson, 'Six Stages of Mending a Face' (1792), the Wellcome Library, London

We may justly wonder, however, whether the attacks of English dentists on French practitioners in England did not comprise a back-handed compliment to the reputation that the latter were acquiring. As the successes of the Ruspinis and the Talmas attested, Frenchness had a cachet among the growing public for dental services which the Berdmores of the world were seeking to master. 'The reputation of having studied abroad', one medical journal noticed in 1844, was 'a passport to immediate practice'.[52] Frenchness also became a marketing feature of dental products: the Hemets sold 'Hemet's Dentifrice', and the Ruspinis their own *Dentifrice* too. The Anglo-Saxon term 'tooth-powder' would establish itself only slowly, though this too could benefit from French branding, as with 'Madame du Barry's Tooth-powder', on sale in the English shires.[53] Scratch at least some French dentists moreover and one discovered something altogether more homely: in the 1830s, one 'Monsieur Le Dray', whose publicity announced that he had served numerous members of Europe's reigning houses, was in fact 'Joseph Jordan, the quack doctor of Houndsditch'.[54] Imitation was the best form of flattery that French dentistry achieved from their English *confrères*.

If English dentists hated French dentists, many English teeth loved them. The message behind Rowlandson's 1811 print was that the success of French dentists in England was due to the unhealthy xenophilic and fashion-crazy willingness of the English public (and especially females, it seemed) to be duped by quacks. The fact was that French dentists were often preferred because they were better than their English analogues. Whether English dentists liked it or not, moreover, the French smile was catching on among English clients. For English visitors to Paris, a visit to a Parisian dentist had become virtually a part of the Grand Tour by the eve of the Revolution: the diarist Mrs Cradock in 1786 told how she had been taken to see the famous dentist, Étienne Bourdet:

> and we waited for more than half an hour before he daigned to appear, his valet having come to advise us that his master never entered the operating room before he had had his morning coffee. The moment arrived . . . this grand person (but excellent dentist) [attended to me].[55]

Yet it was also becoming less necessary for English teeth to have to migrate to the French capital. Help was closer at hand, as the French dental diaspora demonstrated. Englishwomen in particular were becoming emulative with their French sisters. A Swiss visitor to England in the late seventeenth century had considered one of English women's main deficiencies that of 'not taking care of their teeth'. In 1783, an English visitor to France, John Andrews, noted that 'there are two objects of which the French ladies are peculiarly solicitous to make a display, their eyes and their teeth; in the brilliancy of the first and the whiteness of the latter they think no woman can surpass or equal them'. Yet he gallantly noted that 'of late years . . . the English women

are become more careful of their teeth then they were used to be'.[56] The French smile was making progress on the lips of English women.

Much of the dental literature which appeared in the first half of the nineteenth century stressing that the healthy mouth was also beautiful played nicely to underlying *mentalités* which figured Frenchness as irrevocably bound up with love, fashion and women. Though the gentlemen were not forgotten, it was the ladies who basked most in the new concern for a healthy and beautiful mouth. 'Many ladies owe the acquisition of a husband to the attraction of a beautiful set of teeth,' noted Edward Breham in 1816.[57] The ladies were a target audience for most dental advertising – as lovers, spouses or mothers. Hugh Moise's *An Appendage to the Toilet* was dedicated to 'British ladies' who 'have justly held the palm of beauty in preference to those of other nations', while Jacques-René Duval's *The Dentiste de la Jeunesse* was specifically designed for 'the more intelligent orders of parents and guardians'; and John Nicholles's *The Teeth in Relation to Beauty, Voice and Health* apostrophized 'the mothers of the rising generation'.[58]

There was less consumer resistance to the French smile than English dentists would have liked – as Dubois de Chémant's fortunes showed. As early as 1797, he claimed to have sold over 3,000 full dentures in the previous decade, and by 1816, the number had risen to 12,000.[59] By the 1840s, half a million dentures were being exported from Paris, presumably by the heirs of Dubois and his rivals; many of the contraptions were doubtless heading for English mouths.[60]

Yet Dubois de Chémant's career failed to live up to its promise. He ran into business trouble when former servants stole his techniques and set up in competition in London, while in France he had to endure a further bout of litigation over intellectual property rights.[61] Da Fonzi's invention of individual porcelain teeth threatened the solvency of Dubois de Chémant, while the latter also suffered rivalry from an unexpected quarter, his old enemy Dubois-Foucou, who had had a latter-day Damascene conversion to the cause of the porcelain denture. English dentists meanwhile conducted a vicious campaign against him. In 1802, Joseph Fox thought Dubois de Chémant's dentures rather unnatural, and mocked their manufacturer's pseudo-science:

> Considerable latitude must be given when we place the partial statements of an author possessing much vivacity and disposed to regard his own invention as meriting a decided preference.[62]

John Gray, the author of a book entitled *Preservation of the Teeth*, nodded agreement:

> The things called mineral, or Jews' teeth [*sic*], are now plentifully manufactured of porcelain; but they always look what they are and can never be mistaken for teeth . . . and by acting as a whetstone on any of the natural teeth soon wear them away.[63]

Given this blanket of hostility – with Dubois de Chémant's porcelain teeth viewed as a reprehensible invention which was exotic, foreign, 'Jewish' and (worst of all) French – it is perhaps not surprising that Dubois de Chémant's porcelain dentures did not firmly and durably establish themselves in English mouths in the first decades of the nineteenth century. The continuing and vigorous trade in non-porcelain teeth showed the absence of a marketing breakthrough – porcelain teeth would only come fully onto the dental agenda much later in the nineteenth century.

We should not underestimate the importance of design faults with Dubois de Chémant's porcelain teeth which his rivals highlighted – and which his clients must to some degree have suffered: the problems of fit, with all the pain and discomfort which that caused; the erosion effect on opposing teeth; problems of the appearance of the teeth; the decidedly off-putting noise of grinding porcelain while chewing. The weight of the full set also was a factor which caused discomfort and possible embarrassment in some mouths. Further research and development would have been necessary to overcome these problems, but precious little was forthcoming, certainly in England, where the institutional environment for such work was unsupportive.

Sadly, in France too by the early nineteenth century, scientific dentistry was living off its capital. When it reorganized the medical professions in 1802, the French government failed to institutionalise the teaching and practice of dentistry. This meant that the specialism would not attract good candidates in the future. Skilled dentists had to compete in an almost wholly unregulated field and on grounds of legal equality with the untrained and charlatans. Napoleon I had his teeth looked after by an offshoot of eighteenth-century dentistry (Dubois-Foucou, in fact); but by mid-century Napoleon III would turn to North American dental expertise for his own and his family's mouth care. The French smile was fading.

Dental history has tended to be written in the past as a classic Whig history of scientific progress, with the lineage of Great Dentists and Great Technical Breakthroughs offering an unbroken upwards ascent. Yet the early nineteenth century looks like a definite period of regression, as the leading sector of world dentistry (France) hit the rocks, and as no competitors emerged with an adequate scientific grounding and a supportive cultural environment. Perhaps this was the beginnings of a cultural transformation which would gradually move the scientific edge of dentistry out of Europe – and make of the late twentieth and early twenty-first centuries the Age of the Great American Smile, proclaimed from every cinema billboard and political advertising hoarding. But there did not seem much sign of that shift by, say, 1840.

Did this have to be the situation? Did England have to continue to lose out in the dental stakes? The collapse of indigenous French dentistry in the early nineteenth century offered a wonderful opportunity for the English

to develop their dental science in ways which would allow them to assume former French hegemony in the field. The long-term presence in England of some very decent French practitioners was an asset in this respect. But it was an asset which was largely unexploited. The English had been hostile to (or worse, ignorant of) French dentistry when the latter was at its peak, and missed a trick when it was in decline. Just how and why English dentistry proved unable to capitalize on the collapse of French dental science – and how the US eventually came to take over this vacant slot – is a question worth pondering. And in pondering we will do worse than to reflect on the message of the Rowlandson cartoon on which I focused at the beginning of this chapter. When it comes to cross-Channel intellectual and cultural exchange, we would be wrong to rule out of court the role played by corporative selfishness, insularity and sheer bloody-minded, xenophobic prejudice.

We might also finally wish to reflect on what all this signified in terms of 'the patient's view' – one of Roy Porter's favourite vantage-points of course on the history of medicine.[64] Good old-fashioned English prejudice against French dentists had its price, I am suggesting – and that price was paid in English mouths, by English teeth. Rowlandson makes a joke of French dentistry by highlighting an English mouth made ridiculous by the newfangled, Frenchified porcelain teeth and caught in the unwittingly hilarious rictus of a French smile. But if we cast our eyes over the English gentleman observer of the new dentistry at the right of the engraving, we see that his mouth, frankly, and indeed to put it mildly, also leaves a great deal to be desired. By 1811, when Rowlandson engraved this cartoon, the English may well have been getting the edge over their continental neighbour in the second Hundred Years War between the two countries. But Rowlandson also makes us realize that if the English were winning the war, they were still at risk of losing the teeth.

7

Hotbeds and Cool Fruits: The Unnatural Cultivation of the Eighteenth-century Cucumber

Anne Secord

'Why the most natural actions of a man's life should be call'd his Non-Naturals' was a paradox that delighted the eighteenth-century writer Laurence Sterne, and continues to amuse us today.[1] The 'non-naturals' were precisely those things essential to health – air, food and drink, sleep, exercise, excretions and the passions – that were not considered natural in themselves because they were liable, through abuse or accident, to become the cause of disease. They affected but were not part of a person's constitution. Physicians therefore encouraged individuals to regulate their daily lives through knowledge of these effects on their own bodies. The non-naturals had to be used variously and judiciously to stimulate the nurturing and healing qualities of nature. It was the correct functioning of the body that was considered 'natural'.[2]

This notion of the natural, increasingly evoked to condemn the prevalence of an artificial and 'unnatural' life of luxury, was, as contemporary commentators declared, difficult to pin down. Not least, the harnessing of nature's powers could be claimed to underlie the production of luxury goods themselves.[3] The luxury object whose skilled workmanship adds to its appeal is usually represented by the trinket, gadget or novelty, rather than by a product of nature. But the artifice that turned natural objects into luxury goods illuminates even more clearly the ambiguous meaning of 'natural' and 'unnatural' in the eighteenth century, by revealing the intersection of debates concerning the health of the human body and that of the body politic. Such luxuries included not only exotics such as grapes, pineapples and melons, but also the production of forced (that is, before the natural season) fruits and vegetables, or what gardeners termed 'novelties of the spring'.[4]

This chapter explores this theme by considering one such 'novelty' that required great skill to raise. While the cultivation of pineapples or grapes in England excited attention and admiration, the real test of a gardener's skill was seen to lie in the forced production of cucumbers. Investigating why this was the case, and how excellence was judged, reveals the importance of display not just among the upper classes, but also among gardeners themselves. The aspiration of estate owners to improve their prospects in all senses of the word depended on their ability to employ skilful gardeners. How they acquired the best gardeners is intertwined with the story of the cucumber. The conspicuous consumption and power of polite society – over the natural as well as the social world – was quite literally expressed in the fruits of these servants' labours.

Unlike the still visible remnants of English pleasure gardens artfully refashioned in the eighteenth century into 'natural' landscapes, the products of kitchen gardens were ephemeral. Thinking of fruits and vegetables as material objects, however, enables us to explore not only their political significance as representations of luxury, but also to examine the knowledge and technical skills required for their production, and the risks involved in their consumption. Cucumbers are particularly significant because they not only shared characteristics with exotics, but also differed from them in crucial ways. The cultivation of grapes and melons in England depended on artificial sources of heat at all times, and pineapples, the acme of 'polite' gardening, required to be downright 'indulged' in their pine stoves.[5] Cucumbers, in contrast, did not require artificial means to produce a glut in the summer when they were as available to the poor as to the rich. The fruit was made special by being produced and consumed out of season: cucumbers were scarce, expensive and desirable only when forced on hotbeds well before summer. The cucumber therefore became an ambiguous object – a vulgar foodstuff that could be transformed into a luxury item and, as we will see, a choice dish which was a potential poison.

Cucumber time

By 1785, according to the *Classical Dictionary of the Vulgar Tongue*, 'Cucumbers' denoted 'taylors', an association derived from tailors being reduced to living on cucumbers in the summer when the departure of the gentry from London left them without work.[6] The satirical writer Ned Ward had referred to tailors who 'for want of Trade, Make Cucumbers their daily Bread' in 1721, while in an earlier poem he implied that cucumbers were undesirable food, fit for those to be tortured.[7] Cucumbers, plentiful and cheap in the summer, stimulated a trade among the poor who hawked them to those in the seasonal trades. The actor Ned Shuter, born in the notorious rookery

of St Giles in 1728, stated that his mother 'sold Oysters in the Winter, and Cucumbers in the Summer'.[8] By 1775, the cucumber seller was sufficiently familiar to find a place in the *Cries of London* and to remain a stock character into the next century (Figure 6). The stark meaning of the cucumber for its impoverished summer consumer was made clear by the engraver and print-seller George Bickham. His print of a tailor composed of the tools of his trade follows an artisanal tradition of sexualizing the implements of one's craft. The cucumber, however, remains innocent of this charge, separate from the tailor's identity as a craftsman, and emblematic of nothing so much as want (Figure 7).

GREEN AND LARGE CUCUMBERS.

Figure 6 'The cucumber seller', *Cries of London* (1804), Houghton, Library Harvard University

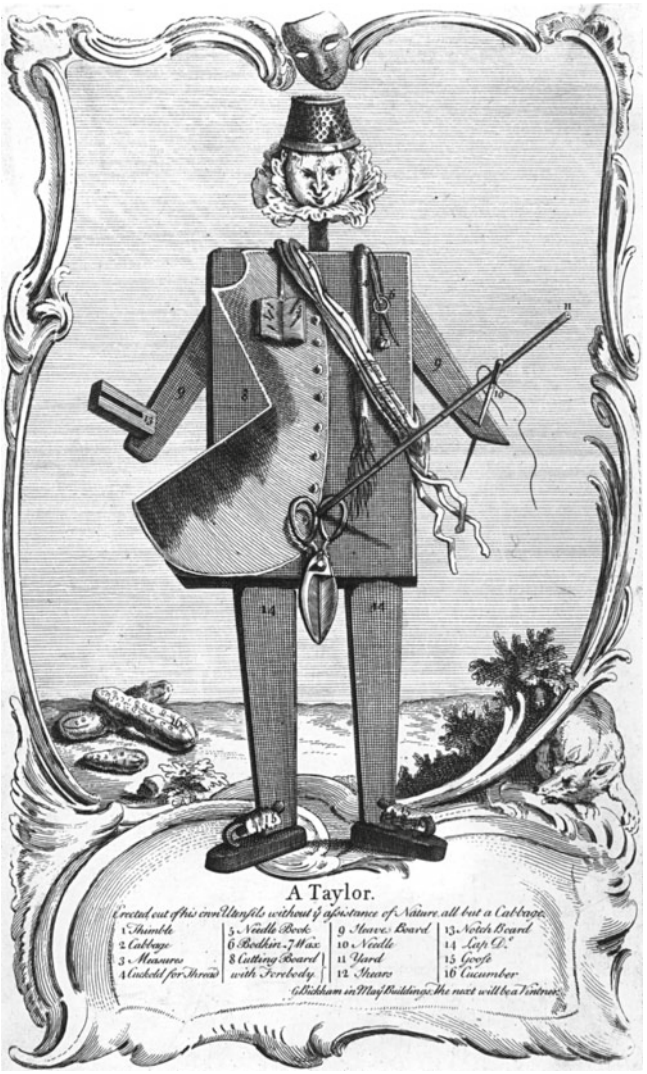


Figure 7 George Bickham, 'A Taylor' (*c.* 1750). © the Trustees of the British Museum. Among the accoutrements listed in the key, no. 15 'Goose', was slang for the tailor's Iron; no. 2 'Cabbage', referred to the leftover pieces of cloth that the tailor purloined to patch together for sale during the slow summer season in the tailoring trade when all he could afford to live on were cucumbers, no. 16.

The impact of being reduced to living on cucumbers was heightened by their reputation as a dubious foodstuff, as a warning in the *Cries of London* made clear:

Green cucumbers, however nice,
By all who prudent are and wise,
And health prefer to choicest dainty,
Will ne'er be eaten in great plenty;
Their properties, so deadly cold,
Agree not with the human mould,
But oft to dangerous sickness move,
And sometimes actual poison prove:
Yet some will readily prefer
To wholesome food a cucumber,
Studious alone their taste to please,
And careless of their health and ease.[9]

In the late seventeenth century, so deleterious were cucumbers thought to be that death itself was considered cowardly when claiming a summer victim whose blood had been chilled by '*Cucumbers*, and *Cabbedge Roots*'.[10]

Despite changes in medical theories, little altered in eighteenth-century practical advice concerning diet. Health, regarded as a natural balance or harmony of the humours, was endangered by sudden transitions of temperature or by food with qualities inappropriate to those of the consumer. Thus imbibing cold fluids when hot could result in 'immediate death'. 'Neither is it safe when warm to eat freely of raw fruits, sallads, or the like', warned the physician William Buchan, whose work was aimed at the labourer as well as the rich. Unusual in drawing attention to occupations that were injurious to health, Buchan pointed out the dangers of sedentary trades. Tailors, who were particularly susceptible to 'a total vitium of the humours' through sitting cross-legged, constantly bending and neglecting to take exercise, should 'avoid food that is windy, or hard of digestion'.[11] With their constitutions undermined by their occupation, tailors were 'cooler' and possessed thinner blood than healthier persons. Cucumbers, with their cold and moist qualities, were the last thing they should be eating in any quantity.[12]

Such views may not have directly contributed to suspicion of the 'deadly cold' qualities of the cucumber among the poor, but they certainly informed gardeners' perceptions. Kitchen garden manuals, reflecting an increased emphasis on the pleasures of the table, often included culinary advice. The early cucumber, increasingly fashionable though it was, appeared to compound the fruit's undesirable qualities. The seedsman Stephen Switzer maintained that the 'great encouragement given by the nobility and gentry . . . towards the accelerating of garden vegetables' had stimulated the forcing skills of kitchen gardeners, but he declared that as cucumbers were cold to the fourth degree and 'the next degree whereof would be poisonous', they 'ought never to be eaten before *May*, not after the latter end of *July*, or some small time in *August*, except it be very sparingly'.[13] Nor could the garden writer Batty Langley recommend eating cucumbers as early as February or March, 'they being much too cold for the Weather of those

Months'. 'All Sorts of Cucumbers are temperately cold and moist in the second Degree', he pointed out, 'and therefore not good'.[14]

Physicians warned that the specific dangers of the cucumber could be difficult to assess. Although all plants of the 'low pomiferous Kind, as *Melons, Pompions, Gourds, Cucumbers*, contain a cooling Juice', John Arbuthnot held that unlike melons and pineapples, which are 'rich and cordial', the 'Juice of Cucumbers is too cold for some Stomachs'.[15] The physician and poet John Armstrong, however, pointed to something altogether more mysterious and sinister:

> . . . there often lurks some nameless, strange,
> Peculiar thing; nor on the skin display'd,
> Felt in the pulse, nor in the habit seen;
> Which finds a poison in the food that most
> The temp'rature affects. There are, whose blood
> Impetuous rages thro' the turgid veins,
> Who better bear the fiery fruits of Ind,
> Than the moist Melon, or pale Cucumber.[16]

In some cases the cooling qualities of melons and cucumbers were advantageous in overcoming too much heat in the body. Italians and Spaniards were thought to consume melons to overcome lust; a similar effect from cucumbers was no doubt implied in *Tristram Shandy* when innocent and impetuous uncle Toby receives instructions in the art of love-making.[17]

If cucumbers were to be eaten out of season, measures needed to be taken to counteract their unwholesome cooling qualities. Large ones with big seeds were to be avoided, 'excepting by such Persons whose Stomachs are very hot'.[18] Coldness had to be counteracted by 'hot' ingredients such as lemon juice, pepper, vinegar and onions, though more often, it seems, by serving cucumbers as a hot dish.[19] Moderate quantities were always advised. As the pleasures of the table increased with a new cuisine using novel and forced produce, the physician George Cheyne condemned 'the luxurious artfulness of cookery', while Buchan thought it 'were well for mankind if cookery . . . were intirely prohibited.' Mixing ingredients together produces 'almost a poison'; plain roasting or boiling is all that 'nature points out'.[20] This providential view held that to eat anything other than local seasonal food was to upset the natural economy of both the individual body and that of the nation. Cheyne, whose aim was to reform the eating habits of his rich clientele away from fattened livestock and other artificially produced foods, made the point reflexively:

> Perpetual foulness and cramming, gross food and nastiness, we know, will putrify the juices and mortify the muscular substance of human creatures; and sure they can do no less in brute animals, and thus make even our

> food poison. The same may be said of hot beds, and forcing plants and vegetables.[21]

Such protests are perhaps the best indication of how widespread this form of food production was, and how prevalent the idea that eating and drinking fuelled the body, just as the luxury trade stimulated the national economy.[22] As explicit critiques of the very thing sought by many in polite society, namely luxury, these medical texts may have explained why forced cucumbers were so unwholesome, but did little to dampen the fashion for out-of-season produce among the well-to-do. Throughout the eighteenth century and into the nineteenth the forced cucumber remained an object of desire. Distinctions in both quantity and quality needed to be maintained between the food of the rich and that of the poor. This could be managed only by consuming cucumbers out of season when the difficulty of production meant that they were always expensive. It was only in summer, with the slackening of trade, that tailors drifted into what came to be known as 'cucumber time'. The aristocracy and gentry, having first experienced the forced fruit produced in London's market gardens, began to expect their own gardeners to raise early cucumbers. For the gardener, timing the appearance of cucumbers became an extreme test of skill in the practice of his art.

Dung, toil and attention

By the 1720s, a gardener unable to produce early cucumbers 'however so ingenious otherwise, must be stigmatiz'd for one unknowing in his art or business'.[23] This implied a widespread understanding of hotbed production, involving copious amounts of fresh dung with its first 'nauseous heat'.[24] The quality of air – one of the non-naturals – was tainted by emanations from manure piles, and this, together with the large number of labourers required for the management of hotbeds, may explain the eighteenth-century practice of moving the kitchen garden away from the house.[25] Gardeners had, of course, long used dung as manure, and London market gardeners' reliance on the contents of the capital's privies and on stable dung is revealed by a bargeman's taunt in 1700:

> Foh, you nasty dogs that get your bread by the drippings of other people's fundaments; well you pray for the dunghill, for if that should fail you, no turd, no gardener. Who was that, you rogues, that dung'd his own cap at stocks-market, and carried home the old gold to enrich his radish-bed?[26]

Although still associated with dung, gardeners seized an opportunity to improve their image and status with the increasing desire for forced produce. An enormous amount of manual labour was required for preparing and maintaining hotbeds and for watering the garden, and the skills of the

kitchen gardener depended on having such resources at his disposal.[27] As the demand for forced produce increased and the gardener's year became 'one continued circle of labour and toil', employers were informed in no uncertain terms that a good gardener, able to supply the kitchen in all seasons, should control the money allocated to the garden, be as well paid as 'the best Servant' and not have to deal with trouble caused by employers leaving garden labourers unpaid.[28]

With such resources, a gardener could undertake the production of hotbeds. Like physicians, gardeners were often critical of luxury, warning against the 'immoderate' use of dung as manure because the practice imitated the 'intemperate and luxurious Way of Life' that produced ills in humans and 'immature Death'.[29] This censure, however, did not extend to the copious use of fresh 'steaming' dung as a source of heat, by which means the skilful gardener could imitate the climate of a foreign plant or encourage unseasonal growth. At all stages of early cucumber production huge quantities of dung had to be managed correctly to control the 'Fury of Heat' and produce beds with a moderate and lasting warmth. The frames and glass lights placed on these beds had to be manipulated to protect the plants from the effects of too much heat from the 'Steam of Dung rising' or from falling drops of 'cold Steam' (condensation), both of which meant 'present Death'.[30] The condensation inside the lights needed to be wiped away every morning, and 'your finger should be often thrust into the bed' to judge whether the temperature needed adjusting by the addition or removal of dung.[31] Precise judgement was also required in knowing when best to open the lights to allow in air, and the extent to which to cover them with mats at night or as protection from extreme cold (Figure 8). Cucumbers, being mainly composed of water, required clean water from rivers or cisterns, not from horse-ponds, but overwatering would increase their cold qualities. Any misjudgement had potentially lethal consequences.

The earlier the cucumbers and the less exposed to the air the more essential the task of impregnating the female flower. Gardeners were instructed to insert the anthera of the male flower 'into the center of the female flower, touching the stigmata thereof, and twirl it about'. If done as soon as the blossoms appear, this 'completes the business' for the production of fruit.[32] Richard Bradley, practical horticulturist and professor of botany at Cambridge, was the first to suggest, in the 1720s, that the wind carried 'male Dust' to the female blossoms. Langley, who claimed not to be 'Philosopher' enough to judge Bradley's view – let alone accept a sexual theory of plants – still advocated watching closely for the cucumber blossoms to arrive. He recommended immediate removal of all the 'false' (male) ones, which do not result in fruit, and 'never could observe, that for want of their familiar Conversation with the female Blossoms at the ends of the Fruit, did ever lose one single Cucumber thereby'.[33]

Figure 8 Thomas Mawe [John Abercrombie], 'Frontispiece', *Every Man His Own Gardener,* second edition (1767). By permission of the Syndics of Cambridge University Library.

On the left is a hotbed, whose heat was preserved by layering 'straw waste hay, or fern' round the sides of the bed. The month (judging by the tulips) is probably May when it was safe to lift the glass lights and allow the air to reach the mature plants.

By 1789, despite such disagreements, the practical advice about controlling the 'rancid dungy steam' of hotbeds and the continual attention and judgement required for raising early cucumbers had not changed.[34] The value of experience and minute observation was constantly reiterated. Even as

thermometers became more widespread, gardeners remained impatient of any perceived theoretical encroachment or abstruse language that would give a false certainty to practices and mask the slow and essential attainment of tacit knowledge: why use 'caloric'? demanded nurseryman John Rogers. 'Why not say a warmth?'[35]

Mastering the art of raising early cucumbers and the acute attention this required generated its own pleasure, in which moral and economic values combined in a utilitarian aesthetic. Attention, moreover, 'infused its objects with affect'.[36] While apparent in gardening manuals, this is perhaps most clear in a quite different text: William Cowper's widely read and much loved poem *The Task* of 1785 may have stimulated the interest in growing early cucumbers. The poet readily admitted that:

> To raise the prickly and green-coated gourd,
> So grateful to the palate, and when rare
> So coveted, else base and disesteem'd –
> Food for the vulgar merely – is an art
> That toiling ages have but just matur'd,
> And at this moment unassay'd in song.

Cowper, however, justified his eulogy to a plant that also provided food for the poor, by claiming that his poetical powers

> Pant for the praise of dressing to the taste
> Of critic appetite, no sordid fare,
> A cucumber, while costly yet and scarce.

He then devoted 82 lines to describing the culture of the cucumber in exact detail before ending with the admonition:

> Grudge not, ye rich, (since luxury must have
> His dainties, and the world's more num'rous half
> Lives by contriving delicates for you)
> Grudge not the cost. Ye little know the cares,
> The vigilance, the labour, and the skill,
> That day and night are exercis'd, and hang
> Upon the ticklish balance of suspense,
> That ye may garnish your profuse regales
> With summer fruits brought forth by wintry suns.[37]

Cowper knew whereof he spoke. In 1777 he had managed to raise cucumbers by the end of March.[38]

Another keen gardener and most attentive observer of nature, the Reverend Gilbert White of Selborne, could attest not only to the difficulties and failures of cultivating early cucumbers, but also to the pleasure of selling his earliest fruits in London at two shillings apiece in mid-April 1763. White, whose acute observational skills were honed by his 'radical' early gardening and his experiments with ventilating the frames on his hotbeds, managed to obtain the 50 cartloads of dung he required each season by developing a local natural economy. Farmers would provide fresh dung for White's garden, which, after use in hotbeds, was in a suitable state to be returned to the farm as a general manure.[39]

Earlier in the century, the lack of dung or even a garden had not stopped one Sir Thomas More who was so desirous of early 'Salleting, Cucumbers, Melons, &c' that he devised a method of using only basins of Thames water covered with perforated cork to grow them in a '*Bed-Chamber*, *Closet*, or *Dining-Room*'. His claimed success showed 'that the Almighty has put it into the Power of Man, with due Application and Diligence, to govern the Vegitable World to a much greater Improvement, Satisfaction, and Pleasure than ever was known in all the former Ages of the World'.[40] Although it appears that this method remained unique to More, by 1790 writers claimed that most people owning a garden attempted to raise early cucumbers.[41]

Hotbeds of genius

The forced cucumber as desirable commodity cannot be separated from the labour involved in its production, for it was the gardener's artful diligence and attention that imbued nature with value. According to the scriptures, the state of humanity – fallen from grace and susceptible to luxury and vice – resulted in the need to labour in order to make the land productive. It was the exercise of this moral duty, the Reverend William Hanbury claimed, that made cultivated trees superior to wild ones: 'It is with them, as with Men; Care and Culture refine both: The Nursery is a Sort of Education, and the Tree which is transplanted from thence, will as much excell all others, as the Man of Learning will outshine the Peasant.'[42] This labour, according to eighteenth-century land improvers, assisted nature through directing and regulating, rather than animating, the fertility of the soil. As fertility came to be seen as a property of the land and not a result of God's direct intervention, increased productivity or excellence of produce was attributed to the skill and diligence of the cultivator.[43] In the kitchen garden, the gardener's expertise consisted in persuading nature to assist his efforts in overcoming the English climate and in making exotic foods safe to consume.[44] With respect to cucumbers – in the opinion of many, already worryingly close to being poisonous – failure to produce a good crop was attributed to 'ignorance or inattention'. The whole success of cucumbers depended upon 'perpetual watching'.[45]

A balance had to be struck, moreover, between 'forcing vegetables in such a manner as to eat near as well as when they come natural' and gardeners' ambitions both to satisfy demand and to demonstrate their skill at manipulating nature by producing cucumbers at ever earlier times of the year.[46] Even though many gardeners thought the very early cucumber worthless, the deployment of their expertise provided pleasure and exhibited accomplishment. Switzer thought cucumbers produced before May 'have by no means a good taste' and did not 'pay for the care and expence we are at about them . . . being very little better than poison'. None the less, he praised those who possessed the 'knowledge and mystery' of forcing cucumbers, claiming it was 'to the immortal honour of our present Kitchen Gardiners, we see the great inclemency of our climate regulated, and nature taught, by their industrious hands, to outdo herself . . . and the unwearied and laborious Gardiner undaunted, even in the midst of the severest weather that can happen'.[47] Philip Miller of the Chelsea Physic Garden also thought fruit appearing before April would be 'crude and unhealthy', having been produced by the rancid steam of the dung rather than the sun, but acknowledged that, despite the expense, raising early cucumbers 'is what most Gentlemens Gardeners have an Emulation to exceed each other in'.[48]

To this end, just as the nobility displayed the fruits of their gardens on their dining tables, so the display of produce was an important way for gardeners to exhibit their skills among themselves. Melon and cucumber feasts were frequently held, with prizes awarded for the best fruits, which 'were productive of much active emulation' and 'a stimulus to further improvement'.[49] By 1797, the Winchester Annual Cucumber Feast was being held at the Dolphin Inn as early as 21 March.[50] This striving to cultivate what many considered a useless product was sufficiently well known to become symbolic of any worthless effort. In 1780, just as he was completing his *Lives of the Poets*, Samuel Johnson disparagingly remarked of Thomas Gray's *Odes*: 'They are forced plants, raised in a hot-bed; and they are poor plants; they are but cucumbers after all.'[51] For gardeners, however, the cucumber came to represent their skill. Their feasts became occasions not only to compete with one another, but also to display their expertise to nurserymen who frequently attended.

Noblemen may have sought to demonstrate the benign fertility of English land through the more spectacular and surprising produce of their kitchen gardens, but they did not battle to produce cucumbers or pineapples themselves. Instead, they strove to acquire gardeners who possessed the knowledge and experience required for the successful cultivation of such fruit. For aristocrats these superior servants were commodities in themselves, and marketed as such through information networks involving nurserymen, employers and gardeners.[52] For their part, gardeners knew that success in growing exotic or out-of-season fruit was the way to climb the career ladder by allowing them to seek employers higher up the social scale with larger

estates and more money to lavish on such enterprises. It was the 'liberal spirit' of such men of fortune 'that called forth the abilities of the servant', and employers were warned that without such liberality 'no man can shine'.[53]

While few could afford to grow pineapples, the cultivation of early cucumbers was in the reach, if not the ability, of all, and became a way for gardeners to define their status and jockey for positions. Demand for forced cucumbers in London must have persisted since, in February 1797, a gardener was tried for stealing 'nineteen cucumber plants, valued at 10s.' from another garden.[54] Nor did the nobility lose their appetite for early cucumbers, as the experience of James McPhail shows. Appointed gardener to the politician Lord Hawkesbury at Addiscombe Place, Surrey in January 1785, McPhail was dismayed to discover that gardeners in the area sowed their cucumber seeds on 20 October. He set to immediately in the hope of raising a later crop, but through mildew his cucumbers came to an 'untimely end'. 'Thus', confessed McPhail, 'the first year of my servitude in this place passed over with no small anxiety on my side; because I failed in having that vegetable either good or in plenty, which was required in the family.' In that year and the next he followed the practice of his neighbour and planted seeds on 20 October for fruit in January or February. But although McPhail 'was allowed plenty of dung, and had horses and carts at my command to bring it, and labourers enough to work and prepare it' he failed to produce cucumbers 'so early as they were required in the family'. In desperation, for none 'but those who sensibly experience it, know the anxiety of mind felt by a servant calumniated, and in danger of losing both his place and character', McPhail paid his neighbour 5 guineas to learn that pouring water on the hotbed reduced the burning heat around the roots. Even with this tip, success remained 'very precarious' and dependent on 'continual attention'. McPhail therefore began experimenting with using dung in expensive brick frames to heat the air rather than the earth in which the cucumbers were grown, ultimately perfecting a method that resulted in Christmas cucumbers that were 'firm and well-flavored' rather than 'watery and ill-tasted'.[55]

Despite, or perhaps because of, McPhail's innovation, demand for early cucumbers continued to tax the gardener. The 80-year-old veteran gardener and nurseryman John Rogers, whose long career included working in George III's Royal Gardens, could still grumble in 1839:

> In winter, when the snow is deep on the ground, a cucumber, either in its green or stewed state, highly embellishes the table of the higher and more wealthy classes, who grudge not the expence; and cucumbers produced fine at that season of the year, add greatly to the gardener's reputation. When the rage for very long and uncouth-looking cucumbers abates, much unnecessary care, labour, and anxiety will be saved to the gardener.[56]

Conclusion

The qualities that make the eighteenth-century cucumber seem an unlikely and ridiculous object of luxury are the very aspects that allow us to widen our understanding of social relations and views of nature in this period. Attitudes towards luxury, Paul Langford claims, come close to being a history of the eighteenth century if politics is taken to be 'about the distribution and representation of this luxury, religion about the attempt to control it, public polemic about generating and regulating it, and social policy about confining it to those who did not produce it'.[57] The history of the unnatural cultivation of the cucumber, by which means a common product was transformed into an object of desire through horticultural artifice, not only epitomizes this analysis of luxury, but also allows the historian to tap the voices of those rarely considered in connection with such debates. The attitudes of the poor, the views of medical men and the opinions of gardeners reveal with unusual clarity the contrivance and contradictions involved in making a luxury.

For the wealthy, the justification of luxuries like early cucumbers lay in the means by which the art of gardening could render both the social order and the products of the soil natural. In this period, 'natural' did not denote a distinction between wild and cultivated nature, but rather could be applied to both if the artifice of the cultivator was thought to operate in accordance with nature. Historians have analysed how improving country estates to look as natural as possible became an index of taste and political ideology, focusing their attention on gardens that were remodelled to express English freedom under the Hanoverian succession and to reflect landowners' fitness for political leadership. Likewise, the political import of kitchen gardens has been investigated in relation to debates about imperialism, utility and improvement. In contrast to the landscape garden this was achieved by the very boundedness and order of the kitchen garden. This self-enclosed space was conceived of as an English Eden, where the skilful management and improvement of plants from foreign countries provided parallels with British imperial policy through which alien peoples were subsumed into the British order.[58] Just as Britain's great houses and estates were on display to the polite eighteenth-century tourist, so too were the hothouses or 'Stoves of great men', which 'present the astonished spectator with the chief produce of the whole globe'.[59]

Such impressive landscape gardens and great stoves were relatively few, however, and it is the forced produce of the kitchen garden that better illustrates the connection of gardening and politics in the eighteenth century by bringing together themes of consumption, production and luxury. This is apparent from kitchen garden manuals, whose authors addressed their works to 'gardeners and nurserymen' as well as the 'nobility and gentry' of England. Writing for both audiences at once reflected the inability to separate employer and servant, consumer and producer, and manuals were

designed to educate each to the requirements of the other. Employers needed to provide the resources which allowed gardeners to exercise their art; gardeners had to possess the necessary skill to make best use of these resources. It is clear that few of those employed as gardeners regarded the cucumber as a desirable object, but, by creating and maintaining the demand for vegetable 'novelties' through ever earlier production of the fruit, they could exploit the desires of the rich in order to secure better positions for themselves. Gardeners' made explicit the skill required to channel nature's powers and supply produce from the kitchen garden the whole year round. It was this 'labour of the brain', they claimed, that entitled a good gardener to be considered 'a philosopher and a politician, and one whose employ ought to place him very near the eye and favour of his master'.[60]

For wealthy employers, the gardener's art allowed the early cucumber to be regarded as natural; for gardeners, art was manifested in the power to imitate natural processes while the resulting fruit was considered by many as unnatural. These differences relate not only to ideas concerning cultivation and horticulture, but also to the interior landscape of the eighteenth-century body, both in the medical sense of the effective conversion of the non-naturals into naturals through suitable regimen and in terms of the embodied skill required of the kitchen gardener. The significance of the eighteenth-century cucumber is illuminated only if it is regarded as a material object to be consumed in all senses of the word.

8

Arguing Disability: Ex-Servicemen's Own Stories in Early Modern England, 1590–1790

Geoffrey L. Hudson

> Your Petitioner beinge striken and wounded [was] one of the first fallinge downe for dead . . . smeared with his one bloud and . . . cloven through the scull of the head with a Pollaxe insoemuch that his braines appeared to the viewe of many, [and] had fower more disperate woundes giuen him but beinge past sence knewe not with what . . . till about the middle of the night your petitioner was inforced to travell . . . throughe frost and snowe, which had soe perished his bodie that his woundes were thought uncurable, yet in length of time throughe much paine and sorrowe and likewayse to his greate cost and charges his woundes were healed . . . . [Your petitioner seeks a stipend] . . . soe that others thereby may be encouraged to use the like endeavors [and] be preserued from beinge a aspectacle [*sic*] of want and indigency.[1]

Englishmen who survived the havoc war wrought on their bodies needed and demanded relief. Over 400 years ago the English Parliament responded, creating Europe's first state system of benefits for the rank-and-file disabled in 1593. This pension system, administered by county justices in the seventeenth century, was in the eighteenth replaced by the Royal Hospitals of Greenwich and Chelsea. The emergence and workings of these two early systems, and the ways in which their would-be beneficiaries worked them, offer a useful perspective on the history of disability and social relations.

Uncovering the petitioner's perspective

The relief of the disabled poor, including its medical aspects, has been described to a great extent from the perspective of the creators and managers of the Poor Laws and their institutions. This is mainly because the nature of the sources has given us their view – and thus left us peering from on high. Until very recently scholars have paid little attention to the attitudes

and actions of disabled people seeking relief, assuming instead that the Poor Laws and those in authority successfully perpetuated traditional deference and submission to one's 'betters'.[2] For example, Anne Borsay, in a recent article on disability before the Industrial Revolution, stated that 'impairment served to strengthen the vision of submission'.[3] Historians interested in the perceived problem of why common people did not create more trouble for their governors have until recently addressed it through analysis of the nature of patriarchal and paternalist theory and practice using sources such as household conduct books and catechisms.[4] Others, some of whom work with the richer sources available for the later seventeenth and eighteenth centuries, have pioneered a new approach. They have been more attentive to demonstrations of initiative and independence among the poor who used statutory poor relief.[5] New evidence from the disabled veteran schemes, including that which I present here, contributes to this debate, challenging the functionalist argument that the poor and disabled were deferential and acquiescent. In addition, the hundreds of petitions, supporting letters, court and hospital records, and council minutes existing for these systems (and providing information on over 10,000 men) enable us to investigate the nature of disability itself.

Someone other than the veteran – vicars, parish clerks, military commanders and schoolmasters – usually wrote petitions, and, of course, the petitions were written to achieve an end: support for the petitioner. Accordingly, they said what the justices and hospital admission boards wanted to hear. Ex-servicemen's petitions must therefore be read, to a certain extent, as a form of fiction. But there is considerable evidence that the maimed soldiers had to appear in court and before admission boards to petition. They had to confirm their written stories and garner support from patrons. The petitions were scrutinized, and therefore they must also be read as valid representations of their bearers' – and contemporary readers' – realities.[6] In practice, then, the submissive tone in many petitions should not be equated with the veterans' own attitudes. Alterations in some petitions reveal that their tone was adapted in order to secure relief; in these instances one can literally read between the lines. In early 1657, for example, Nicholas Amery made 'his addresse to' the bench on return from service. Tellingly, a word was added between the lines: his address was qualified as 'humble'.[7]

The county system

The Elizabethan legislation provided for pensions as a right based on disability experienced in military service. The legislation relied on the maimed ex-serviceman and his military superiors to create the application; local JPs were to use this documentation rather than create their own. Once a pension was granted, justices were not permitted arbitrarily to revoke, diminish or alter it without just cause (i.e. contrary to the statutory provisions). Disabled

men could claim stipends as a right without submitting to physical examination or a means test.

Towards the middle and end of the first decade of the century, however, ex-servicemen *and* their champions began to introduce arguments based on extra-statutory criteria. Poverty was cited. Such citations included mention of the inability of the ex-serviceman to labour or otherwise maintain himself, age, or that he had a wife and children to support. Some argued that he had done all he could to avoid reliance on public funds.

Many of these new criteria are found in a parish certificate forwarded to the Kent bench in January 1617/18 in support of an application for a county pension. The parish declared that the man, a carpenter, had been maimed in the hand in France in the service of Queen Elizabeth. On his return 'by his painfull labour in his calling' he 'kept & mainteyned himself his wife & manye Children so long as he was able to frome his worke'. But now, unable to work because of his war wound, he had fallen into 'great want & povertie'. On the reverse was a certificate from the daughter-in-law of the man's commander in France as to his service in which he was 'hurt . . . which now in his age is a great trouble unto him'. She also stated that he 'hath ever lived an honest labouringe man'.[8]

These changes in the application procedure and the basis for application are reflected in two long-term developments between 1593 and 1641. Increasingly, applicants cited their inability to work rather than to serve again in the military, and the latter disappeared as a stated reason why a pension should be granted. There is also evidence that justices in sessions increasingly expected men to reveal their disabilities on application for pension benefits.[9] In addition, men were increasingly denied pensions, or had pensions withdrawn, because they had other means of subsistence.[10]

Need, associated with life-cycle poverty, became the basis for the application and award of relief and pension level increases, rather than the statutory basis of mutilation in recent military conflict. The meaning of disability within the context of the county military pension scheme became, in time, almost identical to what Tim Wales and Margaret Pelling have shown was its meaning in the parish poor relief system. Relief was granted to those who were disabled by age or casualty, in order to maintain themselves and their families.[11]

JPs were quick to implement the 1593 legislation. But when the numbers of eligible men increased, the justices failed (despite pressure from the Privy Council) to use the statutory powers given to them in 1598 and 1601 to raise the rates. The political pressures are apparent: it was one thing to set a rate, another to collect it and yet another to increase it.[12] Instead of raising taxes, the justices resorted to traditional practices used by institutions such as cathedral almshouses and guilds. They restricted the number of pensions available, granted pensions in reversion and awarded one-time composition payments in lieu of future stipend payments.[13]

After 1647 many of the discrepancies between law and practice of the pre-1642 period were incorporated into legislation. These changes reflected the concerns of county and corporation benches in a couple of significant respects: it was now legal to discriminate on the basis of need and to require certificates from local justices as to the suitability of veterans for relief. In addition, discrepancies between law and practice continued because of a continuing lack of funds to pension adequately all those entitled to a stipend.[14]

Lists of new pensioners in 1647 reveal the disabilities ex-servicemen claimed, and justices were prepared to recognize, immediately after war. Hands, arms and legs – crucially important in the pursuit of manual labour – were the most affected.

Evidence from Quarter Sessions petitions from Cheshire and Devon, shown in Table 8.2, reveals that *most* veterans mentioned explicitly that they had been physically maimed in battle. They narrated their service history, the battles and their injuries, describing the ways in which their wounds were physically disabling.

Table 8.1 New pensioners and their disabilities, 1647[15]

	Somerset	**Cheshire**
One limb inoperative	8	16
One limb lost	4	1
More than one limb inoperative	2	2
Head wound	2	2
Sickness	0	1

Table 8.2 Disabilities of Applicants for Pensions, Cheshire and Devon[16]

	Cheshire 1593–1642	**Cheshire 1646–60**	**Cheshire 1660–80**	**Devon 1660–92**
General description of maims in service	25	44	71	216
One limb inoperative	9	35	16	33
More than one limb inoperative	3	9	8	10
Cold/diseas	7	4	5	11
Disabled by age, not military	6	3	15	9
One limb lost	1	5	4	11
More than one limb lost	1	1	0	1
Visual	2	2	4	11
Hearing	1	0	0	0
Mental	0	0	0	2

Interestingly, particularly in comparison to modern models of disability, some also cited colds and disease, violent fevers and fluxes, which they had contracted in service. These illnesses continued to plague them, making labour difficult. Such citations of cold and disease correspond to contemporary understandings of the humoral theory of the body, a theory that was understood and accepted by lay people of different social classes as well as medical practitioners.

Within humoralism the body was believed to be a semi-permeable, irrigated vessel in which moved the four humours (blood, phlegm, yellow bile and black bile). Individuals could maintain their health by preserving their internal stability through evacuation of bodily fluids and the avoidance, if possible, of environments and conditions of life that upset the humoral balance. By contrast, lengthy exposure to perilous extremes – of weather, diet or activity – could damage the body permanently.[17] In 1656, for example, John Rigby declared that service 'hath brought him to much weaknesse by the length of the march & heats and colds therein so [he is] not able to labour'.[18]

In addition to the effect of cold and heat, men sometimes mentioned loss of blood, which, if significant, could have a permanent effect on the body.[19] The blood was believed to carry the humours, and as such an inordinate loss could irreparably upset the internal balance for life, especially as one got older and started to dry out.[20] In all these cases, the diagnosis was offered by the disabled veterans themselves in their petitions, confirmed by patrons and supporters, and by and large accepted by those administering the pension system. The meaning of some illnesses, as with battle wounds, was cast in historical terms by petitioners. What we would deem medically insignificant events in service triggered long-term disability. Thus they illustrate the close relationship between a given society's model of the healthy body and the constitution of the 'disability' category.

It is also clear from the petitions that although age was important in contemporary notions of pensionable disability, one had to be old *and* disabled to be considered: the men expected to work as long as possible. There was no notion of mandatory retirement.[21]

In addition to the effects of cold, disease, blood loss and age, specific causes of wounds were cited. Evidence from Cheshire and Devon indicates that gunshot wounds, notoriously dangerous, were cited most often – 27 of 32 citations in Cheshire between 1660 and 1680 in which the cause of the wound was mentioned, 52 of 60 citations in Devon between 1660 and 1692.[22] In one case a surgeon's certificate for a Humphrey Briscoe stated the man had been 'shot with a Brace of Bullets verie dangrously into the shoulder out of which came many Bones and he a longe time in Cure . . . ' In his petition Briscoe maintained that he was 'disabled to follow his vocation as formerly'.[23]

Like Briscoe, men usually stressed that they were so disabled as to be unable to work. This emphasis again corresponded with contemporary notions of

disability within the parish Poor Law system, based on life-cycle poverty. Although initially, in 1593, the county scheme provided for pensions as a *right* based on disability incurred during military service without a means test, this changed, first in practice, later in law. Disability to work, a physical examination and a means test were introduced. A number of counties made the norm of disability even more systematic by requiring periodic reviews of pensioners to ensure that they were still disabled to work. Surgeons were then hired to examine the men physically, as in Devon after the Restoration.[24] This suggests that scholars who argue that disability was a medical construct are wrong: at the origins of disability as a social and political construct in early modern Europe, the medical men were very much followers, functionaries not innovators of policy.[25]

Manipulation and negotiation in the county system

Louise Gray has postulated that German disabled paupers wanted to work and that their identity was bound up with taking an active role in society.[26] There is certainly something to this: ex-servicemen's survival strategies, and the nature of their by-employments, were described in their petitions and indicate their practice of working if necessary and possible, as well as seeking support from kith and kin.[27] I am more cautious, however: doing what one must is not always doing what one desires. Indeed, in the cases covered by my research, the disabled men acted with zeal in pursuit of county pensions, indicating a preference for secure, permanent relief to continuing to serve or work. A common strategy or expedient utilized by veterans was importunity, displayed as burdensome, troublesome pertinacity in solicitation. Men would return to the court to apply for relief after previously accepting one-off payments in lieu of future benefits. After having had their pensions taken away they would return, often several times, demanding its reinstatement. When seeking a pension increase they would trouble the bench with many appearances. They also were careful to watch for and act on pension vacancies, and informed on each other to create such vacancies.

In addition, they manipulated the system: relying on magisterial amnesia to reappear when they had been told not to; approaching clerks of the peace before the court was in session to smooth the way; seeking the support of prominent justices; acquiring relief in several jurisdictions simultaneously; and going over the heads of the county bench, sometimes appealing to the central government to intervene.

Veterans told central authorities what they wanted to hear, praising their honourable protection of ex-soldiers. They mentioned instances of *lèse-majesté* – neglect of, or words of disrespect spoken by local justices about previous missives from king and council. One veteran commented that despite the previous intervention of the council, the justices 'still will wronge

poore menn with out enie just cause'.[28] After 1642, petitions also drew attention to the issue of allegiance and mentioned legal entitlement. Demands were made: loyalty to state or king must be rewarded. Indeed, the disabled called the pension 'recompence', 'satisfaction' and 'justice'.[29]

Veterans also acted together not only to demand relief but to press for changes in practice and law. At the national level, Parliament was treated to numerous demonstrations and petitions from ex-servicemen at key moments – the early and mid-1590s, 1647, later 1650s, early 1660s (and in relation to the Hospitals, 1770s) – bringing pressure to bear on their betters to relieve them through enacting and/or improving legislation.[30] Consistently, veterans actively sought honourable, secure pensions for life, and in the mid-1600s also secured pensions for women and children as war widows and orphans.[31]

In the counties, pensions were paid to war widows and orphans for approximately 30 years from the late 1640s. In the city of London in 1671, 89 war widows (and orphans) comprised 62 per cent of the pensioners, receiving 62 per cent of funds expended. One major difference between men's and women's petitions was that women very rarely mentioned their occupation; instead they were more likely to mention their children – orphans in contemporary usage – enumerating them and whether they were small (that is, under seven years of age). War widowhood meant that a woman became a widow earlier than usual and was likely to be left with more and younger children. Justices discriminated among applicants on the basis of need, and the number of children – the size of the woman's charge in contemporary language – was an important factor in her favour. The Restoration did bring a change. Outside London women were rarely granted stipends and had to settle for gratuities. This seems to reflect the attitudes of the Royalists to women and the armed forces – both were to be returned to firm control (the army to the king, war widows to their parish officers). Royalists were reluctant to break with the tradition established in 1593–1642 of granting war widows parish relief. They were unwilling to accord women the same status as disabled veterans and grant them state military pensions.[32]

The Royal Hospitals: narrating disability, countering containment

The Royal Hospitals at Chelsea and Greenwich eventually replaced county relief and demanded new strategies from would-be and actual pensioners. Hospitals' selection procedure required military certificates and physical examination by hospital medics. Their definition of pensionable disability continued, as in the county scheme, to focus on disability to work. As with the county system, the majority of pensioners had been wounded in service to such an extent that they were disabled to work, with a minority suffering from illness (see Tables 8.3 and 8.4).

Table 8.3 Chelsea Disabilities, 1715–27[33]

General description of maims in service:	20
Wounds/lost use of: hands, arms or legs	1,414
Wounds: chest, shoulder, back	211
Lost arm, hand, leg, foot or leg	45
Head injury	513
Bruised	201
Rupture	104
Visual (165 blind)	214
Deaf	44
Cold and/or humours	87
Rheumatism	118
Palsy	25
Worn out	306
Consumption	26
Convulsions	20

Table 8.4 Greenwich disabilities, 1749–90[34]

General description of maims in service:	34
Wounds/lost use of: hands, arms or legs	1,454
Wounds: chest, shoulder, back	180
Lost arm or leg	263
Lost both legs	12
Head injury	168
Rupture	697
Visual (50 blind)	129
Deaf	26
Palsy	10
Dropsy	2
Rheumatism	6
Fits	5

Table 8.5 Chelsea wounds, 1715–27[35]

Shot	1,607
Bombshell, grenade	122
Cut, stab	1,151

The records show that gunshot wounds continued to predominate, as they had in the previous century.

Scholars have argued that the term 'disabled' prior to the twentieth century 'mostly denoted honourable decrepitude, often allied to old age'.[36] This link with age, however, is not entirely accurate. An examination of 9,194

Greenwich pensioners from 1749 to 1790 shows that 28 per cent were under 50, 13 per cent under 40. Some were as young as 12.[37]

A comparison of the early and the later eighteenth century shows a dramatic decline in explanations founded in humoral theory, with mention of the effect of bad humours, as well as cold and heat in service, disappearing. The 1720s admission books include references to the humours that would not have been out of place in the 1600s: 'quite deaf, happened to him as the officer says by colds got in the camp; humour fallen into his right leg'; 'lost hearing; almost lost his sight; lost memory; by cold at the camp near Sterling'; 'very lame by rheumatism on right side received at Newcastle by cold'. By the late eighteenth century, however, humoral references *are gone*. Instead of citing a common frame of reference, located previously in humoral theory, sources increasingly cite the surgeon's opinion, based on his clinical examination and diagnosis of damaged or diseased body parts. When, for example, John Miller petitioned for a pension in 1786, he did *not* tell his life story or refer to the humours. Instead, he asked the governor of Chelsea Hospital to order that he be 'inspected' by a hospital surgeon so that it 'will fully appear' that he was incapable of further service.[38] Thus the humoral theory disappeared even from petitions, to be replaced by explanations based on professional physical diagnosis instead of patient/petitioner narrative. This shift is in part related to contemporary developments in military medicine, as evidenced in John Pringle's *Observations on the Diseases of the Army* (1753). Pringle was Physician General to His Majesty's Forces Oversees in the 1740s. In his *Observations,* Pringle described the incidence, prevention and treatment of disease. Several of the maladies that troops suffered from in and after service, previously located in an imbalance of the humours, were now located in the so-called inflammatory diseases: bronchitis, pleurisy, pneumonia, rheumatism and consumption.[39] The cause of these diseases was believed to be putrid air experienced in service, rather than humoral imbalance. A principal aim of military medicine was therefore the avoidance of pestilential vapours via hygienic measures, ventilation, decent clothing and food.

The disappearance of humoral theory even from petitions over the course of the eighteenth century, and its replacement by a medical practitioner's physical diagnosis (instead of a patient narrative), is a significant finding given the current historiography. Mary Fissell has argued that this change started to occur in the mid-eighteenth century as a result of the medicalization of infirmaries and hospitals, later abetted by the late eighteenth-century importation of Parisian diagnostic techniques.[40] What Fissell does not discuss is the potential earlier impact of military medicine, manifest in the Royal Hospitals of Greenwich and Chelsea and other military hospitals. The argument of Fissell and others for a mid-eighteenth-century locus for the development of new hospital-based medical practices is not sensible for the history of military medicine.

Hospitals, field and fixed, had been a crucial component of military medicine for the British forces since the reign of William III and the War of Spanish Succession. This was the case in other European countries as well, as the size of forces increased, and national governments got increasingly involved in medicalized military hospitals from the late seventeenth century onwards.[41] Phillip Mills asserts that for the British army, 'the single most important continuity in military medicine provision throughout the eighteenth century was reliance upon a hospital system . . . The military hospital patient was above all a medical patient.'[42] The hospital suited the military environment: one of the principal benefits of hospital care was the coordination of medicine and the control of the patient.[43] Misdemeanours, such as the refusal to take medicines or follow dietary advice, were subject to punishment. The impact of military experience on civilian practice is an important research question that needs further attention. Günter Risse has suggested, for example, that elements of practice (e.g. ventilation and cleanliness) at the new infirmary at Edinburgh, built in the 1740s, reflected military experience.[44]

Harold Cook and others have looked to developments in the mid- and late seventeenth century, arguing that military medicine was influential in the development of a fundamental shift in the history of medicine. Cook argues that military medicine emphasized identifying diseases as entities and developing corresponding (and quick) medical and surgical cures for the masses in the forces. This was arguably in contrast to traditional learned medicine with its emphasis on physic, the individual and his or her own particular disease, focusing on the humours. In the military, it was neither possible nor desirable to apply humoral therapeutics, based on intimate knowledge of patient histories and idiosyncrasies, for each and every serviceman.

In the mid-seventeenth century, these impulses had influenced several military medicine practitioners to be involved in chemical medicine (e.g. John French, physician at the Parliamentary Savoy House Hospital). After 1688 changes were mediated via reform of military medical institutions, including hospitals, influenced by Dutch practices. These changes also included a move to take on board the clinical ideas of Thomas Sydenham: clinical emphasis on observation, note-taking and the identification of disease from symptoms.[45] Thus, Cook suggests, military medicine over several decades after 1688 legitimized medical empiricism, and the development of standardized diagnosis and treatment. For military practitioners the natural-historical approach, combined with experiments, promised a better basis for rules of diagnosis and therapy. Furthermore, the significantly enlarged armed forces in the late seventeenth and the eighteenth centuries offered significant encouragement to medical practitioners who sought to offer new and effective treatments for the masses.[46] It is possible that the decline of humoral medical theory in the Royal Hospitals, as evidenced in the petitions and other supporting documentation, was a product of these

developments and, perhaps, the beginning of a decline of humoral theory in popular culture as well.

Such changes notwithstanding, within the Royal Hospitals the veterans acted out and argued, as had their predecessors within the context of the county scheme. They practised importunity when they petitioned for pensions, tried to get back in after running away or being expelled, and in order to achieve better conditions.[47] Men pleaded poverty, disability and ignorance of the rules based on illiteracy or bad eyesight in order to secure relief. And quite often, after returning, men would again commit the offences they had promised never to do again : drinking, having sex with local women, fighting, thieving and questioning the authority of hospital officers.[48] Even submission could be an illusion. The Greenwich Lieutenant Governor reported that a John Noble offered several times to beg the Governor's favour on his knees towards his being restored to the Hospital. But finding no encouragement, Noble 'said God damn you, I am a better man than you', and stood up.[49]

Bureaucratic deficiencies within the hospitals provided ex-servicemen with opportunities which they seized, with men impersonating deceased out-pensioners and bribing hospital officials.[50] To support themselves and their families better, pensioners sought by-employments, working in taverns for example, despite a prohibition on such outside work. Others rejoined the forces, seeking hospital readmission years later if their disabilities made it impossible for them to continue.[51]

Intimacy between pensioners and the war widow nurses was not unknown. One nurse, for example, was quoted as declaring in the act that her lover 'was the most charming man in the house'.[52] Forbidden to marry, pensioners and local women had clandestine marriages in the liberty of the Fleet in London; canonically legal, these marriages were conducted without banns, a licence, and outside the couple's diocese. By these means the pensioner and bride undermined hospital control of the process of family formation, thus avoiding expulsion from the hospital. Others had sex and children without the benefit of marriage.[53] In Greenwich 60 per cent of pensioners were married, 35 per cent with children.[54] Despite early hospital intentions that war widows and orphans would be relieved, this did not come to pass, save for appointing war widows as nurses and creating a school for orphan boys at Greenwich later in the century. Building and maintenance costs were high, and widows were left to apply to the parish like other destitutes.

As well as resisting social control, pensioners resisted – albeit less successfully – the development of a medical monopoly. Seventeenth-century county pensioners were given money to help pay for their medical bills, leaving them with considerable freedom of choice. Initially, hospital pensioners similarly had room to pursue their own courses of treatment on the outside. Thomas Nichols, for example, was under the care of a Mrs Price of Deptford for his blindness in 1707.[55] This leeway to go outside the institution was rapidly curtailed, however. Pensioners were punished if they resisted

the course of treatment dictated by the hospital surgeon or physician, or sought alternative care. The buyer–seller relationship between patient and medical practitioner disappeared for disabled seamen, for instance, even while it remained predominant in society. One Roger Pannell, ordered to go to the surgeon to be bled in April of 1711, refused saying, 'he would not [go] for the surgeon had kill'd one man by letting his blood very lately'. The council that administered discipline at Greenwich seriously considered expelling him, only to relent when Pannell apologized to the surgeon and the latter intervened on his behalf.[56] In another example, Richard Mercer presented himself to the physician and surgeon in 1720 for treatment, but was told he was not ill. He responded by going outside the hospital for treatment and medicines. He also went to the civil officers of the house and asked to be placed in a lower ward so that he could more easily enter and leave the institution. The civil officers agreed, only for the medics to protest in council 'that the said Mercer has no distempers that may occasion his being admitted into the Infirmary'. The council determined that Mercer had acted in contempt of the hospital officers 'by applying for Medicines abroad' and he was reprimanded.[57]

As with the county scheme, ex-servicemen within the hospitals actively and regularly went over the heads of hospital officers. They sought reversal of decisions by hospital admission boards, and councils of discipline, acquiring intervention from military patrons, the king and Members of Parliament. In doing so, they complained of ill-treatment, a lack of justice and misbehaviour on the part of hospital officers.[58]

It is clear from the actions of pensioners in seeking outside intervention, and in their words and actions in mutiny, that they felt as entitled to their pensions as had seventeenth-century veterans.[59] This sense of entitlement was similar to that expressed by veterans decades earlier. As one Mathew Boreman stated in a petition to the Admiralty seeking pension reinstatement, 'your Petitioner, late a Pensioner in Greenwich Hospital . . . was . . . justly entitled to the Injoyment, of the same dureing the Remainder of his days, an Indulgence due for his having faithfully and Cheerfully spent his Youth and Strength in the Service of this Crown and Government ...'[60]

Conclusions

The county and hospital schemes tell us much about the effect of war on the body in this period and reveal contemporary notions of chronic disability and the declining role of humoral ideas. Long-term physical disabilities caused by gunshot wounds and sword cuts overwhelmingly predominated over illness. Ageing was a process of very gradual withdrawal from economic productivity and self-support. They reveal too the extent to which authorities and the impoverished shared the belief that, in the end, the only recognized 'disability' was the disability to work. Men expected and sought work or

continued service even though they had been affected in body by their wartime experiences.

Under both systems, the disabled, as individuals and in combination, demonstrated through their actions and words a firm sense of entitlement. And they were effective in securing changes in law and practice, as well as benefits for themselves. Men gained skills and developed attitudes in military service that enabled them to work the pension scheme aggressively. Indeed, they may have set a significant precedent for England's impoverished: one of war bringing about long-term changes to patterns of deference and social relations.

Nor were England's governors the only actors in this play. They experienced significant pressure from veterans, locally and nationally. It has been suggested that the origins and development of the Poor Laws may be better understood as 'the result of pressure from both directions'. The poor felt entitled to relief, and the governors recognized the legitimacy of the needs and demands of their inferiors, and had the means and 'sense of benevolence' necessary to act.[61] Nevertheless, the nature of the behaviour of the lower orders in putting pressure on their governors is open to interpretation. Some scholars have argued that in petitioning for relief, the poor were accepting a role that had been dictated to them by the wealthier in society. Each time they asked for and received relief they re-enacted their subordination. They acquiesced in order to be succoured.[62] Drawing on the evidence of these ex-servicemen, it is possible to take this analysis one step further: subordination and negotiation are different. Certainly a mask of deference was worn by the governed, but when those governed require the governors to exercise some of the practical duties implied by elite paternalism, both groups found themselves bound by that social system. Indeed, the collective and individual action of disabled veterans forced the governors to act and thereby enabled the governed to improve their lot: new laws were passed and practices were altered to the benefit of the disabled poor. These were not simply exercises in social control. The governed were certainly conditioned by a patriarchal linguistic and historical heritage (petitioning in the established mode), but they also knew from their shared life experiences at work and at war that things could be better. The play unfolded according to established conventions but the governed could, on occasion, be the protagonists.[63]

9

Lunacy and Labouring Men: Narratives of Male Vulnerability in Mid-Victorian London

Akihito Suzuki

This chapter will argue that Victorian labouring men in London who were sent to the Middlesex County Asylum at Hanwell during 1845–50 suffered from intense fear of poverty and deep anxiety about their economic future. Such fear and anxiety were not given prominent place in the writings of middle-class social commentators or psychiatric authors at that time, but were reported by the patients' families to be the major cause of each man's insanity. I suggest that these fears and anxieties were the psychological price of the new working-class respectability and the concomitant notion of manhood.

Introduction

What kind of mental disease did Victorian working-class men suffer from? We do not have a ready answer, although we know they were confined *en masse* in public asylums when they became insane.[1] No iconic disease exists for Victorian working-class men to parallel the examples of the melancholia of male elite in the Renaissance, the demonic possession of young women during the Reformation and the 'English disease' of the prosperous class during the Enlightenment. Victorian middle-class women had hysteria as the disease that symbolized their place in the separate spheres, while middle-class and well-educated men in the late nineteenth century had neurasthenia, 'shattered nerves' or even male hysteria.[2] If one proceeds to the early twentieth century, shell-shocked soldiers and officers on the Western Front played the most prominent part in the present historiography of mental diseases. Perhaps because of this difference, less educated men's experience of mental diseases in the nineteenth century so far have been left unexplored.[3] Here I begin to fill this historiographical blind spot to reframe our chronology of mental diseases of men.

Psychiatric treatises and annual reports of county asylums certainly refer frequently to the experience of the working-class male patients. Such sources tell us more about the views of medical superintendents than about those of the patients and their families. In order to examine the perspectives of lay people, I have used different sources, namely the casebooks of the Middlesex County Asylum at Hanwell.[4] Although written by the asylum physician, they contain rich information about lay people's understanding of madness: from 1846 the doctors on the male side of the asylum assiduously and systematically interviewed the patients' families who visited the asylum. Almost certainly this practice was an idea of John Conolly, the asylum's most famous superintendent, who at that time held the office of visiting physician.[5] He conducted this time-consuming survey in order to find the cause of the disease in the life-history of the patients, as reported by their families. Thanks to painstaking record-keeping, we now possess detailed information about the lives of almost all male patients during the period under consideration. For analysis I have selected the casebooks written between October 1845 and September 1850, which contain 197 interviews. Through the examination of these narratives, I have identified patterns of lay interpretation of insanity.[6]

Domestic economy and madness

The lay narratives of insanity which emerged from the Hanwell are extremely diverse and defy simple characterization.[7] Certainly, there existed stereotypical figures from the stylized portrait gallery of nineteenth-century madness: hard-drinking labourers, over-proud maniacs, love-sick Werthers, religious fanatics, bankrupt businessmen, and so on.[8] Most representations in the casebooks did not, however, conform to such stereotypes. This is unsurprising: the stereotypes did not so much reflect reality as allay fears by drawing a fictional line between the normal and the abnormal, between the Self and the Other.[9] The sources consulted here have a different origin and purpose, for they contained the view of those who had to live with the madness of their family member. While stereotypes of madness gave the images of insanity seen from outside, these are representations of madness seen from inside and experienced intimately. From the viewpoint of those close to the patient, things were more complex, and occasionally completely different. The Hanwell narrators were directly involved in the unhappy situation of the patients' mental disturbances for a considerable period, and experienced directly the patients' unstable, unpredictable and troublesome behaviour. Moreover, as members of the family, the narrators were co-sufferers of the hardships to which the patients' mental diseases were attributed. Thus narrators looked for the cause of insanity through the framework of *their own* present calamity. Consequently, the lay and domestic exercise of psychiatric aetiology was less about applying a ready-made stereotypical

Table 9.1 Types of causal attributions

			Number	%
Internal	Mental	Defective characters	43	21.8
External	Mental	Grief	106	53.8
	Mental	Anxiety	96	48.7
	Mental	Distress	26	13.2
	Somatic	Illness	74	37.6
	Somatic	Drinking	35	17.8
	Somatic	Injury	24	12.2
	Somatic	Fatigue	22	11.2

∗ The total number of patients = 197

pattern of the causation of madness, and more about searching for a particularized account of the hardship which the patient and the narrator shared.

Table 9.1 shows that two broad frameworks existed in which the narratives were told. One is an 'internal' model, which placed a premium on the individual's power to control his own behaviour and saw madness as a natural continuation of the defective personality. Madness conceived in this model traces the disease to the troublesome character of the individual. Forty-three narratives (21.8 per cent) used this model. Here, I will not discuss narratives employing this framework. Suffice it to say that they register the family's criticism of the patient himself. The other is an 'externalist' model, which was used by the majority of narratives. It emphasized the impact of circumstantial forces on the mind in the causation of madness. Individuals became insane according to this model because the mind could not endure shocks, stresses or mishaps that assaulted it from outside. Those patients conceived of in accordance with this model had become insane passively. These two approaches apparently coexisted in the popular understanding of madness in early Victorian period. Indeed, condemnatory and sympathetic feelings towards the patient often coexisted in a single narrative, for the line between madness that deserved sympathy and that which did not was fine and ambiguous. Tocqueville's dictum on English pauperism applied to insanity, too: '[Nothing] is as difficult to distinguish as the nuances that separate unmerited misfortune from an adversity produced by vice.'[10] Below I focus on these externalist accounts. They are central as the dominant explanatory mode, as well as for their historiographic import.

Table 9.2 further divides the 'external' attributions of the cause of madness. The number of cases in which psychological causes were mentioned is greater than cases in which somatic causes were given, a common finding in statistics of insanity at that time.[11] Among psychological causes, issues related

Table 9.2 Types of causal attributions: external and mental

Category	Subjects	No.	%
Grief	Economic loss	42	21.3
	Unemployment	37	18.8
	Bereavement	27	13.7
	Total	106	53.8
Anxiety	Poverty	40	20.3
	Work	23	11.7
	Domestic	20	10.2
	Others	13	6.6
	Total	96	48.7
Distress	–	26	13.2

∗The total number of patients = 197

to economy and domestic situations were by far the most prominent factor in the family narratives of madness. Anxiety rooted in poverty and work, and grief over economic losses and unemployment, loomed large, as did the psychological strains caused by stressful domestic situations.[12] The two major categories of economic concerns and domestic anxieties were naturally intertwined, for unemployment affected the entire household and domestic concerns frequently led to inability to work. Lay attributions of madness in labouring men were thus framed round the intersection of a man's work and economy and his domestic and emotional life. Here, I focus on the former, although I will necessarily touch on some issues related to the latter.

Among the stereotypes of 'economic' madness, the most firmly established was that of madness caused by excitement at huge gains and shock at huge losses.[13] The rapid industrialization and economic growth beginning in the late eighteenth century brought about opportunities for large-scale capital investment, exposing many to the morbid excitement of economic speculation. Particularly from the 1830s, the sudden and immense expansion of the railway as well as foreign government loans attracted frenzied investors, many of whom were to lose heavily.[14] These two types of investment were mentioned by Benjamin Skipper's wife as the causes of her husband's madness: Skipper had become insane, his wife told the asylum, because he had lost all his savings in speculative investment in Spanish bonds and railways, as well as in heavy gambling.[15] Skipper and many others were perceived by their families as psychiatric casualties of the boom-and-burst cycles of the early phase of the Industrial Revolution. What is remarkable here is how widespread this concept was in the social spectrum: proof that the oscillating economy affected not just a handful of wealthy investors or the social elites, but a great many individuals in the lower-middle and working classes.

Stories of the speculator gone mad are, however, minorities in my sample. Much more common were accounts of chronic poverty. Rather than the sudden swings of fortune, the endless struggle to earn enough was prominent in family narratives of madness. The wife of Henry Allison, a Methodist shoemaker with five children, told a typical story: 'though he worked hard from an early hour in the morning to a late one at night he was extremely poor and found it very difficult to support his family.' Allison's mental breakdown finally came with the abject failure of a desperate attempt to earn money:

> [One] evening he took out a pair of boots which he had made hoping to sell them, he called at several ready made boots and shoe shops but could not dispose of them, he wandered about the streets till after midnight (it was unusual for him stay out late) came home told his wife that he had got employment in the city which would yield him half a guinea a day.[16]

This fairy tale ending turned out to be untrue. One cannot tell whether his story of getting a good job was a delusion or a desperate attempt to please his wife, who must have waited for him with anxiety and expectation. The next day, his madness was expressed in a most spectacular fashion: 'the following day (Sunday) he went to Chapel, when the service was concluded he stood up said he was going abroad to preach the Gospel to the heathen nations, [and said] that he could be absent for six years and that on his return he should have a large congregation in London.' The picture emerging from his wife's narrative is clear: an honest, sober and hard-working man, who rarely stayed out late (note well the emphasis on this sign of his respectability in the passage quoted above), cracking under the pressure of poverty, anxiety and disappointment.

Likewise prominent in many narratives was the psychological risk of overwork and exhaustion. Many families attributed the madness of their male family member to his physical and mental exhaustion. Here lies the greatest discrepancy between madness seen from the inside and madness seen from the outside. The effect of hard labour on mental health was a blind spot in the contemporary middle-class discourse on disease and the hazard of working-class life. Although much had been written on the wretched bodily/physical state of the overworked urban poor since the early 1830s, the *psychiatric* risk of overwork was rarely discussed.[17] Few social commentators, even when sympathetic to the poor, considered the possibility of labourers working themselves into lunacy. Somatic diseases such as consumption came to symbolize poverty and exhaustion (think of Dickens' Little Nell and Engels' *Condition of the Working-Class in England*), but mental diseases were notable for their absence from contemporary analyses of the plight of labouring men.[18] When sanitary reformers did talk about what can be called the mental health of poor workers, they discussed the prevention of moral depravity

through environmental means.[19] Indeed, idleness, not overwork, was seen as the major risk to the 'moral health' of the workers: as Gareth Steadman Jones has argued, middle-class concern over the degeneration of the moral health of the London poor focused on the alleged lack of will to work due to the debilitating effects of indiscriminate charity.[20]

Authors of psychiatric treatise, on the other hand, had long been acquainted with the danger to the mind of overwork. In the pages of medical treatises, however, it was middle-class men and women who usually monopolized the privilege of possessing vulnerable psyches subject to damage from overwork.[21] Over-exertion of the brain, rather than the muscles, was understood by psychiatrists to be a major cause of psychiatric diseases. The division between brain work and manual work thus underpinned a theoretical psychiatric epidemiology of the middle and working classes: middle-class occupations were psychologically high-risk activity, while manual labour was physically demanding but psychologically wholesome. It thus makes sense that intemperance was one of the most commonly invoked cause of working-class lunacy by psychiatric authors. The contrast in the aetiology enabled middle-class doctors to sympathize with their social peers and blame their social inferiors: middle-class mental patients were regarded as too sincere followers of a rigorous work ethic, faulted only for their zeal, while their working-class counterparts were explained as abusers of their leisure time.

The picture of physical labour/working-class life was very different when seen from the other side. The family narratives told at Hanwell clearly demonstrate that working-class families often ascribed the insanity of their menfolk to (blameless) toil. Indeed, narrators often expressed resentment or anger against the employers. Robert Taylor's wife, for instance, cast her insane husband as a victim of his employer's decadent lifestyle:

> 12 y[ea]rs ago whilst coachman to the Duke of Somerset, he was much out at night driving his Grace's daughters to balls and waiting in the street for them till four or five o'clock in the morning. [Want] of sleep and exposure to cold brought on nervousness and despondency, in which he attempted suicide and inflicted a wound in his throat with a razor.[22]

Here is a potent resentment against the aristocratic frivolity that put working men's physical and mental health at risk. John Blake's wife similarly conveyed barely concealed resentment at his intolerable workload:

> he had the care of a large garden, a lawn, two green houses, also a horse and gig, his master was out everyday in the gig on business, and having married lately he went out in the gig with his wife in the evening also, this caused the patient additional trouble in cleaning the horse and gig.

Accordingly, Blake 'was constantly occupied from morning till night, and had scarcely any time to himself even on a Sunday'. His wife told doctors succinctly: 'overwork and anxiety to please his master in the performance of his work so as not to be out of employment again are supposed to be causes of his malady.'[23] In these cases, the madness of a family member provoked a protest against his employer, framing the patient as a victim of a vicious industrial relationship. Madness induced by overwork was an idea woven into working-class consciousness of their place in society, not the nonentity that appeared (only to be dismissed) in the writings of middle-class social commentators and psychiatric authors.

If overwork was the major physical cause of madness cited by families, anxiety was the major psychological cause. Among the sources of anxiety, matters related to the economy and particularly to employment were again the most significant. Numerous families described how anxiety about chronic poverty 'preyed on' the minds of the patients.

Relatively well-off working men might have been exempt from physically debilitating overwork and the corrosive psychological effect of long-term distress, but they too faced devastating anxiety, their families told at the asylum. Labouring men were well aware how easily things could go wrong and were vigilant for possible danger of economic ruin.[24] The narratives at the Hanwell Asylum reveal that working-class people lived in constant fear of ruin. Stories were repeatedly told about anxiety and morbid fear of ruin eventually leading to madness. The madness of Bartholomew Joseph, a well-off tailor who took lodgers to supplement his income, was traced to the frequent difficulties he had had from not having enough lodgers to pay 'heavy rent and taxes'.[25] James Knowles ran a hotel-*cum*-tobacconist's and was modestly well-off. However, when his business suffered a downturn, his anxiety eroded his sanity: '[he] was afraid he should become bankrupt, his rent and taxes being heavy, not being able to pay off some little money'. Consequently, he grew 'greatly depressed, could not take pleasure in anything', while his appetite failed and he grew thin and pale. As if to prepare for the worst to come, he entertained a delusion that he was a king and would 'reform the hospitals and workhouses, and benefit the poor'.[26]

Lower-middle-class occupations were by no means free from similar risk and anxieties. Many men of lower-middle-class occupations were reported to have become insane due to business failure. Edward Benjamin Shawe failed as a school teacher and became dependent on his mother, which drove him into madness.[27] John Newman was driven to insanity from disappointment and anxiety after the Lunacy Commissioners withdrew his licence to keep a private madhouse.[28] Edward Fuller's case exemplifies how precarious the job of a clerk was, and what it was like to live under the fear of losing one's position. Fuller was a copying clerk with 'constant employment and good earning', but he started to worry when his firm appointed another clerk and his own income diminished. This 'preyed upon his mind and irritated him

greatly, [and he] expressed fears that he should lose his situation altogether and be in distress'.[29]

Even with secure employment, there were numerous other sources of economic anxiety.[30] Among them, household expenditure was a major concern for a labouring man. An intemperate and extravagant wife seems to have been a common source of male anxiety at that time, although we are much more familiar with the opposite pattern. William Garnham was greatly troubled by 'his wife's extravagance and fondness for dress', which contributed to his downfall and insanity.[31] 'Grief at difficulties arising from the extravagance and intemperance of his wife' was the reported cause of John Langford's madness.[32] Likewise, Richard Thomas Jones lost his mind with worry over his household members' alleged lack of financial discipline: his wife was addicted to drink and his cat ate too much.[33] His sanity was corroded by the fear that his female and feline companions would eventually bring him to economic ruin.

The most unavoidable risk of all was old age. Naturally enough, old age and the attendant reduction or loss of income loomed large in the mental landscape of working-class men. William Frederick Kilsby had had good employment as a compositor, but age and the cumulative effects of attacks of paralysis eventually rendered him incapable of work, which preyed on his mind.[34] John Mahon, too, was getting poorer due to his age; and his anxiety over his reduced circumstances was the reported root of his mental disease.[35] With 'his strength [beginning] to fail' in his mid-fifties, Samuel Hyde 'expressed fears that he should not be able to keep his situation [as a waiter], appeared over-anxious about his duties, got up 3 o'clock in the morning to clear knives, forks, etc.'.[36]

The narratives at the Hanwell thus tell the story of lives rife with fear and anxiety of ruin. Behind these stories, one can clearly sense the dark shadow of the New Poor Law and its stigmatization of poverty.[37] Economic ruin, destitution and the stigma of pauperism were now something to be avoided *at any cost*. The workhouse in particular, that dreaded stigmatizing space for the poor, was the centre and source of enormous anxiety for London's labourers. The image of the workhouse as English 'bastilles' firmly established itself immediately after the introduction of the New Poor Law. Carlyle's criticisms, Dickens' caricatures (as in *Oliver Twist*), the widely publicized and revolting starvation of inmates at Andover – all painted a picture of grim horror. Admittedly, these stereotypes do not comprehensively reflect the realities of the New Poor Law: research has revealed regional differences, oppositions at the local level and the survival of outdoor relief.[38] Nevertheless, cases from the Hanwell demonstrate that *fear* of the workhouse was genuine for the labouring poor in London, and even for relatively well-off working men.

Take the example of Richard Godwin, a 48-year-old house painter. He was greatly disturbed by the prolonged illness of his wife and 'abscess of the face' of his daughter. He feared his wife's death and the loss of his daughter's

job. His spiralling anxiety culminated in his morbid fear that 'he himself would be overwhelmed by pecuniary embarrassments and obliged to go with his family into the workhouse'. These fears made him 'pass sleepless nights and cry like a child'.[39] Cases in which the fear of the workhouse was a prominent part of the narrative abound. John Blake became 'dull, taciturn, [had] sleepless nights, dread of being destitute and of being in a workhouse'.[40] Another patient remarked that 'he would rather drown himself than go to the workhouse'.[41] The workhouse was often the centre of horrible delusions, too. George Wilson had long been tormented by ill-health and unemployment; finally, he called on his sister and asked her to hide him, stating that 'they wanted to send him to the workhouse to smother him between two beds'.[42] The workhouse was among the most frequently mentioned symbols of anxiety and fear of economic ruin which led to the mental diseases of those sent to the Hanwell Asylum.

Conclusion

Seen from inside, the picture of the madness of working-class men was much more nuanced than, and sometimes significantly different from, that conceived by middle-class social commentators and psychiatric practitioners. Narratives told at the Hanwell were, more often than not, sympathetic accounts of the economic hardships which had befallen the patient and which were shared by his family members. According to these narratives, most labouring male patients at Hanwell in the mid-nineteenth century were driven mad by their anxious concern for economic independence. Such interpretations differed markedly from the perceptions of distant middle-class observers, who were haunted by the cost of pauperism and conceptualized the psychiatric risk of labouring men more in terms of idleness, moral depravity and lack of will. Indeed, overwork and anxiety over poverty, two major causes attributed by the family, were mocked as vulgar errors by a leading alienist in the nineteenth century, who instead emphasized hereditary predisposition:

> Taking up the first hospital report within reach, and turning to the table of causes, I find in it hard work, fear of poverty, and jealousy, to mention no others. . . . But dropping, if possible, all former impressions, and looking upon the matter afresh, by the light of sure and unquestionable knowledge, we shall scarcely find any warrant, I think, for believing that the incidents here named can, in the nature of things, exert a morbific effect on the brain.[43]

Doctors and families thus clearly used very different interpretative models. The interpretative schism between the classes has so far prevented us from recognizing a widely held framework for understanding madness, which

traced the disease to the deep-rooted anxiety about poverty, loss of independence and the failure to live up to the ideal of respectability.

My sources suggest the historical psychiatric records, when used with caution, are useful for investigating the mind-set of the people in the past. We cannot know how scientifically correct were the family's attributions of madness to those socio-psychological causes. We can be reasonably sure, however, that the anxiety reportedly felt by the male patients was so intense that it induced the family to see a causal link with the disease. An analysis of the sources of anxiety thus helps to throw light on the structure of expectation and fear of working-class men in the mid-Victorian period.

I should like to argue that fear of poverty and destitution, and anxiety about unemployment or ruin, were closely related to a newly forged code of behaviour for labouring men. To begin with, one needed the capacity of *foresight* to be anxious about one's future economic state. Those living only for the pleasure of the moment would not worry about impending economic ruin. Foreseeing economic troubles was the ultimate basis of the new virtue of responsible prudence preached by both middle-class social commentators and working-class reformers such as Francis Place.[44] For the former, economic prudence in working-class men would secure a more orderly workforce, reduce pauperism and bring about more stable society. For the latter, it would empower the working class to achieve independence and self-respect. It should be noted that this new pattern of behaviour was driven by optimism for improvement on the one hand and dark fear of destitution on the other. My sources illustrate that aspirations for respectability made people deeply, and sometimes morbidly, anxious over their futures. The anxiety must have been intense, for economic prudence was still a virtue easier to praise than to practise without considerable psychological stress: the economy was still unstable; incomes for many were still too low to enable forward planning; the welfare state was still to come.[45] Many of the patients at the Hanwell Asylum, who were reported to have become insane due to economy-related anxiety, were thus paying the psychological price of the capacity of economic foresight. 'The rise of respectable society' took its psychological toll in the form of worry over one's economic future.[46]

Another historical factor that lay behind the anxiety-driven cases of madness is the new working-class notion of manhood. From the early nineteenth century, a notion gained ascendancy which dictated that the male head of the family should take economic responsibility of supporting his dependants on his shoulders alone. The rise of the concept of a 'breadwinner's wage' and the notion of domesticity in the early and mid-nineteenth century restructured the working-class family in terms of the responsibilities and expected behaviour of each member. Generally speaking, married women relegated themselves, often but not always willingly, to the subservient role of a housewife, although they continued to contribute to the family economy in various ways.[47] Men enjoyed the privileges due to

the master of the family, but at the same time were expected to bring a stable income to the family. The responsibility of keeping the family from ruin was now squarely on his shoulders, and many men felt the burden of this responsibility acutely. The breadwinner ideal thus brought both self-respect and vulnerability. The new manhood assured a working-class man the status of master of his home, which he was no longer able to find in his subservient status of employee. At the same time, he was under constant pressure to live up to the ideal, and the fear of failure to provide for his family, and the dread of stamping the stigma of pauper on himself and his family, tormented his psyche.[48]

The iconic mental disease of Victorian working-class men, as perceived by their families and peers, was rooted in and caused by their new quest for respectability, independence and manhood. That interpretation was tightly enmeshed with a new consciousness of working-class men. The fact that this widespread lay aetiology was not properly noticed, or even dismissed by psychiatrists, suggests a wide gap between the working and middle classes in the mid-nineteenth century, as well as between patient and caregiver in Victorian psychiatric provision. It also reinforces arguments that for proper understanding of history of medicine, the viewpoint of 'medical history from below' is indispensable, a lesson which will remain the most enduring legacy of Roy Porter.

10

'Arrows of Desire': British Sexual Utopians and the Politics of Health

Lesley A. Hall

Sexual utopia has been a recurrent, if often marginalized, tradition within British radical and socialist thought. Conceptualizing sex as a benign and creative force, such thinkers regarded its negative aspects (possessiveness, jealousy, exploitation, violence) as distortions produced by oppressive social institutions and economic inequality. In their ideal imagined state, sexual associations would no longer be contained within restrictive parameters enforced by repressive institutions, but would be a matter of free and equal choice, continuing only as long as desired by both parties. Such relationships might not even be confined to conventional monogamy, although the underlying utopian vision was quite antithetical to promiscuous libertinism, itself identified as yet another manifestation of inegalitarianism.

This chapter considers in particular Edward Carpenter, Havelock Ellis and their younger contemporary Stella Browne, all active in the late nineteenth and early twentieth centuries: the three combined a commitment to this tradition of sexual utopia with passionate concern for bringing about a society healthier in all respects. They saw liberation from conventional constraints on sexuality as not merely desirable and beneficial to the individual, but an integral part of a vision of public health that extended far beyond better sewers, vaccination programmes and similar interventions.

They were part of a longer tradition, going back at least to the ideals of sexual equality and free liaisons expressed by thinkers such as William Godwin and Mary Wollstonecraft, and in the works of Romantic poets such as Shelley.[1] These ideals were often associated with an interest in issues of health and a critical attitude towards conventional medicine. M. L. Bush's account of the early nineteenth-century radical Richard Carlile illuminates a persisting milieu of free thought, free love and alternatives to accepted authority in matters of health and personal relations as well as the political sphere. Carlile argued that sexual activity was healthy and desirable for both sexes. But his arguments remained largely based on concerns over individual well-being rather than public health.[2]

Similar ideas featured in the British utopian socialist movement associated particularly with Robert Owen, and influenced by related developments on the Continent, especially the French reformers Henri Saint-Simon and Charles Fourier. This movement had some sympathy for neo-Malthusian arguments for 'preventive checks' (contraceptives). Arguments that smaller families would be beneficial to women's health and improve their ability to mother their children were advanced alongside the economic case made for the benefits to the family as a unit. This discourse was largely one of self-improvement and individual well-being, rather than public health. However, George Drysdale also argued that by enabling earlier marriage, contraception would obviate the need for prostitution and therefore diminish the epidemic levels of venereal disease. This was probably the beginning of the convergence between radical sexual reform and the wider concerns about public health which became a major feature of British public life from the 1840s.[3]

Issues of 'free love' continued to be debated in progressive and radical circles. The focus was on an essentially private, monogamous heterosexual relationship with which church and state had no right to interfere, alongside the critique of the propertarian nature of marriage and the legal disabilities of women within it. Karen Hunt has described such debates occurring even within the Marxist left-wing Social Democratic Federation, as well as less theoretically rigorous lifestyle-oriented bodies such as the Fellowship of the New Life. Some SDF couples did choose free unions (including Eleanor Marx, although this was largely due to her partner Edward Aveling's inability to obtain a divorce). This practice was not without risk though: Edith Lanchester was incarcerated in a lunatic asylum by her family for transgressing gender and class norms by announcing her intention to live with a male comrade of lower social status.[4]

A rather different utopian theme of the *fin de siècle* was that of the feminist maternalist society, strong on motherhood but largely negative about sex. Probably the best known fictional example is the American feminist and social reformer Charlotte Perkins Gilman's *Herland*, though a native and much earlier British version is Elizabeth Corbett's 1889 novel *New Amazonia*. Matthew Beaumont has drawn attention to Corbett's strategy of 'simultaneously desexualising her ideal women and exalting their maternal status' within a future state, 'nurturing but rigidly rationalistic', known as 'The Mother'. The polemical writings of Mrs Frances Swiney, a significant feminist thinker influenced by theosophy, adumbrated similar arguments, eventually creating her own breakaway female-centred 'League of Isis'. Swiney claimed that 'Marriage is necessary to woman only as it affects the reproduction of the species – her organism is not dependent on it'. She believed that '[a]s the lights of knowledge regarding physical science become more diffused, women will undoubtedly take this matter of the rights of maternity exclusively into their own hands' with evolutionary and spiritual benefits.[5]

There was a ferment of ideas about, and even practical experiments with, new agendas for human relationships in the final decades of the

nineteenth century. Pre-existing ideas about reforming relationships were increasingly influenced by concerns about public health and the nature of the healthy society. These concerns were strongly, if not explicitly, influenced by the debates about the state and health which had been generated by the successful campaign against the Contagious Diseases Acts of the 1860s (suspended 1883, repealed 1886). The Acts had legislated forcible examination of women identified as prostitutes in designated garrison and port towns to protect the health of the army and navy; women demanding a single standard of sexual morality were prominent advocates of repeal. Less successful campaigns against compulsory smallpox vaccination and outcries against vivisection demonstrated similar concerns over the real meaning of social health and the limits of science, as well as about increasing government intervention into personal and family life.

The failures of the sanitarian project for the control and eradication of disease were picked out all the more clearly in the glare of its obvious successes. If Darwin's doctrines of the struggle for existence and the survival of the fittest had at first seemed to confer a scientific and evolutionary basis for the superiority of the industrialized West, they also held the threat that more robust and fitter organisms might eventually supersede the present fittest. Once at the pinnacle of evolution where was there to go, but down? Did the endemic persistence of tuberculosis, cancer and syphilis indicate that the current lords of creation were ultimately betrayed by what was false within? Or were these diseases the outward manifestations of an unjust capitalist patriarchal society, and therefore subject to eradication, or at least amelioration, through social reform?

Among the critics of this state of affairs, the writings of Edward Carpenter (1844–1929) were widely read within the late nineteenth- and early twentieth-century socialist movement. Initially educated for a career in the Church, Carpenter resigned his curacy and became a socialist, anti-imperialist, pacificist, humanitarian, vegetarian and advocate of the 'simple life'. He is probably best remembered for being a very early champion of the rights of homosexuals: he lived relatively openly with his male partner.[6]

Carpenter was deeply disenchanted with the culture around him. In 1884 he characterized England as

> Rotten at heart, and penetrated with falsehood, her aristocracy wrapped in selfishness, luxury and self-satisfied philanthropy, her Government offices . . . effete, plethoric, gorged.

'The redemption of England from the curse which rests upon her' lay in a programme of being:

> helpful towards each other, forgetful of differences, scrupulously honest in yourselves and charitable even to your enemies, but determined that

> *nothing* shall move you from . . . the righteous distribution in society of the fruits of your own and other men's labour, the return to Honesty as the sole possible basis of national life.

This was not just a political programme; it was an ethical and social agenda.[7] It can also be viewed as a quest for an alternative version of Englishness and of masculinity opposed to both effete, bloated decadence and militarist macho-imperialism. It was not just about a more inclusive politics or greater economic justice, but about creating a healthy society: transforming personal and social relations was an integral part of this project.

Carpenter's ideas about the health of society were most explicitly expressed in his *Civilisation: its Cause and Cure*, first published in 1889. He argued that civilization itself was a kind of disease and that the individual microcosm and the societal macrocosm mirrored one another:

> For as in the body disease arises from the loss of physical unity which constitutes health, and so takes the form of warfare or discord between the various parts, or of the abnormal development of individual organs, or the consumption of the system by predatory germs and growths, so in our modern life we find the unity gone which constitutes true society, and in its place warfare of classes and individuals, abnormal development of some to the detriment of others, and consumption of the organism by masses of social parasites.

He identified health (both personal and social) as unity and wholeness – a positive ideal – whereas disease represented a pathological break-up into multiplicity.[8] This was a rather different take on the standard discourse of the 'diseases of civilization', which saw certain ailments as generated by unhealthy urban life-styles, but usually framed this within notions of degeneration and social decay.[9] For Carpenter, however, the decline was not inevitable.

He set forth a vision of the achievable ideal society as 'a return to nature and community of human life'. He argued for a radical simplification of lifestyle, emphasized open air, rational dress and a healthy diet based on 'fruits, nuts, tubers, grains, eggs, etc. . . . and milk in its various forms'. And in these utopian communities he envisaged 'mutual help and combination will . . . have become spontaneous and instinctive'.[10]

In *Civilisation*, Carpenter was relatively silent about sexual arrangements; it is, however, suggestive that his vision of the new domestic architecture was 'various for the various wants of men, whether for a single individual or a family, or for groups of individuals or families'. He also suggested that bodily and spiritual love would not be antagonistic but absorbed into one another, as 'passions take their place perfectly naturally', and that what were considered vices under contemporary conditions would no longer be

'blemish[es] and defect[s]' when the 'proper command' of 'a clean soul in a clean body' was regained.[11]

He was more explicit in *Love's Coming of Age* (first published 1896). The unions which would replace marriage in his ideal future state of society would not be 'stagnant double selfishness', but more open attachments permitting 'wide excursions of the pair from each other' pursuing 'objects of work and interest' – even the reaching out to bestow 'some boon of affection to those who need it more'.[12] He argued that there was 'an immense diversity . . . in matters relating to sex and love'. There were 'hundreds of thousands' of different kinds of men and women and therefore many possible kinds of union:

> there are no limits of grace or comeliness, or of character or accomplishment, within which love is obliged to move; and . . . there is no defect, of body or mind, which is of necessity a bar – which may not even . . . become an object of attraction.[13]

Carpenter took the contemporary pathological discourse found in Krafft-Ebing's *Psychopathia Sexualis*, in which deviations of desire and fetishistic cravings were depicted in terms of degeneracy and perversion, and reinterpreted these as benign variations from the norm.[14] To Carpenter, variety and variation were natural, and ultimately healthful, rather than sick excrescences on the body politic.

Carpenter's friend and colleague Havelock Ellis (1859–1939) took a somewhat different line on the improvement of the health of society. As with Carpenter, he was involved with many of the social movements of the day, although his intense shyness and reclusive temperament meant that much of his connection with them was by way of friendship, in person or by correspondence, with more activist individuals. He was himself a medical man, though he devoted himself to investigating sex and did not practise medicine.[15] Ellis is best remembered for his massive, seven-volume *Studies in the Psychology of Sex* (1897–1928), and associated contributions to sexual enlightenment, but was also concerned with broader issues concerning human well-being. *The Nationalisation of Health* (published in 1892) embodied his views on how society could be rendered healthier for the majority of its citizens. Like Carpenter, he considered that 'a great civilisation is ill built up on the bodies of men and women enfeebled and distorted by over-work, filth and disease'. However, unlike Carpenter, he did not identify civilization *per se* as the problem; indeed, his solutions were positively pragmatic: 'a rational and organized system of securing the conditions of health as against the private, individual, sporadic lack of system'. For Ellis, '[t]he key-word of our modern methods is not *cure* but *prevention*' and medical science was pointing the way towards 'a gigantic system of healthy living'. Like Carpenter, he had a glowing holistic vision of health: 'Fresh air and reasonable garments,

cleanliness in the full sense of the word, pleasant work and varied exercise, the healthful play of the secretions and the excretions.'[16]

Throughout his lengthy career and numerous published works, Ellis emphasized education and encouragement rather than state intervention and, like Carpenter, idealized diversity rather than regimentation. This was particularly noticeable in his writings on eugenics and allied matters, which contrast sharply with the writings of other contemporaries such as H. G. Wells. In *A Modern Utopia* (1905), for example, Wells foregrounded reproduction and suggested that in his ideal state, women who failed to produce healthy intelligent children should lose status.[17] In the short pieces collected in *Questions of Today* (1936), Ellis took a contrary position, commenting that '[t]o be ill born is not necessarily Fate's last word . . . . It is to the ill-born that we owe many of the greatest achievements the world can show'. He added that it was important to 'make the right conditions for the life once created.' This commitment to human variety was embodied in his suggestion that 'queernesses and defects' that diverged from the apparent norm (sexual diversity is surely a subtext here) should 'be cherished'.[18] The inspiring utopianism of his vision was recognized by his contemporary Stella Browne, who commented in 1922 in a review of his *Little Essays of Love and Virtue* that 'direct action of the most thoroughgoing description is necessary, before the lessons he teaches can be assimilated and his ideals "made flesh"'.[19]

Stella Browne (1880–1955) was a generation younger than these two figures. Born in Halifax, Nova Scotia, she was educated in Germany and then at St Felix School, Southwold before attending Somerville College, Oxford, achieving second-class honours in Modern History in 1902. She was for some years a militant suffragette, was a life-long socialist (though she left the British Communist Party, of which she was a very early member, over the issue of birth control) and a campaigning pacifist. She was also profoundly committed to the rights of the individual. She is probably best known as a vigorous campaigner for birth control and abortion law reform. Though she 'never held that family limitation would *alone* abolish poverty', in her view the advent of the socialist millennium would not render redundant the question of women's reproductive choice.[20] A definite philosophical basis underlay her campaigns and activism.

On questions of public health, she demonstrated clear affinities with the ideas of Carpenter and Ellis, while contributing a specifically gendered perspective. Browne argued from a left-wing critique of orthodox medicine influenced by alternative theories on health. She did not hold the medical science of the day in awe, pointing out that '[o]fficial medical opinion has changed its mind very often in the past', for example, 'about anaesthetics in childbirth, about asepsis, about psychotherapeutics, about osteopathy'. She protested against 'medical monopoly under pretext of "safeguards", hygienic or "moral"'. Orthodox medical thinking, Browne argued, conflicted with the

ideas held by many about, for example, 'a diet including flesh meat and the moderate use of alcohol' and on 'the fundamental questions of vaccination and vivisection'. She voiced concern over the generally unhealthy conditions in which so many were doomed to live, and had some particularly trenchant things to say about housing. She saw the problem as not simply one of poverty, but of the wider neglect of women's domestic needs. 'Not even [in] the most finely-equipped and organised household' had she ever found 'a convenient and well-planned kitchen sink'.[21]

Like Carpenter and Ellis, Browne did not believe that sex could be reformed in isolation from the other ills of society. Rather, she consistently depicted the factors that made for ill-health and suffering as innate by-products of the way in which society was organized on all levels, from the planning of kitchens to the highest emotional and spiritual concerns. She rejected 'a social order which puts necessary work, justice, creative art and science, love and breeding on a cash basis', and demanded 'revolutionary changes in all departments . . . the development of hitherto isolated human harmonies, or intense and vivid variations of faculty and type'. For Browne, as for Carpenter and Ellis, variety and diversity were the symbols of the truly healthy society, and health would spring from a holistic synthesis of many elements.[22]

If in *Civilisation* Carpenter was a little cagey about the domestic establishments in his imagined communities of the future, in *Love's Coming of Age* he mooted the possibility of relationships not based on the traditional couple. Ellis recognized that though not everybody was fitted for marriage or parenthood, they had a part to play in human society. Browne's utopia included women with lovers but not babies, women with babies but no husbands, and even foresaw lesbian mothers. 'Two of the most intensely maternal women' she knew were 'congenital inverts': she suggested that 'in the future, the resources of developed chemistry and biology' would enable 'artificial fertilisation' in such cases.[23]

While the arguments advanced by these thinkers invoked common terms of the day, such as 'breeding', 'eugenics' and 'race', they took a very different line from the mainstream eugenics movements and the less coherent but still pervasive contemporary views about 'good' and 'bad' breeding. It is obvious that they did not measure social value by reproductive competence and personal biological contributions to 'the race' (and were all, themselves, childless). Carpenter, especially in *The Intermediate Sex*, made a cogent case for the benefits that 'uranians' might convey to society as a whole: 'A vast number of intermediates do actually perform most valuable social work . . . partly on account and by reason of their special temperament.' Beyond their roles in art and literature, as educators and social workers, he suggested that

> Uranian people may be destined to form the advance guard of that great movement which will one day transform the common life by substituting the bond of personal affection and compassion for the monetary, legal and other external ties which now control and confine society.[24]

Throughout her long career, Browne made strident attacks on orthodox visions of eugenics, which she considered patriarchal and class-bound, and not in line with the developing understanding of genetics. She vigorously challenged the Eugenics Education Society as early at 1912 in a letter published in *The Freewoman*, and in 1914 she invoked the recently rediscovered work of Mendel on the 'highly complex nature of . . . hereditary traits' against ' "Neo-Darwinian" eugenists'. In her 1935 plea for legal abortion she argued, against the orthodox eugenic case which would have allowed abortion on grounds of poor heredity, that 'Heredity is a much more intricate problem than pre-Mendelian Darwinism supposed', meanwhile claiming that women's free choice of motherhood would release 'possibilities of genetic variety and superiority'.[25]

This utopian tradition of diversity and variation contrasts with a couple of contemporary alternative visions of health in society. In 1922, the communist doctor and sex-reformer Eden Paul, who with his wife Cedar translated many works of continental sexology into English, made a disturbing case for a left-wing eugenics aimed at producing, presumably, heroic proletarians as depicted on Soviet posters of the period. Arguing that 'the "artificial" or *economic* man is largely removed from the selective influence of nature' leading to the proliferation of 'persons with grave eye defects, short-sight, astigmatism, etc.', he conceded that 'there is a large measure of truth in the eugenist argument'.[26] The Pauls were very much part of the same circles as Ellis and Browne. Yet their response to society's ills – their attribution of those ills to the capitalist system notwithstanding – contrasts strongly with Ellis's advocacy of the cherishing of 'queernesses and defects', or Browne's consistent scepticism about the validity of eugenic projects for the improvement of society.

In support of her argument that abortion might be an act of responsible motherhood, Browne cited the work of two figures well known for their vision of an organic community, Dr George Scott Williamson and Dr Innes Pearse, the founders of the Pioneer Health Centre, Peckham (which flourished, with some hiatuses, from 1926 to 1952). Williamson and Pearse were endeavouring to discover the factors promoting health (rather than simply preventing disease) through periodic health investigations and the creation of a local social centre. However, the Peckham vision had no place for those who were not at some stage of what its founders believed to be a biologically defined familial cycle. There was no place for single people or the voluntarily child-free couple. Browne, though largely approving of the work they were doing, commented that 'their scale of values seems rather obsessed with Parenthood!'[27]

Peckham, situated in an inner-city suburb rather than the countryside, might be considered an unusual example of the utopian-influenced experiments in living which emerged during the late nineteenth and the first part of the twentieth centuries, and ranged from the garden city movement to smaller agrarian communes. But studies of these communities

suggest that such experiments were very diverse, some being founded on strong if unorthodox religious principles, and others rejecting marriage in favour of free union and scandalizing locals with naturist practices. Thus the religiously based Bruderhof community saw motherhood as woman's most important function, whereas the Whiteway anarchists preached and practised birth control. However, the central relationship model in all cases appears to have been monogamous heterosexuality, with gender roles remaining fairly static – the men worked the fields and the women did the cooking. The Elmsett pacifist community during the Second World War did include at least one male couple (although both partners eventually fell in love with and married women when the gender balance of the community became more equal).

The interest in sexual relations evinced by these communities reflects broader commitments to the creation of a health-giving reformed society. Health concerns, broadly construed, were often integral to these projects. For example, in the garden city movement there was a strong commitment to ensuring housing was well ventilated, had a sunny aspect, was designed to reduce both domestic drudgery and places for dirt to accumulate and included adequate bathrooms. There was a pervasive emphasis on a healthy, often vegetarian, diet, and provision of facilities for open-air work and recreation. The idea of prevention of ill-health through right living was usually implicit, and sometimes formalized, as through proposed garden city schemes for preventive health services. The Bruderhof community in Ashton Keynes included its own small hospital, though it is not clear whether this was run on orthodox medical principles – they did have a trained veterinary surgeon to care for the farm animals. The views of these very diverse communities on orthodox medicine and their relations with officials of the state public health system remain somewhat occluded, given that much research remains to be done in this area. Difficulties with officialdom were experienced by several communities over failure to obtain planning permission for community building, and refusals to register births or send children to school. There may well also have been clashes over public health issues such as vaccination or isolation of cases of infectious disease.[28]

How influential within British left-wing and progressive thinking more generally was this strand advocating a holistic vision of health combined with sexual liberation? It was sometimes ignored, or even ferociously attacked, as in George Orwell's diatribe against 'every fruit-juice drinker, nudist, sandal-wearer, sex-maniac, Quaker, Nature-cure quack, pacifist and feminist in England'.[29] Sex was largely ignored in the provisions of the welfare state instituted by the Labour government elected in 1945. While there were some gestures towards the inclusion of sex education within the educational curriculum, actual implementation was extremely haphazard. Birth control was virtually omitted from the National Health Service; it was treated as a personal indulgence, except for cases of dire medical need, until

1967, and only fully incorporated in 1974. Even treatment for venereal diseases remained largely on the basis established in 1917, free and confidential but in clinics hidden in hospital basements or annexes. Marriage guidance, like birth control, was left in the hands of voluntary bodies, though approved organizations received government grants.[30]

However, the impact of this utopian discourse had a profound impact on the individual and local level. Chris Nottingham in *The Pursuit of Serenity* makes a strong case for Ellis as a neglected influence on at least one generation if not more. Carpenter's works were widely read. Browne lectured to many meetings of Women's Cooperative Guilds and women's sections of the Labour Party, as well as being active in the Fabian Society and in more specifically sex-reforming bodies such as the Malthusian League and the British Society for the Study of Sex Psychology.[31]

The extent to which this discourse offered inspiration to those toiling for sexual enlightenment in specific practical ways is extremely difficult to assess. A case can be made for its underlying significance in the reforms of the Labour government of the 1960s concerning birth control and abortion, homosexuality, decensorship and divorce. One obvious connection, though there were doubtless many more subtle links, was through Gerald Gardiner, Lord Chancellor in the Wilson administration, active in the British Society for the Study of Sex Psychology as a young man.[32]

In the Jerusalem these thinkers wanted to build in England's green and pleasant land, health had a wider meaning than merely the absence of disease and fitness to reproduce. A concern for values that can only be described as spiritual was their major measure of the health of society. Ellis spoke for all in articulating his hope for and vision of 'a juster social organization, and . . . the realization of ideas which until now have seemed but the imagination of idle dreamers'.[33]

11

'Twenty Centuries of Christianity Weigh Heavily on Women's Brains . . .': Anarchism, Science and Women's Nature in Spain at the Turn of the Twentieth Century

Katharina Rowold

> Your former and current condition as men's slaves was and is due to the fact that all religions and systems of government were formed by and made up of men who were ignorant of the scientific truth and firmly believed that women were essentially and organically inferior to men . . . [1]

This pronouncement, made in 1903 by José Prat to an audience of women, was indicative of the anarchist engagement with the question of women's emancipation in Spain at the turn of the twentieth century. The anarchist, or libertarian, movement with its strongholds in rural Andalusia and industrial Barcelona was exceptionally successful in Spain, eventually growing into one of the largest left-wing movements in Europe in the early twentieth century.[2] It was a movement that showed a significant interest in the question of women's emancipation. There was no organized women's movement in Spain until the foundation of the National Association of Spanish Women in 1918. None the less, 'the Woman Question' (*la cuestión femenina*) was widely discussed and highly controversial in the late nineteenth and early twentieth centuries. Debate on the subject was fostered under the auspices of other movements and encouraged by individual women and men.[3] The Woman Question became an ideological battlefield where traditionalist Catholicism, liberal secularism and anarchism confronted each other.

The fusion of anarchism and feminism was itself not without problems. Historians have noted that of the principal anarchist political theorists, William Godwin, Pierre-Joseph Proudhon, Mikhail Bakunin and Peter Kropotkin, only Proudhon addressed the question of women at any length, and he is well known for proclaiming women's physical, intellectual and moral inferiority.[4] Scholars of anarchism in Spain have highlighted that

although the movement there acknowledged the importance of women's emancipation to the anarchist project, few members gave priority to these concerns. Eventually the feeling that not enough attention was paid to women's emancipation, that women in the movement were not treated as equals, and that they needed to be empowered to claim their places in the movement led to the formation of the anarchist women's organization *Mujeres Libres* in 1936.[5] But despite the complexity of the relationship between anarchism and feminism, there was ample interest in and support for women's emancipation to make it a frequent topic of discussion in Spanish anarchist publications at the turn of the twentieth century.

In considerations of women's emancipation, anarchists reflected on the nature of the differences between the sexes.[6] Historical scholarship has neglected a critical dimension of turn-of-the-century Spanish anarchist approaches to women's emancipation: the impact contemporary medical and scientific theories on sexual difference had on their constructions of female nature. Here, I will explore this impact. Science played an important role in anarchist thought, and when considering women's social position, anarchists privileged notions of the natural as defined by science.[7] There exists an extensive historiography that has illustrated the anti-feminist implications of late nineteenth- and early twentieth-century scientific and medical theories on women's nature.[8] Anarchists, however, considered science to have emancipatory qualities. Although it was not uncommon for turn-of-the-century European feminists to appropriate and reconstitute aspects of medico-scientific theories on female nature to support their demands for women's rights,[9] there were few who matched the conviction of Spanish anarcho-feminists that science essentially demanded women's emancipation. Indeed, science constituted anarchists' most important tool in the ideological struggle of redefining women's nature which involved a rejection of religious doctrine on womanhood. This chapter discusses the role of science in anarchist approaches to women's emancipation and explores the ways in which anarchists' understanding of femininity came out of an engagement with European medical and scientific theories on the female mind and body.

Anarchism, science and the 'Woman Question'

Roman Catholic ideals of womanhood were exceptionally influential in the shaping of women's social roles in Spain. In the last decade of the nineteenth century, however, science came to play an increasingly important role in negotiations of gender roles, similar to developments that had already taken place in some other European countries. The scientization of the Woman Question became so persuasive that by the beginning of the twentieth century there were even some Catholic writers who bolstered their views on womanhood with scientific constructions of femininity.[10]

This was possible because, as Frances Lannon has observed, Catholic and scientific ideas on sexual difference could neatly overlap.[11] The Church became increasingly interested in the Woman Question at the turn of the century. The majority of practising Catholics were women, and the Church sought to secure their continued loyalty and active support, as well as promoting its understanding of womanhood,[12] in which devout women accepted paternal power and recognized marriage and motherhood as their God-given mission in life.[13] Contemporary scientific discourse on female nature did not necessarily contradict religious ideals of womanhood. Rather, religious and scientific theories on gender could reinforce, as well as challenge each other.

Anarchists, however, were uncompromising in their rejection of religious doctrine. According to them, the Catholic Church and science had been locked in a struggle for centuries: while the Church sought to contain progress and the liberation of humankind, science promised just this.[14] Indeed, to anarchists science was a key element in the struggle against exploitation, be it economic, political or cultural.[15] It followed that to anarcho-feminists religious and scientific notions of female nature clearly challenged each other: whereas the Catholic Church was guilty of nurturing inegalitarian power relations between the sexes, science promoted women's emancipation. Although there were anarchists, often drawing on the writings of Proudhon, who believed that the 'work of a woman is to be a mother' and that emancipation required the reconsideration of women's role in the home,[16] it was more common for turn-of-the-century anarchists to claim that women were the equals of men, and ought to have the same rights and duties.[17] Vital to this approach was the assertion that science proclaimed the equality of the sexes.[18]

There was a widespread belief in late nineteenth- and early twentieth-century Spain, held not only by traditionalist Catholics, but also many liberals, that the advent of Christianity had liberated women from the status of servitude. Anarchists thought differently. According to them, 'Christianity is the worst enemy that women have had'.[19] The fraught relationship between women and the Church had grave consequences: 'twenty centuries of Christianity', declared Julio Camba (1882–1962), a well-known anarchist of the turn of the century, 'weigh heavily on women's brains'.[20] Anarchists believed that for revolution to succeed, it was necessary first to win the minds of people, change their attitudes and empower them.[21] They thus sought to subvert Catholic and 'bourgeois' notions of women's inferiority.[22] Science offered an alternative. Teresa Mañé ('Soledad Gustavo', 1865–1938 [or 1939]), the daughter of a republican politician, who had turned to anarchism in her youth, thus emphasized in 1890 that the 'progressive' way to consider the question of women's social position was by being a 'student of the natural', as opposed to a 'student of the sacred'.[23] Some years later, she told her readers that it was the anatomist's scalpel that had provided

proof that 'we have a brain . . . in contrast to the thesis put forward by many saints and the Fathers of the Church'.[24]

This commitment to science, however, did not render libertarians oblivious to the fact that the relationship between contemporary scientific knowledge and women's emancipation was not straightforward. Rather, they took a nuanced view. Camba, for instance, was well aware that there were contemporary scientists who claimed women to be naturally inferior to men. But Camba, an advocate of evolutionism, believed that they did not do justice to the female sex, as they failed to inquire into the factors that had fostered inferiority.[25] More frequently, libertarians approached the issue somewhat differently. In 1904, the influential anarchist journal *Natura* published a translated article by the French anthropologist Léonce Manouvrier, in which he contended that 'there exist prejudices of sex, race and caste or social class, of profession, which unconsciously form part of the mentality, even in men of science', and that there was no doubt that the 'prejudice of sex' had entered publications which, in the guise of scientific theories, proclaimed the intellectual inferiority of women.[26] This was a common understanding in anarchist circles. It did not, however, threaten the status of science as a means to truth; rather, it was seen to mean that prejudice led some authors to mistreat anatomical and physiological facts.[27] In this vein, Anselmo Lorenzo (1841–1914), a printer who was one of the leading anarchists in Barcelona, was pleased to report on the autopsy of the German professor of anatomy and physiology Theodor von Bischoff. Well known for claiming that women's lesser absolute brain weight indicated mental inferiority,[28] his autopsy, according to Lorenzo, revealed that von Bischoff's own brain weighed 'five grams less than that of any illiterate woman'. Absolute brain weight was thus of no consequence. Von Bischoff had neglected to consider what had long been known: what really mattered was relative brain weight. Unprejudiced scientific studies of the brain, Lorenzo thus asserted, 'did not prove a fundamental inequality in mental ability between the sexes'.[29]

Anarchism and female nature

The interest in science meant that in anarchists' discussions of women's emancipation there was an awareness of and engagement with theories on sexual difference as articulated by European anatomists, physiologists, anthropologists and evolutionists. The views on female nature that underlay these discussions were informed by anarchists' understanding of women's subordination. Anarchists saw women's subordination as an economic phenomenon, and it became an articulated aim in 1872 to incorporate women into the paid workforce on equal terms with men.[30] Many in the movement saw women's subordination also as mediated through institutions such as the Catholic Church, the state and the family.[31] But the

'enslavement' of women by men, according to anarchists (and in contrast to Friedrich Engels' theory)[32] pre-dated the emergence of private ownership of property and the monogamous family. Women, 'regardless of the class [they] belong to', had lived in varying degrees of social inferiority to men throughout human history.[33] Drawing on conceptions of the gendered body that were commonplace in late nineteenth- and early twentieth-century medico-scientific discourse in Europe, Spanish anarchists saw women's initial subordination as originating in the biology of the female body and the superior muscular strength of the male. The female body was weak in terms of muscular force, and it was debilitated by its physiological functions.[34] Indeed, this view could on occasion lead to the endorsement of the notion that women were 'invalids' during 'certain periods of their sexual lives'.[35]

The subjection of women when 'physical strength was a sign of power' had developed in men the instinct of domination and feeling of superiority. Teresa Claramunt (1862–1931), a textile worker, who like Teresa Mañé was among the most important and prolific anarchist writers of the time, pointed out that this meant that women's subjection, which originated with the sexed body, was the foundation of all other forms of inequality.[36] Neo-Malthusian anarchists, at this point a minority group based in Catalonia, interpreted this as meaning that women's true emancipation would only be possible through the use of contraception.[37] More generally, anarchists emphasized the need for women to take responsibility for their own emancipation and assert themselves. Particularly, anarchist women also argued that men, who as priests, capitalists, legislators and husbands were responsible for women's subordination, had to change to make possible the emancipation of the female sex.[38]

There was thus a strong belief that women so far had lived in unfavourable social conditions. Anarchists thought that this state of tyranny, which would be but a 'distasteful memory' in the new society,[39] shaped gender difference, as will be discussed below. None the less, in anarchist representations of female and male nature, notions of underlying natural differences were present. Teresa Claramunt, for example, maintained that women and men possessed different aptitudes, equally necessary for the march of progress.[40] Furthermore, turn-of-the-century anarchists were careful to distance themselves from one of the most common charges levelled at the European women's movements: that they sought to masculinize women.[41] Teresa Mañé made it clear that feminism was not, nor should it be, about masculinizing women. It was about women freeing themselves from 'the tutelage of the stronger sex, but not about invading its dominions'.[42] Reassurances in this direction were not only indicative of an adherence to notions of natural differences, but also point to a rejection of lesbianism increasingly associated with gender inversion by sexologists.[43]

Many anarchists explicitly and vehemently refuted the view that women were by nature mentally inferior to men, however.[44] 'Nature', José Prat

confidently proclaimed, 'has nothing to do with it'.[45] But the assertion of equality between the sexes did not necessarily imply a concept of gender similarity. It was often part of an 'equal but different' approach that depended on notions of sexual complementarity. In Spain, traces of the ancient Greek model of hierarchical sexual difference survived into the later part of the nineteenth century, as Galenic and Hippocratic theory was tenaciously defended in the fight against materialism.[46] By the turn of the century, however, the concept of sexual complementarity, which was prevalent in European medico-scientific discourse on gender difference, had become widespread in Spain. Liberal 'krausists', who aspired to the cultural, social, economic and political regeneration of Spain, played an important role in disseminating these theories in the second half of the nineteenth century. For many anarchists, it was a scientific model of sexual difference with which to contest notions of female inferiority and male superiority they associated with Catholic doctrine. Had not, Anselmo Lorenzo for instance pointed out, the German evolutionist Ernst Haeckel noted that men and women constituted different organisms that complemented each other?[47] The idea of sexual complementarity also informed the thinking of Francisco Ferrer Guardia (1848–1909), who in 1901 opened the 'Modern School' in Barcelona, the best known rationalist school based on libertarian educational principles, which was co-educational on principle, in contrast to normal practice in Spain.[48] The provision of co-education, important to the anarchist educational project, was thus not necessarily based on views of gender similarity. In a theory reminiscent of that espoused by the British evolutionists Patrick Geddes and J. Arthur Thomson in *The Evolution of Sex* (1889), Ferrer Guardia claimed that in men 'thought and a progressive spirit' dominated by law of nature; in women, it was 'intense emotion and the conservative element'. But Ferrer Guardia did not think that women's role should be confined to the home and was careful to note that these differences did not lend support to 'the ideas of reactionaries': it did not mean that women were unable to think about serious matters or that their minds could only embrace 'superstitions'. On the contrary, it made it all the more important to provide the female sex with a rational, scientific education. This way, women would come to play an important role in the 'conservation' of progressive ideas.[49] Anarchist women writers embraced similar ideas on sexual complementarity.[50] Teresa Mañé, for instance, declared that women were more emotional, empathetic and selfless, but less likely than men to produce 'great ideas' and 'sublime thoughts . . . of progress'. However, by virtue of their moral sway, women 'enhance and even give permanence to these same thoughts, these same ideas'.[51]

Many anarchists thus adhered to notions natural sexual complementarity. Yet there was also a firm belief that the female mind was adversely affected by the education women received and social circumstances they lived in.[52] According to José Prat, the outcome of this was such that women were hardly

'in a position of being able to understand us'.[53] It was the environment women lived in that was responsible for this sorry state of affairs, though. Had not, anarchists liked to point out, María Dolores Aleu, one of the first qualified female physicians in Spain, highlighted in her doctoral thesis that there was 'no difference between the male and female organisation in the earliest period of intra-uterine life'? And had she not noted that differences in aptitudes became apparent when girls and boys started to undergo distinctive education?[54] The final verdict on the nature of the differences between men and women could thus only be given once women received the same education as men and enjoyed the same freedoms.[55] 'I believe', Teresa Claramunt declared, 'that it is not possible to assert our inferiority as long as women are kept in a restricted milieu and given as sole *education* a collection of nonsense, sophisms, and superstitions that shrivel our intelligence, rather than awakening it.'[56] Natural differences between the sexes were distorted by existing authority relations. The new society, where women would be able to 'freely satisfy all [their] wishes and needs as beings who feel and think',[57] would provide the context in which women's true nature could unfold.

Some anarchist writers suggested that the adjustment of differences between the female and male mind would not and could not happen within a single lifetime, however. Instead, they thought that the female mind had been moulded by centuries of subordination. The impact of evolution on the differences between the male and female mind was a hotly debated topic in Europe.[58] Evolutionary theory had a great impact on anarchism (not least because it provided a way to explain the origins and history of humankind without reference to God),[59] and there were anarchists who agreed with the increasingly commonplace turn-of-the-century assumption that European men were more advanced than women in evolutionary terms. Yet, in the anarchist view, this was not inevitable. This interpretation was informed by the Lamarckian concept of the inheritance of acquired traits.[60] Camba, for example, declared in 1903 that it was clear that woman's organism could not follow the 'evolutionary current that led us to our present degree of perfection'. However, he continued by asserting that women were victims of the restricted environment that men had imposed on them. Throughout history, their brains and muscles had hardly been exercised. For men, however, this had been different and thus they 'underwent great evolution and [their] brain developed progressively'. If women, according to Camba, had been able to 'use all their organs they would undoubtedly nowadays be at the same heights as us'.[61] Women were less evolved than men, but this was a Lamarckian consequence of the environment they had lived in.

The idea of the evolutionary backwardness of women became intertwined with a preoccupation with degeneration, so prevalent at the turn of the century, which also spread in anarchist circles.[62] According to anarchists, the organization of capitalist society led to degeneration, as the 'the mutual

exploitation of individuals' put too much strain on physical and mental forces. Degeneration theory for anarchists thus meant a 'terrible condemnation' of contemporary society.[63] It also supposed a specific condemnation of women's social position. In turn-of-the-century degeneration discourse in Europe, much was made of the idea that women's emancipation would lead to socio-biological decline. Spanish anarchists saw things differently: it was women's subordination, not their emancipation, that was the problem. 'Our moral and material misery', so it was suggested by Camba, might be the result of women's condition. 'Being of weak body and spirit, the men she engenders come to the world with all her weaknesses.'[64] More specifically, the idea of degeneration informed anarchists' ideas on the reorganization of sexual and family life. Loveless marriages of convenience, it was argued, were 'unnatural' and could not provide 'robust' offspring. How could the fruit of such unions possibly be 'beautiful and healthy'?[65] 'Free love' (i.e. freely established relationships confirmed by mutual agreement, rather than sanctioned by the Church or state), on the other hand, would engender healthy and robust new generations. The mission of love, Teresa Mañé declared, 'is to perfect the race, physically and intellectually'.[66] Yet free love was only possible if women were neither economically nor legally dependent on men and could thus 'freely express their thoughts and their desires'.[67] Women's emancipation therefore became a necessity for the biological health of the 'race' and its progress.

Conclusion

Anarchist theory provides a rich field for the examination of the multifaceted interaction of science and feminism. Even if the question of women's emancipation was not a matter of priority for many members, it was an issue that was addressed by many well-known libertarian writers of the turn of the century. In the absence of an organized women's movement, anarchism was one of the movements that provided the most coherent discussion of women's subordination and emancipation in Spain in this period. This interest in the Woman Question led anarchists to partake in a debate held across much of Europe on the nature of the female mind and body and how it related to women's position in society. Anarcho-feminists were not unanimous in their views on the female mind and body, but they shared a profound faith in the central role of scientific knowledge in women's emancipation. It has been well documented that turn-of-the-century science often justified continuing inequalities between the sexes. But libertarians saw science as a tool to challenge existing gender relations. Science, as the example of turn-of-the-twentieth-century Spanish anarchism makes clear, could thus be as much part of the language of those who sought women's emancipation, as well as of those who deplored it.

Anarchists expanded the limits of the discourse on the rights and duties of women, displaying a theoretical commitment to women's equality that was rare in turn-of-the-century Spain. Seeking to change the attitudes of women and men, for anarchists rethinking gender relations, involved contesting Roman Catholic ideas on womanhood. None the less, their attempt to reconceptualize gender difference remained influenced by a cultural context in which gender roles were rigidly demarcated and their approaches to women's liberations became implicated in the turn-of-the-century scientization of the Woman Question. Anarchists turned to science to crush traditional prejudice, but their constructions of female nature absorbed many of the cultural assumptions about femininity and masculinity reflected in contemporary scientific theories.

12

'A Band of Lunatics down Camberwell Way': Percy Lane Oliver and Voluntary Blood Donation in Interwar Britain

Kim Pelis

> It is a matter of national pride that ours is the only country in the world, with the exception of Holland, Sweden and some of the Dominions, where voluntary bodies of donors are the rule and not the exception. In most countries the voluntary services of Great Britain are regarded as hopelessly Utopian and as characteristic a national phenomenon as the British Constitution.[1]

The Oliver Twist

In 1946, Britain launched its National Blood Transfusion Service (BTS), an early and famous component of its National Health Service.[2] The National BTS's reliance on unpaid, voluntary donors who made their blood a 'gift' to dying strangers contrasted strikingly – and often favourably – with the American tendency to *pay* 'donors' for blood. The sociologist Richard Titmuss seized on this contrast, turning it into an eloquent defence of altruism in his classic study, *The Gift Relationship* (1970). Here, social conscience played hero to self-interest's villain. The National BTS, Titmuss argued, could 'only be understood within the context of the Health Service'. The NHS, an exemplar of 'socialized medical care', was 'the most unsordid act of British social policy in the twentieth century', encouraging 'sentiments of altruism, reciprocity and social duty . . . [in] all social groups and classes'.[3] For Titmuss, the NHS extended the communitarian spirit and material responsibilities generated in wartime. The National BTS exemplified this spirit. Titmuss made no mention of blood donation's pre-NHS history, so freeing the reader to conclude that the system of voluntary donation arose *de novo* from this post-war movement. It did not. It was, instead, the product of interwar Britain, shaped by its much older tradition of medical charity.

Titmuss's analysis was permeated by the thinking of the economist John Maynard Keynes. During the interwar period, the Keynsian critique of laissez-faire capitalism became increasingly influential. Keynes believed that the 'Invisible Hand' needed visible guidance. His brother Geoffrey, a surgeon at St Bartholomew's Hospital, London, was at this same time the public face of the city's Red Cross BTS. Even before 1926, when this service was created, Geoffrey had offered his own critique of paid blood donation – one that owed no small debt to his brother's ideas.[4] Blood donors, he argued, should be unpaid volunteers, overseen neither by hospitals nor by the state, but by a voluntary organization, such as the Red Cross.[5] Carefully organized voluntarism would make visible – and usable – arms, freely bared for blood donation. Yet, despite his important role in the ultimate success of voluntary blood donation, Keynes did not establish the emerging service; nor did the hospitals or the state. What of the Red Cross itself? Here in fact lay its origins, but not in the places of power where one might expect to find them.

In 1923, a year before Keynes appealed for voluntarism, Sir Walter Morley Fletcher, secretary of the then young Medical Research Council, had suggested to Sir Arthur Stanley, chair of the British Red Cross, that Stanley's organization should concern itself with 'a piece of direct life-saving work ... the creation and support of an organization for the supply of human blood from suitable "donors" when blood transfusion is needed'.[6] Stanley replied: 'It will interest you to know that one of our Divisions in London is actually doing this work. I only heard of it by chance a few days ago.'[7] That division was in Camberwell, and its commitment to voluntary blood donation had been formulated and championed by its honorary secretary, Percy Lane Oliver. Oliver was a medical outsider who held a day job in the finance department of the local town hall. His grassroots organization not only served a useful medical function and came to be a national and international model; as we shall see, it eventually dictated transfusion technique to the autocratic medical consultants of London's prestigious voluntary hospitals. Its power arose less out of a 'grand vision' than from the combination of a strong commitment to voluntarism, a search for personal and professional identity in peacetime, and a series of nasty local political struggles, all in the context of changing medical and economic needs.

Blood shed in war and peace

The Great War determined the timing of blood transfusion's reintroduction to British medicine. Although Britain had pioneered the practice of transfusion during the nineteenth century, its doctors came, by the end of the century, to believe that saline infusion was an effective and efficient substitute.[8] Shortly thereafter, American surgeons became convinced that saline offered but a temporary solution to haemorrhage and shock, and led a return to blood.[9] The British, however, were having none of it, and continued to

resist blood transfusion until well into the Great War.[10] Resistance finally gave way in the summer of 1916: 'in the treatment of the Somme casualties', commented the British *Official History*, saline infusion 'was found most disappointing'.[11] Just at that juncture, as the *BMJ* published its first transfusion article that century, young British medical officers in France were being converted by witness to the power of transfused blood.[12] One such officer was Geoffrey Langdon Keynes.

Geoffrey Keynes had been pulled out of his graduate medical training by the war, which launched him into the kind of education in trauma surgery then still rare in peacetime. Other British surgeons had embraced transfusion more publicly during the war, but Keynes would take the lead in finding its place in civilian medicine. Despite his junior status, Keynes's family connections gave him entry to the British medical elite. When he wrote the first British book on transfusion in 1922, his social position helped ensure its respectful consideration. His book drew attention to the growing similarities between trauma in war and in peace. The haemorrhagic shock so common to war was, he noted, being 'faithfully reproduced in the victims of train or street accidents, in patients who have undergone certain severe operations, and in women suffering from post-partum haemorrhage or a ruptured ectopic gestation'.[13] Keynes (later Sir Geoffrey) quickly became the uncontested *medical* voice of transfusion in interwar Britain.

As transfusions became more popular, London's hospitals faced a growing crisis: the availability of blood donors. During the nineteenth century, the procedure was uncommon and blood types unknown, making it fairly easy to find a donor among the patient's family, servants or the medical spectators who came to witness the bold operation.[14] Twentieth-century donor options were more complicated. Transfusions were more frequently performed and the existence of blood types was recognized.[15] At the time, only four blood types, labelled 'I' to 'IV', were known.[16] Contrary to expectations, familial relation was no guarantor of blood type compatibility. Where, then, would donors be found? The Americans approached the problem with typical entrepreneurial flair: hospitals paid donor 'volunteers'. Most popular on individual hospital donor lists were men with type 'IV' blood. This blood (now labelled 'O') seemed to be universal, fit for any patient. Particularly in New York City, donor payments soared.[17]

Initially, London hospitals followed the American pattern of donor procurement, but soon found they were unable to sustain it. For the city's voluntary hospitals were in financial crisis: medical costs were mounting, but philanthropic gifts had declined.[18] Occasionally, the voluntary tradition offered its own options. At the London Hospital, for instance, the General East End Tradesmen's Association expanded its annual hospital donations to include money to pay blood donors from 1922.[19] But generally there was little money with which to pay donors. It was in this context that Fletcher appealed to Stanley and the Red Cross to organize and pay donors. Even

Keynes was still thinking that donors would be paid for their troubles. By this time, however, Percy Oliver's all-volunteer donor service was up and running.

The politics of giving

There is an accepted story – a standard narrative – of the creation of the voluntary blood donor service. It tells of Oliver receiving a phone call 'on an evening in October 1921' from King's College Hospital, requesting that he or one of his volunteers donate blood. Oliver dated the birth of his donor service from this call.[20] The hospital may well have called that evening; however, the call was *not* Oliver's introduction to the therapeutic potential – or exigencies – of transfusion. A full ten months earlier, in December 1920, the Camberwell Red Cross Division minutes mentioned a similar request by the hospital. In fact, 'a number of members had already volunteered' to give their blood. At issue was the matter of payment. Oliver's policy was absolute: 'in the event of any payment being made for such services it be retained by the Division.'[21] Clearly, he was aware of the need for blood donors *before* October 1921. The standard narrative telescoped events, giving them brevity, drama and an air of visionary revelation. The origin-truth is more complicated, and more revealing, than this origin-myth.

Percy Lane Oliver's parents were teachers in Cornwall, where he was born in 1878. The family moved to London in 1883. At the age of 14, he took the Civil Service exam, coming first among nearly 500 applicants. His weak heart, however, led the Medical Board to reject him and he became an assistant librarian to the Camberwell Borough Council. In 1901, he moved to the finance department of the Town Hall. A founding member of the Camberwell Division of the Red Cross, he was appointed honorary secretary in 1910.[22] Oliver was organized and energetic, continually producing new schemes to raise funds and assist the local community. His wartime efforts earned him an OBE and the commendations of his superiors. In short, Oliver was a bright, capable man, and a firm believer in voluntarism.[23] He was, however, limited in his reach by his social station. But his unshakable belief in his voluntary work sometimes led him to sidestep social convention – and his immediate superiors – to achieve his goals. This inclination helped lay the foundations for the voluntary donor service.

In the years after the war it was not just transfusion that seemed to lack an obvious peacetime niche: the Red Cross too sought appropriate activities. As early as December 1918, Oliver had devised a scheme for his Camberwell Division to help its local hospital, King's College, with everything from paperwork and linen changing to surplus medical supplies.[24] To fund these efforts, Oliver looked to expand the donations he was receiving from the London County Council's (LCC) 'Sunday Cinemas' programme. This had been instituted in 1916 to distribute Sunday cinema profits to designated

charities.[25] Oliver negotiated contributions from two local cinemas and, by the autumn of 1920, was looking to add a third.[26]

Meanwhile, the London County Branch, which oversaw the Camberwell and other local divisions, had undergone its own restructuring. Its Finance Committee intended to take a more active role in distributing funds between divisions. Accordingly, the County Branch rejected Camberwell's addition of a new cinema to its contributors. An indignant Oliver ignored the County Branch and appealed directly to the LCC, requesting that contributions 'be sent . . . to the beneficiary direct, instead of through the medium of the County Branch'.[27] Tensions escalated. In June 1921, Oliver and his Executive Committee took their case directly to the 'central body' of the British Red Cross. Branding County policy 'bad in principle, and eminently calculated to discourage the efforts of Divisions to raise funds locally', Camberwell appealed for 'protection from such high-handed action'.[28] That October, lacking a ruling from headquarters, the County Branch condemned Camberwell's actions as a 'breach of discipline' and demanded an apology. Oliver's division demurred.[29] The County in turn withheld *all* of Camberwell's Sunday Cinemas proceeds, and Camberwell responded by discontinuing all formal Executive Committee meetings.[30] The Division's official silence would last for more than four years, but its activity levels increased. For the same month the Division went silent, King's College Hospital rang Oliver again, looking for a blood donor. Camberwell set to work.

The band of lunatics

Percy Oliver and his equally industrious wife, Ethel Grace, were the administrative and ideological heart of the new BTS. The pair organized all aspects of donation. When hospitals called, they contacted donors by telegraph, constable, taxi driver – and sometimes by bicycle. Occasionally, the Olivers were fortunate enough to find a vehicle-owning volunteer nearby: one day, 'just as a call had been received', a Mr Hoddle called and was subsequently impressed to spend 'the whole afternoon conveying the Divisional Secretary around Peckham and Brixton in his sidecar, calling on various volunteers in the pouring rain'.[31] The local clergy also pitched in.[32] Moreover, the Olivers donated their own blood. These personal experiences of donation would come to inform their ideas about donor identity and their demands for better donor treatment.

Initially, the project was manageable as an offshoot of the Division's general Red Cross work. In 1922, its first full year of operation, the BTS supplied donors on 13 occasions. Word of their work spread. In August, when no donor could be found for a dying man at Guy's Hospital, the man's wife, 'desperate in her distress', was 'reduced to stopping strangers in the street to ask them to give their blood . . . . Among those to whom she appealed was a member of the Camberwell Red Cross who told her to

apply to Mr. Oliver.' She did, her husband was transfused, and improved. 'From that time on', Mrs Oliver recollected, 'the word seemed to go round in Hospital circles that there was a band of lunatics somewhere down Camberwell way willing to give their blood to any necessitous patient in hospital'. By 1923, the service provided donors to six hospitals and covered expenses with the proceeds from tin foil collections and patient gifts. As administrative demands mounted, they hired an 'efficient woman donor' to assist. The service continued to grow.[33]

By the spring of 1924, Camberwell's donors were being requested in numbers greater than the limited membership could sustain. Oliver took his case to a local newspaper. His article was read by a Rover Scout, who 'interested his organisation in the movement'.[34] In June, it was decided that Oliver's Red Cross division would continue to administer the donor service, while 'the London Rovers would obtain sufficient donors from their members to meet the requirements of all the London hospitals'.[35] This alliance signified the formation of the *London* BTS.[36] By September, 55 Rovers had signed up; that year, the Service answered 62 hospital calls.[37] Eventually, Oliver would make alliances with a wide range of groups[38] – even the British Union of Fascists signed on.[39]

Oliver also made two strategic *medical* alliances to support his growing service. The first was with Dr Ronald Canti, a pathologist at Bart's and an innovator in both organization and technique.[40] The second alliance was with the London division of the British Hospitals Association, which represented the interests of voluntary hospitals.[41] Thereafter, when Oliver dealt with the doctors and administrators of London's hospitals, he did so backed by the medical and social authority he personally lacked. Such alliances helped him root his service in the complex terrain of interwar voluntarism and hospital politics. This grounding, coupled with a rising medical need for blood, assured the Service's continued growth: in 1925, it boasted a donor force of 180 members, who were called to 47 of London's voluntary hospitals on 428 occasions. But, astonishingly, 'at the end of 1925 the Camberwell Red Cross gave up the work'.[42]

The London Red Cross BTS, founded in 1926, is generally presented as a continuation of the Camberwell Service. In the most important sense of 'continuation' it was just that. It embraced the earlier BTS's organization, ideology and leadership. Yet technically, it was a different service. Moreover, although there is truth in the general assumption that the 1926 Service sprang directly from Camberwell's success, it is not the whole truth. Before the 1926 Service started, the Camberwell Service was formally disbanded. The story takes us back to the battle between the local and county Red Cross branches.

Four years had passed since Oliver's transfusion service had begun sending donors to London's hospitals. Still, there had been no resolution to the struggle between the Camberwell Division and the County Branch over

Sunday Cinemas contributions and divisional autonomy.[43] The Camberwell Executive Committee continued its official silence. This meant, technically, that the Division had done no Red Cross work in four years; not even the transfusion service 'counted'.[44] In the autumn of 1925, the four commandants under Oliver's Executive Committee decided to use this technicality to seize power of the Division, formally protesting to the mayor that the Division had conducted 'practically no Red Cross work' in years.[45] Oliver was livid. Responding indignantly in a detailed report, he reiterated the Division's reasons for official silence and described its many projects. The first project listed was the BTS: 'over 200 lives saved already by testimony of Hospitals. Scores of grateful letters from patients and relatives, and resolutions of thanks from Institutions. Similar bodies being formed everywhere, all taking Camberwell scheme as a model.' Oliver could not restrain himself from noting acerbically: 'County Branch endeavoured to appropriate Scheme as soon as Camberwell had made it a success.' After summarizing other works, he emphasized his personal sacrifice in making Camberwell 'famous throughout the Kingdom. More than half my annual holidays are sacrificed to Red Cross work, and it is only with the utmost difficulty that I can manage to get an evening off with my wife.'[46] All this was prefatory to his main point: his resignation, effective from 31 December. The other committee members followed Oliver's lead.[47] The commandants took over as the new Executive Committee, leading the Division down an unspectacular path of standard service. The County Branch, between the lines of their Annual Report, breathed a collective sigh of relief.[48]

The central body of the British Red Cross, meanwhile, was well aware of Oliver's service. Stanley seems to have been watching the Division since 1923.[49] Shortly after the Camberwell resignations, the British Red Cross scheduled a meeting with the Rovers and the British Hospitals Association to discuss the continued provision of blood donors. The initial meeting was to take place on 18 January 1926. By this time, however, politics had taken their toll on Oliver, who had become quite ill.[50] Though absent from the meeting, Oliver emerged victorious. His service was revived as the 'British Red Cross Blood Transfusion Service' and he was named as its guiding force.[51] Moreover, he was to be assisted by a medical adviser: Geoffrey Keynes. True to his role as transfusion's British voice, Keynes soon called for donors on British radio.[52] Thereafter, he would lend weight to Oliver's ideas about the nature – and treatment – of the voluntary donor.

The etiquette of technique

The interwar conjunction of rising medical need for blood and the continuing financial problems of London's voluntary hospitals helped sustain demand for Oliver's unpaid donors. But what could inspire individuals, in sufficient numbers, to volunteer their blood to save a stranger?

To have a sense of what was being asked of donors, we must have a clearer picture of what donation entailed in the 1920s.

Quite simply, many people perceived blood donation to be a dangerous and mysterious process. At the very least, it was inconvenient: there were no blood banks, so donors had to get themselves to a hospital whenever a patient needed blood. In the years before the First World War, American surgeons had developed a number of techniques to move blood between bodies before it clotted. All of these relied on the proximity of donor to patient and required that a 1- to 2-inch cut be made in the donor's arm to expose a vein. This was commonly referred to as 'cutting down'. During the war, however, doctors began adding sodium citrate to the blood to hinder its coagulation. With citrate, blood could be withdrawn in advance, collected into prepared bottles and drawn through a mere puncture wound. But even when citrate was used, interwar transfusion still relied on donors going to hospitals.

Released from the demands of war surgery, doctors tended to employ the 'simpler' procedure of cutting down on donor veins. This was particularly the case in the US, where donor payment guaranteed that a steady supply of blood would flow into needy patients. Indeed, a group of 'professional' donors emerged in 1920s New York. These donors – men, of type IV (O) blood – collected fees of up to $100 a 'donation'.[53] Newspapers portrayed them as 'heroes' who risked pain and danger to sacrifice their blood (and collect their fees). With donors so notoriously well paid, hospitals and surgeons felt justified in subjecting them to pain and inconvenience. Further, the kinds of men who came forward to give blood in this system were not necessarily of the most trustworthy character. Syphilis in particular was a constant problem.

Oliver knew from long experience with voluntarism (and somewhat briefer experience as a blood donor) that unpaid volunteers would never return to such a system. Indeed, his conception of the voluntary donor serving 'suffering humanity' was defined *against* the image of the heroic American blood-cowboy. Individuals volunteered for an assortment of reasons, but tended to share certain characteristics. Many were women; particularly in these years after the war had depleted the country's supply of younger males. The poor as well as the wealthy were inclined to give. Oliver boasted that, in his service, 'donors are drawn from all classes of the community – independent and professional men and women, barristers, journalists, shopmen and girls, labourers, apprentices, roadmen, maid servants and charwomen'.[54] Yet such individuals would not come forward if they perceived blood donation to be a painful and dangerous process. Further, they would not continue to donate blood if they were injured, mistreated or frightened. 'The efficient continuation of the service', Oliver asserted, 'depends absolutely upon the consideration and skill with which the members are treated'.[55]

With the official formation of London's BTS in 1926, Oliver codified his beliefs, shaping them into rules for how doctors and hospitals were to treat

blood donors. These rules ensured that a kind of buffer, in geographic space, mental consideration and physical treatment, was in place to protect the voluntary donor. The donor, Oliver insisted, was neither a 'vendor' nor patient, and was to be treated with special consideration. To this end, all doctors were required to type patients before requesting donors. The BTS would then send a donor of matching type, allowing all donors to be used. Upon arrival at hospital, donors were to be greeted by a member of the staff, who would guide them to a special bleeding room. They were never to encounter the patient and were always to be bled lying down. And never, under any circumstances, was the doctor to cut down on donor veins. Afterwards, the doctor was required to complete a form giving the anonymous details of the patient's history and case outcome. When doctors delayed, they received stern reprimands from Oliver, complete with active threats that further donations would be withheld.

Of these dictates, the rule against cutting down on veins – a veritable 'etiquette of technique' – is most striking.[56] Early in the BTS's history, cutting down was standard practice. However, it was painful, increased risk of sepsis and, with continued practice, rendered the veins worthless for future use. Ethel Oliver herself could no longer give blood after fewer than a dozen donations.[57] Shortly after Canti and Oliver had formed their alliance, Canti offered an alternative: the citrate method, which was simple, safe and could be performed with a puncture alone. It would preserve donor veins – and thereby, preserve donors. When Keynes concurred, the move to citrate became absolute. No surgeon was *ever* to cut down on a donor's vein. Doctors and hospitals generally followed Oliver's rules, which had the backing not only of Geoffrey Keynes, but also the BHA and the Red Cross. Besides, any doctor – indeed, any hospital – that transgressed would face losing its free blood supply. As the years went by, the prospect of medicine without blood donors became increasingly inconceivable. And so, while American doctors cut down on the veins of their well-paid donors and debated the safety and efficacy of transfusions performed with citrated blood, few such operations and debates took place in London. Oliver's BTS dictated medical practice, London's doctors complied and the service grew. Soon, it was supplying donors to all of London's voluntary hospitals. In 1930, the BTS formally extended its donors to London's municipal hospitals, charging these hospitals '£1. 1s. a call'.[58] Poetically, it was approved to receive funds from the Sunday Cinemas programme in that same year.[59]

Conclusion

During the 1930s, the foundations Oliver had laid in the 1920s were rapidly expanded into a 'British' service, rooted in local communities. A handful of 'provincial' organizations in 1931 grew in number to 46 by 1938.[60] With their expansion, the London and provincial services faced tribulations

common to most small organizations in times of rapid growth. To contain problems of donor discontent and doctor disrespect, and to hold at bay the looming spectre of fee-for-service donation – what Oliver derided as 'professional' donation – the BTS itself increasingly professionalized. In 1931, it obtained funding from the Medical Research Council to appoint H. F. Brewer as the service's medical officer. Brewer's position was more formal than that of Keynes and Canti. He performed physical examinations of all donors, conducted research on the effects of blood loss, oversaw the provision of low-cost, high-titre blood typing serum from Burroughs, Wellcome, and acted generally as a focal point for donors' medical complaints.[61] In 1932, the BTS established the Voluntary Blood Donor Association and was insuring donors.[62] Oliver also understood that donors needed information if they were to continue to supply their blood without financial reward. In 1933, he started the *Blood Transfusion Service Quarterly Circular*, a newsletter that featured statistics, donor stories, medical reports and accounts (teeming equally with concern and hubris) of the difficulties faced by 'vendor' services in other countries. Oliver also used the new *QC* to shore up the identity of the voluntary blood donor, printing 'press absurdities' and poking cartoon fun at those who thought blood donation dangerous.

In 1936, the *British Medical Journal* included an article on the 'problems of blood transfusion'. It had been written, the journal noted in an editorial commentary, 'by a layman who has special claims to the notice of the profession: Mr Oliver'. Oliver had 'acquired a unique position as an authority . . . and his words must be received with respect'.[63] The following autumn, Oliver and his service – now seen as 'the British system' – received similar praise at the Second International Congress on Blood Transfusion in Paris, if tempered by pragmatic scepticism: 'the British system of voluntary donors … received handsome tribute on all hands, but was *evidently regarded as hopelessly Utopian in most other countries*'.[64] When the world again went to war, Oliver aided in the efforts to establish a national emergency transfusion service.[65] He died in 1944. After the war ended, England and Wales instituted a national health service that included the National BTS.[66] The government service, too, used unpaid donors.

This, then, is the history that Richard Titmuss omitted from his comparative analysis of blood donation. The British donor service was a product of interwar voluntarism, rather than of the later state system that largely subsumed it. Though Titmuss ignored Oliver, he sounded much the same on the advantages of the unpaid system. It was less wasteful and suffered fewer shortages of blood and donors:

> [I]n commercial blood markets, the consumer is not king. He has less freedom to live unharmed; little choice in determining price; is more subject to shortages in supply . . . . Far from being sovereign, he is often exploited.[67]

Voluntary services, on the other hand, preserved a social space for altruism; and altruism in turn opened society to true social change. Titmuss and Oliver, though working in different social contexts, thus shared important assumptions about the meaning and significance of preserving blood as a social gift and preventing its abandonment to market forces. Neither of these influential voices for centralized voluntarism was medically trained. And it was Percy Oliver, a layman from south London, who first breathed life into this voluntary donor system.[68]

Part III

Minds, Identities and Social Order

13

Murder, Melancholy and the Insanity Defence in Eighteenth-century Hamburg

Mary Lindemann

Let me begin with two strikingly similar cases of homicide, separated by two centuries. In his book on madness in sixteenth-century Germany, Erik Midelfort relates the story of Conrad Herman, a master dyer from the town of Laufen on the River Neckar. On 12 January 1590 Herman murdered his wife and attacked his four slumbering children. The subsequent investigation revealed that he 'often displayed melancholy and practised weird nonsense'. Perplexed by Herman's behaviour, the authorities consulted the Tübingen legal faculty. In the sixteenth century, such practices had become, according to Midelfort, 'routine … in all difficult cases'. The jurists in Tübingen accepted the existence of an insanity defence, referring to the then thousand-year-old legal code of Justinian, yet concluded that despite his obvious 'melancholic disorder', Herman had been *compos mentis* when he slew his wife 'fiercely, but … knowingly and intentionally'. They based that opinion on his demonstration of 'good understanding' before and after the crime, and their suspicion that his madness was counterfeit. He was beheaded. Midelfort observes that in this case, '[n]o one thought of consulting a physician, because the legal question was framed in such a way that lawyers and laymen themselves could judge the relevant issues'.[1]

The crime Johann Georg Rüsau committed in the early hours of 15 August 1803 was equally shocking. Rüsau's maid discovered the bodies of his wife and their five children, all 'swimming in their [own] blood'. Rüsau could not be found. Two duck-hunters stumbled over him (literally) later the same day, lying near the Alster lake: his clothes were sodden and several wounds marked his throat and hands. During his initial interrogation, he admitted the six murders 'calmly' and showed 'no trace whatsoever of mental derangement'. 'With a light shudder' he identified the bloodied knife found by the bodies as the one he had used.[2]

Few crimes, before or since in Hamburg, evoked the sensation Rüsau's multiple murders did. The numerous pamphlets dealing with the horrific

incident fragmented in opinion. Many delineated the details of the crime for the curious; some constructed more or less accurate 'life histories'; others commented piously on a life gone wrong; yet others fictionalized Rüsau's conversation with his wife and children 'in the kingdom of the dead'. Many pamphlets addressed the insanity defence. Rüsau's case is, of course, by no means the only one during the eighteenth century where the question of the criminal's 'responsibility' (*Zurechnungsfähigkeit*) was disputed. Yet it offers a point at which we can take stock of the evolution of the insanity defence over two centuries.[3] This chapter, therefore, analyses the debates over the insanity defence that occurred in Hamburg at the very opening of the era of anthropological psychiatry. What Roger Smith observes for a later period, a different place with a different legal system (Victorian England), that trials of individual crimes reveal the constitutive elements of medical and legal discourses, is equally true of eighteenth-century Hamburg. '[E]ach trial was a debate in itself.'[4]

As Midelfort observes, the insanity defence was already old in the sixteenth century. The German criminal code of Charles V – the famous Carolina of 1532 – remained the basis for criminal law throughout the eighteenth century. Its existence did not, however, preclude vigorous debate on the subject. Especially contested was the idea of *Zurechnungsfähigkeit,* translated variously as 'accountability, sanity, and soundness of mind'. The Carolina and Hamburg law provided for a reduction or no penalty for those judged *Zurechnungsunfähig* (not competent). Many factors contributed to diminished capability: extreme youth, senility, drunkenness, fever, idiocy or femaleness. Insanity, too, could acquit a perpetrator.[5] Yet the determination of insanity, the basis for deciding whether or not someone was aware of and, therefore, accountable for his or her actions, remained a perplexing task (as, of course, it does today). Indeed, the question of who determines mental derangement and its treatment forms a major theme in historical treatments of madness, whether they discuss early seventeenth-century England, Victorian Britain and America, or post-Reformation Germany.[6]

The insanity defence has virtually always evoked controversy and suspicion, although rarely has the debate bifurcated cleanly into opposing camps. Charles Rosenberg, in his classic analysis of *The Trial of the Assassin Guiteau*, points out that the 'late nineteenth-century controversy over criminal responsibility was … more complex than a simple struggle between conservative and liberal, orthodox believer and agnostic'. The battle was 'utterly incomprehensible' unless one understood the evolution of medical ideas about insanity. Even the acceptance of the McNaughten rule in the 1850s (as formulated in England in 1843) – that a person was to be viewed as responsible if he knew what the nature and consequences of his actions were and that the law forbade them – did not create clarity; nor did it determine the outcome of individual cases, especially as many early psychiatrists came to 'regard the McNaughten test as essentially irrelevant'.[7] Similar uncertainties

had been generated in the late eighteenth and early nineteenth centuries with the formation of concepts of partial insanity and monomania.

While an extended discussion of the eighteenth-century understanding of madness is not called for here, it is relevant to point out that a general – if not universal – tendency was to accept Locke's secularized belief that insanity was linked to 'abnormal patterns in the association of ideas which profoundly disturbed the individual's judgement'. By the end of the eighteenth century, however, this general agreement was breaking down. Concepts of partial or temporary insanity came to account for the many instances of madness 'where the patient did not seem to dwell in a delusional state'.[8] The idea of temporary or partial insanity had momentous consequences for the shaping of the insanity defence.

Certainly, no standard procedures existed for dealing with criminal insanity in eighteenth-century Hamburg. Moreover, jurists, magistrates (who were not always jurists), commentators and a broader public disagreed on what constituted exculpatory madness. Curiously enough, enlightened writings and thought could advance a quite narrow definition of what insanity was. Determining whether madness was feigned muddied the waters even more. In a comprehensive twelve-volume treatment of the Hamburg legal system which appeared at mid-century, Johann Klefeker glossed the incongruities in how Hamburg disposed of insanity cases. Despite the explicit provision that insanity 'in all crimes creates exculpation', Hamburg's courts moved inconsistently when deciding such cases; at times sentencing the accused to death, at times ordering close confinement.[9] This disparity arose from the lack of clear guidelines for determining legal or criminal responsibility. For instance, the practice of consulting a legal or medical faculty never became the norm in Hamburg, despite being routine elsewhere in Germany since the sixteenth century.

Hamburg's legal system was, of course, embedded in the city's political structure and culture. The entire freighted relationship between the courts, the political system and ideas of rights and privileges in Hamburg weaves a fascinating but lengthy story that cannot be told in its entirety here. Two courts dealt with criminal matters directly: the Lower Court (*Niedergericht*) and the Upper Court (*Obergericht*, which was identical in membership to the City Council, or *Rat*). Throughout the eighteenth century, political conflicts in Hamburg turned on who *exactly* exercised sovereignty in the city: the City Council or the Citizens' Assembly, from which latter body most members of the Lower Court were appointed. Not surprisingly, in this climate individual cases, and especially those involving capital crimes, frequently triggered clashes between the two courts, representing as they did antagonistic views of sovereignty and about the exercise of power in the city.[10]

A second complicating feature was Hamburg's political *culture*. Hamburg considered itself a 'republic' of citizens, proud of its heritage as a free, imperial city in which lived 'no nobles, no patricians, no slaves, no, not even

subjects. All true Hamburgers belong to and acknowledge only one class, that of *Bürger*, nothing more and nothing less.'[11] While this paean ignored crucial disparities of wealth and power separating citizens from non-citizens, the rhetoric coloured virtually all political discourse in Hamburg. Pride in a free, self-governing citizenry linked with disdain for professional, and thus venal, administrators and bureaucrats. Paying officials to govern betokened an appalling atrophy of true republican values. Political commentators prized the ability of each citizen, of each member of society, to act competently and responsibly. Thus, criminal cases that brought the competence of individuals into doubt or that suggested that some people must not suffer the consequences of their actions, were especially vexatious because they shook the basis of Hamburg's political edifice. While raving madness raised no problem for the courts, diagnoses that made judges feel unable to determine responsibility and culpability, and thus to require recourse to experts, was a ticklish situation that evoked unease, scepticism and even mistrust because of its momentous political implications.

Hamburg's Lower Court was a bench composed of nine members selected from the City Council (two) and the Citizens' Assembly (seven). An investigating magistrate initiated cases and presented the results of his inquiries to the full court. Only then did the accused's lawyer produce a written defence. It was up to the defender to frame an insanity plea if he felt the situation warranted it. The Lower Court then reached its finding. In capital crimes, the sentence passed to the Upper Court (the court of final appeal). After 1712, the City Council consisted of 24 senators of whom half were to be trained in law. Once the Upper Court received the ruling of the Lower Court it had three options: it could confirm the finding, overturn it or reform it. Reforming could go in either direction, mitigating a penalty the Council judged too harsh or exacerbating a penalty viewed as too lenient. The Upper Court never became a rubber stamp. It exerted its authority *especially* in cases where insanity was alleged.

In 1725, for instance, a shopkeeper named Daniel Sottmann killed his four-year-old stepson seemingly for no reason whatsoever. The magistrates portrayed his action as the result of 'a savage passion that suddenly welled up within him'. One minute he was calmly discussing the day's business with his wife, the next he was plunging a knife into the child's body. He confessed 'with tears and sighs'. The Lower Court sentenced him to be beheaded, after first having his flesh torn with hot pincers. His defender sought to have him freed by presenting evidence of Sottmann's 'occasional sad and melancholic disposition'. This plea failed to sway the Lower Court. The Upper Court confirmed the original verdict, although it mitigated the severity of the sentence to beheading.[12]

Twelve years later, however, another case demonstrated a very different outcome. Here the events involved a once prosperous merchant, Nicolaus Lipstorp, who stabbed a beggar and left him to bleed to death. The

Lower and the Upper Courts agreed that Lipstorp could not be punished and merely ordered him safely confined. Lipstorp's madness, however, seemed completely transparent. Credible witnesses testified that he had been afflicted with a 'melancholy that had grown ever stronger' over the years and that eventually completely 'scrambled' his wits. He believed himself to be, among other things, the true Messiah. Some years before the murder, his family, unwilling to lock him up, had removed him to the countryside to reduce the chance that he would cause mischief.[13]

Fratricide, like parricide and infanticide, counted as one of those extraordinarily frightful crimes for which Hamburg law set particularly severe penalties – like breaking on the wheel. The brother-murder Lucas Wiegers committed in 1747 produced an extensive defence based on the argument that 'the same Wiegers had for a long time both before and during the deed, not been in full possession of his senses'.[14] Wiegers was a 22-year-old corporal on leave and had been living with his mother for some time. His elder brother, Christoph, having recently completed his theology studies, had just returned home. The mother was unable to board both sons and she made it clear to Lucas that he would have to give way. At this point, Lucas decided to eliminate his rival. Fully clothed (a crucial detail), he placed a pistol in each of his two pockets and entered the room where his brother slept. The pistols misfired, but he dispatched his brother by slashing him repeatedly. When arrested, he confessed the murder with 'a distinct "Ja"'. Thereafter, he began 'to act foolishly'. His mother produced witnesses who swore that since his youth he had done all sorts of crazy things, had never been fully in control of his actions and 'was also often very melancholy'. Attests from (unspecified) medical and judicial faculties obtained by his mother argued that he should not be punished but merely confined. And so the Lower Court decided.

His defence, however, found less sympathy in the Upper Court. It concluded he had committed 'this wanton act with evil intention and deliberately' and sentenced him to an exemplary and 'well-deserved penalty' of decapitation. The deliberations in the Upper Court demonstrate how Hamburg's governors thought about insanity as a legal plea at mid-century. The defender had contended that no sane person could commit such a ghastly crime. The Upper Court conceded that 'a raving madman [*furiosus*]' could not *de regula* be punished. To be considered a *furiosus*, however, the person must have suffered an entire 'mental alienation' present at the time of the action and permanent. Wiegers intended to murder his brother, went about the act 'purposefully', and thus could be punished. The circumstances of the crime clearly manifested intent.[15] A *furiosus* was, by definition, incapable of intent. *Furor*, or rather determining if someone was or was not a *furiosus*, however, broached a difficult point. For Cicero, who coined the term, it meant a 'total blindness of mind', a madness greater than any other. Such people were not responsible and therefore not culpable. For those

sixteenth-century jurists dealing with cases of *furor*, the problem proved no less nettlesome, for they 'were concerned to discover the exact meaning of *furor* and to discount, as culpable, other disturbed mental states'.[16]

Legal theory and practice had worked out several methods of ascertaining intent as well as of testing the veracity of testimony in such cases. Confessions must be closely scrutinized for clues about the suspect's mental condition: Was he able to relate the deed coherently? Did his story vary with repeated telling? Did his version correspond to the facts as otherwise known? Other people, including physicians, could provide pertinent evidence. While the Upper Court admitted 'the established practice of a verbal examination by physicians', it regarded such evidence as neither sufficient nor necessary. Physicians, moreover, must observe and question persons not just once but several times and should never base their opinions solely on written statements, hearsay or reputation. One must interrogate witnesses carefully. The closer the noted instances of fury occurred to the actual criminal incident, the more credibility they bore. Moreover, actions that were exceptionally odd or grossly evil did not alone prove *furiosum*. 'Intervals' of insanity demonstrated not *furiosum* (and non-accountability) but rather melancholy (and thus culpability).

The Upper Court chided the Lower for resting its conclusion in the Wiegers case on the advice of the medical faculty in Göttingen. This *responsum*, it charged, contained only 'suppositions' and these had, moreover, been derived solely from written depositions. No one in Göttingen had interviewed Wiegers. The judicial opinion, also from Göttingen, simply repeated the medical one. Neither faculty took into account that the accused had perhaps fabricated his 'deranged talk'. Thus, the Upper Court dismissed both opinions.

The Upper Court constructed another version of the mental condition of Wiegers before and during the attack. His mental state admittedly manifested 'a certain degree of rage'. Nevertheless, rage did not prove insanity. As the Upper Court read the case, all the facts that the defence had marshalled in arguing that Wiegers was a *furiosus* or *dementus* were sufficient only to suggest 'fatuity, arrogance, irascibility, or melancholy'. They compared Wiegers' case to the earlier ones of Lipstorp and Sottmann. Lipstorp, 'a madman, who never came to himself', could not be punished at all. Sottmann was 'melancholic' and therefore was spared an 'intensified' penalty, but he was executed.

Melancholy was the sticking point. Benedict Carpzov in the seventeenth century stressed that a melancholic, even one who experienced intervals of fury, could be decapitated for a capital crime. Samuel Stryk, at the beginning of the eighteenth century, offers a nosology of melancholy that seemed to set useful categories for legal judgments. Melancholics of Stryk's 'second group', whose peevishness made them 'sullen and desperate', could be punished. Other melancholics whose mental state was coupled with dementia were not

culpable, however. Thus Wiegers, whose mania was neither continuous nor attended by 'delirium', was legally competent and his execution justified.[17]

But what about Rüsau? Did he, too, fit into Stryk's 'second class' and was he, therefore, also competent?

A week after the murders, Hamburg's acting city physicians examined the accused. They visited him three times in six days, each time speaking with him for about an hour.

> We found a man of fifty-two years, of medium height, very thin, with a jaundiced and atrabilious complexion, [having] a black beard and hair, small dark-brown eyes and a gloomy expression. His brow showed signs of a lingering sorrow.

Although this analysis regarded expressions, temperament and physical appearance as important, Rüsau's observers also devoted much of their time with him to 'dialogues'. In these extended conversations, they encouraged Rüsau to reflect 'on his [previous] life, on his health, and on the appalling deed he had committed'. Rüsau described his modest lifestyle, portrayed himself as a person not given to heating drinks or anger and as a man who enjoyed a game of cards but did not gamble. He admitted a tendency to miserliness which, he insisted, did not stem from a deep-seated character flaw, but from concern for the well-being of his family.[18]

Rüsau had begun his adult life as a theology student. In Hamburg, he received a place as catechist at one of the city's charitable institutions but was not considered for a more prominent position as pastor of a major congregation. After abandoning the ministry, he married and started a school with his wife which, for a while, went on excellently. Then his troubles began: the number of students shrank appreciably, he started a business partnership in textiles that 'did not do as well as he had hoped' and he ran into debt. As it turns out, his financial circumstances were not especially alarming; his assets significantly outweighed his liabilities. This was not how he saw it though, for 'he was always in good humour when things went well, yet fell into a depression at the least sign of trouble or impediment'.

In the weeks before the murders, dark thoughts of ruin plagued him day and night. His future seemed bleak, even calamitous:

> He feared that he would lose everything, slip into grinding poverty, and watch his wife and children die a disgraceful death of hunger. . . . All reasonable arguments he made to himself remained ineffectual . . . . He often tried to pray and found himself unable to. Almost every night he lay sleepless and [would break out] in an enervating sweat.

He continued in this state of mind until, obsessed by this 'ghastly picture of the future', he decided to 'save' his family from the terrible fate he believed awaited them.

He related that at the time of the murders 'his mood was more despondent than violent, but soon he lost his reason entirely, completed the deed as in a delirium, and with the greatest haste'. For 48 hours thereafter, he expressed 'the deepest regret for his crime', although he continued to assert that his dire financial situation afforded him little choice. On the third day, however, he became contrite and convinced himself that it was all a 'horrible mistake' and that a 'mere delusion' had driven him to annihilate his entire family. The physicians concluded that at the time of their examination, Rüsau demonstrated 'not the slightest sign of any mental derangement' and thus they declared him 'now' fully competent.

When they turned to his mental condition directly before and during the deed, however, they found it 'very probable that he . . . then suffered from dementia'. They admitted that this mental state did not belong to the most familiar types of mental derangement: he was neither feeble-minded (*blödsinnig*), manic (*toll* or *manisch*) nor simply crazy (*närrisch*). His insanity was altogether different, 'namely that of melancholy or monomania'.[19] The physicians explained that such people were in general sane, *except* in one regard. They based their assessment on Johann Christian Reil's definition of monomania:

> a partial disfunction [or inversion] of the ability to reason, which relates to a single thing or a series of similar things. This inversion . . . restricts [therefore] the subject's freedom of will and, according to his obsession, determines his actions.[20]

Thus, it was not significant that Rüsau's financial condition proved less serious than he perceived; it sufficed that he was completely convinced of its gravity. Rüsau's physique had always shown 'a melancholic habitus' and, therefore, he had a 'natural tendency' to this affliction. Moreover, his 'petty [nature], weak character, lethargy, solitary lifestyle, and petulance' supported their conclusion. The physicians argued that 'before and during the deed' Rüsau had suffered from a melancholy or partial insanity that paralysed his will. Indeed, the fact that in prison he had come 'to give up this obsession entirely' only reinforced their diagnosis. Numerous physicians suggested that a major 'shock' – such as the murder of his entire family – could indeed cure monomaniacs. *Sensatio major taxit minorem*!

Rüsau's defender, the advocate Schleiden, shaped his argument on the notion of partial insanity or monomania. Drawing on Reil, Styrk and Carpzov, he argued that Rüsau laboured under the kind of melancholy that absolved him of responsibility for the crime 'even if no sign of its presence previously manifested itself, and [even if] the criminal acted entirely normal in all other respects'. Schleiden called on the court to request expert opinions from medical and legal faculties at 'foreign' universities. (Hamburg did not have a university at the time.)[21]

The Lower Court, however, pursued a different tack. Instead of sending the documents to a faculty for its *responsum*, its president, Dr Hasche, convened an extraordinary meeting of nine prominent residents. This conclave, held at Hasche's home, was made up of three theologians, three lawyers and three physicians (one of whom was Dr Rambach, a co-signatory of the report on Rüsau's mental condition). Three men – two lawyers and Rambach – believed Rüsau to be 'actually crazy' and therefore not responsible. They proposed that he be confined for life. The other six argued that Rüsau was 'fully sane' and should receive the death penalty, although without the wheel 'considering the circumstances'. The Lower Court concurred, insisting that Rüsau's act was 'cowardly' (*feige*; underlined twice); he had given in to 'doubt and sorrow', yet he had also committed 'an intentional crime'. The Upper Court reformed the opinion and pronounced a considerably harsher sentence. Sixty years later, the prominent physician and sanitary reformer, Hermann Gustav Gernet, observed: 'He was mentally ill and, properly, belonged in a madhouse'.[22]

Few people in the early 1800s would have concurred. The vast majority of public and learned opinion agreed with the execution. The reasons why illustrate the sometimes strange effect the Enlightenment had on shaping opinions about criminal insanity, individual volition and human responsibility. They reveal as well the resistance of a city-state and a 'republic' like Hamburg to relying on the judgment of experts (worse yet, 'foreign scholars'); the difficulty of accepting the idea of monomania; as well as the lingering fear of being taken for fools.

The case prompted enormous public discussion. There were, of course, those who regarded the insanity defence as a disgraceful sham. The author of the sensationalist pamphlet *Rüsau, the Murderer of His Family* referred to Rüsau as a man who 'stalked' through his house, a naked blade in his hand, 'panting for blood'. There was no madness here except one with method in it. Others believed that even the execution of someone who was insane could serve a worthy purpose. 'Not even a madman would slink into the chamber of [his] sleeping [victims] to murder them . . . if he knew that his crime would [without exception] be punished.'[23]

Most commentators took the insanity defence more seriously, and if they fully believed Rüsau guilty of his crime – and therefore believed that he should be executed – they did not dismiss the insanity defence *tout court*. They found Rüsau a deeply flawed human being. The Enlightenment, with its stress on humanity and education, came to play a rather peculiar role that is best represented in the 95-page piece the pedagogue Johann Daniel Curio published in 1804. Hamburg's enlightened public received it warmly.[24] Curio expressed the horror every decent person felt when confronted by such an event; it was 'abominable and villainous'. Yet the murderer remained 'our brother'. His depiction of Rüsau reviewed the case from its 'practical, moral, and psychological' sides and analysed Rüsau's faults as

deriving from an inappropriate education. Rüsau represented one of the many poor students who exhibited no real talent for learning. An education based on superficial knowledge often led the possessor down false avenues, such as the mysticism and astrology that fascinated Rüsau. Rüsau lacked 'common sense' (*sens commun*), a 'manly firmness of character' and the attributes of a 'humane, educated person'. Such qualities would have allowed him to deal with the unforeseen circumstances of life that instead crushed him. Likewise, he was unable to obtain solace or comfort from rational religion.[25]

Curio had no patience with the crowds demanding Rüsau's death and who thrilled at the thought of the 'exquisite spectacle' of a grisly execution. While he abhorred the barbarity of the wheel, he insisted that Rüsau's blood had flowed 'legally'. Rüsau had never displayed the slightest indication of insanity before that fateful night. A 'gloomy and taciturn personality' could not, Curio argued, be considered the equivalent of madness. If bad temper and melancholy excused murder, where would that leave human society? Strong passions drove Rüsau, but surely not every passionate man was insane? If one accepted the idea of monomania in Rüsau's case, then, according to Curio:

> no judge can condemn the voluptuary who murders his father in order to obtain the estate he needs to slake his lusts. Then no judge can punish the ambitious man who sweeps away everyone in his path nor [can he] punish the child molester whose monomania is pederasty, nor the arsonist whose mania tells him to set fire to his house . . . Thus the judge can punish no vice, no crime if it is [considered] the result of a monomania.[26]

Curio's faith in the powers of 'true' education to improve all members of the human race led him to view Rüsau's crime as a result not of insanity but of poor instruction, of allowing himself to be ruled by his passions and of a bent towards religious enthusiasm.

Other commentators raised objections to the insanity plea. The author of a critical review of Schleiden's defence contended that all criminals who committed heinous crimes were momentarily 'insane' but that did not excuse them. He referred to several prominent cases in the recent past, especially that of Deborah Traub, a Jewess who had poisoned her mother and stepmother. Wasn't she, too, he queried rhetorically, 'in a condition of temporary insanity' during the act? No one had hesitated to condemn *her* to death. Thus, an Enlightened text that pressed for religious toleration and the civic emancipation of Jews demonstrated little understanding for the insanity defence. Moreover, the author continued, the practice of requesting expert opinions from university faculties proved unsuitable for a republic in which laws applied equally to all and in which all should judge and be judged without needing the jurists' 'flowery speech' (by implication an

obfuscating language that served the interest of tyrants). Thus, the insanity plea as a defence forged by experts was a method that, by absolving men of the consequences of their acts, also prohibited them freedom of will. A republic like Hamburg could not sustain itself if it admitted the diagnosis of temporary or partial insanity into its calculus of penality.[27]

Perhaps the most subtle and reasoned of the reactions to the insanity defence came from a lawyer, A. C. Wolters. His book echoed the moral sentiments Curio stressed; emphasizing especially steadfastness of character and courage in confronting unforeseen events under the slogan 'Who does not dare to fight, does not deserve to live.' Self-control and victory over adversity came from the proper deployment of human reason and from a steadfastness of character attained only through a proper upbringing and careful religious training. Rüsau, with his 'morose disposition' and his 'lethargic spirit', faltered when confronted with the daily irritations of life. Therefore, 'his own faults' led to his unhappiness.[28]

Wolters' analysis was, however, physical as well. He accepted that 'the natural form and organization of the body clearly exerts a very important effect on human thought and conduct'. Still, one could not immediately conclude from the 'shape of the head alone' what the character of a man was. Here, Wolters found himself more in line with the early work of Gall than with that of Joseph Caspar Lavater's physiognomy (*Von der Physiognomik* appeared in 1772). Wolters realized that 'physical states' must necessarily exert a 'strong influence' on a person's free will. If physicians could ascertain exactly how the interior parts of the body were put together, 'this would greatly benefit the criminal magistrates', he opined. He believed that Rüsau's body harboured a 'natural tendency' to melancholy and 'if an autopsy of his body could have been done, so perhaps it would have shown that he had a certain flaw in internal organs' that determined his behaviour.[29]

Although Wolters regarded Rüsau as melancholic, he none the less insisted that Rüsau was completely sane when he committed the deed. The fact that Rüsau could accurately recall the details of the crime and elucidate his motivation spoke for his rationality. Were he insane, Wolters reasoned, 'he would not have been able to remember his intentions'. Moreover, during the investigation, Rüsau exhibited absolutely 'no trace' of mental derangement. Wolters concluded that the court had every legal right to condemn Rüsau to death and even to the wheel. While Wolters accepted states of mind that were neither fully insane nor completely sound and believed that every mental condition between sane and insane was somehow pathological, that pathological state did not excuse crimes.[30]

Many factors combined to weaken the willingness of jurists and other observers to accept the insanity plea. Clearly, shifting ideas about mental states and the introduction of the confusing diagnosis of partial insanity assumed critical importance. But the Enlightenment played a curious role as well by narrowing the limits of exculpatory madness. So, too, did the republicanism of Hamburg which relied on the principles of non-expert opinion

and individual responsibility for its very *raison d'être*. Thus, neither medical or legal discourse nor the sentiments of humanitarianism and the reforms it fostered determined the outcome of trials in which insanity was alleged. It was rather the interplay of these discourses with local particularities in the character of the Enlightenment and of political culture in Hamburg that decided the fate of criminals.

14

'One of the Best-Known Identity Crises in History'? John Stuart Mill's Mental Crisis and its Meanings[1]

Chandak Sengoopta

Nineteenth-century intellectuals often fell victim to a strange kind of physical and mental collapse which later generations would call a nervous breakdown. Twentieth-century historians have laboured to explain the psychological and sociocultural contexts of these crises.[2] This chapter seeks, in a very modest way, to focus on a single, famous 'case' of a mental crisis and questions the utility of using psychiatric theories popular at any one time to explain such events. While the aetiology of mental disorders is a subject of undoubted importance for clinicians, there is little need to include particular biological, psychiatric or psychological theories in the historian's toolkit. Does it matter to the intellectual historian whether Nietzsche suffered from the neuropsychological sequelae of syphilis or some other kind of mental disorder? I would suggest not, even though some medical historians – especially those whose allegiances and interests are more medical than historical – are likely to disagree.

The 'case' of John Stuart Mill (1806–73), I would argue, illustrates the futility of 'historical psychology' and retrospective diagnosis. Mill's 'mental crisis' is mentioned in every account of his life, largely because Mill himself accorded it great importance in the development of his mature thought. His chapter on the crisis in his posthumously published *Autobiography* is the single major source for whatever we know about the event. Mill scholars and biographers have interpreted this testimony in widely different ways, and most have been anxious to identify *the* cause of the crisis with the help of whatever psychological theory was fashionable at the time. The credibility of those diagnoses, as we shall see, has tended to fluctuate with the market value of the theories, and while there could, of course, be no 'plain' history transcending *all* theory, Mill's fate demonstrates the dangers of historians playing at being psychiatrists or psychologists playing at being biographers. This essay, I should emphasize, does not purport to be a comprehensive

critique of psychohistory or its cognates; nor, needless to say, does it seek to contribute to studies of Mill's thought except in very peripheral ways.

The 'case history'

Mill's breakdown began when he was around 20 with 'a dull state of nerves, such as everybody is occasionally liable to; unsusceptible to enjoyment or pleasurable excitement'. He had hitherto lived and thought, he admitted, as a follower of Jeremy Bentham and his sole aim in life had been 'to be a reformer of the world'.[3] Initially, Mill had lived happily with this objective, working at India House and, in his spare time, writing, debating, teaching his brothers and sisters, contributing to radical journals and editing Bentham's voluminous manuscript, *The Rationale of Judicial Evidence*, which was finally published in five hefty volumes. He worked enormously hard, of course – 'a close look at his labours in these years', says a noted Mill scholar, 'is enough of itself to bring on a state of depression in the viewer' – but there is no evidence to conclude that he felt overwhelmed or strained.[4]

During the 'dull state of nerves' in 1826, Mill asked himself quite casually whether the immediate institution of all the intellectual, social and political reforms that he was working towards would make him happy. It was a rash thought, for

> an irrepressible self-consciousness distinctly answered, 'No!' At this my heart sank within me: the whole foundation on which my life was constructed fell down. All my happiness was to have been found in the continual pursuit of this end. The end had ceased to charm, and how could there ever again be any interest in the means? I seemed to have nothing else to live for.[5]

The hitherto unquestioned confidence of the young reformer collapsed completely. His agony was acute but it remained internal. Mill carried on with his usual work 'mechanically' and the well-trained mind did not outwardly falter. (The crisis has in fact been called his 'secret breakdown.'[6]) His heart was no longer in it, however, and his soul – not that Mill would have used such a mystical expression even at this low point – had lost its way. Even his revered father's wisdom now failed to impress. 'Even if he could be made to understand it, he was not the physician who could heal it . . . If I had loved any one sufficiently to make confiding my griefs a necessity, I should not have been in the condition I was.'[7]

The more Mill thought about it, the more it seemed to him that his present ills were to be ascribed to his single-minded training in analytical skills. 'I now saw', he recorded, 'that the habit of analysis has a tendency to wear away the feelings: as indeed it has when no other mental habit is cultivated, and the analysing spirit remains without its natural complements and

correctives.'[8] Instead of speeding him to success, glory and fulfilment, his rigorously intellectual training had drained him of the emotional strength required to attain true goodness or greatness.[9] Mill's crisis, in other words, could be seen not merely as an unhappy episode in his own life, but as one recent scholar has put it, as 'the breakdown of the Benthamite mind itself' – a collapse of the linear and impersonal framework of Bentham's rationalistic universe.[10]

Mill was to languish under this dark cloud for about six months and then, when he had almost despaired of recovering, there was a sudden break. Purely by chance, he read the French dramatist Jean François Marmontel's *Mémoires d'un père* and was moved to tears by the description of the death of the author's father and his determination, at a tender age, to take care of his bereaved family with all the love and devotion of the father. With the unexpected tears came a lightening of the heart. Mill realized that he was not devoid of all feeling after all: 'I was no longer hopeless: I was not a stock or a stone.'[11] Slowly, and without ever denigrating the intellect, he now began to cultivate his emotions through poetry and music, emerging from the spartan world of utilitarian orthodoxy into a sunnier universe governed by Harriet Taylor, for a long time his intimate friend and subsequently his wife. The crisis, Mill believed, had transformed his whole view of life; scholars and biographers agree, some of them adding that it marked the end of James Mill's reign over his son and the inauguration of Harriet Taylor's.[12] Mill would be afflicted with depression and ennui periodically for the rest of his life, but never again would he acknowledge sinking as low as he had at the age of 20.[13]

This is the essence of the story as recounted in Mill's *Autobiography* and there are more than a few lacunae in it, some remediable, others less so. To take the most major omission, Mill makes no mention of the fact that he was brought down by a far more severe crisis ten years later. In 1836, as his father was dying, Mill collapsed and this time he could not carry on with his work. He had to take a long leave of absence from India House and developed nervous tics that persisted for life. He was sent to Brighton and later, after his father had died, to the continent by his physician. He even thought of quitting his India House job.[14] It did not come to that, but medically speaking, the 1836 episode was a far more worrying breakdown than the earlier one; yet, it was his youthful passage through confusion and dejection that Mill himself found more significant and it is on that episode that biographers have understandably concentrated over the years.

Quests for a diagnosis

John Mill, it is well known, was used by his father James Mill (1773–1836) as an educational guinea-pig. James Mill's dream was to rear *the* intellectual, political and moral heir for himself and his master, Jeremy Bentham. For

Benthamites, 'education was all-powerful in the formation of character'. Mill Senior had declared that

> as soon as the infant, or rather the embryo, begins to feel, the character begins to be formed; and that the habits, which are then contracted, are the most pervading and operative of all. Education, then, or the care of forming the habits, ought to commence, as much as possible, with the period of sensation itself; and at no period is the utmost vigilance of greater importance, than the first.[15]

Young John Stuart was never sent to school but educated at home by his father. Even the simplest description of that education arouses horror in the contemporary breast, and predictably enough, Mill's childhood experiences have appeared to offer the key to his later breakdown.

Despite their fondness for analysing 'great men', and in spite of the fact that the young Sigmund Freud had translated some of Mill's essays into German, it was only around the middle of the twentieth century that psychoanalysts put Mill on the couch.[16] A pioneering article was published by a Chicago analyst, A. W. Levi, in 1946. Levi found the all-important clue to Mill's crisis in his response to Marmontel's tale of a son resolving to take the place of his dead father. Mill, the psychoanalytic interpretation assumed, must always have subconsciously resented being under the iron fist of his father. That dammed-up resentment – but not the associated emotions – finally burst forth at the age of 20, bringing about the breakdown. The cure was abreaction: the release of the repressed emotions, which commenced with the tears that flowed after reading Marmontel. 'Dimly at first, but with greater clarity as times [*sic*] goes on, John Stuart Mill begins to recognize that the very existence of his father is a threat to his own ego. And in an instant there flashes across his mind the idea of his father's death! Here is the only conclusive solution to his problem!'[17]

The psychoanalytic approach to Mill and his relations with his father have not been confined to such observations. Bruce Mazlish's mammoth work *James and John Stuart Mill: Father and Son in the Nineteenth Century* (1975) is far more than a study of Mill's mental crisis; it is a psychobiography of the two Mills, an investigation of nineteenth-century father–son conflicts, and, ultimately, a rousing defence of the genre of psychohistory.[18] Since this brief chapter cannot hope to do justice to those grander themes, let us concentrate solely on Mazlish's interpretation of Mill's crisis. Endorsing the essence of Levi's analysis of Mill's crisis, Mazlish agrees that it was no more than an expression of long-repressed oedipal conflicts. The following quote should be sufficient illustration of Mazlish's approach to the theme:

> Did young John say to his father, as so many of our own children (if we observe them closely), 'Let *me* get into bed with Mommy, and you move

> out'? Did he masturbate, for example, and was he severely reprimanded and perhaps threatened?[19]

Once one has attained this kind of insight, going back to traditional history, declares Mazlish, 'is like listening to music on single-track after one has heard stereo'.[20]

Psychoanalysis, however, was not the first psychological lens through which Mill's crisis was viewed. The earliest 'scientific' explanation of the crisis was that of the pioneering psychologist Alexander Bain (1818–1903), who knew Mill well and was a familiar figure in radical utilitarian circles.[21] Bain was impressed as well as appalled by the rigours of Mill's childhood education. That Mill could bear such strain for so long, said Bain, attested to the power of his brain and the robustness of his constitution. The challenges the boy had faced so indomitably, however, had sapped his inner strength. 'That his health suffered, we have ample evidence . . . That his mental progress might have been as great with a smaller strain on his powers, I am strongly inclined to believe,' asserted Bain. His disapproval of James Mill's pedagogical regime, however, had little to do with the emotional and existential questions that Mill himself had highlighted.

In all his psychological work, Bain argued for the synthesis of physiology and psychology. He had received some training in nervous physiology at William Sharpey's laboratory in University College London and fully accepted the pertinence of physiology to the study of the mind. 'The time has now come,' he had declared in his celebrated 1855 treatise *The Senses and the Intellect*, 'when many of the striking discoveries of physiologists relative to the nervous system should find a recognized place in the science of mind'. In his other famous work *The Emotions and the Will* (1859), he had emphasized the physical basis of feelings and emotions.[22] There was, to be sure, much more to Bain's psychological doctrine than physiology and his physiological concepts were not entirely original or 'startling', but as the psychologist and historian L. S. Hearnshaw argued in 1964, Bain's was the first 'systematic physiological psychology based on a reasonably sound physiology'.[23] Roger Smith sees Bain as working towards 'non-mentalist theories of efficacious human activity' and translating the moral concept of effort into 'physiological terms of motor discharge.'[24] Unsurprisingly, therefore, Bain interpreted Mill's crisis in neurophysiological terms.

Bain was appalled not only by the quantity of material that the young Mill was expected to imbibe, but also by the 'rapid and unbroken' succession in which James Mill had moved his son from one topic of study to another. This was bad even from a narrowly pedagogical viewpoint, because the quick moves impeded the physiological learning process. 'The nervous currents when strongly aroused in any direction tend to persist for some time,' Bain explained. 'In the act of learning, this persistence will count in stamping the impression; while part of the effect of a lesson must be lost

in hurrying without a moment's break to something new, even although the change of subject is of the nature of relief.'[25] Impressive as Mill's education was, therefore, its value had been impaired by Mill Senior's belief that the young brain was a virtual sponge that could take in as much intellectual nourishment as could be provided.[26] This contravention of the laws of nervous physiology had reduced the value of Mill's education and may well have damaged his developing nervous system. James Mill could have produced an even greater – and definitely healthier – prodigy with a more focused regimen of study.

Bain's discussion of Mill's eventual crisis was also neurological. He criticized the account in the *Autobiography* because it portrayed the event as 'purely spiritual or mental; the physical counterpart being wholly omitted.' He acknowledged that Mill had stated that the crisis had commenced during a 'dull state of nerves', but that was not enough. Mill had simply failed to acknowledge the baneful consequences of working too hard. 'Nothing', Bain sighed, 'could be more characteristic of the man. There was one thing he never would allow, which was that work could be pushed to the point of being injurious to either body or mind.'[27] Bain proceeded to a detailed pathographic portrait of Mill, the minutiae of which need not detain us here. The fundamental point to grasp is that Bain eliminated – or, at least, downplayed – the spiritual and existential dimensions of Mill's crisis and turned it into a typical late nineteenth-century nervous breakdown.[28] 'The nervous system', observed Bain, 'was habitually kept at a high tension all through; this cannot be done for nothing'.[29] It was not Mill's crisis that was surprising, but the fact that he had somehow managed not to become a complete invalid, despite all the tortures he and his father had imposed on his nerves.

In the later nineteenth century, 'overwork' had become the standard explanation for cases of mental breakdown.[30] Mill's case, Bain argued, was no different. It was 'beyond all reasonable doubt' that the mental dejection chronicled by Mill in the *Autobiography* was caused by physical factors, the chief of which was 'over-working the brain'.[31] Mill was fortunate, opined Bain, that the major symptoms of the crisis were relieved in a few months, but it was clear that his nervous system was permanently weakened as a result.[32] Bain's 1880 account was typical of the new concern with nerves, nervous energy and nervous exhaustion that marked the latter decades of the nineteenth century; in this new neurocentric age, and in the hands of a neurologically oriented psychologist, it was virtually inevitable that Mill's spiritual and intellectual crisis would be transmogrified into the malady of the era: the nervous breakdown. This overwork theory, however, was unceremoniously rejected by the psychoanalysts: simple overwork, without some underlying 'neurotic tendency', could not precipitate a breakdown. 'No one, I think, has ever suffered a nervous breakdown from mere overwork,' asserted A. W. Levi in 1945, echoing, whether knowingly or unwittingly, Mill's own firm conviction.[33]

Instead of a diagnosis: a reactionary conclusion

A closer look at Mill's breakdown suggests that it meant something rather different to him than to the many who have since tried to explain it, and such an examination also shows how interpretations of the crisis have themselves changed with shifts in medical and psychological fashion. The only contemporary witness who recorded the details of the crisis was Mill himself, but he did so more than a quarter of a century after the event.[34] Apart from the obvious difficulties of providing an objective first-person account of a personal crisis, this one, therefore, is also potentially vulnerable to a host of other dangers, ranging from common forgetfulness to retrospective falsification. None the less, we shall never find a more direct account of the breakdown and must take it, distortions and all, as our primary text.

Let us, like the psychobiographers, start with Mill's fabled education under the supervision of his father. It is salutary to begin with the reminder that Mill was, on the whole, delighted with the education he received and although he eventually expressed reservations about the way he was brought up by his father, he always cherished the learning he obtained in childhood. As the great Mill scholar John Robson warned, 'anyone feeling tempted to tears or rage about Mill's education should glance . . . at other personal accounts of childhood education in the nineteenth century, which are marked by woeful memories of neglect, boredom and very often brutality, especially at public schools'. The opening pages of Mill's autobiography, on the other hand, tell us repeatedly of the delight and enjoyment he obtained from his education. Moreover, Mill's childhood education, although unusual, was by no means unique: apart from Bentham himself, Thomas Babington Macaulay and Mill's own siblings underwent similar training.[35] We should, in short, be chary of explaining Mill's breakdown or any deficiencies in his mature character as simple and direct consequences of undue intellectual pressure at an early age.[36]

In the first three chapters of the *Autobiography*, Mill described how he was educated – indeed, he justified the very notion of chronicling 'so uneventful a life' by the desirability of leaving 'some record of an education which was unusual and remarkable' and it is from these that we derive the bulk of whatever we know about James Mill's pedagogical project.[37] It commenced with classical Greek when John Stuart was three years old. The pupil was moved to Latin, algebra, chemistry and differential calculus at the age of eight, and began to learn logic and political economy at twelve. As he learnt his subjects, he was charged with teaching them to his eight younger siblings.[38] The target was set very high – 'my father, in all his teaching, demanded of me not only the utmost that I could do, but much that I could by no possibility have done'. Nevertheless, Mill emphasized, his had not been 'an education of cram . . . Anything which could be found out by thinking, I never was told, until I had exhausted my efforts to find it out for

myself.'[39] The boy was protected from association with children of his own age to prevent him from 'the contagion of vulgar modes of thought and feeling'. James Mill was an agnostic, so John was also protected from religion. Holidays were banned, although the boy was not denied some leisure every day.[40]

Mill later averred that his education gave him 'an advantage of a quarter of a century' over his contemporaries, although he admitted that he had been taught more 'to *know* than to *do*'.[41] We approach the subsequent chapter on the crisis, then, with certain preconceptions, some of which that chapter reinforces. Mill himself considered his crisis to have played a valuable role in his mental development – the full chapter heading is 'A Crisis in My Mental History: One Stage Onward'. Interestingly, Mill, who had thought long and hard about the matter, did not implicate his workload *at all* in the causation of his crisis. More importantly, *none* of the 'symptoms' he recorded could actually be accommodated in Bain's reductive neurological template. However, instead of trying to understand what Mill had experienced, Bain simply assumed that he *must* have had a nervous breakdown, which *must* have been brought about by overwork. So much for the biological psychology of the late nineteenth century.

What, however, of psychoanalysis born supposedly in reaction to the soulless psychology of nerves? We need to see the works of psychohistorians like Mazlish as expressions of the expansive and optimistic style of psychohistory associated with the historian William Langer's identification of psychology as 'the next assignment' for historians to tackle and the psychologist Erik Erikson's declaration that history is too important to be left 'entirely to non-clinical observers and professional historians who often all too nobly immerse themselves into the very disguises, rationalizations, and idealizations of the historical process from which it should be their business to separate themselves'.[42] Psychohistory claimed to illuminate what historical actors tried to conceal and their tool in this task was the putatively rational, objective and scientific theory of psychoanalysis. Mill's case was redefined from a nervous breakdown into an oedipal crisis solely on the strength of Freudian theory and one needs to have total faith in psychoanalysis to accept it. Otherwise, it seems to be as antiquated and futile as Bain's all but forgotten neurological explanation.

I would, therefore, argue that 'stereophonic' analyses of Mill's crises have failed and we would be better off with what Mazlish might call 'single-track' analyses. Instead of trying to psychoanalyse long-dead people, we would do well to stick to the surface of their narratives and seek to identify *what* meanings those events might have had in their lives and *how* they – and others – gave meaning to those events in their narratives. What was it like for Mill to go through the crisis, what did he think it did for him and why did *he* think he suffered from the breakdown?

So much has been written about every aspect of the thought and influence of John Stuart Mill that one is in danger of forgetting that in the 200 years

since his birth, there have really been only two comprehensive biographies of the man. The reasons are not relevant to the argument here, although one notes in passing that there are few straightforward biographies of many 'great' philosophers – think of Kant, Hegel or Locke.[43] References to various aspects of Mill's upbringing and early experiences – most notably his putatively monstrous education – are, of course, common in all works on Mill but rarely accompanied by any deep exploration. One rare analysis is to be found in the recent biography of Mill by Nicholas Capaldi. For Capaldi, the crisis 'marks the transition from Mill the radical proselytizer of Benthamism to Mill the Romantic synthesizer. It marks the discovery of his soul and of the woman who would become the great influence in his life when his father's influence waned.'[44]

Capaldi's well-informed but wholly unpretentious and 'unscientific' reading gives us everything that truly matters about Mill's crisis. The effects and contexts, in this case, are far more interesting and important than the causes. We do not, of course, have any reason to doubt that Mill was a very hard worker, but do we have any reason or need to assume that it had a causal relationship to his crisis? Similarly, it would be wrong to ignore the awe with which the young son had regarded his father or to underestimate the emotional difficulties he may have experienced in outgrowing it. Must we, however, view those difficulties through psychoanalytic spectacles? For those who hanker for 'scientific' method, it may be depressing to contemplate that the Oedipus complex is now almost as much a historical curiosity as the idea of 'nerve force', but that, sadly, is the case outside certain sections of the humanities. One could, of course, continue to use psychoanalysis as a hermeneutic method, but it is hard to combine that approach with any strong claim for the superiority of psychoanalytic interpretations over other readings.

Instead of building theoretical castles in the air, we would, I am convinced, do better to stick to the straight and narrow pursuit of meanings, connections, consequences and stories at a level of sturdy but socioculturally sensitive common sense. In his *Social History of Madness* (1987), Roy Porter emphasized that in exploring individual accounts of insanity, he was not guided by psychiatric or psychoanalytic aims. It was not the remit of his book to 'decode what mad people said, wrote and did in the light of some or other psychiatric theory . . . '[45] The snarks of neurophysiology or psychoanalysis, Porter knew well, are all Boojums – historians vanish as 'softly and suddenly' as Lewis Carroll's unfortunate Baker when they flirt with such sirens, and we should emulate Porter's caution when tempted to produce 'causal' explanations of the agonies and ecstasies of historical figures.

15

Murder by Hypnosis? Altered States and the Mental Geography of Science

Emese Lafferton

Something went terribly wrong that September evening in 1894, at the Salamon castle in remote north-eastern Hungary. It began as usual. Countess Szirmay, Baron Jungfeld and several members of the Forgách family were present, the dinner table was elegantly laid and excitement was mounting about the performance that was to come. For Ella, the 22-year-old daughter of the landowner Tódor Salamon, however, this night was different from others. She was thrilled by the fact that Dr Vragassy, chief doctor of the Viennese voluntary ambulance association, was expected to arrive just for the event; he was, after all, a trained eye to judge her performance, another possible convert who could enhance her already splendid reputation within the country and beyond its borders.[1]

The audience took their seats at 7:40 pm and the show began. It took about 12 minutes for Ella to fall into a hypnotic sleep. Franz Neukomm, the successful German machine-factory owner and well-digger from Versec (in southern Hungary) – widely reputed for his miraculous healings and séances, and certainly a charismatic magnetizer with his long hair, dark beard and piercing eyes – then declared that he would attempt an especially curious experiment.

Curious demonstrations had hardly been missing from Neukomm's repertoire. Earlier in the same year of 1894, in front of a similar audience of aristocratic family members and friends, Neukomm had ordered the hypnotized Ella to find a piece of precious jewellery lost by a female relative weeks before. Following his order, Ella's obedient soul left her body, seeking and eventually finding the jewel in a crack in the staircase. During another such telepathic search, her soul travelled several kilometres, entering another castle. Ella suddenly began to relate the extraordinary events she 'saw' in that distant place: a thief entered, sought valuables, collected three pieces of silverware, then hid under the staircase. Eventually he rushed out of the castle, concealing the silver in a haystack.[2] Hearing this wonderful account, the audience rushed to their carriages and rode to the castle, found the silverware was indeed missing, and then recovered it from the haystack. In

other séances, Ella allegedly gave information about the perpetrators of a murder that had for some time troubled the local police.[3] No wonder that everyone was fascinated with the master-magnetizer's extraordinary power and the girl's wonderful capabilities.

This time, however, Neukomm was attempting a different sort of telepathic experiment. One of his brothers lived in Versec and had been vomiting blood for a long time. His doctors were unable to give a diagnosis, hesitating between tuberculosis and stomach-bleeding. Neukomm was convinced of the former, but now was looking for confirmation from Ella. In an authoritative voice he told the girl that they were in Versec. To guide Ella's searching soul, Neukomm explained in which house his brother lived and in which room he was to be found. Once Ella said she was there, Neukomm requested a detailed description of his brother's condition.[4]

Ella found the man seriously ill. In the words of Dr Vragassy – as a professional medical man, considered to be the only reliable eyewitness –

> What followed was really incredible. The medium began a scientific description of the lungs, giving a minute account of their diseased condition with technical particulars which even an ordinary doctor would not give and which might only be expected from an experienced specialist. With a full command and correct use of technical expressions she … declared that the prognosis was very unfavourable, as against that kind of disease medical skill was powerless.

Elsewhere, he reported: 'She named the diseases as oedema pulmonum acutum hydropicum suffocativum.'[5] When Neukomm finally asked Ella about the probable outcome of the disease, the girl slowly mumbled: 'We have to be prepared for the worst!' Having uttered these words, Ella shrieked and collapsed. All medical attempts to bring her back to consciousness failed and, after some time, she died.

This story, from a distant castle in deep provincial Hungary, speedily reached all major European cities (and even America), stirred public feelings and muddied professional-medical waters concerning the wonderful – and perhaps all too dangerous – phenomenon of hypnosis. Major newspapers covered the story via their Budapest and Viennese correspondents.[6] Prominent medical mandarins, legal professionals and social commentators of the day expressed diverse views, demonstrating the intricate social ramifications of hypnotic practice. The heated interest in the case was due partly to its fascinating details and partly to its uniqueness: it was claimed by many to be the first case of death caused directly by hypnosis.

What followed in the Hungarian press was symptomatic of *fin-de-siècle* society and culture. The daily press sensationalized the story for public consumption, enticing readers with titillating headlines including 'The Fatal Hypnosis', 'Playing with the Brain', 'Death under Hypnosis' and even

'Murder by Hypnosis'. The case proved highly complex and subsequent reports were full of surprising twists, supplied by novel information received each day. A court investigation was launched – not by the family, who stood by Neukomm, but by the authorities to clarify Neukomm's responsibility in the death.

The case intensified concerns over the spiritualist séances then flourishing in the country. Within two months and as a direct consequence of the case, the Hungarian Ministry of Interior, at the recommendation of the National Public Health Council, passed a resolution that greatly limited the practice of hypnosis, in the country. It prohibited anyone unqualified for medical practice from using it and strictly defined the circumstances under which even doctors could employ hypnotism. Strikingly, in a crucial decision unprecedented in Europe, it allowed for the medical use of hypnosis *only with the aim of a cure.*[7] This prohibited not merely staged demonstrations but also academic research experiments in hypnosis.[8]

Ella's case occurred at a time when hypnosis was omnipresent in society, generating an intense cultural interchange between diverse spheres of *fin-de-siècle* intellectual and social life. With its power to fascinate, hypnosis made its way into provincial village households and aristocratic castles, urban psychiatric clinics and medical societies, and public entertainments as well as criminal courts. In Europe, learned journals were founded for the scientific investigation of hypnosis, and semi-professional and lay societies organized for psychic research.[9] Newspapers sensationalized hypnotic cases and literary works on hypnosis were widely read: George du Maurier's best-selling novel *Trilby*, in which the sinister musician and hypnotist Svengali uses his powers to enslave and exploit the heroine Trilby, sold prodigiously all over Europe and America.[10] *Trilby* was published the same year of 1894; its hypnotist-protagonist was a Hungarian Jew.

Here I outline a map of scientific and social exchanges in order to recover how hypnosis was embedded in intricate social relations. As a phenomenon, hypnosis cannot be understood separately from the national and international contexts of medical legislation and laboratory research, forensic practice, the mass media and the social anxieties that surrounded it. I propose both real and symbolic geography (by which I mean the complex web of relations interweaving the social and cultural space) as keys to the interpretation of Ella's story. Such a perspective emphasizes the fact that hypnosis necessarily occurred in a particular locality and highlights the commerce between different locations and the costs of exchange. The Salamon castle in provincial Hungary was the epicentre of the telepathic events and Ella's death, as well as of the consequential waves of influence the case exerted. Media sensationalism swiftly propelled the story across national and social boundaries, compelling European psychiatric and medical mandarins and social commentators to respond. Reconstructing these influences enables us to reconsider notions of centre and periphery in nineteenth-century Europe

and renders visible the relationships between geographic and symbolic areas of statehood and science, expertise and lay practice, 'central' and local medical interests.

To uncover the grave social and professional anxieties provoked by hypnosis, I first explore aspects of the intricate relationship between medical and laboratory hypnotic research, lay hypnosis and sensationally reported forensic cases like Ella's. I then demonstrate how psychiatrists utilized their experimental results and knowledge in famous forensic cases. These indicate the controversial nature of such scientific experimentation; show how scientific and lay hypnosis remained alarmingly intertwined in the period; and render visible the painful lack of professional unity in areas where the social utility of psychiatric expertise could have been bolstered.

Finally, we will revisit Ella's story from the perspective of European expert opinions and the local court investigation. This will shed light on the *locality* and *status* of medico-legal expertise. The local court resolution reveals the complexity of the relationship between European versus national versus local knowledge and interests. The diversity of European expert speculations concerning the role of hypnosis in Ella's death, and the powerlessness of European or even Budapest centres of expertise to define the outcome of the court case, lend an unquestionable centrality and importance to its local, provincial resolution.

Dog, vinegar and chicken thigh: medical research on hypnosis

At the centre of the controversy around Ella's case stood the question of the magnetizer's responsibility for the girl's death, an issue inseparable from claims about the physically destructive and morally harmful potential of hypnosis and its possible malicious or negligent use. How great was the power of the mind over the body? What were the limits of the power of one person's mind over another's? If crime was committed under hypnosis, who was responsible: the hypnotizer or the hypnotized?

Precisely these questions were scrutinized in many contemporary psychiatric laboratories in the 1880s and 1890s, when hypnosis was a fashionable research area in European medical circles. An abundance of publications on the topic in English, French and German investigated the nature of hypnosis and contested its overall significance and possible uses. Prominent places of research, such as Charcot's psychiatric ward in Paris or Bernheim's in Nancy, attracted students and practitioners from all over Europe and America, who migrated to these centres to witness the spectacular hypnotic phenomena.[11]

In Hungary, the first systematic psychiatric research on hypnosis was undertaken at the new psychiatric clinic attached to the Department of Mental Health and Pathology at the Budapest Medical Faculty, established in 1882 and headed by the eminent doctor Károly Laufenauer (1848–1901).[12]

The results of these experiments were published in 1895 as the first book on the subject in Hungarian by Laufenauer's young assistant, the neuropathologist Károly Schaffer (1864–1939).[13] Laufenauer and Schaffer focused mostly on physiological phenomena, using hypnosis in the laboratory experimental set-up as a *tool* to investigate the functioning and intricate interrelation of the nervous system, senses and muscular reflexes. The experimenters found the body open to the power of suggestion, obediently revealing its inner mechanisms. The reflex functions, the entire sensory mechanism, movement functions and the higher associative functions – all could be affected by hypnotic suggestion, producing amazing effects. These included cramps, contractions, seizures, paralysis, bleedings, sweating, raising the body temperature, as well as sensory illusions, blindness, deafness, hallucinations, memory delusions and mood changes.[14]

Outside the psychiatric clinic, the most important doctor investigating hypnosis was the internist and neurologist Ernő Jendrássik (1858–1921) of the Budapest Medical Faculty. In his hypnotic research, undertaken between 1887 and 1892, Jendrássik studied similar physiological and organic phenomena. He also reported 'branding' experiments on one of his hystero-epileptic patients, Ilma: he touched her skin with an ordinary cold object which she was told was red-hot, producing blisters and serious burn wounds with scabbing and secretion.[15] Jendrássik considered these 'burning experiments' the 'most exciting' ones, although he reported the serious pain they had caused.[16] Such complicated and highly spectacular experiments demonstrated the extent to which psychic influence could generate objectively observable organic changes.

To investigate the power of suggestion itself, Jendrássik conducted 'behavioural' experiments that explicitly focused on free will and the forensic implications of hypnosis. To prove that a hypnotized person did not retain even a shred of free will, Jendrássik suggested to Ilma that she was a dog, and thus she went down on all fours and barked.[17] He fed another hypnotized patient a raw potato, saying that it was a roast chicken thigh. The patient 'ate with great appetite'.[18] Hypnotized patients drank vinegar as if it were the most delicious Tokaji wine. Under hypnosis, Ilma was asked to write love-letters to a doctor who, she was told, had murdered her father. She wrote them without hesitation. In another experiment, he suggested that she murder one of her doctors, at which she crept up behind him and tried to stab him with a roll of paper she had been given.[19] Jendrássik concluded that his experiments proved the lack of free will in the hypnotic state: it was the overriding will of the experimenter that directed the actions of the hypnotized person.

Unlike experiments that produced observable or verifiable organic changes in the body, such behavioural experiments challenged notions of scientific credibility and objective proof. Many doctors in the audience questioned their authenticity, some of them going as far as to suggest that the

experimenter was simply fooled by the manipulation and simulation of shrewd hysterical patients of dubious character.[20] Psychiatrists who successfully conducted similar experiments all over Europe, however, remained convinced that the experiments were genuine and provided scientific evidence of the overriding power of the hypnotizer over the hypnotized.

Medical experimenters repeatedly emphasized the objectivity and highly controlled conditions of their experiments. Controversially, however, there were few differences between many of these demonstrations and those of lay magnetizers. Neukomm, for instance, successfully produced muscle contractions, anaesthesia and a large burn wound on Ella's hand with a cold hairpin; made her forget her mother-tongue; and regressed her to earlier ages of childhood.[21] And like his medical counterparts, Neukomm also used hypnosis for treatment.[22] Such marked similarities notwithstanding, doctors drew a sharp divide between legitimate, scientific experimentation and illegitimate hypnotic performances.

Also striking is the fact that the psychiatric profession in Europe was itself divided on how to apply their experimental results and knowledge as experts in the court arena and the wider public sphere. Many doctors across Europe believed that experimental knowledge supported the claim that 'every conceivable crime' may be committed on or by a hypnotized person who 'may not know anything about it later on'.[23] Others, equally widespread, denied that hypnosis completely overruled free will, and strictly distinguished real cases from clinical laboratory experiments. The latter, they claimed, only proved the carrying out of 'fictitious crimes' with paper rolls instead of real knife, or harmless dust instead of poison. In the 'real situation', a 'faint consciousness' was always retained.[24] The applicability of hypnotic experimental results to real criminal cases was therefore contested even within the community of experimenters who attested their authenticity and scientific standards.

Wider professional and social anxieties

In spite of divided expert opinion, hypnosis (as well as the psychiatric expert) figured centrally in criminal cases that captured both the public and medical imagination; extensively covered by the press, these were recycled in learned articles and books on hypnosis. At the nexus of psychiatric expertise, crime and the mass media, these cases revealed deep social anxieties.[25]

Persons susceptible to hypnosis were seen as highly vulnerable to the malicious intentions of magnetizers and criminals, so much so that Liégeois seriously proposed a kind of 'moral preventive inoculation': the testing of everyone's susceptibility to deep hypnosis, with the most susceptible then being told under hypnosis that nobody will ever be able to hypnotize them again.[26] Such preventive measures were, practically speaking, impossible, and numerous examples proved that crimes were committed on and by

hypnotic subjects. Famous trials showed that women especially were seen as potential victims of 'offences against morality' in assaults carried out 'against her will and without her consciousness'.[27] One case, that of Ceslav Lubicz-Czynski, tried in Munich, received much international public attention.

Originally a private teacher of French in Cracow, Czynski gave public lectures on hypnosis in Dresden from 1893 and advertised his hypnotic cures. The 38-year-old virtuous, deeply religious and impressively rich Baroness Hedwig Zedlitz sought help from him for her nervous stomach problems and headaches. During the hypnotic treatments, however, Czynski started to proclaim his love for the Baroness, strategically kindling her 'great disposition' to spiritualism and occultism, and cunningly using sophisticated literary and religious symbolism to win her heart. He achieved such a power over her that she eventually 'completely yielded to him'.[28] Czynski even staged a mock marriage ceremony. Indeed, the Baroness demonstrated strong attachment to Czynski even in the initial period of the investigation, after her father and brother had brought him to court.

Expert opinions varied. Some termed Czynski a 'hypnotizing pathological swindler' and regarded the Baroness's love as hypnotic and not freely willed .[29] Others believed that hypnosis played no part in the case, and that the woman had indeed fallen in love.[30] Czynski was in the end acquitted from the charge of offence against morality, but was punished with three years in prison for the sham marriage.[31] His lower social background made the situation even more unsettling, not only undermining morality by exploiting female suggestibility as a form of gendered weakness, but also threatening the social order.

Other cases suggested that any man possessing the power of hypnosis could be a threat to female virtue, whether he was a beggar, a bicycle manufacturer or a photographer.[32] But most common suspects were from the varied ranks of healers, magnetizers, magnetopaths, nature-doctors and quacks who advertised their services in newspapers. Reports of abuses by such practitioners circulated in the daily as well as the learned press.[33]

In such cases, hypnosis appeared as a potential threat to the entire moral and contractual basis of bourgeois society. Sinister hypnotists used their powers to seduce fair girls, to make respectable wives abandon their marriage agreements or to induce abortion by suggestion to conceal illicit affairs.[34] The hypnotic state could be exploited to 'get property illegally', to make people 'sign promissory notes, deeds of gift' and to steal the intellectual property of others.[35] Hypnosis also breached rights when minors were involved without parental authorization or knowledge.[36]

Experts suggested that even legal procedure was endangered: the memory of witnesses might be manipulated not only by lay evil-doers, but by legal investigators themselves, through their liability to influence and falsify the memory of confessors and witnesses by their own (conscious or unconscious) questioning. The French guru of medical hypnosis in Nancy,

Hyppolite Bernheim's best example of this was the famous Hungarian Tiszaeszlár case in 1882, when the local Jewish community was accused of the ritual murder of a young Christian girl to obtain blood to mix into the unleavened bread of the Passover. The key 'eye-witness' was a 15-year-old Jewish boy who claimed to have seen with his own eyes the girl's throat being cut by members of the Jewish community, and accused his own father of complicity. For Bernheim, who believed that the information was obtained from the boy through the highly questionable interrogation by a 'commissioner of public safety' (an 'expert in extorting confessions'), the case proved the mind's susceptibility to suggestion and its possible exploitation in the legal domain.[37] This case directly inspired Bernheim to conduct a wide range of clinical hypnotic experiments in which the doctor suggested certain facts and crimes to patients who fully incorporated even the most morally controversial ideas. He called these 'retroactive hallucinations'.[38]

These cases captured the public imagination. The extensive press coverage and the considerable lay and expert comment they generated certainly helped to create something of a craze around hypnosis, suggesting its omnipresence. Albert Moll, a Munich doctor and author of one of the most comprehensive and synthesizing contemporary books on hypnosis, observed that 'laymen endeavour to ascribe to hypnosis anything they find very peculiar ... unpleasant, or that they cannot quite understand'.[39] In other words, Moll saw hypnosis itself becoming the suspect for anything puzzling or disruptive to social order:

> Just as some strong perfume used to be considered the overpowering agent in cases of criminal assault in the train, so nowadays hypnosis is unjustly blamed; though we must at once admit that such assault may be, and frequently have been, made during hypnosis.[40]

The press played an enormous part in fuelling the hypnosis panic. When the case of Johann Berchthold, a Munich triple-murderer, came to court in the mid-1890s, a portion of the Munich daily press swiftly took the investigation into its own hands. One of the newspapers invited anyone who had any information on the case to report it to the editorial staff, 'promising the strictest confidence'.[41] Based on information thus acquired, the newspaper 'declared … before the magistrates had completed their preliminary investigations, "that there was practically no doubt that Berchthold was the murderer" '.[42] Soon further witnesses came forward and gave evidence on oath with the most apparent contradictions, including seven who admitted to committing some of the murders ascribed to Berchthold. Forel identified all these phenomena as 'falsification of memory'.[43] The case prompted warnings about suggestibility in legal cases. The press itself epitomized hypnotic power, capable of generating a 'psychical epidemic' on the social scale .[44]

Sensationalized criminal cases, particularly those involving healers, also threatened the contractual basis of medicine between doctor and patient. They potentially undermined the patient's trust in the expert responsibly providing medical treatment. Representatives of the gradually strengthening psychiatric profession strove to demarcate and safeguard their expert domain with the strict exclusion of lay hypnotists as charlatans. But the medical rejection of the therapeutic use of lay hypnotism is perhaps less surprising than the fact that even the 'purely' scientific and medical use of hypnosis was seriously contested: the psychiatric and broader medical community were deeply divided over the issue.

By the end of the 1880s, the risks and benefits of hypnosis were widely discussed in Hungary as well as in international psychiatric forums.[45] Neurologists and neuropathologists like Laufenauer, Schaffer and Jendrássik, who were experimenting with hypnosis for both scientific and therapeutic purposes, explicitly claimed that the experiments were not dangerous to the patient, since, as Jendrássik repeatedly stressed, 'an expert does not go that far'.[46] But Jendrássik's hypnotic experiments provoked Ilma to escape from his ward after her physical and psychic condition severely deteriorated – she gained about 30 lb in weight, became especially restless and her hysterical attacks became as frequent as 13 a day. Her willingness to take part in the experiments also diminished.[47] Based on such examples, other doctors suggested that precisely 'due to these numerous experimentations, these unfortunate persons very soon find themselves in lunatic asylums'.[48]

In Hungary, serious medical dissent came from the ranks of mental asylum doctors. Eminent doctors, like Ödön Blum and Jakab Salgó, stressed that hypnosis both presupposed and negatively affected an unbalanced nervous system, the further excitation of which was greatly detrimental to the patient. Hypnosis added to the patient's existing problems: loss of appetite, headaches, increased heartbeats, great restlessness, fits and seizures.[49] Doctors using hypnosis only aggravated their patients' conditions.[50] Doctors who agreed with Salgó that 'hypnotic experiments and treatments are not so important for science so that we should reduce human beings to the value of experimental animals' asserted that hypnosis undermined the primary traditional role of the doctor to heal. For them, not only the lay practice of hypnosis, but also its scientific use should be greatly curtailed.[51] It is probably due to this deep divide within the Hungarian psychiatric community that the 1894 Ministerial resolution triggered by Ella's case banned not only lay practice, but also scientific research in the field. However, the 1894 resolution was not particularly effective. Psychiatrists complained as late as 1903 that, while the resolution 'tied the hands of the researcher doctor', 'hypnosis was thriving in the country at secret meetings' and hypnotists with foul reputations still carried on their 'trade' as if there had been no ban.[52]

Diverse expert opinions and the local resolution of Ella's case

Ella's death did not undermine her family's belief in telepathic and clairvoyant phenomena.[53] However, articles by social commentators appeared in the daily press ridiculing the telepathic 'miracles' Ella performed.[54] Even Ella's former violin teacher was tracked down in the capital, to satisfy the incredulous readers with additional evidence of Ella's teenage 'innocent weakness of liking to fascinate ... with striking performances'. This former teacher claimed that Ella conspired with Neukomm just as she had done with her music instructor years before.[55] Meanwhile, most psychiatric experts dismissed the telepathic aspect of the event as a topic unworthy of consideration. The Viennese doctor Moriz Benedikt, for instance, believed that 'nobody in his right mind can suppose that while under hypnotic influence she displayed medical knowledge of which she was ignorant in her natural state'.[56] Benedikt was also convinced that Ella had herself hidden the stolen objects she then miraculously 'found'.[57] In face of such general scepticism, the eminent Professor Laufenauer's view that the telepathic phenomena reported from the séances were 'facts' was surprising and much criticized in the daily press.[58]

Expert speculations about the role of hypnosis in Ella's death varied. Early press accounts contained at least one false fact, namely that Neukomm suggested the *actual disease of tuberculosis* to Ella, who then possibly produced the lethal symptoms of tuberculosis in its highly advanced state and died as a direct consequence of his suggestion.[59] But even once it was clarified that Neukomm had not suggested the disease to the girl, the mere 'image' of the disease was seen as potentially fatal if fully 'incorporated'. The Swiss expert Forel stated, for example, that the 'terrifying conception of a diseased lung, which the somnambulist might possibly have for the moment thought was her own' could have caused the death.[60] Given the incredible influence of the mind on the body – it was, after all, capable of producing striking organic changes under experimental conditions – it is no surprise that the doctors who had conducted such experiments thought it possible that the death derived directly from the suggestion.

Some experts, like the Viennese Krafft-Ebing, blamed the death on an 'emotional psychic shock': Ella's sudden death resulted from her delivery of a difficult medical verdict – the death sentence on Neukomm's brother. Elsewhere Krafft-Ebing claimed that 'there can be no doubt that death resulted from the anaemia of the brain', from a real or 'nervous' brain stroke. (Based on the first autopsy, at the family's request conducted only on the brain, the official diagnosis was cerebral anaemia and oedema.[61]) Whatever the actual cause, he and Benedikt agreed that hypnosis had a potentially 'murderous' influence.[62]

The Munich doctor von Schrenck-Notzing and Laufenauer's younger colleague, Ernő Moravcsik, instead emphasized the role of Ella's hystero-epilepsy and nervous predispositions. However, while the former admitted that the extreme excitement may have triggered a lethal fit, Moravcsik denied a direct connection between hypnosis and death.[63]

In contrast to such disagreements, eminent French psychiatrists unanimously denied a direct connection between death and hypnosis in this case. They emphasized their extensive experience with hypnotic experiments and treatment, not a single case of which had been harmful when properly conducted by an experienced professional. Doctors at the Salpêtrière in Paris, like Bernheim in Nancy, acquitted hypnosis of Ella's death: 'All that we can say is that a nervous subject suffering from a cardiac condition could have felt a very strong emotion that was capable of provoking death.' This, however, would have happened even without hypnosis.[64] Bernheim's eager insistence that there was no direct connection between hypnosis and death, however, is perhaps worth a note here. As Forel pointed out, Ella's death was not the first to occur during (and possibly due to) hypnosis: it had also happened in Bernheim's practice.[65] Bernheim's patient in fact died three hours after he woke up from hypnosis, during which he began to suffer from terrible apprehension. None the less his death was still professionally highly embarrassing for Bernheim, whose dying patient had stated that 'hypnotism was killing him'.[66]

The English, meanwhile, paid little attention to the Hungarian case. While the *Lancet* reported on it, urging legislation to ban lay hypnosis, the *British Medical Journal* dedicated only a few lines to the case, lightly introducing it with the sentence: 'Just about this time of year the sea-serpent and the foreign hypnotizer make their appearance with annually renewed vigour.'[67]

Despite their differing positions on the case, practically all of the doctors quoted above agreed on certain facts: that Neukomm made a mistake by conducting his experiments on such a highly nervous and excitable patient; that Ella became extraordinarily irritated and exhausted during the séance; and that had Neukomm been medically trained, he would have stopped – or never begun – the experiment. There was complete medical agreement that no layperson should be allowed to hypnotize.

Initially, Neukomm seemed doomed. It looked likely that he would be convicted, if not of murder, at least of gross negligence in his orchestration of the fatal séance. Top legal experts, however, soon revealed how complex the legal assessment of the case was. First, medical experts had to establish the relationship between hypnosis and death, and then Neukomm had to be proved negligent. The latter would be especially difficult. Could Neukomm have had *reasonable* knowledge of the harmful or even fatal consequences of hypnosis? Did the presence of an eminent doctor (Vragassy) lessen Neukomm's responsibility? Legal opinions on these and related issues were widely divided.[68]

The Nyíregyháza (the seat of Szabolcs county) Royal Court investigated the case. The Royal prosecutor requested expert advice from the Forensic Medical Council, as was usual in cases involving possible medical malpractice or where medical re-examination was necessary. The Council found hypnosis – and therefore Neukomm – responsible for Ella's death. But the magnetizer was eventually acquitted, against the expert opinion of Hungarian and international psychiatric, medical and legal mandarins.[69] Something else proved to be more influential on the outcome of the court investigation.

András Jósa, the eminent and powerful Szabolcs county chief doctor, who himself was present at the autopsies, defended Neukomm throughout the case and seems to have been instrumental in influencing the court verdict. Jósa knew Ella well, having been invited to the Salamon castle to treat her nervous symptoms.[70] Jósa's report, written at the request of the Interior Ministry only days after the tragedy, stated that a year earlier he had revealed to the family that, due to Ella's increasing nervous illness and the poor condition of her vascular system, her death could occur 'at any moment, without any external cause'. Jósa concluded elsewhere: 'if, at the moment when Ella Salamon's death occurred, a fly had flown in front of the girl's face, even that would have caused her death.'[71]

Jósa repeatedly insisted that Neukomm had successfully used hypnosis with Ella for *therapeutic* purposes. On at least six occasions in 1894, Neukomm used hypnosis with Ella to relieve her nervous symptoms.[72] During the investigation, Neukomm too increasingly emphasized his therapeutic intent, as evidenced by the final verdict a year later.[73] The court established only 'accidental connection' between hypnosis and death in the case, almost literally repeating Jósa's opinion that sudden death could have occurred at any time. Ignoring the extensive press coverage of Neukomm's telepathic experiments with Ella, the opinion stated that hypnosis was:

> medically prescribed ... therefore Neukomm hypnotized Ella not for experimental purposes, but directly at [Ella's] own determined request, which was so strong, that even [its] mere denial – according to medical opinion – could have become fatal.[74]

The court further noted that, prior to Ella's case, no regulation controlled the practice of therapeutic hypnosis except when conducted for monetary gain; therefore, Neukomm's practice was not illegal. To the likely dismay of psychiatrists who had been experimenting with hypnosis at prominent institutes in the capital for almost 15 years, the court also stated that the effects of hypnosis on the human organism were as yet undiscovered by science. Therefore, the court considered claims that Neukomm could have had knowledge of the possible detrimental or fatal influence of hypnosis unfounded. The charge of gross negligence could not be proved.[75]

From the mosaic of information published in the newspapers, one is led to suspect some degree of initial conspiring among those present at the

séance in the Salamon castle. The aristocratic adherents of telepathy and hypnosis 'negotiated' how the news should be made public through the night following the tragedy.[76] Moreover, the young pathologist Dr József Lovrich, invited from Budapest University to perform the autopsy in the presence of the local medical authority, Dr Jósa, on the day following Ella's death, was told that Ella died of a sudden hystero-epileptic fit. No one mentioned hypnosis.[77] Furthermore, Lovrich later declared that, although he proposed a full autopsy of the body, *local authorities* decided to follow the family's request that only the brain be dissected (hence a second autopsy had to be performed later at the special order of the investigators). He also insisted that his autopsy results were not described as Jósa had presented them to the Ministry, adding: 'I have to assume that it is a faulty quotation taken from memory.'[78]

It is noteworthy that, in a letter written by Neukomm to his family in Versec two days after the tragedy – but still before the case became public – the magnetizer claimed that Ella died suddenly and *before* the hypnotic séance. The letter, obtained and later published by a journalist, stated: 'It is fortunate that it happened this way and not during or after hypnosis, otherwise people would have said that the hypnosis was the cause of the tragedy.'[79] People were suspicious of Neukomm's intent, and a few days later, amidst great embarrassment, Neukomm had to admit that the letter was false.[80] Evidence therefore suggests that those present at the performance initially wanted to cover up the story, whether to defend the reputation of the family, or of hypnosis and the telepathic séances they so deeply believed in.

Dr Jósa's support of Neukomm is perhaps even more striking. Far from the capital's prestigious university psychiatric laboratories, this well-connected and widely beloved provincial doctor, known for his selfless dedication to healing the needy and his eccentric love of his provincial homeland, was an impressive and powerful authority.[81] It must have suited Jósa's genuine beliefs as well as his immediate interest to stand by Neukomm: for a few years prior to Ella's death, Jósa had successfully employed hypnosis in treating his patients from all strata of society. In Ella's case, he not only found hypnosis therapeutically highly effective, but originally *he himself* recommended it![82] Furthermore, as he revealed in his memoir, Jósa had become a convert to hypnosis after he had witnessed some of Neukomm's séances in Nyíregyháza, and learnt the method of hypnotizing directly from the magnetizer.[83]

The outcome of Ella's story, and particularly of Neukomm's trial, demonstrates that local interests and expert authority in provincial Hungary could overrule medical expertise emanating from eminent centres in both the Hungarian capital and other European cities. In the provincial setting, the charisma and dedication of the local chief doctor was a greater influence than any national or international expert authority.

This reconstruction of Ella's case tests historians' tacit understandings of the power assigned to the 'centre' and the workings of influence in social

space. Much historical scholarship endorses notions of 'centre' and 'periphery' which attribute power primarily to the former. In some older intellectual or institutional histories, centres of knowledge or practices appear as enclosed repositories of monolithic power, isolated both from the social space that surrounds them and from other 'peripheral' locations. The periphery is interesting only in so far as it serves to enhance the importance of the centre through its contrasting backwardness. In this shared model of how power and influence are exerted in space, the direction of influence always points from the centre to the periphery. But power and the notion of the centre are situational and relative; they are dynamically interrelated and make no sense in isolation. As Ella's story demonstrates, the remote, the peripheral and the local may affect phenomena in ways beyond this conventional understanding.

Here, by temporarily shifting the 'centre' (where the telepathic séance, the investigations and the final resolution of the case take place) to what is normally considered the margin (a provincial castle and town in the north-eastern region of the Hungary), one can intricately map scientific and social exchanges, revealing a complex picture of the social dynamic of scientific practices. Real and symbolic geography is therefore key to my reconstruction of Ella's story and its wide-ranging reverberations. It demonstrates how a provincial hypnotic/telepathic séance was connected to lay hypnosis and the spiritualist movement in the country, to medical legislation in the Budapest government, to experimental research by psychiatric and medical gurus in prestigious institutes all over Europe, and to the courtrooms from which forensic cases invaded private homes with the help of the media. It also shows that a case from the periphery could amaze millions of newspaper readers, mobilize remote experts and enter medical textbooks; in other words, could gain significance and power through its interconnectedness.

From this new, albeit temporary, 'centre' located in the periphery, one can also see the fragility of expert knowledge produced within prestigious Western institutions. This case undermines the centrality and power of expert knowledge as well as the disciplinary unity of late nineteenth-century psychiatry (often overestimated by historians). Although most doctors who involved themselves in hypnosis research endeavoured to gain exclusive control of a field formerly claimed by spiritualists and a variety of healers, their disunity was apparent in the laboratory, the clinic and the wider social space. They disagreed over the interpretation of certain hypnotic phenomena produced in the laboratory setting, over the applicability of such results in public forensic cases, over its potential therapeutic uses in asylums and clinics; and in specific cases, such as Ella's, they produced a wide variety of conflicting explanations, shaped by institutional or personal interests.

Ella's story therefore simultaneously raises questions of what counts as a centre and what power is attributed to it and demonstrates how hypnosis was embedded in intricate social relations. But by demonstrating the complex

ways in which the remote can potentially affect (or the local override) the 'centre', the case demands more than a new mental map with reversed direction of influences. We must revise not only our sense of geography, but also our 'physics': here the qualities of the effect also appear in new light, as more varied and complex, and therefore the case necessitates a reconsideration of our tacit understanding of how influence works in space.

16

Maladies of the Will: Freedom, Fetters and the Fear of Freud

Daniel Pick

1

The history of the self, Roy Porter observes in his poignant, posthumously published book *Flesh in the Age of Reason*, is commonly told as the tale of growing individualism and personal autonomy, a saga of 'the maturing self-consciousness of the self-determining individual'. Porter allocates Freud a couple of cameo roles and describes psychoanalysis as the culmination of a romantic impetus to know and tell all about the self. Freud 'opened up new horizons of selfhood, or rather plumbed the psyche's oceanic depths, exposing a hidden world of secret desires and treacherous drives'. Freud, Porter concludes, located the self in a 'terrifying subterranean battleground of the id, ego and superego'.

In another brief diversion, Porter observes how various twentieth-century philosophies have questioned the proposition that there is any 'core self' to discover. Pointing to the work of Ryle, and, across the Channel, to Barthes, Foucault and Derrida, he describes diverse challenges to the 'ghost in the machine' and to the entire 'conventional humanistic understanding of subjectivity – the individual agent, writer or reader possessing a mind of his or her own and exercising free will through thought and action'. The ascent of selfhood, it now seemed, was but 'idle teleological myth, a humanist hagiography'.[1] Porter's snapshot of Freud is placed in the context of these divergent traditions of thought: those committed to 'realizing' human autonomy and those concerned with debunking the notion that there is such a thing as a 'core', authentic self. But how was the Freudian 'self' first received? And against what intellectual backdrop was it judged? This chapter ponders some early twentieth-century cultural reactions to psychoanalysis and links the dismay that it evoked to certain, historically earlier and scarcely less provocative explorations of the self and the will, particularly those emerging from three traditions of nineteenth-century thought, which could loosely be gathered together under the headings 'magnetism and hypnotism', 'evolutionary naturalism' and 'degenerationism'. I suggest that public disquiet

about psychoanalysis was fuelled by the fear that it disregards or even incapacitates free will. Psychoanalysis was reviled in part because it was perceived to threaten a set of residual Victorian values that were in crisis well before Freud set pen to paper.

There is certainly no difficulty in recording public expressions of consternation and distaste about the invention of psychoanalysis, not least among English doctors.[2] But it is easier to demonstrate this than fully to account for its causes, or to generalize about 'national cultures', on the basis of descriptions of the audiences who notoriously greeted Freudian discussion of the unconscious in startled and hostile silence before 'voting with their feet'.[3]

Sometimes arguments for the dangers of psychoanalysis were focused on particular charismatic individuals who were perceived to have committed 'boundary violations' of one kind or another, leading to explosive stories of sexual and moral transgression. Needless to say, there were numerous precedents for those concerns with unwarranted 'therapeutic' interference, including many derived from the annals of mesmerism and hypnotism – from tales of Mesmer's own supposed kidnapping of a patient in Vienna in the 1780s, to stories about the sinister prowess of the stage hypnotist known as 'Donato' a century later (in some countries his performances were banned).[4] Of course, mesmerism and hypnotism have complex histories of their own, but here we are concerned with how psychoanalysis was sometimes crudely bundled together with other procedures and movements from the past. Freud reaped not only the whirlwind of his own provocations, but some of those already discernible in the culture and science of the previous century.

Although British press coverage during the 1920s and 1930s was by no means universally hostile, there were marked and consistent criticisms of the beguiling nature of Freudian therapy, and of the supposed lack of scientific rigour in its theory, the putatively slipshod methods of its investigations, the self-proving arguments and 'cultish' fervour of its adherents. In the early heady days, the boundaries of orthodoxy and fringe were anything but clear. Even so, the adequacy of 'training' and the disposition of the practitioner were by no means the only qualms. Hostile commentators lamented the degree of interest Freud's books had generated and the 'popular craze' for psychoanalysis that – it was suspected – had now taken hold. Psychoanalysis was sometimes excoriated because it was thought to overexcite the patient and the public at large, apparently undermining the stable boundaries of the self and the group. Thus the jubilant claim that Freud's theory would prove as revolutionary as that which Charles Darwin gave to the world, aroused the ire of G. K. Chesterton in 1920.[5] Chesterton lambasted the 'fashionable fatalism' that Freud apparently encouraged; the professor, he remarked, was but the latest in a line that could be traced back to Haeckel in the nineteenth century and Calvin in the seventeenth:

> Already some are professing to find in [psychoanalysis] a whole encyclopaedia of excuses. They declare that psycho-analysts have told them

> that it is always unwholesome to repress an impulse . . . They say that certain deeds are done by the sub-conscious self and not by the real self. They open up again the yawning abyss of the old argument about human responsibility, about fate and free will.[6]

(Compare that with Tennyson some half a century earlier, who offered this warning against old Calvinists and modern evolutionary materialists who thought they could dispense with the crucial human ingredient: 'Take away the sense of individual responsibility, and men sink into pessimism and madness.'[7]) Announcements that psychoanalysis was on the march, or, as the *New Statesman* put it in 1923, that 'we are all psychoanalysts now',[8] was just the sort of thing that Chesterton regretted. He predicted that the fad would soon pass.

Psychoanalysis represented a radical departure from the world of Darwinian and materialist scientific thought in which its founder had grown up, but it was also the heir to a deep Victorian *angst* about the possibility of moral freedom. Freud provoked several kinds of disquiet that can be compared and contrasted with the invective previously directed against the various nineteenth-century systems of thought highlighted here. Critics debated whether psychoanalysis disclosed and/or exacerbated a contemporary predicament of regression and weak moral fibre. Some worried that psychoanalysis ensnared its patients in – rather than freeing them from – neuroses, and appeared apprehensive that this new theory had so muddied the meanings of the will and the self as to undermine an essential human dignity.

The concept of 'the will' had been a central (if vexed) reference point in many nineteenth-century psychologies and philosophies; but it found little explicit place in the psychoanalytic literature. It was not a key theoretical term for Freud. Perhaps in the 'death drive' and the myriad clinical elaborations that have followed Freud's famous formulation in *Beyond the Pleasure Principle*, psychoanalysis has produced its own increasingly intricate versions of what the Victorians had previously placed under the broad and ominous heading of maladies and perversions of will. The psychoanalytic evocation of deep and mysterious unconscious forces running counter to life, of destructive psychic processes that break up a sense of human connection, of perverse impulses that stifle creative activities or of those violent repudiations of our deepest dependencies all draw something from – even as they transform – an older diagnostics of the will.

In the period during which Freud first developed his theories, a sense of malaise about modern 'weak will' was matched by anxiety about the state of the concept itself. Some were convinced that a philosophical and/or psychological crossroads had been reached around the *fin de siècle*. This can be seen in a book that appeared in 1898 (by coincidence shortly after Freud coined the term 'psychoanalysis'), a panoramic study of the will, in which Archibald Alexander traced debates about freedom and necessity from the Greeks to

the Victorians, and suggested that the concept was no longer coherent. The movement above all others that had contributed to this contemporary difficulty, he believed, was Darwinism – as a result of the theory of natural selection, he pointed out, no psychical state could be taken for granted as a 'given'. The will, he warned, was at risk of disappearing in an age committed to decomposing everything and relocating each mental process in another deeper and more primitive layer.

Whether or not we see Freud as the presumptive intellectual successor of Darwin,[9] the problem of the meaning and efficacy of the will, so spectacularly stirred up by Darwinism, was radically extended in Freud's psychology of the unconscious.[10] Psychoanalytical accounts of conflicting intra-psychic 'agencies', coloured by fantasy and largely unconscious, created substantial new uncertainties for any account of the will. 'If someone asks what 'the psychical' really means, it is easy to reply by enumerating its constituents: our perceptions, ideas, memories, feelings and acts of volition – all these form part of what is psychical,' Freud declared in 'Some Elementary Lessons in Psycho-Analysis' in 1938.[11] Yet much of our psychical life, he demonstrates here, as he had so often before, is entirely unconscious. As many of his interpreters observed, acts of 'volition' seemed hard to reconcile with psychoanalytical theory itself. Once 'will' became a feature of the 'terrifying subterranean battleground' of the id, ego and superego, it tended to lose its basic meaning, hitherto bound up with an idea of conscious intention.

Anxiety about the erosion of free will was not of course the only source of indignation about Freud, but in fact even many of the other publicly stated objections to psychoanalysis turned in the end on related questions about human dignity and the personal responsibility that could or should be attributed to the individual. Sexual disgust evidently played its part in the repudiation of Freud. Fear of 'Teutonic' and/or Jewish infiltration is clearly also of relevance in understanding ambivalence towards – or even outright rejection of – psychoanalysis. In both cases objections to Freud turned on fears of an outsider's demoralizing, confusing or invasive effect. Alarm about sexual licence on the one side, and of a 'Jewish' infiltration of the Gentile mind on the other, was expressed in several significant commentaries, and sometimes even found its way into the criticisms made by erstwhile allies.[12]

To take an early example, note how sexual and racial distaste are combined in a review of *The Interpretation of Dreams* which appeared in *The Athenaeum* in 1913: 'Englishmen have little to learn about the manner of telling dreams, however deficient they may be in interpreting them'; Freud is apparently preoccupied with the morbid rather than the healthy dreamer and betrays, in his tendency to introspection, the hallmarks of an 'Oriental heredity'. All in all, it seems, there is nothing much for the English here and the whole book is swiftly dismissed:

> His conclusions are sometimes far-fetched and fit the premises incompletely, while an atmosphere of sex pervades many parts of the book

> and renders it very unpleasant reading. The results he reaches are hardly commensurate with the labour expended, and reveal a seamy side of life in Vienna which might well have been left alone.[13]

Some urged that a revised (bowdlerized?) version of the treatment and theory was required. Important 'mediators' of Freud's work, such as the anthropologist, 'shellshock' doctor and psychologist, W.H. R. Rivers, sought a more palatable ('British'?) compromise. He balked at the sexual theories of psychoanalysis, at least for his patient group, even while promoting the clinical relevance of the unconscious and the 'talking' method during the Great War.[14] Despite such endeavours mainstream psychiatry remained largely untouched. To this day, as Porter himself has so often chronicled, traditions and approaches other than Freud's have largely held sway. These include various forms of behaviourism, occupational therapy and, above all, organic psychiatry. The latter had little truck with Freud, even though, from its different angle, it too was decried for threatening morality, undermining responsibility and free will.

With the appearance of Freud's landmark book, *The Ego and the Id* (1923), the superego came to prominence. Ferocious attacks on the ego at the behest of the superego, impossible idealized demands, profound unconscious guilt, savage prohibitions of which we are unaware increasingly seasoned the psychoanalytic account of the internal world. Desire, anxiety and guilt were all the more crippling because they were not only unknown but in part always unknowable. Small wonder that in a leader column headed 'Psychoanalysis', *The Times*, in 1925, acknowledged that Freud offered some interesting theories, but lamented the loss of the 'mind doctor of the "old school"' who conceives it as his duty to 'strengthen his patients' will by every means in his power' and 'looks askance at the idea that there are elements of the mind lying outside the scope or range of conscious thought, and hence beyond the controlling influence of the will'. Citing the case of a young barrister who had endured psychoanalytic treatment only to commit suicide, *The Times* warned that Freud had inaugurated a highly dangerous clinical practice.[15] Many psychoanalysts no doubt would have accepted the view that Freud dethroned the will, if not the negative verdict on the procedure itself.

Institutional 'interests' and turf rivalries were factors (psychoanalysis as potential incursion into the theory and client-base of medico-psychiatry, for instance) driving cultural suspicions of Freud. In the English case at least, native empiricism and an entrenched tendency to anti-theoretical philistinism have also been mooted as explanations for 'cool' attitudes to Freud.[16] But this form of comparative 'national' approach only takes us so far. For a start, one can easily supply instances of contemptuous Continental critics of Freud or of horrified audiences – and patients – heading away in protest; but one can also present, on this side of the Channel, significant

instances of enthusiasm and respect. Recent research on the reception of Freud in early twentieth-century Britain shows the variety of figures who were drawn to psychoanalysis.[17] Pioneers included the botanist and ecologist Sir Arthur Tansley, the philosopher and mathematician Frank Ramsey and the historian Louis Namier.

Examples of cultural engagement with Freud range from the interest expressed by some Edwardian 'psychical researchers' to the endeavours of various feminist campaigners and dissatisfied doctors to apply the 'talking cure'; from the zeal of avantgarde educationalists to the fascination of modernist writers; from the entrepreneurship of 'Bloomsbury' publishing initiatives to the close attention of poets. Auden wrote in homage on Freud's death: 'To us he is no more a person / Now but a whole climate of opinion'. The curiosity of inter-war scientists, anthropologists, economists and philosophers, as well as various writers and artists, testify to the claim that Freud fuelled and reflected what the celebrated American critic, Lionel Trilling, in a book on Freud later termed 'the crisis of our culture'.

But while many *literati* and some scientists were deeply absorbed, others saw Freud's project as a siren call to anarchy and depravity. Take this disgruntled correspondent and former analysand who, in 1926, wrote to the *Church Times* (where better?) to denounce the Freudian assault, 'the blinding, fascinating, alcoholic effect' of the therapy and the patient's enforced discovery that 'all roads lead to sex'. To lie on the couch was apparently to poison the mind, encourage hatred and shatter all spiritual comfort.[18] Freud was repeatedly to evoke such horror.

Some had their cake and ate it. Awe and derision were memorably combined in D. H. Lawrence's essays on the unconscious in the early 1920s. Here the novelist celebrated Freud's courage in setting off into an unknown land of dreams, only to castigate him for the excremental filth he uncovered in the course of his analysis:

> What dreams dear heart! What was there in the cave? Alas that we ever looked! Nothing but a huge slimey serpent of sex, and heaps of excrement, and a myriad repulsive little horrors spawned between sex and excrement.[19]

Intellectuals were divided as to whether to revile or admire Freud for showing us our debasement in that 'cave', and for explaining how our liberty was fettered from within. Virginia Woolf also expressed conflicting views. Soon after Freud's death in 1939, and not long before her own, she noted herself 'gulping up' his ideas. Her compressed diary entries refer to Freud's effect in 'enlarg[ing] the circumference' of her thought, giving her 'brain a wider scope' and unsettling conventional ideas of religion, civilization and, crucially, freedom. The impact was powerful – 'reducing one to a whirlpool; & I daresay truly'. 'If we're all instinct, the unconscious, what's all this about

civilisation, the whole man, freedom &c? His savagery against God good. The falseness of loving one's neighbours. The conscience as censor. Hate . . . But I'm too mixed. I'm going to begin Mill on Liberty.'[20] A nice juxtaposition, Mill and Freud, not least because, in his youth, the latter had translated some essays by the former.[21] Where Mill had offered a monumental defence of liberty, Freud seemed to cast the very concept into doubt: 'what's all this about civilisation, the whole man, freedom &c?' And yet the contrast is not as sharp as it looks. For just as Freud sought, in one sense, to enlarge the individual's liberty, so Mill was by no means sure how many of us could really think our way free of baser motives, and of the corrosive and nefarious influence of others.

2

Let us follow Woolf, who turned to a dose of Mill after 'gulping up' her Freud. Mill had celebrated and theorized liberty, and had described the individual's entitlement to freedom *from* the interference of the state. This assumption that 'non-interference' was a social good and that the individual was the appropriate unit of analysis came pointedly to be challenged by a variety of thinkers in the late nineteenth and early twentieth centuries. But in many ways Mill himself had already complicated clear-cut distinctions between the operations of the individual and the group. 'Individuality' was potentially threatened by conformism within our consciousness.

Mill described his painful struggle to grasp free will and to reconcile it with an account of causation, in his *Autobiography*: 'I felt as if I was scientifically proven to be the helpless slave of antecedent circumstances; as if my character and that of all others had been formed for us by agencies beyond our control, and was wholly out of our own power.' Mill concluded that

> though our character is formed by circumstances, our own desires can do much to shape those circumstances; and that what is really inspiring and ennobling in the doctrine of free will, is the conviction that we have real power over the formation of our own character; that our own will, by influencing some of our circumstances, can modify our future habits or capabilities of willing.[22]

But he was frequently to express concerns about the impact of modern life itself on our thought. Tremendous psycho-social damage could be wrought by demagogic politics, the corrosive nature of 'low' cultural values, crude moral conformism and mystification. All such factors could deprave or deform the citizen. How far might the bewildering and compulsive – even hypnotic - force of 'public opinion' (in a vulgar and/or exploitative market-place of ideas) insidiously fashion the will? In Mill's philosophy, civil society

(and not just the formal state) worked its effect for good or evil, and might undermine our possibility of freedom.

Mill argued that the world might become better in future through benign moral education. Drawing on Benthamite assumptions, he argued that the function of chastisement and encouragement was to refashion the will. Bad education could stunt or pervert it. Mill promised that if new habits could be formed, the will could be beneficially recast. He showed in his touching account of his recovery from the melancholia that laid him low as a young man how he could change and his will to life be restored. 'Will, like all other parts of our constitution', he remarked, 'is amenable to habit'; 'we may will from habit what we no longer desire for itself, or desire only because we will it.'[23]

Mill's book *On Liberty*, published in 1859 (the same year as the publication of Darwin's *Origin of Species*), supported belief in the potential power and autonomous endeavour of the individual. Political constraints and obstacles, religious shibboleths or the yoke of debased and vulgar public opinion still hampered the full realization of individual freedom, Mill eloquently insisted, but the vista of mental autonomy and self-understanding had never been more powerfully described.

Other accounts, such as the laissez-faire economic arguments of Richard Cobden and John Bright, the strictures on the natural rightness of governmental non-interference provided by the great evolutionary populariser and liberal Herbert Spencer, or, in a different vein, the exhortations to independence provided by Samuel Smiles in books such as *Self-Help* (also 1859) suggested that individual will had a bright future. At their most extreme, such paeans to liberty assumed that we can become, psychologically and somatically, as well as politically and economically, discrete individuals, or even fully formed self-knowing characters, capable of willed action, of reason and judgement, exercised at the various moral crossroads of life. How far the caprice of biological endowment, the onslaught of a hostile environment or the cruel vagaries of social chance could quash the determination of this supposedly self-made and self-possessed figure was one of the great talking points of the period. For every statement of hope, however, there were other anxious inquiries into the nature of human incapacity and failure of will.

In the 1860s and 1870s, Darwinian descriptions of the evolutionary processes and the law of 'natural selection' that shaped, or perhaps misshaped, our faculties appeared, in many eyes, to threaten human distinctions, as well as the biblical account of creation. Are we but creatures of the material world, of natural history, even as we believe ourselves to be exercising free choices? Darwin's writings suggested the illusions of voluntary action. Crucially, his central concept of natural selection envisaged the likely reward of survival and reproduction for those who just happened to possess 'an edge' over others; it was not a prize that nature bestowed on those who took the trouble to better themselves by reaching upwards or doing the right

thing. Altruism had its place in securing the survival of the group, but this quality was conceived in instinctual terms. In his more uncompromising discussions of materialism and evolution, Darwin seemed to leave little, if any, room for virtue or beauty as ends in themselves. A viable political philosophy and moral system, it was argued in turn, would need to reflect rather than precede these given truths of nature.

Darwin's work constituted one among several powerful jolts to the idea of 'self-fashioning' and it provoked a plethora of horrified comments and rearguard actions. Some responses took the form of a wholesale attack; others sought to accept many aspects of the new account, while still seeking to retain free will. The eminent physiologist and Unitarian William B. Carpenter balked at the strident materialist views advanced by Darwin's 'bulldog', Thomas Huxley. Carpenter's response was to fall back on that common sense which, he pointed out, Huxley had himself previously advocated: namely that we know we have free will, from ceaseless daily experience of ourselves, exercising choice.[24]

But in place of a social model centred on the activity of discrete individuals, a host of commentators now insisted that the impersonal, the collective, the instinctual or the unconscious decisively shaped human action.[25] Even the capacity to work came increasingly to be seen as a consequence of structuring economic conditions, rather than of the moral striving – or failing – of free agents. Such discussions led many to return to that doctrine of 'Philosophical Necessity', which Mill himself so painfully reconsidered: thus to conclude that free will was an illusion.[26] The vision of liberty, autonomy and freedom, so promisingly set out in the philosophical productions of the mid-century, seemed to many writers in the Victorian twilight years a dream in ruins.

With the Enlightenment and Romanticism, as Roy Porter observed in *Rewriting the Self*, freedom had come to be elaborately extolled and probed. Liberty of thought (with all dependences and early influences on that individual cast aside)[27] was sometimes presented as a panacea for all social and psychic ills. A form of individualism crystallized, in which the single being was pictured as free and alone. And yet, as we have glimpsed, alongside the great pronouncements of 'classical liberalism' lay alternative visions of the self, not least a literature that stressed our fundamental reliance on and interconnection with others. Dependency might be pictured in benign or malign form, but many commentators showed that individual minds were really adrift in a sea of mysterious natural or interpersonal forces. By the late nineteenth century, several philosophers of liberalism, alert to the significance for the individual, of market forces, inexorable international political rivalries and remorseless subterranean psychological processes vigorously criticized their predecessors who had placed an excessive faith in the powers of the self.[28]

3

In his study of the will, the philosopher Brian O'Shaughnessy has noted how nineteenth-century thought imbued the mind with a quasi-organic vitality: the will could be seen as 'healthy' or 'diseased', alive or dead. For some, 'will' signified the striving of a specific individual, with a distinctive system of beliefs, desires and values; for others, 'will' implied processes pertaining to the group or even the species.[29] Some Victorian debates about will were indeed linked to questions about collective health, even to the diagnosis of 'national character'. O'Shaughnessy focuses on how earlier romantic celebrations of life-promoting nature gave way to a preoccupation with destructive and self-destructive forces.

> 'The will' as such could therefore hardly any longer be idealised, and natural illness and even natural evil tended to appear in naturalistic conceptions of the human psyche. … 'We are all ill', said Freud, somewhat as other ages have said that 'we are all sinners'.[30]

A development particularly relevant to this shift from a language of sin to medical illness had occurred around the mid-century, with the emergence to prominence of a new scientific literature on degeneration that brought several existing threads of thought and socio-biological alarm together. A vast spectrum of inheritable pathologies was set out in a novel treatise by a French psychiatrist, Bénédict-Augustin Morel, in 1857. His work inaugurated a venerable tradition, a remarkable range of investigations into supposedly linked moral, psychological and physical illnesses now afflicting society. Morel was a Catholic believer, but many of his followers dispensed with references to 'the Fall', confining themselves instead to descriptions of modern pathologies of body and will. Numerous late Victorian commentators drew inspiration from that set of ideas, as they explored the 'degenerate's' relative (or absolute) incapacity of will.

Alarmist diagnoses of individual character failings merged with inquiries into the 'sickness' of modern civilization at large. Thus Friedrich Nietzsche drew on various specialist works on degeneration and criminality as he portrayed the racial malaise, enervated beliefs and bad nerves of the nineteenth century. 'But what is prone most of all to illness and degeneracy in these mixed breeds is the will', he explained in *Beyond Good and Evil*.[31] The European of the future was pictured as 'the most intelligent slave animal, very industrious, at bottom very modest, inquisitive to excess, multifarious, coddled, weak-willed – a cosmopolitan chaos of affects and intelligences'.[32] Numerous other writers around this time documented epidemics of 'fatigue' and ennui that apparently vitiated modern society. A child not habituated to will his own strength and to resist negative social forces, warned Dr Achille-Adrien Proust (father of Marcel Proust and author of *L'Hygiène du*

neurasthénique, 1897), would in all probability succumb to languor, sadness, lubricity, feebleness and other such conditions. The sad end that Proust *père* anticipated was that 'moral paralysis called *aboulia*'.[33] (Needless to say, in a variety of political movements across the twentieth century, regeneration of the will has been presented as the supreme social good.)

In *Maladies of the Will* (1883), Théodule Ribot documented many instances in which the connections of mind and body, intention and behaviour fell apart. Ribot had been a student of the École Normale Supérieure, a lecturer at the Sorbonne and the editor of the *Revue Philosophique*, before he crowned his career with a chair at the Collège de France. Consciousness, he argued, was but one form of psychic manifestation and it emanated from bodily sensations. The ostensibly voluntary act – I will – was materially produced, and a manifestation of evolutionary processes. This 'will' described a faculty that could normally (and for good evolutionary reasons) override the instant inclinations of our gross appetites. In states of illness this propensity to restraint could disappear even in the most advanced human beings.[34] He argued that 'the will is not an entity reigning by right of birth, although sometimes disobeyed, but a resultant always unstable, always ready to decompose itself, and, to say truly, a happy accident'.[35] Ribot discussed states in which the will faded or ceased to operate altogether. Some were incapacitated by torpor (for instance by drug addiction – here he cited Thomas De Quincy on opium), others by the spell-binding influence of another person. Still more were paralysed by strange and as yet unexplained internal states.

One of the new disease entities that had emerged in the second half of the nineteenth century was 'abulia'.[36] Here, according to Ribot, the patient knows how to will 'interiorly', mentally, according to the dictates of reason, but is unable to transform the will into action, leading to a state of motor incapacity or effective will-lessness in practice. It is a 'pure disease of the will' in which the capacity to act disappears; the patient is reduced to a state of impotence. Much of Ribot's discussion was given over to understanding states of mentally induced paralysis. In the clinical conditions he described, the will was shown to be excessively strong in some, feeble or even entirely extinguished in others. But whether or not suffering a malady of the will, singular identity in Ribot's discussion, looked ever less discrete; the 'I' anything but a 'first person singular'. Ribot argued that the brain was 'like a laboratory full of movement, where thousands of tasks are performed at once. ... Consciousness is the narrow gate through which a very small part of this work appears to us.' Or as he summed up these startling revelations, 'the conscious person is a compound, a result of very complex states'.[37] This prominent work represented the tip of an iceberg: many other researchers contributed to the burgeoning scientific literature suggesting the internal hiddenness of the personality *from itself*.

Such studies by Ribot and his colleagues in the 1880s and 1890s compounded the sense, already so evident in the varied literature of

mesmerism, evolutionary naturalism and degenerationism alluded to above, that the self and the will would have to be reconceptualized. Of course, there were specific nuances and inflections to such works and debates in different countries (of which the present discussion takes no account), but my point is that from this miscellany of images – of gruesome simian criminals, savage 'survivals', degenerate urban denizens and exhausted and enchanted subjects – the possibility of a new picture emerged of, as it were, the extreme peculiarities and anomalies of normal psychic life. It was not only spectacular grotesques that gripped these Victorian scientists, but also the ordinary transgressions of the self, the puzzles of memory and forgetting, of consciousness and unconsciousness, of desire and fascination.

In this period, the historian and psychologist Hippolyte Taine referred to the mind itself as a kind of rowdy theatre, comprising many conflicting voices.[38] Other 'theatres of the mind' had been proposed by earlier philosophers and story-tellers, of course (and not least by David Hume in *A Treatise of Human Nature*, in the eighteenth century), but what marked out so much late nineteenth-century thought was the uneasy movement between philosophies of the will, biology, physiology, expressions of medico-moral anxiety about the direction of society, and specific clinical and forensic material illustrative of a modern crisis of 'abulia'.

Such inquiries paved the way for psychoanalysis. This is not to say that these Victorian psychic dramas were psychoanalytic *avant la lettre,* but they did show, in their own terms, how actions and thoughts 'centre stage' were disturbed and complicated by a variety of other voices, memories and desires, emanating from the dim interior.

Metaphors of physical disintegration, social anarchy and political conflict have been deployed many times since Freud to characterize this disturbing internal landscape. Recent examples of such appropriations of socio-political terms for the discussion of the 'internal world' are 'mafia gang' and 'pathological organization' – these are used in clinical and theoretical accounts, as part of an endeavour to describe malign unconscious forces, exerting their powerful and even enthralling grip on our mental functioning and better selves.[39]

In tracing these developments from century to century, there is a familiar danger of 'Whiggism', where we view, in celebratory but constricting fashion, Victorian mesmerism and hypnotism, or the scientific literature on evolution and degeneration, as but staging posts to the psychoanalytic model of the mind. None the less it is important to recognize that psychoanalysis was born in part from such developments and, crucially, from the shrill history of contestation that surrounded them. From the sufferings and excitements of 'mesmeromania', and despite all its solemn magnetic theories, which now seem so antiquated, so *ancien régime*, there emerged, little by little, other possibilities for representing the mysterious nature of our connections to and disconnections from others. Visions of evolutionary 'throwbacks' pointed to

the limits of reason and the fragility of the will, just as Ribot's depictions of abulia sufferers contributed to new questions about the enigmas of the 'I', and the gap between conscious intention and unconscious motive. Out of such diverse portraits of the normal and the pathological, the ill and the well, the individual and the group, emerged a different science, concerned with psychological rather than biological regression, with the exploration of the interior psychic significance of emotional ties, rather than with the literal analysis of 'magnetic' attractions or the 'tyranny' of biological organization. Mesmerism, Darwinism and Freudianism each in their different way aroused a cacophony of painful protests, intense and enduring fears that the self and the will were being driven apart by pernicious philosophy, or that they were inevitably falling apart by dint of the internal crisis of 'mind' itself. From this uncertain terrain – of primitive impulses, irresistible desires and multiple identifications – there would emerge a language of 'the internal world' that we can recognize as our own.

17

Two Cultures Revisited: The Case of the *Fin de Siècle*

Mark S. Micale

One of the myriad ways in which Roy Porter broadened our historical understanding of early modern Europe was by emphasizing how science, medicine, philosophy and the arts continually intersected. Porter showed that the leading figures in these cultural domains did not just engage one another personally; rather, they continually participated in a shared medico-literary milieu of ideas, theories, texts and images. Britain during the seventeenth and eighteenth centuries was exceptionally rich in these cross-fertilizations: Robert Burton, the Oxford divine, drew on a mass of scientific learning to produce his encyclopaedic *Anatomy of Melancholy* in 1621. John Locke, the pre-eminent psychologist, epistemologist and political philosopher of the late Stuart era, was schooled in medicine and associated with Sydenham. Bernard de Mandeville, physician, philosopher and social satirist, showcased a long fictional dialogue between a doctor, patient and family members in his *Treatise on the Hypochondriack and Hysterick Diseases* (1711). The Scottish novelist, playwright and historian Tobias Smollett doubled as a London surgeon. And the leading English literary intellectual of the third quarter of the eighteenth century, styled *Dr* Johnson, was so well versed in classical and modern medical texts that he contributed learned entries on the subject to his *Dictionary of the English Language* (1755). The entire eighteenth-century culture of sensibility was simultaneously literary, moral, psychological, theological and physiological. Because of the great scope of his own learning, Porter was uniquely well equipped to bring to life the integrated humanistic world that was Enlightenment Britain.[1]

And what of 'the two cultures' in later periods of Western history? C. P. Snow's formulation has been part of the Anglophonic intellectual world since the spring of 1959, when Snow delivered his famous harangue as the Rede Lecture at Cambridge University. In the mid-twentieth century, Snow asserted, a deep suspicion, even incomprehension, divided 'literary intellectuals' and 'natural scientists', who remained sceptical of each other's methods and ignorant of their work.[2] Snow's cultural diagnosis generated

extensive debate, and his pamphlet has recently won new admirers.[3] Surprisingly, however, the specifically historical aspects of his thesis have received little exploration.

After cohabitating comfortably in antiquity, the Renaissance and the eighteenth century, just when *did* the West's sciences and humanities drift apart? Snow looked to the Industrial Revolution in England of the late eighteenth and early nineteenth centuries. Other scholars have identified key moments in the divorce as René Descartes' drive in the 1630s to mathematize human knowledge, the founding in 1660 of the British Royal Society and the publication of *Principia* by Isaac Newton in 1687 or the 'Preliminary Discourse' by the French encyclopaedist Jean D'Alembert in the 1750s. The historical witnesses most often called to testify to the emerging cultural rift are the British literary Romantics. We have grown familiar with the passages: Wordsworth's observation that 'Our meddling scientific intellect / Mis-shapens the beauteous forms of things; / We murder to dissect'; Blake's drawing of a god-like Newton methodically reducing the cosmos to geometry; Coleridge's assertion that 'Poetry is not the proper antithesis to prose, but to science'; and Mary Shelley's proto-feminist critique of modern science's destructive pretensions to self-sufficient creativity in *Frankenstein* (1818).[4]

It has been argued repeatedly that these statements reflect both Romantic aesthetic ideology, with its new emphasis on the expressive powers of the individual imagination, and fundamental changes in the scientific world. Concerning the latter, during the early nineteenth century university-based medical training, an objectivist methodology, official licensing standards, technical journals and textbooks, professional specialization and an objectifying clinical gaze came into their own. Consequently, a self-consciously 'scientific medicine' took form, leaving little room for the supposedly self-indulgent subjectivism of literature and the arts.[5]

Admirable monographic studies have examined nineteenth-century literary figures who, in various ways, continued to engage the sciences of their day. What we lack is a general history of the two cultures. Such a broad-ranging account, I believe, would in fact disprove the idea of a permanent post-Enlightenment bifurcation between the worlds of art and science. That historical view rests far too exclusively on the short and unreliable statements of three or four literary elites who found it useful to define their own artistic identities in ideological opposition to the operations of science. In the paragraphs that follow, I show that the historic dialogue between the two cultural realms – a dialogue that thrived in early modern Britain but had largely shut down in the educational world of Snow's postwar Oxbridge – flourished a good deal *later* than is widely believed. The forces that would push the sciences and humanities apart gathered as the 1800s wore on, to be sure; but a heavy traffic in ideas, images, personalities and publications across these cultural fields still ran in the *fin de siècle* and pre-war periods.

What is more, that rich and complex interplay was a crucial and constitutive part of European avant-garde thought and culture during these eras. This cultural phenomenon can be found throughout the sciences, including in the subject Porter made his own, the sciences of the mind.

*

The science/medicine/culture nexus a century ago took many forms. A remarkably large number of novelists, painters and philosophers from these decades had received medical training.[6] In the literary world, still more figures were either linked by family affiliation to physicians or read deeply in the medical literature of their day. (Flaubert, Hesse, Hofmannsthal, Lawrence, Lewis, Proust, Turgenev, Woolf, and Zola come to mind.) The most familiar of these boundary crossings involve men and women of the arts drawing creatively on recent science in their own artistic projects. We think of Charles Darwin's influence on the writing of George Eliot and Thomas Hardy; Émile Zola's programme to apply Claude Bernard's laboratory methodology to 'the experimental novel'; Georges Seurat's study of Maxwell's, Helmholtz's and Chevreul's theories of optics and colour psychology in the creation of his pointillist, neo-Impressionist style; Pablo Picasso's familiarity with work in physical anthropology as he painted *Les Demoiselles d'Avignon*, and so on. These examples reinforce John Limon's observation that science is typically regarded as the seminal force in Western civilization whose influence spreads outward culturally. But, as Gillian Beer insisted in her brilliant study of Darwin and mid-Victorian literary culture, the interchange of ideas across the latter half of the nineteenth century was in fact bi-directional.[7] During the penultimate decade of the nineteenth century, novels, poems, play and paintings did not simply and secondarily voice languages of the self, psyche and pathology that had been created elsewhere. Rather, *fin-de-siècle* and *aube-de-siècle* scientific intellectuals were as influenced by cutting-edge philosophical thought, cultural representations and popular stereotypes as novelists and artists were informed by science and medicine. Further, these interactions involved more than individual scientists and physicians simply partaking in the activities of the 'other culture'. They informed the subjects, models, methods and language deployed in the sciences.

Jean-Martin Charcot, for instance, became 'the founder of neurology' and the great hysteria theorist of the French *belle époque*; but he also maintained passionate artistic interests. He collected paintings – his taste was conservative, the Old Masters, rather than the Impressionist innovators exhibiting around him – and he illustrated his personal letters in his own hand. Politicians, poets, novelists, architects, painters and sculptors were as likely as doctors and scientists to grace his Tuesday evening *soirées* in the Faubourg Saint-Germain. He pioneered the application of photography to medicine and established a full-time photographic *atelier* on the grounds of the Salpêtrière hospital, the first of its kind in the world. Most substantively, Charcot's clinical epistemology, with its distinctive construction of

diagnostic *tableaux*, was informed by a highly artistic sensibility. Observers at the time, including Freud, characterized Charcot as an intellectual '*visuel*', whose clinical portraits of neurological disease syndromes, such as tabes dorsalis and amyotrophic lateral sclerosis, had a crystalline, descriptive quality that makes them classics in medical history.

The artistic and the scientific in fact overlapped continually in Charcot's career: in 1887, he co-authored *Les démoniaques dans l'art*, which analysed neuropathologically past paintings and engravings. André Brouillet's dramatic 1886 canvas of Charcot displaying a hypnotized hysteric patient to a rapt audience of listeners includes in the assemblage two novelists, a politician and an Impressionist art critic. And Paul Richer was Charcot's *interne* whose 800-page *Études clinique de l'hystéro-épilepsie* of 1881, replete with dramatic illustrations of the stylized hysterical fit, systematized the Charcotian model of hysteria. Richer was later appointed professor of artistic anatomy at the Paris École des Beaux Arts.[8]

In turn, Charcot's teachings doubled back and exerted a strong, if somewhat sensationalist, attraction on literary intellectuals. Jules Claretie, Léon Daudet, Axel Munthe and Leo Tolstoy all depicted the 'Napoleon of the neuroses' in their imaginative writings. On the popular level, novel after novel featured Charcot-style hysteria, including Maurice Rollinat's *Les névrosés* (1883), Daniel Lesueur's *Névrosée* (1890) and Armand Dubarry's *Hystérique* (1897). The best-known play about life at the Salpêtrière was *Une leçon à la Salpêtrière* by André de Lorde, the son of a physician, who dedicated the work to the psychologist Alfred Binet.[9] A generation later, the Surrealist theorists André Breton and Louis Aragon christened Charcotian hysteria 'the most poetic discovery of the nineteenth century'.[10] And then there is Marcel Proust. Proust's sprawling, multi-volume masterpiece *In Search of Lost Time* (1913–27) is the leading document in French literary modernism. Scholars have reconstructed Proust's 'sources' and include Charcot on hysteria, Bergson on time, Janet on psychological automatisms, Binet on fetishisms, Ribot on maladies of the will and Egger on the stream of consciousness. Proust's father Adrien was professor of public hygiene on the Paris Medical Faculty and co-author of a book on nervousness.

An intriguing German example of these art/science linkages is provided by Ernst Haeckel (1834–1919). Professor of zoology at the University of Jena, Haeckel is best known today for his championing of Darwinian theory in German Europe, for formulating the concept that 'ontogeny recapitulates phylogeny' and for coining the term 'ecology'. He was one of the most popular and prolific scientific *savants* in the late nineteenth and early twentieth centuries. Haeckel, it turns out, was also an accomplished visual artist who, in the finest Goethean tradition, celebrated the truth and beauty of nature in countless drawings, watercolours and oil paintings. In his 1899 volume of illustrations titled *Arts Forms in Nature* (*Kunstformen der Natur*), Haeckel recorded in 100 folio images his phantasmagorical vision of insects, reptiles and marine life. His representations of coral, sponges, starfish,

anemones and jellyfish in particular, many of them observed by him microscopically, were far more than lifeless, mechanical illustrations of the text. Rather, Haeckel lavished his aesthetic attention on the colour and design of these extraordinary organisms: in strict visual terms, their decorative, naturalistic detail calls to mind the drawings and etchings of Albrecht Dürer. The exotic, imaginative quality of the plates is reminiscent of the make-believe animal figures in the paintings of Hieronymus Bosch, and their ornamental intricacies recall the horticultural wallpaper designs of the Victorian poet and Arts and Crafts theorist William Morris[11] (Plates 1 and 2).

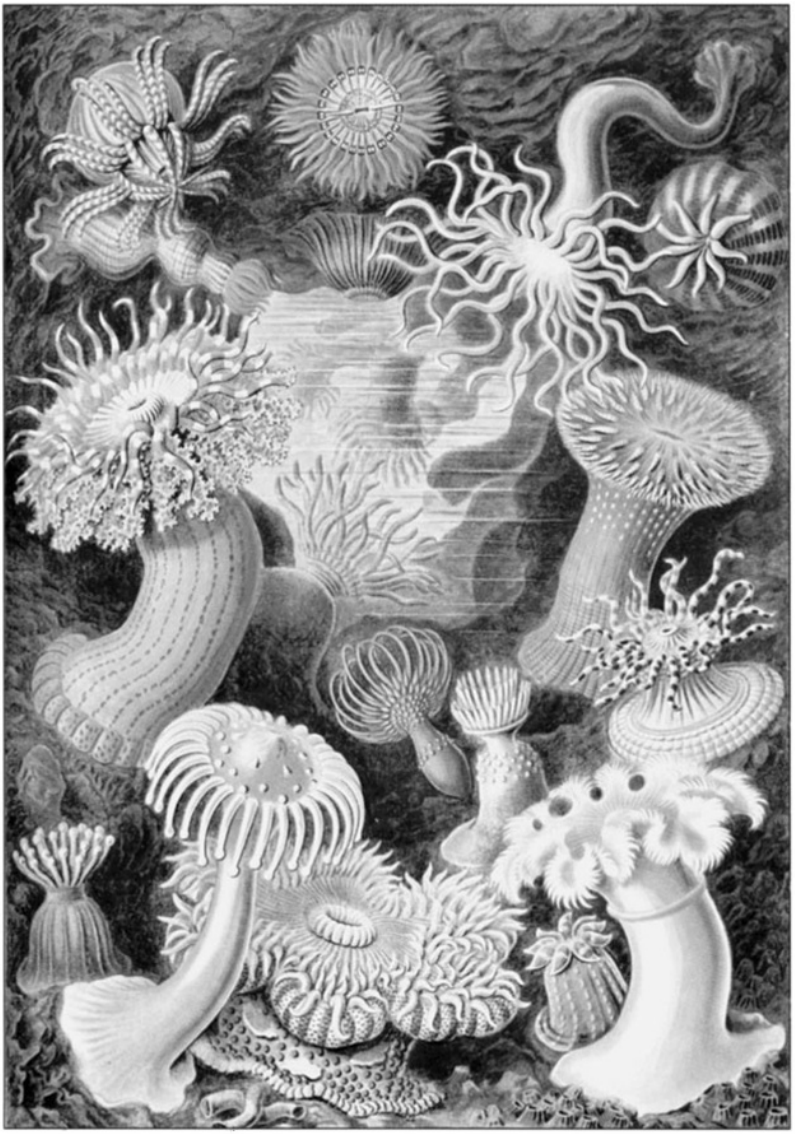
Haeckel's gorgeous hand-coloured drawings and etchings made their way into innumerable books and magazines, and from there informed popular understanding of evolution and inspired an entire movement of 'evolutionary aesthetics'. Furthermore, Christoph Kockerbeck has shown that Haeckel's spiny, extraterrestrial images were borrowed by several art nouveau and *Jugendstil* artists, including the Parisian architect René Binet, who used them as inspiration for a pavilion built at the 1900 Universal Exhibition in Paris, and the German graphic artist Hermann Obrist. The American dancer Isadora Duncan was likewise an ardent admirer, and during her tour of Germany in 1903–4 she partly based her expressive 'dance of the future' on his evolutionary ideas.[12] Haeckel is also believed to be the first person, using his own drawings, to illustrate discussions of Darwin with a 'phylogenetic tree', which became a common diagrammatic conceptualization in textbooks of evolutionary biology.

Richard Noll emphasizes that Haeckel pursued and popularized the philosophical implications of his work too. As the new century began, he published his bestselling *Riddle of the Universe* (*Die Weltraetsel*). Here Haeckel advanced his philosophy of monism, derived from the extensive study of zoology and botany. Haeckel intended his new monistic belief system to replace all earlier dualisms, including matter/spirit, mind/body and the divine/human. With his self-styled science-based faith, Nature itself would be worshipped through a new aesthetic vision. All scientists would develop the skills and sensitivities of artists, and artists would celebrate natural life as it flourished evolutionarily.[13] Haeckel in part envisaged his extravagant, pantheistic religion as a way to synthesize Beauty, Nature and God, i.e., to unify art, science and religion. Not surprisingly, his monistic teachings attracted many artists and intellectuals.[14]

Yet another telling example is provided by the Spanish neurohistologist Santiago Ramón y Cajal (1852–1934). Cajal is renowned for his brilliant work on the structure and function of nerve cells, for which he eventually shared the Nobel Prize. Indicatively, Cajal's first passion had been painting, not science, and for years he struggled with his parents before giving up the palette for the microscope. In his autobiography, he insisted that he first sharpened his observational skills of the natural world as a painter in his youth.

From the start of his career in neuroanatomy and neurophysiology, Cajal was consumed with the question of 'the fine structure' of brain tissue,

1. William Morris, furnishing fabric 'Wey' (1883). Courtesy of V & A Images/ Victoria and Albert Museum.



2. Ernst Haeckel, 'Actiniae' (1904). Plate 49 from *Kunstformen der Natur.*

that is, with what microscopic nerve cells actually look like. European laboratory scientists had for decades struggled to find a chemical fixative that would make nerve cells stand out against their tangled, undifferentiated background. Working in Barcelona in 1888, Cajal employed an elaborate, multi-step procedure of soaking, staining and mounting tissue samples using a silver nitrate solution as the key ingredient. For the first time in medical history, the intricate mass of nerve cells and fibres leapt out in high resolution to the observer's objectifying gaze. What Cajal saw astounded him: scientists previously had hypothesized that cells in the brain and spinal column formed a single, interconnected network of undifferentiated grey matter. Cajal's technique, however, highlighted an infinite number of discrete, atomistic nerve cells whose fibres extended outward, but did not interpenetrate. In what became the inaugural event of modern neuroscience, Cajal had isolated the neuron, the basic cellular unit of brain function. This structural observation then opened up the way for new ideas about function: 15 years later, Charles Sherrington advanced the concept of the synapse which leapt electrically from one individual neuron to another.

Revealingly, Cajal did not just write about what he observed in the late 1880s; he also drew it as precisely as possible. At a meeting of the German Anatomical Society in Berlin, it was less his lecture than his exhibition of slides and drawings on paper with coloured inks that drew his colleagues' attention. In subsequent publications, Cajal made his own photographic plates and used his earlier knowledge of lithographic engraving to illustrate accurately his own scientific papers. Cajal's drive to visualize form and function culminated in his *Texture of the Nervous System of Man and the Vertebrates* (1899–1904), which is full of fascinating, exquisitely detailed illustrations more reminiscent of the wiry line drawings of Paul Klee than a textbook of neuroanatomy (Figure 9). Decades later, in his *Recollections of My Life* (1937), Cajal used a multitude of aesthetic metaphors to describe his scientific work. 'If the true vocation of the artist is to see, Cajal's great discovery was that of an artist,' writes a recent commentator. 'The neuron was a painter's discovery, made by observation and draftsmanship.'[15] Like Charcot's and Haeckel's illustrations, Cajal's intricate visualizations spread to the non-scientific world, where they were widely reproduced and where they appear in paintings by artists such as the Norwegian Modernist Edvard Munch.

In these three cases, art and science are not just compatible; they are coextensive. The work of the French clinical neurologist Charcot, the German evolutionary biologist Haeckel and the Spanish neuroanatomist Ramón y Cajal literally looks different because of its encounter with art, broadly considered. Exactly how 'the artistic' figured in their work – as philosophical resource, as epistemological modality or as representational technique – varied. Furthermore, in each instance, the scientific work in question went

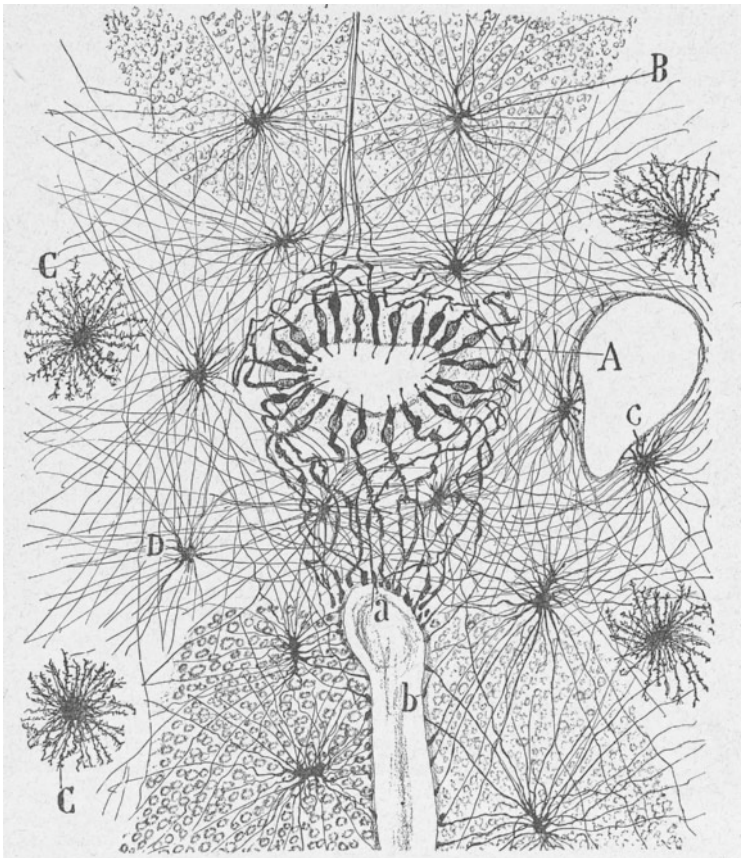


Fig. 77. — Neuroglia de la región central gris y porciones vecinas de la substancia blanca de la médula espinal de un niño de ocho días (método de Golgi).—A, células del epéndimo ; B, neuroglia de la substancia blanca ; C, neuroglia de cortas radiaciones ; *b*, engrosamientos terminales de las hebras neuróglicas.

Figure 9 Santiago Ramón y Cajal, 'Neuroglia de la region central gris . . . (metodo de Golgi)', in *Textura del sistema nervioso del hombre y de los vertebrados* . . . (1899). Wellcome Library, London.

on to become culturally available for artistic reappropriation. The model of influence is less one- or two-directional than circular.

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The above examples concern science, medicine and the visual arts. Imaginative literature was of course another major arena of interaction. Throughout the Victorian and Edwardian eras, it was common for physicians to write

prolifically, including novels, poetry, autobiographies, medical memoirs, biographies of other doctors and social commentaries of their own time. The pattern again holds across Europe during the *fin-de-siècle* decades. Henri Beaunis became the first director of the psychological laboratory at the Sorbonne, where he published extensively on the anatomy, physiology and pathology of the central nervous system during the years of the early Third Republic. Along the way, Beaunis found time to write poetry, short stories and plays, a collection of which was published near the end of his life.[16] Alfred Binet became France's leading experimental psychologist of the period and is best known today for his work on measuring the intelligence of children; Binet was a playwright too, and at least one of his dramas was performed at the Sarah Bernhardt Theatre.

Interestingly, in German-speaking central Europe, it was poetry rather than prose that attracted most scientific authors. Based in Heidelberg and then Munich, Emil Kraepelin was by the 1890s the best-known psychiatric clinician in Germany, who introduced the diagnostic categories of manic-depressive psychosis and dementia praecox (i.e. schizophrenia) and formulated the longitudinal case history. In his spare time, as it were, Kraepelin wrote poetry. He was talented enough to get an entire volume of his verse, consisting of 60 poems, published under the rather Hegelian title *Werden – Sein – Vergehen*.[17] Analogously, Theodor Meynert, professor of psychiatry at the Vienna Medical School, has the unenviable reputation today, fostered by Freud's biographers, of being a hard-nosed, dogmatic somaticist who refused to take the neuroses as a subject of serious study. It comes somewhat as a surprise, then, to learn that the eminent neuropathologist was also an accomplished poet, who cultivated Shakespeare, Byron, Schiller and Herder.[18]

Doubtless in some instances the late nineteenth-century physician-*littérateur* was a dilettante figure. In other cases, however, the two activities again directly informed one another in ways that were quite substantive. Freud is the best-known example, and we should pause briefly to appreciate his case. In Vienna during the 1870s and 1880s, Freud received medical training and began his career as a laboratory neuroanatomist and clinical neurologist. Earlier, however, he had been schooled in the broad, central European tradition of the classical humanities. In creating psychoanalysis, he drew key terms and concepts – catharsis, cathexis, the Oedipus complex, narcissism – from Shakespeare and the Greek tragedians. In Sophocles' *Oedipus Rex*, Freud found a guide to the universal experience of a male child loving his mother and resenting his father. Among the literary moderns, the Swiss novelist Carl Spitteler's *Imago* (1906) furnished psychoanalysis not only with the name of its cultural journal, but with the concept of the idealized and illusory character of a beloved's image as seen by a lover. Similarly, Freud's concepts of catharsis and abreaction, which are key in modern psychological healing, originated in ancient Greek theatre and were much

discussed in *fin-de-siècle* Viennese cultural circles. Many of Freud's books received reviews in literary and philosophical as well as medical journals. Writing in the Preface to *The Interpretation of Dreams* of 1900, Freud insisted, somewhat defensively, that he was 'not a poet but a natural scientist [nicht Poet sondern Naturforscher]', and later in his life the city of Frankfurt awarded him the Goethe Prize for his German prose writing, whereas a Nobel Prize for medicine eluded him. Whether Freud was actually more open than other psychologists to, or just more honest about, his borrowings from non-scientific sources is a matter for debate. Whichever, countless creative writers and artists during the twentieth century were in turn influenced by his ideas.

Perhaps the most significant convergence of the medical and the literary during the late nineteenth and early twentieth centuries involves form and genre. Before the final quarter of the 1800s, psychiatric cases tended to be short and mechanical recitations of hereditary background and symptom profile, diagnosis and prognosis. New models of mental functioning emerging during the *fin de siècle*, however, were centrally concerned with consciousness and the inner mental life of patients; as a consequence, the new aim of psychiatric case histories beginning around 1890 was the representation of individual emotional experience and intrapsychic subjectivity. Clinical reports, in short, became studies of minds through time. This change, needless to say, required new presentational strategies. In a parallel development, language, the spoken word, increasingly became the primary therapeutic agent as doctors and clients began using introspective and free associative techniques. Case history writing during this generation, in other words, became much more 'literary'.[19] As a result, the period 1880–1920 witnessed the publication of a sequence of highly readable, individualized, often book-length case histories – Eugène Azam's Félida; Janet's Irène and Madeleine; Josef Breuer's Anna O.; Jung's Hélène Preiswerk; Théodore Flournoy's Hélène Smith; Freud's Dora, Wolf Man and Rat Man; and Morton Prince's Miss Beauchamp.

The new novelistic case histories of this generation were demonstrably indebted to ancient and Modernist literary practices. In chapter 9 of his *Principles of Psychology* (1890), the physician/philosopher William James coined the phrase 'stream of consciousness'; the term caught on and was later applied to Joyce, Proust, Woolf and Faulkner, among many others. Furthermore, the interior monologue has a literary history long pre-dating modern psychiatry, and introspection and story-telling are the creative writer's methods.[20] In these psychiatric and literary texts alike, the distinction between 'introspective knowledge' and 'objective knowledge', which Michel Serres has highlighted, began to dissolve.[21]

If *fin-de-siècle* clinical literature became more narrative, novel-writing at the same time probed human nature quasi-scientifically. Jacqueline Carroy has studied the mutual literary and medico-psychological exploration of double and multiple personality during these years. Robert Louis Stevenson's *Strange*

Case of Dr. Jekyll and Mr. Hyde of 1886, Carroy points out, was only the best known of many novels about split personalities at a time when physicians from Morton Prince in Boston to Frederic Myers in London to Pierre Janet in Paris were also exploring the subject.[22] In parallel fashion, scholars have brought out the extent to which an entire 'culture of hysteria' formed during *belle époque* France, in which religious, medical, legal, popular, visual and journalistic images of hysteria continually circulated. Henri Ellenberger once commented on the striking descriptive homologies between clinical personalities and literary characters throughout this era. Elsewhere, I have tried to bring out the many ways in which the literary/artistic and medical/scientific avant gardes a century ago collaborated in a common probing of sexuality, subjectivity and self-identity.

Again, varied examples abound: in Vienna, Arthur Schnitzler trained in medicine from 1879 to 1885. Like his contemporary Freud, he specialized in psychiatry and studied with Meynert. Soon after graduating, however, Schnitzler abandoned medicine and began to write plays and novels, eventually becoming one of Vienna's leading Modernist authors. Schnitzler's dramas and fictions are full of dreamy, neurotic characters with overstimulated sex lives. Freud admired Schnitzler's psychological portraiture and dubbed his erstwhile colleague his 'double [*Doppelganger*]'.[23]

Similarly, Frederick Van Eeden was the leading exponent of the new psychological approaches in the Netherlands. The first major biography of Van Eeden (who deserves to be better known) has recently appeared.[24] Van Eeden, we learn, pioneered the modern psychotherapy of the subconscious and in 1887, with his colleague Albert Van Roentergam, opened the first institute of psychotherapy in Amsterdam (perhaps the first in all Europe). Van Eeden began to study dream psychology systematically in the later 1890s; but he went further even than Freud in embracing literary methods and insights. In 1909, he published *The Bride of Dreams*, which condenses his clinical observations into novel form. He also wrote poetry voluminously, as well as art and literary criticism.[25]

Finally, Charles Richet was a member of the Salpêtrière school of neurology. It was Richet who first aroused Charcot's interest in hypnosis and who later won the Nobel Prize for physiology and medicine for his research on anaphylaxis, or severe allergic reaction to an outside agent. Richet, too, wrote poetry and novels under the pen-name Charles Epheyre. Among his many novels, *Possession* of 1887 and *Soeur Marthe* of 1890 interweave the themes of love, hysteria, hypnotism and somnambulism. Carroy has argued that, read in chronological order, Richet's medical and literary productions provide parallel explorations of the same set of themes in human pathology and psychology. Richet/Ephyre worked simultaneously and symbiotically as an artistic and a scientific intellectual.[26]

The examples I have adduced hail from Paris, Jena, Munich, Heidelberg, Vienna, Barcelona, Amsterdam and Zurich. They involve numerous fields of

scientific endeavour, including clinical neurology and psychiatry, psychoanalysis, histology, neurophysiology, zoology and botany, psychology and internal medicine. Many other examples could be included. Clearly, the two cultures were in continual and highly fertile communication throughout this period.[27]

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How were these myriad cultural criss-crossings possible half a century after the Romantic generation that Snow and his followers pinpointed as decisive in the history of the two cultures? What were the conditions that continued to make an integrated medico-literary culture possible throughout the European *fin de siècle*? There were no doubt many reasons. In the last section of this essay, I want to flag three sets of circumstances that were relevant.

First, throughout this period the tradition of a shared classical humanistic education endured. Across the 1800s, European governments put in place national systems of education, which differed significantly by nation. None the less, through the early twentieth century, physicians across Europe received a broad liberal education, learned to read multiple languages, became well versed in classical and modern literature and were knowledgeable about the history of the arts. This intellectual training was highly formative. As Allan Janik has reminded us regarding the Vienna of Freud, Kraus, Schoenberg and Wittgenstein,

> it is difficult to exaggerate the importance of the classical gymnasium: years spent translating Latin and Greek classics sensitized the Viennese pupil both to rhetorical structure and architectonics as well as to the nuances of style that make elegant self-expression possible. However tedious this may have been for the pupils, this rigorous regimentation produced minds of a caliber seldom seen today. . . . Rigorous classical education was absolutely crucial in making *Alt Wien* into the creative milieu it was.[28]

In France, many figures who went on to distinguish themselves in science, medicine, philosophy and literature were educated side by side at the École Normale Supérieure. At the prestigious secondary schools, or *lycées*, a thorough grounding in Latin was *de rigueur*, as was a familiarity with Shakespeare, Goethe and the French literary classics. The final year of lyceum training for all students was devoted to philosophy. In fact, an undergraduate education in Greek and Latin was required to get into French medical schools until as late as 1902, at which time the classical baccalaureate became optional. At the same time, most university graduate *facultés de lettres* in France required courses in philosophy and psychology. In Britain, throughout the Victorian and Edwardian years higher-ranking professionals and government officials continued to receive a liberal arts education as a sign of

class status and financial privilege. In other words, well into the twentieth century, individuals who would later engage in divergent professional activities across the cultural spectrum had been trained, institutionally and intellectually, in the same or similar environments. No historical change was more consequential for the eventual cleavage between the two cultures, I believe, than the rapid decline of classical liberal arts education in the post-First World War years.

A second fostering circumstance involves the infrastructure of European culture during the closing decades of the 1800s. The *fin de siècle* was a unique cultural moment during which ideas, texts, theories, images and opinions were exchanged across national boundaries and disciplinary frontiers at greater speed and in higher volume than ever before. Throughout Europe, cities exploded in size, with London, Edinburgh, Glasgow, Amsterdam, Copenhagen, Stockholm, Paris, Berlin, Vienna and St Petersburg thriving as cultural centres. Literacy rose dramatically throughout the century, and new universities were founded. More and better translations of books appeared all the time. Mechanized systems of transportation, especially train travel, allowed much easier and faster movement between cities. This was also the great age of international scientific congresses, where papers were routinely delivered in four or five languages. In Vienna and Paris in particular, those epicentres of Modernist experimentation, the sheer spatial intimacies were remarkable: libraries, laboratories, galleries, museums, salons, universities, bookshops, institutes, publishers, concert halls, opera houses and amphitheatres were often located in the same neighbourhoods, and a remarkable number of cultural celebrities from the period personally knew and interacted with one another. Well-travelled polyglot *savants* in medicine and the sciences, like Charcot, Rudolf Virchow and William James, conducted far-flung correspondences with scores of cultural figures. Furthermore, dozens of publications brought the latest data and ideas of science to the general educated public in an accessible, non-technical language. These circumstances virtually ensured continual cross-cultural interactions.

Third and finally, I suspect that a development in scientific methodology and epistemology further encouraged this interplay. *Fin-de-siècle* intellectual history continues to be associated with what H. Stuart Hughes two generations ago termed 'the revolt against positivism'.[29] During the period 1870–1914, the Enlightenment/Victorian scientific worldview came under critique as excessively realist in one field of thought and culture after another, including in the sciences themselves. Physicians and philosophers from Mach and Bergson to Poincaré and Einstein began to emphasize the degree to which science provided less a literal and direct account of the external world than a collection of models and representations of that world mediated by human perception, cognition and language. In avant-garde physics, epistemology, moral philosophy, novel writing and painting alike, the subjectivities and relativities of nature, society and the human psyche

were highlighted anew. Assuredly, this cultural climate was congenial to a rapprochement between the sciences and humanities. The rationalist certainties and positivist principles of Descartes, Newton and Comte had facilitated the first drifting apart of the two cultures during the late eighteenth and early nineteenth centuries; conversely, the epistemological crises of early European Modernism opened up a cultural space where the two could re-engage. It is no coincidence that perspectivalism, a psychological turn inward and a critical interrogation of the inherited models of knowledge and rules of practice mark both the arts and the sciences of the European *fin de siècle*.

By the 1920s, these three sets of circumstances were undergoing rapid change. Scientism reasserted itself in the form of logical positivism. Scientific discourses became more and more limited to technical experts and less available to general educated audiences. As professionalization advanced, science increasingly became an occupation. Institutions of higher learning set out branches of knowledge in the form of specialized academic 'disciplines' and the Enlightenment ideal of a unitary, pan-European culture of humanism waned after the carnage of the First World War. Above all, across Europe, schooling from the time of adolescence became channelled into mutually exclusive vocational tracks. Once art and science came to involve wholly different types and trajectories of education – different mental formations from young adulthood onward – the essential precondition for a shared cultural conversation disappeared. To be sure, some productive art/science linkages continued between the world wars – in the mental sciences, the existential and phenomenological psychiatry of the 1930s and 1940s are outstanding examples – just as some still do today; but there is little doubt that *the degree of differentiation* between the two enterprises was growing rapidly, just as C. P. Snow came into his own. Cultural relations between the two realms changed less between 1800 and 1914 than they did between 1918 and 1950.

At the outset of the twenty-first century, it is instructive to update Snow's cultural account. In the mental sciences today, various forms of biological psychiatry are ascendant, and the leading collateral fields of knowledge are judged to be chemistry, genetics, endocrinology, pharmacology and molecular biology, not literature, philosophy, painting and poetry. Some present-day neuroscientists have even suggested that, as types of human mental activity, art and science are located on different sides of the brain. A neurobiological theory of the two cultures!

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The epigraph of this commemorative volume, 'for he wrote *de omni scribili*', is drawn from Laurence Sterne's *The Life and Opinions of Tristram Shandy, Gentleman* (1759–67). Sterne's novel is said to have been Roy Porter's all-time favourite. Sterne and his prose masterpiece are relevant for a reason

specific to this chapter, too. Sterne was a cleric by occupation, but his comic, whimsical narrative is saturated with science and medicine, from the famous opening excursions on his conception and birth, to his characterizations of midwives and quack doctors, to the endless satirical digressions on health and sickness. The book's medicalized worldview is key to the protagonist's (and the author's) mental and emotional autobiography; it is also, scholars have determined, well informed by the medical learning of the day.[30] Sterne, in other words, neatly exemplifies the synthesis of art, science and autobiography that prevailed in Georgian Britain. Porter – an exceptionally widely read historian of medicine and a gifted story-teller – may well have been drawn to the book in part for this reason. Most readers today will have to repair to a Latin dictionary to translate Sterne's phrase. But it would immediately have been understood by the cast of eighteenth-century physicians that Porter celebrated – and by the European medical and scientific community of the *fin de siècle*.

18
Roy

William F. Bynum

Roy's sudden death took everyone by surprise. He seemed indestructible and vulnerable at the same time. For years, especially in winter, you could tell that he was in the building by his rasping cough carried down the corridors. But then this was nothing new, and three days before his death, he was showing someone into his room as we exchanged greetings for the first time in months (he had been in Peru). I knew he looked ill, but I too was shocked with the news of his death late on the Sunday evening.

One of Roy's most amazing characteristics was his energy. I used to get into the Wellcome Institute in the morning shortly after 7 am, and he generally had been at his desk for some time. Even when he had been out the evening before lecturing, he always took the early train to London, and he especially liked lecturing in Scotland, when the night train got into Euston around 6 am. Only since his death has Natsu Hattori told me that he didn't actually like getting up so early. He was human, just more driven than most of us.

Roy was the only child of a Bermondsey jeweller. He described his home as being without books and liked to say that his intellectual precocity led to a family legend that he was a changeling. A history teacher at Wilson's grammar school, Camberwell, recognized that precocity and encouraged his intellectual development. He would return year after year to his old school to talk to its students and try to repay the debt that he felt.

His starred double first in history at Cambridge (1968) led to a junior research fellowship at his (and Charles Darwin's) college, Christ's. There, his life-long love affair with the eighteenth century had been awakened by Sir Jack Plumb, with whom he always had an affectionate, if ironic, relationship. I heard his lectures on the English Enlightenment in 1972. They were the beginnings of what I think he then saw as his *magnum opus* and was to be one of his last books: *Enlightenment: Britain and the Creation of the Modern World* (2000). It was a source of amusement that I was able to supply him, years afterwards, with copies of his typescripts for these lectures. It was just luck, since I am a notoriously bad filer and he was an excellent one.

The fact that there were typescripts of these early lectures provides a clue to his later success. He knew what all comedians know, that spontaneity has to be practised. Whenever he had a lecture to give, he would close his door and rehearse it. Practice and his brilliant memory led to the appearance of spontaneity. Sometimes the lecture was so much a part of him that the text could be dispensed with altogether. Probably the most impressive performance I ever heard him give was an exposition of the ideas of Michel Foucault. His only props were two books, in which quotations he wished to use were marked. He spoke for 50 minutes, without (as the radio programme has it) hesitation, deviation or repetition. He had complained to me earlier that this lecture was giving him particular trouble. The audience certainly couldn't tell it.

Roy's Cambridge decade was highly successful. Robert Young's special subject on 'Man's Place in Nature' attracted him to the history of science, and his PhD on geology in England during the long eighteenth century appeared as his first book. His junior research fellowship at Christ's was followed by a proper teaching fellowship and directorship of studies in history at Churchill. By the time he left Cambridge for London, he was also Churchill's Dean: it was both amusing and a sign of the times that so secular a man could hold such a title. As a Londoner, however, he had found Cambridge too cosy, and in 1978, the Wellcome Trust agreed to fund a new post with what they considered the very dangerous title of social historian of medicine. When Roy applied, it seemed to me unnecessary to interview anyone else. It wouldn't happen today, but the UCL officials agreed to the suggestion, as long as someone from the History Department vetted the applications. Professor I. R. Christie was to cast the deciding vote, so I duly gave him the bundle of applications and told him that I thought one was head and shoulders above the rest. I returned the next day, and he handed back the parcel with the droll words, 'I suppose you mean Porter.'

Except for a year as Clark Professor at UCLA, he stayed at the Wellcome until his retirement. There were lots of offers, but Roy knew when he was well off. At the Institute, he had light teaching duties, bright students and wonderful facilities. Besides, it was in London, where he always felt at home. His history of his native city, *London: A Social History* (1994), is in many ways his best book. Unusually for Roy, this book was not produced on the date the contract specified. He started working on it not long after he came back to London, and for several years, he devotedly spent his weekends exploring different parts of London. His knowledge and love of the metropolis and its people shine on every page. In its closing pages, with his powerful analysis of the pettiness of Margaret Thatcher's vendetta against Ken Livingstone's Greater London Council, he allowed himself a political voice that was mostly hidden from his writings. I knew him well for more than 30 years, but never knew how he voted.

His history of London was one of a number of books that kept his 'proper' credentials with the 'real' historians alive. At the same time, Roy played a

major role in the wider historical acceptance of health and medicine as real, vital subjects of sophisticated historical enquiry. Such was his charm, he also was equally successful in convincing the group that medical historians generally call 'old docs' that the history of medicine was of real moment. I had one old doc, now himself deceased, weeping on the telephone to me a couple of days after Roy died.

Roy was a natural scholar, but the nature of his scholarship changed over the years. The media discovered him, and the hectic pace that had gradually accelerated over the years became almost frantic. Roy possessed that rare quality in an academic: a journalist's commitment to deadlines. It made him an editor's dream. Books would often arrive by courier at his desk late in the afternoon, and the following morning, a thoughtful review would go out. He wrote faster than most people read, and he read even faster than he wrote. Whenever he was confronted with the fact that, perhaps, he was doing too much, he would simply reply that he accepted only a fraction of the things that were offered to him. Invitations for lectures, radio and television programmes, PhD examining, puffs for books: these and things of like ilk filled his mailbox each morning, and each evening his desk was clear. It was the capacity for work, combined with his organization and amazing intelligence, which set him apart.

The pace told on him, of course. In his later years, he became much more protective of his time. He had five minutes for practically anyone, but then he wanted to get the business over and return to his computer, or dash off for a lecture or broadcast. He loved the fax, and replies to letters were generally a few words scribbled at the bottom, and faxed by return. He disliked the phone for business, and email was a very late addition to his communication repertoire. Few people would have announced yet another divorce (there were four) by a 'post-it' on the mailroom notice board, but it was completely in character with the man. Efficiency was all.

He produced more books than most of us produce papers. Some of his edited volumes have come to light only after his death, and even the single-authored books that have appeared posthumously would have been enough to secure tenure for most people. He edited *History of Science* seemingly effortlessly for more than two decades, founded and co-edited *History of Psychiatry*, and edited several series of monographs. His *Greatest Benefit of Mankind*, the book that many people will remember, was simply one of a series of monographs of which he was the general editor.

Roy took early retirement partly because he knew he could live well by his wits, and academic life had become full of bureaucracy. Even in his short retirement, he continued to live in the fast lane. But he always favoured the bicycle as his preferred mode of transportation. It was by his bicycle, with a handful of daffodils from his allotment, that he died, all too soon.

Notes on Contributors / Remembering Roy Porter

Not only was Roy Porter a brilliant scholar and a dedicated colleague, he was also a remarkable, generous and idiosyncratic individual – in fact, he was exactly the kind of person about whom Roy himself loved to write. We were all lucky that our lives were enriched by his presence. In this section, conventionally reserved for descriptions of ourselves, many of us have also added favourite memories of Roy, or moments when his influence shaped our ideas and careers.

Roberta Bivins is Wellcome Lecturer in the History of Medicine at Cardiff University. The topic of her most recent book,*Alternative Medicine? A Global Approach* (2007), was suggested to her by Roy Porter many years ago. She is now studying the reciprocal impact of immigration and medical research/healthcare delivery in the US and UK since the Second World War.

'As a nervous PhD student, I came to Roy via the most tenuous of connections: my American supervisor happened to know Roy's wife. It is typical of his generosity that Roy invited me to London sight unseen, simply as a favour to his wife's acquaintance. Dutifully, he scheduled regular lunches with me to check on my progress. Alongside the sandwiches and the inspiration, Roy offered me, in his inimitable way, a much-needed sense of belonging. One lunchtime, he dropped into his chair, grinned, and reported: "My mother has a question for you: did the royals use acupuncture?" I was surprised – but the idea that Roy's mum knew all about my dissertation suddenly made me feel right at home in London. [The answer was, "Quite possibly."]

In later years, when I returned as one of his many post-doctoral fellows, Roy could always console me for the slings and arrows of academic life. After a dire job interview during which I had been scolded for looking too young, Roy told me a story. In the previous year, he had accepted an invitation to address a provincial medical society. The society's gratified president arranged to meet his distinguished guest at the railway station on the appointed day. Arriving on schedule, Roy watched his fellow passengers disappear into the gathering dusk, until only he and a smartly dressed older gentleman remained. Roy approached, introduced himself – and met with complete disbelief. After further efforts to convince the man of his identity failed, Roy suggested that as he could not look the part, perhaps he had better go home. His companion curtly agreed, and Roy took the next train back to London. I have since told the same story to my own disheartened graduate students, and they too have been consoled by the absurd proof it offers: that sometimes even the finest scholars, like the best books, are judged only by their covers.'

Peter Burke was Professor of Cultural History at the University of Cambridge until his recent retirement and remains Life Fellow of Emmanuel College. His most recent books, both published in 2004, are *What is Cultural History?* and *Languages and Communities in Early Modern Europe*. With Roy, he co-edited *A Social History of Language*. 'I asked him for a contribution, and he was so helpful in suggesting others that I thought it appropriate to take him on board as editor!' He recalls with fondness Roy's floral ties in the early days, his literally as well as metaphorically unbuttoned approach in the later period, and his unique style of correspondence: saving time by writing concise but highly apposite comments on the original letter and sending it back.

Ian Burney is Senior Lecturer in the History of Medicine at the Centre for the History of Science, Technology and Medicine at the University of Manchester. His most recent book, *Poison, Detection and the Victorian Imagination* (2006), is published in the University of Manchester Press's 'Encounters' series.

'My first meeting with Roy Porter, though brief, was profound. I had recently arrived in London to start my PhD research on aspects of what I then described as "the medicalization of the urban social body". In the early months of my research I came across a peculiar institution, the coroner's inquest, and was drawn especially to the "native" language of inquests, its practical embrace of a vision of English particularism that seemed to mark a difference from the bio-power model I had come to find. Yet when I met Roy to give an account of my plans, describing it as an account of medicalizing the inquest, his response went something like this: "Medicalization is overused, don't you think?" I hadn't really thought so then, but as I read further, I began to see his point. I still do.'

William F. Bynum is Professor Emeritus of the History of Medicine at University College London. He has published several articles on malaria in British India, and edited (with Caroline Overy) the correspondence between Patrick Manson and Ronald Ross (*The Beast in the Mosquito*, 1998). His *Oxford Dictionary of Scientific Quotations* (with Roy Porter) was published in 2005. The paperback edition was published in 2006. His *Dictionary of Medical Biography* (edited with Helen Bynum) has just been published. His recollections of Roy constitute our last chapter.

Harold J. (Hal) Cook has written mainly on medicine and science in early modern Europe, especially in England and The Netherlands in the seventeenth century. His most recent book is *Matters of Exchange: Commerce, Medicine, and Science in the Dutch Golden Age* (2007), which connects European activities to the larger world. He is now examining more closely the significance of 'global history' approaches to the history of medicine. In addition, he continues as Director of The Wellcome Trust Centre for the History of Medicine at UCL, a role which Roy too occupied, albeit briefly.

'Roy was known to all as a generous host, and enjoyed a kind of friendly rivalry with some of his colleagues over how many people he could lunch with each week. Mostly, this came from his own pocket – none the less, his managers at the Wellcome Trust sometimes despaired of his habit of taking out endless visiting historians, students (and occasionally their parents) and others. All attempts to talk to him about the consequent expense claims somehow only managed to provoke a smile and a story that smoothed their brows, while changing nothing. Roy was equally generous in saying "yes" when asked to comment on this or that, but the comment usually came back by return post in the form of a few words only – a few sentences if you were lucky – scrawled in longhand, in red or green ink, on the paper or letter that had been sent to him for comment. It was almost always both apt and encouraging, even if laconic. He was one of the few people in modern academic life whose ebullience and personal directness managed to substitute for that dread subject, "accountability". That was a Golden Age, of which we in our degeneracy can now only dream.'

Lesley A. Hall is an archivist at the Wellcome Library for the History and Understanding of Medicine and an Honorary Lecturer in the History of Medicine at University College London. She has published extensively on subjects to do with gender and sexuality in nineteenth- and twentieth-century Britain, including *The Facts of Life: The Creation of Sexual Knowledge in Britain 1650-1950* (1995), co-authored with

Roy. She is currently completing a biography of the feminist sex-radical Stella Browne (1880-1955), a project originally seeded by a chance comment of Roy's.

'One of the many things I remember about Roy is, more than once, watching him chair a speaker and appear to be having a quiet forty winks for the duration of their talk. However, once the speaker wound up, Roy would inevitably leap in with a cogent and incisive comment, demonstrating a complete grasp of the content and arguments of the paper and their ramifications and opening them up for further exploration in discussion. There was also a story that he once walked into a lecture room, asked "What am I supposed to be talking about?", and on being told, gave a coherent lecture on the subject, exactly to time, without notes. This may not be true, but is certainly plausible.'

Geoffrey L. Hudson is Assistant Professor of the History of Medicine at the Northern Ontario School of Medicine (Lakehead & Laurentian Universities, Canada). His research interests are in the areas of the social history of medicine as well as war and society. Hudson is currently completing a study of war and disability in early modern Britain, components of which were initiated while he was Roy Porter's last Research Fellow at the Wellcome Institute/Centre. At the Institute, he had direct experience of Roy's legendary efficiency.

'It *is* true that he announced his fourth divorce with a "post-it" on the office bulletin board. There was not a wasted moment with Roy. I was sent to him for a chat about his sponsoring me for the Fellowship. During our lunch meeting no sooner did he figure out that I knew what I was talking about in answer to his previous question than he interrupted me to fire off another one. Just as he worked hard himself he expected those around him to be similarly devoted. When I first arrived at the Wellcome, Roy told me I should work until 10 pm and consider sleeping in my office to save time from commuting.'

Colin Jones is Professor of History at Queen Mary University of London. Among his books are *The Charitable Imperative* (1989), *The Medical World of Early Modern France* (with Laurence Brockliss), *The Great Nation: France from Louis XV to Napoleon* (2002) and *Paris: Biography of a City* (2004).

'Roy Porter set standards of effervescent but erudite scholarship which left me, and leave me still, in awe. The grace and the warmth of his character are, for me, irreducible to a single anecdote.'

Emese Lafferton is Lecturer in the History and Sociology of Medicine at the Science Studies Unit, University of Edinburgh, and an Affiliated Research Scholar in the Department of History and Philosophy of Science, Cambridge University. She is currently completing a book on *Psychiatry's Dual Monarchy: The Mental Geography of Hungary in the Long Nineteenth Century,* the first comprehensive history of Hungarian psychiatry placed in a wider European framework. Her research interests include the sciences of race and nationalism in Central Europe in the late nineteenth and twentieth centuries.

'I approached Roy as a complete stranger, an MPhil student from Budapest in need of an external supervisor for a project in the history of psychiatry. Along with a letter of introduction, I sent him the draft of a paper I was working on. His wonderfully simple response was that he liked the paper, was interested in learning more about Hungarian psychiatry, and gladly undertook my supervision. He continued as my mentor through my PhD.

Although Roy Porter was a formidable scholar, my most cherished memories of him relate to the humble, almost fragile side of his rich personality. While spending a research year at the (then) Wellcome Institute during my PhD, it seemed natural to me to attend his undergraduate lectures on the history of psychiatry. This was the only course he taught, and it was his last year of teaching. I sensed his surprise and concern that I would be wasting my time in undergraduate lectures, but I was convinced that there was something for me to learn from him, something to gain and take away. If only I could tell him now how much use I made of his stories, images and ways of proposing problems when I first taught my own history of psychiatry course.

On his teaching days, he had two lectures, one in the morning and one in the afternoon, usually with some time for discussion at the end. Once he candidly asked me what I thought of them. I said I found it perplexing that he often appeared to be somewhat perturbed during the first lecture, especially compared to the second, when he seemed more at ease, high-spirited and exuberant. His response was a wide grin and an admission that astonished me. He said it was stage-fright! If the first lecture went well, it inspired and elated him, made him feel comfortable. Absurdly, even after more than 30 years of teaching, and numerous lectures that earned him the reputation of a superbly erudite, witty and funny performer, Roy was still simply nervous before any lecture he gave. These very human anxieties in such a superhuman scholar made Roy lovable as well as admirable.'

Mary Lindemann is Professor of History at the University of Miami, Coral Gables Florida. She has written extensively on early modern German history and the history of medicine. She is the author of four books: *Liaisons dangereuses: Sex, Law, and Diplomacy in the Age of Frederick the Great* (2006); *Medicine and Society in Early Modern Europe* (1999);*Health and Healing in Eighteenth-Century Germany* (1996); and *Patriots and Paupers: Hamburg, 1712-1830* (1990). She remembers Roy's appearance at a scholarly meeting in the US that shall remain nameless. She had pushed for Roy as the plenary speaker. As the time for his lecture approached, one of her co-organizers hurried up and queried, 'Where is our speaker?' Roy *was* there, of course, lounging in the front row in black silk shirt open to the navel, tight leather trousers, hairy chest and all. The organizer practically fainted. But Roy, of course, gave one of his brilliant talks and said sceptic actually cried out 'bravo'.

Mark S. Micale teaches European intellectual history and the history of medicine at the University of Illinois. He has authored numerous books and edited volumes, including *Beyond the Unconscious* (1993); *Approaching Hysteria: Disease and its Interpretation* (1995); and *Traumatic Pasts: History, Psychiatry, and Trauma in the Modern Age* (2001). In 1994, he and Roy Porter edited *Discovering the History of Psychiatry*, which has become a widely cited work in the field. In the late 1980s, Roy also published Micale's first scholarly articles – a set of lengthy historiographical essays on hysteria – in the journals *History of Science* and *History of Psychiatry*.

Kim Pelis is the author of *Charles Nicolle, Pasteur's Imperial Missionary: Typhus and Tunisia* (2006). She has also written a number of articles on the history of transfusion, including 'Transfusion, with Teeth: Re-animation and the Re-introduction of Human Transfusion to British Medical Practice, 1810-1834', in Robert Bud, Bernard Finn and Helmuth Trischler, eds., *Manifesting Medicine: Bodies and Machines* (1999), and 'Taking Credit: The Canadian Army Medical Corps and the British Conversion to Blood Transfusion in W.W.I.,' *Journal of the History of Medicine & Allied Sciences*, 56

(2001): 238-77. She is now speechwriter to the Director of the National Institutes of Health in Bethesda, Maryland.

'I first met Roy Porter when I was a graduate student at Johns Hopkins. He was scheduled to deliver a paper at our weekly symposium and I, though still quite new to medical history, knew that this was a very special occasion. What would this legendary historian, whose publications spilled onto multiple computer pages, be like in person? (This was the pre-Google age.) I was shocked to see a relatively young man, grinning broadly and clad in leather, denim and gold chains, walk into our seminar room. More surprising still was his easy approachability - even when among lowly grad students.

Several years later, as a postdoctoral fellow at what was then the Wellcome Institute, I learned that these first impressions had been accurate. Yet, they did not prepare me for the depth of Roy's enthusiasm for history or of his interest in the careers of young historians. Despite his many projects and deadlines, he always had time to read drafts of our papers, organize rigorous but supportive seminars in which we presented our ongoing research, and help us navigate the often treacherous waters of "the Academy" - even for those of us who were not, technically, his students. For, to Roy Porter, himself an appreciative student of life in all its rich complexity, we were all fellow students.

The chapter I have contributed to this volume was a product of those fortunate years of model mentoring in London. Before he died, Roy read it in draft form. He was greatly amused by the image of the perpetually boyish-looking Percy Lane Oliver riding through the streets of London on his bicycle in search of his latest blood donor. Beneath the charm of the image lay the kind of story Roy loved to tell, of a colourful outcast who nevertheless alters the shape of medical practice in a manner profoundly affected by place and time. Oliver's story, then, is another London tale: a tale set in Camberwell, not far from where Roy Porter grew up, and situated, like Porter's own early life, in the wake of world war.'

Daniel Pick is Professor of History at Birkbeck, University of London. He is an editor of the *History Workshop Journal*, and a member of the Institute of Psychoanalysis. His publications include *Svengali's Web: The Alien Enchanter in Modern Culture* (2000) and *Rome or Death: The Obsessions of General Garibaldi* (2005). He is the co-editor (with Lyndal Roper) of *Dreams and History: The Interpretation of Dreams from Ancient Greece to Modern Psychoanalysis* (2004).

'I recall arriving for a meal once to find Roy telling a joke, uncorking champagne - and then surreptitiously moving sets of proofs out of the way of the bottle. No doubt the manuscripts were tackled later, when the guests left. His impossible workload was played down (he seemed dismayingly undismayed, ever eschewing the idea that his schedule was punishing); his wit played up. This lightness with colleagues was part of his appeal and disconcerting effect. It could not but invite curiosity and admiration, but it also defied easy speculation. His generosity and support to younger colleagues were truly exceptional. I remember him too as a keen if profligate cyclist, surprised each time his bike, left invitingly unlocked around London, came to be snatched. As Colin Jones suggests, it is impossible to confine Roy to a thumbnail sketch. The words "unfathomable" and "baffling" were often used about him, fondly.'

John V. Pickstone has practised the history of medicine at UMIST and the University of Manchester since 1974. He is now Research Professor in the Wellcome Unit and the Centre for the History of Science, Technology and Medicine, which he directed from

1986 to 2002. His present work is mostly on the contemporary history of medicine, including cancer, medical technologies and the NHS. He is the author of *Ways of Knowing: A New History of Science, Technology and Medicine* (2000/US 2001) and co-editor (with Roger Cooter) of the *Companion to Twentieth Century Medicine* (2000/2002).

'Roy visited Manchester many times, with characteristic generosity. The last occasion was a formal lecture in the medical school, when he talked about gout and belly-ache but wore a tie, of sorts. At the dinner afterwards, and after wine, someone enquired why he dressed as he did. Polite and jolly as ever, he replied – to make it easier for other people so to do.'

Dorothy Porter is Professor in the History of Health Sciences and Chair of the Department of Anthropology, History and Social Medicine at the University of California San Francisco. Her last monograph was *Health, Civilisation and the State: A History of Public Health from Ancient to Modern Times*, (1999). She is currently researching 'Avenues and Barriers to Transdisciplinarity in Translational Brain Tumour Research'; writing a history of the relationship between the social sciences and medicine in twentieth-century Britain; and examining the emergence of 'life-style' medicine in the post-war period. Earlier monographs, written with Roy Porter, examined the experiences of health and illness of doctors and patients within the context of the eighteenth century Enlightenment: *Patient's Progress: Doctors and Doctoring in Eighteenth-Century England,* (1989); *In Sickness and in Health: The British Experience 1650-1850* (1988). She has edited numerous volumes on the history of social medicine, medical ethics, public health and the politics of medicine and published widely in academic journals in history, literary studies and in medical journals, including the *Lancet* and the *BMJ*. Before arriving at UCSF, Dorothy Porter was Pro-Vice Master for Research at Birkbeck College, University of London and was also Chair of the Department of History, Classics and Archaeology.

'The last time I spoke to Porter was the day before I left to attend an interview at the University of California, San Francisco. He was thrilled about the SF job and made me promise to make sure he got plenty of west coast lecture invites once I was ensconced in the US. The day I returned from my trip he died, and I never got the chance to celebrate with lots of the hoorahs that he always so generously cheered. Four years on, it's still hard to think that he is not around somewhere churning out the next and the next story that tickled his fancy and got him wound up into another frantic manuscript production. And it's hard to think that he isn't chuckling away at his 10,000th reading of Laurence Sterne's ridiculous book or shaking his head at the latest postmodernist clap-trap. But he is still around in my head, buzzing away with his humour and his wisdom. I guess he is still making me laugh.'

Katharina Rowold teaches European history at London Metropolitan University. She has published in the area of the history of gender and science, and is currently completing *Thinking Women: Minds, Bodies, and Women's Higher Education in Britain, Germany, and Spain, 1860–1914*.

'Roy supervised my PhD thesis and sponsored my Wellcome Research Fellowship. Over the years, he was wonderfully inspiring, and his perpetual enthusiasm was a lifeline. Of his many rather eccentric work habits, one that always intrigued me greatly was that he started his day so early after only a few hours' sleep. He used to get up when other Wellcomers were still innocently snoozing and cycle merrily across town, to appear in his office. I was first introduced to his habit when writing my dissertation proposal: Roy told me that the best time to catch him on the

phone would be before 8 am. At this point, as I was to find out later, he had already a couple of hours of work behind him. Over the years, I was never quite able to believe that someone would willingly sleep so little and get up so early, but I finally found out for myself that it was indeed true the night before I submitted my thesis. I had gone late in the evening to the Wellcome Institute to print and photocopy my thesis. Once there, I suddenly decided that I had used too many German words and that they all needed to be changed into English. By the time I had done this, it was well into the night and the building was eerily quiet. I then struggled for hours with a photocopier that kept jamming. When I was finally making the last copy of the final chapter, bleary-eyed in the early hours, Roy suddenly appeared all chirpy, bicycle freshly chained up, ready to spur me on, crack a joke and set to work.'

Anne Secord was trained in the history of science at London University. She worked as assistant editor of *The Correspondence of Charles Darwin* for the first seven volumes and is currently an Affiliated Research Scholar in the Department of History and Philosophy of Science, Cambridge University. Her research and writings focus on popular, particularly working-class, natural history in nineteenth-century Britain. She has a forthcoming book *Artisan Naturalists*. Roy was her PhD adviser and mentor.

'I could not imagine a better PhD thesis adviser than Roy Porter, nor what it would have been like to write without the benefit of his guidance, humour and encouragement. In the 1990s, after working for several years on *The Correspondence of Charles Darwin*, I had decided to weather a grim period of family illnesses by embarking on a PhD. Being supervised by Roy through the University of London External Programme (which allowed graduates of the university to pursue higher degrees at a minimal cost) was true luxury at a time when higher education in any other form was becoming an expensive commodity.

Everyone knew that Roy's hobby horse was *Tristram Shandy*, but I did not know just how keen a gardener Roy was until after his death. I now understand better why he once told me that in contrast to his discussion of the knowledge of the earth possessed by nineteenth-century miners, my study of artisan botanists dealt with their souls. I hope he would have enjoyed my foray into the eighteenth century via the cucumber, and have seen how he continues to inspire.'

Chandak Sengoopta is Reader in the History of Medicine and Science at Birkbeck College, University of London. He is the author of many scholarly papers and three monographs, of which the most recent is *The Most Secret Quintessence of Life: Sex, Glands, and Hormones, 1850-1950* (2006). His first real job was to cover for Roy Porter during the only sabbatical Roy ever took. 'Don't worry about a thing, I shall leave you my magic jacket to protect you,' Roy chuckled when the appointment was announced. A faded denim jacket was soon delivered, dry-cleaned but still liberally covered with ink stains. It was worn regularly and gratefully but shortly after Roy's death, the jacket, too, showed signs of mortality. Thankfully, however, Natsu replaced it with another well-worn one; used sparingly, it should last for years and shelter the wearer from the elements almost as effectively as Roy himself did.

Akihito Suzuki is Professor of History at the School of Economics, Keio University, Tokyo. He has published many articles on the history of psychiatry, as well as on the history of medicine and diseases in modern Japan. His most recent publications include *Madness at Home: The Psychiatrist, the Patient, and the Family in England*

1820-1860 (2006). He has just helped to translate Roy's *Madness*, which is the ninth of his books to appear in Japanese.

Adrian Wilson took his DPhil at Sussex University and taught at Cambridge and Leicester before moving to Leeds on a Wellcome University Award. He is author of *The Making of Man-midwifery: Childbirth in England, 1660-1770* (1995).

'Like many others, I owe Roy Porter a great deal. He was editor or co-editor of both the books (*Patients and Practitioners*; *William Hunter and the Eighteenth Century Medical World*) in which my first two history of medicine publications appeared; and at the time of his death he was again my most recent editor, this time of a journal (*History of Science*). On each of these occasions, as on every other, he was a model of efficiency and courtesy; I would like to think that all who knew him have been inspired in their professional conduct by his supremely civilized example. He and I had various other dealings over the years, from which I shall select one day and evening in the mid-1990s – a one-day conference on the history of childbirth, held at the Wellcome Institute.

With typical generosity of his time, Roy served as chair for something like half of the conference. It so happened that he had just recently acted as publisher's reader for a MS of mine, and had given valuable feedback which I had already received via the publisher. Come the evening post-conference dinner, Roy suggested that he and I have a chat about the book; of course I took this up, and thus received yet further advice, Roy drawing freely and helpfully on his own (then already considerable) experience of turning scholarship into printed product. All of this I valued at the time, acknowledged in print as best I could, and treasure still, but it is not mainly for this that I remember the evening.

While there was yet some red wine still in the carafes, one particular conference participant who was mentally very sprightly yet rather elderly and therefore a little unsteady on her feet – this, I hasten to add, was nothing to do with the wine – felt it was time to leave. This was going to involve her crossing Euston Road, and she was understandably a little anxious. It's no surprise, but it's worth recording, that it was Roy who promptly interrupted his own conversation, popped up and escorted the lady across the road. He was out of his seat to help her while the rest of us were still thinking about it.

Roy was, obviously, a giant among men; yet his thoughtfulness in the small things was as notable as his perhaps better-known magnanimity in the big ones.

Notes

Chapter 1

1. Roy Porter, *The Enlightenment* (Basingstoke: Palgrave, 2001); Roy Porter, *Flesh in the Age of Reason* (London: Allen Lane, 2003).
2. 'Four Weddings and a Few Books', Hunter Davies interviews, Roy Porter, *Sunday Telegraph Magazine*, 7 December 1997: 8–11. Quoted on p. 14 of the essay on Roy Porter's life by Carole Reeves, at http://www.ucl.ac.uk/histmed/PDFS/Bibiography/Lifeandideas.pdf (hereafter cited as 'Reeves').
3. He dismissed the views not only of orthodox Marxists, but of other deterministic authors such as Richard Dawkins and the socio-biologists, French structuralists and Foucauldians, the Edinburgh 'strong programme' of social constructivism, etc.
4. Roy Porter, 'William Hunter: A Surgeon and a Gentleman', in *William Hunter and the Eighteenth-Century Medical World*, eds William F. Bynum and Roy Porter (Cambridge: Cambridge University Press, 1985), pp. 7–34; Roy Porter, *Edward Gibbon: Making History* (London: Weidenfeld & Nicolson, 1988).
5. Geoff Eley and Kieth Nield, 'Why Does Social History Ignore Politics?' *Social History* 5 (1980): 249–71.
6. Susan Reverby and David Rosner, 'Beyond "the Great Doctors"', in *Health Care in America: Essays in Social History*, ed. Reverby and Rosner (Philadelphia: Temple University Press, 1979), pp. 3–16.
7. Perhaps the single most important advocate for this kind of politics-in-history among those concerned with the history of science and medicine was Bob Young, who lectured for a period in Cambridge and who first introduced Roy to the history of science. Also see Roger Cooter, ' "Framing" the End of the Social History of Medicine', in Frank Huisman and John Harley Warner, eds, *Locating Medical History: The Stories and Their Meanings* (Baltimore, MD: Johns Hopkins University Press, 2004), pp. 309–37.
8. Dorothy Porter wrote that Roy was 'Never really a Marxist intellectually or politically', while he 'identified himself to my mother as politically "an old Labour man", an essential qualification as far as she was concerned to be allowed to date her daughter': Dorothy Porter, 'Obtituary: Roy Porter', *History Workshop Journal* 54, no. 1 (2002): 266; Roy Porter, *London: A Social History* (London: Hamish Hamilton, 1994).
9. For an account of his formative years, see Reeves.
10. 'Acknowledgements' to Porter, *Enlightenment*, cited in Reeves, p. 9.
11. Roy Porter, 'Interview with E. P. Thompson', *Socialist History* 6 (1994): 29–33. This is an edited transcript of the interview first broadcast on 'Nightwaves', BBC Radio 3, 20 May 1993. Cited in Reeves, p. 8.
12. For instance: 'Slowly but surely the constraints of the client economy were being replaced by the rather different controls of the open market. The producer and distributor found that they had a new and more impersonal master with whom to struggle for success.' Neil McKendrick, John Brewer and J. H. Plumb, *The Birth of a Consumer Society: The Commercialization of Eighteenth-Century England* (Bloomington, IN: Indiana University Press, 1982), p. 202. Some of these ideas

were already at work in John Brewer, *Party Ideology and Popular Politics at the Accession of George III* (Cambridge: Cambridge University Press, 1976).

13. Simon Schama, 'The Unruly Realm: Appetite and Restraint in Seventeenth-Century Holland', *Daedalus* 108 (1979): 103–23, and for contingency, see especially his later work, Simon Schama, *Citizens: A Chronicle of the French Revolution* (New York: Knopf, 1989), which was based on lectures he gave in Cambridge in 1969–70.
14. Roy Porter, *English Society in the Eighteenth Century* (Harmondsworth: Penguin, 1982), pp. 358, 359, 362.
15. Roy Porter, 'Medicina e illuminismo nell'Inghilterra del settecento', *Quaderni Storici* 40 (1979): 155–80, reprinted as 'Medicine and the Enlightenment in Eighteenth-Century England', *Society for the Social History of Medicine* 25 (1979): 27–40. Quoted in Reeves, pp. 71–2.
16. This was an ambition to which he later attributed the influence of E. P. Thompson: see Reeves, 12.
17. Roy Porter, 'The Patient's View: Doing Medical History from Below', *Theory and Society* 14 (1985): 175–98.
18. Porter, 'Patient's View', 175.
19. Porter, 'Patient's View'. 181–2.
20. Porter, 'Patient's View', 185.
21. Porter, 'Patient's View', 176.
22. Porter, 'Patient's View', 181.
23. Porter, 'Patient's View', 188.
24. Porter, 'Patient's View', 192, 193.
25. Porter, 'Patient's View', 194.
26. Porter, *English Society*, p. 360.
27. Porter, 'Patient's View', 188.
28. I. S. L. Loudon, 'A Doctor's Cash Book: The Economy of General Practice in the 1830s', *Medical History* 27 (1983): 249–68; Matthew Ramsey, 'The Politics of Professional Monopoly in Nineteenth-Century Medicine: The French Model and its Rivals', in *Professions and the French State, 1700–1900*, ed. Gerald Geisen (Philadelphia: University of Pennsylvania Press, 1984), pp. 225–305; Katherine Park, *Doctors and Medicine in Early Renaissance Florence* (Princeton, NJ: Princeton University Press, 1985), pp. 85–117. In my 1981 dissertation, I drew attention to the importance of the growing market economy of early modern England for explaining medical change; this became the 'medical marketplace' in my *The Decline of the Old Medical Regime in Stuart London* (Ithaca, NY: Cornell University Press, 1986), pp. 28–69.
29. Roy Porter, *Health for Sale: Quackery in England 1650–1850* (Manchester: Manchester University Press, 1989), pp. 41, 39.
30. Indeed, when republished, the book was given a more appropriate title: Roy Porter, *Quacks: Fakers and Charlatans in English Medicine* (London: Tempus, 2000).
31. Roy Porter, *Disease, Medicine and Society in England 1550–1860* (Basingstoke: Macmillan, 1987), p. 35.
32. Porter, 'Patient's View', 185.
33. Beginning with 'Being Mad in Georgian England', *History Today* 31 (1981): 42–8.
34. Roy Porter, *Mind-Forg'd Manacles: A History of Madness in England from the Restoration to the Regency* (Cambridge, MA: Harvard University Press, 1987), p. 166.

35. For example, 'Early specialist mental medicine was eclectic, pluralist, and divided. The unreformed state saw no use for an expert corps.' Porter, *Mind-Forg'd Manacles*, p. 175.
36. Jonathan Andrews et al., *The History of Bethlem* (London: Routledge, 1997).
37. Roy Porter and Dorothy Porter, 'The Rise of the English Drugs Industry: The Role of Thomas Corbyn', *Medical History* 33 (1989): 282; John Brewer, *The Sinews of Power: War, Money and the English State, 1688–1783* (London: Unwin Hyman, 1989).
38. Porter and Porter, 'Rise of the Drugs Industry', 293.
39. Porter and Porter, 'Rise of the Drugs Industry', 295.
40. Porter and Porter, 'Rise of the Drugs Industry', 286, fn. 46.
41. Roy Porter and Mikuláš Teich, *Drugs and Narcotics in History* (Cambridge: Cambridge University Press, 1995).
42. For an example of the kind of work done there, see John Brewer, ed., *Consumption and Culture in the Seventeenth and Eighteenth Centuries: A Bibliography*, Report, compiled by Dorothy K. Auyong, Dorothy Porter and Roy Porter (Los Angeles: The UCLA Center for Seventeenth- and Eighteenth-Century Studies and The William Andrews Clark Memorial Library, 1991); Ann Bermingham and John Brewer, eds, *The Consumption of Culture, 1600–1800: Image, Object, Text* (London: Routledge, 1995).
43. Roy admired the work of Iris Murdoch, for instance: I owe this observation about Roy and Murdoch to Michael Neve. He also tells me that Roy also thought very highly of the work of Luis Bunuel.

Chapter 2

A preliminary version of this chapter was given to the Glasgow Wellcome Centre for the History of Medicine; I thank the Centre for their invitation and the participants, particularly Malcolm Nicolson, for their comments. I am also grateful to Jackie Duffin and Roger White for help with the chapter. Errors are my own responsibility.

1. Michel Foucault, *Naissance de la clinique* (Paris, 1963); *Birth of the Clinic: An Archaeology of Medical Perception*, transl. A. M. Sheridan Smith (London: Tavistock, 1973), hereafter *BC*
2. Roy Porter, *The Greatest Benefit to Mankind: A Medical History of Humanity from Antiquity to the Present* (London: HarperCollins, 1997), pp. 306–14.
3. Porter, *The Greatest Benefit to Mankind,* pp. 263–5.
4. Lawrence I. Conrad, Michael Neve, Vivian Nutton, Roy Porter and Andrew Wear, *The Western Medical Tradition* (Cambridge: Cambridge University Press, 1995), pp. 410–12.
5. Adrian Wilson, 'On the History of Disease Concepts: The Case of Pleurisy', *History of Science* 38 (2000), 271–319.
6. *BC*, pp. xi, xv, xviii.
7. *BC*, p. 146.
8. *BC*, pp. xii, xv.
9. Russell C. Maulitz, *Morbid Appearances: The Anatomy of Pathology in the Early Nineteenth Century* (Cambridge: Cambridge University Press, 1987); Othmar Keel, 'Was Anatomical and Tissue Pathology a Product of the Paris Clinical School or Not?' in Caroline Hannaway and Ann La Berge, eds, *Constructing Paris Medicine* (Amsterdam: Rodopi, 1987/Clio Medica 50), pp. 117–83, and other works there cited.

10. Nancy G. Siraisi, *Medicine and the Italian Universities 1250–1600* (Leiden: Brill, 2001), chs. 11 and 15 *passim*, esp. pp. 237–8, 360.
11. 'In the inquiry which is made by anatomists I find much deficience . . . And as for the footsteps of diseases, and their devastations of the inward parts . . . they ought to have been exactly observed by multitude of anatomies, and the contribution of men's several experiences, and carefully set down both historically according to the appearances, *and artificially with a reference to the diseases and symptoms which resulted from them, in case where the anatomy is of a defunct patient*; whereas now upon opening of bodies they are passed over slightly and in silence.' Francis Bacon, *The Advancement of Learning* (1605), taken from Oxford University Press edition (ed. Arthur Johnston, 1974/1980), p. 110, emphasis added. In this respect Bacon's castigation of previous anatomists was almost certainly unfair, but we so far lack a clear picture of this aspect of sixteenth-century anatomical research.
12. See especially Saul Jarcho, 'Morgagni, Vicarius, and the Difficulty of Clinical Diagnosis', in Lloyd G. Stevenson and Robert P. Multhauf, eds, *Medicine, Science and Culture: Historical Essays in Honour of Owsei Temkin* (Baltimore, MD: Johns Hopkins Press, 1968), pp. 87–95; also Malcolm Nicolson, 'Giovanni Battista Morgagni and Eighteenth-century Physical Examination', in C. Lawrence, ed., *Medical Theory, Surgical Practice* (London: Routledge, 1992), pp. 101–34; Andrew Cunningham, 'Pathology and the Case-history in Morgagni's "On the Seats and Causes of Diseases Investigated Through Anatomy" (1761)', *Med Ges Gesch* XI (1995), 37–61; Wilson, 'On the History of Disease Concepts'.
13. *Anatomie Génerale, appliquée à la physiologie et à la médecine*, (Paris, 1812 edition), vol. i, pp. xcviii-xcix, my translation.
14. Laennec, 'Anatomie pathologique', in *Dictionnaire des sciences médicales* (1812), Vol. II, p. 47, quoted by Foucault, *BC*, p. 135 (Foucault's ellipsis; I have not seen the original text).
15. Thus Laennec's earlier manuscript 'Traité d'anatomie pathologique' (*c.* 1804–8) had illustrated each of its anatomical categories (the various 'accidental productions') with one or more cases of named patients: see Jacalyn Duffin,*To See with a Better Eye: A Life of R. T. H. Laennec* (Princeton, NJ: Princeton University Press, 1998), Table 3.3 (p. 71).
16. *BC*, p. 122.
17. *BC*, p. 146.
18. The opening passage is *BC*, pp. 124–33; quotations here are from pp. 122 (end of the previous chapter), 124.
19. *BC*, p. 126.
20. Quotations here are from *BC*, pp. 127–8.
21. *BC*, pp. 129–32.
22. Oddly, Bichat himself seems to have wavered over this issue, for he advocated a tissue-based classification of diseases in *Anatomie pathologique* and implicitly endorsed this idea in *Anatomie générale*, yet in the *Anatomie Descriptive* he expressed indifference towards classification. See respectively Foucault, *BC*, p. 129; Bichat, *Anatomie générale*, vol. iv, p. 415; Foucault, *BC*, p. 177. But both Bayle and Laennec were enthusiastic about the idea, and Corvisart used it to organize his classic treatise on heart disease: see Duffin,*To See with a Better Eye*, pp. 38, 97–8; Foucault, *BC*, p. 177; J. N. Corvisart des Marets, *Treatise on the Diseases and Organic Lesions of the Heart and Great Vessels*, trans. C. H. Hebb (London, 1813), *passim*.

23. This assertion becomes explicit at *BC*, p. 135. Here Foucault suggests that the application of the 'diacritical principle' to the 'dimension . . . in which the recognizable forms of pathological history [i.e. symptoms] and the visible elements that it reveals on completion [i.e. postmortem anatomical findings] are articulated' required the 'medical gaze' to '*travel along a path that had not so far been opened to it*' (my emphasis). The 'diacritical principle', he has just argued (pp. 134–5), was not in itself new, but as applied in pathological anatomy it had hitherto been confined to comparisons (a) with normal bodies; (b) with postmortems of other patients who had died from the same disease; and (c) 'between what one sees of an altered organ and what one knows of its normal functioning'. Notice, incidentally, that (b) actually implies comparison with symptoms, which undermines the discontinuity claim; but this contradiction, being at one remove, is barely visible. Cf. note 25 below.
24. *BC*, pp. 133–4.
25. Specifically, what happens is that symptoms are replaced in Foucault's discourse by pathological processes, with the implication – though this is nowhere explicitly asserted – that this mirrors what happened historically. As Foucault himself puts it (*BC*, p. 139), 'the density of pathological history' was introduced 'into the specified volume of the body'; this formulation is no doubt accurate in itself, but the implication that the study of symptoms was thereby replaced (or displaced) is fallacious. For an eloquent counter to this picture, see Duffin,*To See with a Better Eye*, p. 303.
26. *BC*, p. 146, replacing 'anatomo-clinical' with 'anatomico-clinical'.
27. To put this another way, Foucault's account of what he calls the 'anatomo-clinical method' has depicted this as wholly anatomical, emptying it of its clinical dimension.
28. Matthew Baillie, *The Morbid Anatomy of Some of the Most Important Parts of the Human Body* (1793).
29. See the works cited in note 9 above.
30. This can be illustrated by selective results of a recent Internet search. In the Catholic Encyclopedia, for instance, it is under the heading of 'ANATOMY IN THE EIGHTEENTH CENTURY' that both Wepfer and Bonet appear; and the reason for this is that pathological anatomy is there presented as an eighteenth-century activity. Another encyclopaedia (Wikipedia), under 'history of anatomy in the seventeenth and eighteenth centuries', does have a few scattered references to 'morbid anatomy', but treats this as a separate activity, quite distinct from anatomy proper. See, respectively, http://www. newadvent.org/cathen/10122a.ht*m* (accessed 1 March 2004); http://en2.wikipedia. org/wiki/History_of_anatomy_in_the_17th_and_eighteenth_centuries (accessed 8 April 2004).

Chapter 3

1. The origins of this chapter go back to 1970, when I gave a paper on 'The social history of social history' to a seminar at Oxford run by the late Raphael Samuel. My thanks to the audience for their questions and especially to John Gillis, also in 1970, for help with the German side of the story. The rethinking and rewriting have been much aided by conversations with Mark Phillips over the last twenty years.
2. Quoted in Renée Simon, *Boulainvillers* (Paris, 1940), p. 48.

3. John Millar, *Origin of the Distinction of Ranks* (1771: second edn, 1779), Introduction; Blair, quoted in Mark Phillips, *Society and Sentiment: Genres of Historical Writing in Britain, 1740–1820* (Princeton, NJ: Priceton University Press, 2000), p. 44.
4. Quoted in Peter H. Reill,*The German Enlightenment and the Rise of Historicism* (Berkeley, CA: University of California Press, 1975), pp. 45, 53–4.
5. Pietro Giannone, *Historia civile del regno di Napoli* (1723); cf. Giovanni Ricuperati, *L'esperienza civile e religiosa di Pietro Giannone* (Milan and Naples 1970), pp. 143–249, and John G. A. Pocock, *Barbarism and Religion*, 2 vols (Cambridge: Cambridge University Press, 1999), 2, pp. 29–41.
6. Giambattista Vico, *Scienza Nuova* (1744), Book 1, section 2, no. xx.
7. Vettore Sandi, *Principi da storia civile della repubblica di Venezia* (Venice, 1765), vol. 1, pp. xvi-xxix. Cf. Francesco Dalla Colletta, *I Principi di storia civile di Vettor Sandi* (Venice, 1995), pp. 99–109.
8. Montesquieu, *Pensée* 954, quoted in Hugh R. Trevor-Roper, 'The Historical Philosophy of the Enlightenment', *Studies on Voltaire* 24/27 (1963): 1667–87, at 1675n.
9. Pocock, *Barbarism*, p. 34.
10. J. G. A. Pocock, 'Gibbon and the Shepherds', *History of European Ideas* 2 (1981): 193–202.
11. Jonathan B. Knudsen, *Justus Möser and the German Enlightenment* (Cambridge: Cambridge University Press, 1986).
12. Montesquieu, *Esprit des Lois* (1748), Book 28, chapter 17.
13. Reill, *German Enlightenment*, pp. 143, 170, 210.
14. Sandi, *Principi*, vii; Karl J. Weintraub, 'Toward the History of the Common Man: Voltaire and Condorcet', in *Ideas in History*, ed. R. Herr and H. T. Parker (Durham, NC: Duke University Press, 1965), pp. 39–64.
15. On 'stadial history', see Ronald Meek, *Social Science and the Ignoble Savage* (Cambridge: Cambridge University Press, 1976); and Karen O'Brien, *Narratives of Enlightenment: Cosmopolitan History from Voltaire to Gibbon* (Cambridge: Cambridge University Press, 1997), pp. 132–6.
16. '[K]ann nie von der Geschichte des gesellschaftlichen Zustandes abgesondert vorgetragen werden': Johann Gottfried Eichhorn, *Allgemeine Geschichte der Cultur und Litteratur des neueren Europa*, 2 vols (Göttingen, 1796–9), vol. 1, vii.
17. Quoted in Georg G. Iggers, 'The University of Göttingen 1760–1800 and the Transformation of Historical Scholarship', *Storia della Storiografia* 2 (1982): 11–37, at 28.
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Chapter 4

Thanks to Chris Lawrence, and especially to Roberta Bivins and John Pickstone, for commenting on previous drafts. This is a revised version of my article 'Medicine and Politics in the Age of Reform', which appears in Arthur Burns and Joanna Innes, eds, *Rethinking the Age of Reform: Britain 1780–1850* (Cambridge: Cambridge University Press, 2004).

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Chapter 5

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Chapter 6

I am grateful to the Wellcome Trust and to the National Library of Medicine in the USA for help in funding research for this chapter.

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49. Cited in Hillam, 'Quackery is in the Eye of the Beholder', *Dental Historian* 29 (1995): 12.
50. See Duffy, *The Englishman and the Foreigner,* esp. pp. 31ff.
51. 'Six Stages of Mending a Face' (1792). The series starts at the top right of the picture and progresses to the bottom left.
52. Cited in Hillam, *Brass Plate*, p. 79.
53. C. Hillam, 'The Availability of Dental Products in Britain at the End of the Eighteenth Century', *Dental Historian* 32 (1997): 64. There are some marvellous examples of dental publicity in the miscellany 'Dental Memoranda collected by T. Purland 1844', held in the Wellcome Library, London.
54. Hillam, *Brass Plate*, p. 89.
55. *Journal de Madame Cradock. Voyage en France (1783–6)*, ed. O. Delphin (Paris, 1906), p. 330.
56. Quotations from Hargreaves, *White as Whale Bone*, p. 9.
57. E. Breham, *Treatise on the Structure, Formation and Various Diseases of the Teeth and Gums* (Leeds, 1816), p. iii.
58. H. Moises, *An Appendage to the Toilet, or An Essay on the Management of the Teeth dedicated to the Ladies* (London, [1798]). Duval was published in London in 1820 and Nicholles in London in 1833. Cf. Breham, *Treatise*, dedication (1816); and B. Walkey, *On the Diseases of the Teeth* (np, nd), dedicated to 'the ladies of Great Britain'.
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61. Dagen, *Documents*.
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63. John Gray, *Preservation of the Teeth, Indispensable to Comfort, Appearance, Health and Longevity* (London 1838), p. 37, n. 6.
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Chapter 7

For helpful comments and suggestions I am grateful to Emma Spary, Sandra Sherman, Patricia Fara, Jim Secord and, above all, Roberta Bivins and John Pickstone.

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3. Adam Ferguson, quoted in Roy Porter, *Enlightenment* (London: Penguin Books 2000), p. 295.
4. Stephen Switzer, *The Practical Kitchen Gardiner* (London: Thomas Woodward 1727), p. 387.
5. Thomas Hale, *Eden* (London: T. Osborne 1757), p. 232; John Abercrombie, *The Complete Forcing-Gardener* (London: Lockyer Davis 1781), p. 11.

6. [Francis Grose], *A Classical Dictionary of the Vulgar Tongue* (London: S. Hooper, 1785).
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11. William Buchan, *Domestic Medicine* (Edinburgh: Balfour, Auld and Smellie, 1769), pp. 61, 139, 142.
12. John Arbuthnot, *Practical Rules of Diet in the Various Constitutions and Diseases of Human Bodies* (London: J. Tonson, 1732), pp. 248, 277.
13. Switzer, *Practical Kitchen Gardiner*, iii, pp. 99–100.
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15. Arbuthnot, *Practical Rules of Diet*, p. 248. Cucumbers, melons, gourds and pumpkins (pompions) are closely related fruits.
16. John Armstrong, *The Art of Preserving Health*. Book II, *Diet* (London: A. Millar, 1744), p. 31.
17. Langley, *New Principles of Gardening*, Part 7, p. 43; Sterne, *Tristram Shandy*, p. 538.
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29. Stephen Switzer, *The Practical Fruit-Gardener* (London: Thomas Woodward, 1724), pp. 7–8.
30. Langley, *New Principles of Gardening,* Part 7, pp. 37, 36
31. Switzer, *Practical Kitchen Gardiner*, p. 372.
32. Abercrombie, *Forcing-Gardener*, p. 66.

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45. *Hints for the Management of Hot-Beds*, p. 5; Switzer, *Practical Kitchen Gardiner*, p. 67.
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Chapter 8

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2. E. M. Leonard, *The Early History of English Poor Relief* (Cambridge: Cambridge University Press, 1900); G. R. Elton, 'An Early Tudor Poor Law', *Economic History Review*, 2nd ser., VI (1953), pp. 55–67; V. Pearl, 'Social Policy in Early Modern London', in H. Lloyd-Jones, V. Pearl and B. Worden, eds, *History and Imagination: Essays in Honour of H. R. Trevor-Roper* (London: Holmes and Meier, 1981); J. C. D. Clark, *English Society* (Cambridge: Cambridge University Press, 1985), esp. p. 76, n. 11; B. and S. Webb, *English Poor Law History, Part I: The Old Poor Law* (London: Longmans, Green and Co., 1927); C. Hill, 'Puritans and the Poor', *Past and Present* II (1952), pp. 32–50; A. L. Beier, *Masterless Men: The Vagrancy Problem 1560–1640* (London: Methuen, 1985); K. Wrightson, *English Society* (London: Hutchinson, 1982); Examples: K. Wrightson, *English Society*, p. 181; K. D. M. Snell, *Annals of the Labouring Poor* (Cambridge: Cambridge University Press, 1985), pp. 104–5; S. Rappaport, *Worlds within Worlds: Structures of Life in Sixteenth-century London* (Cambridge: Cambridge University Press, 1989), pp. 195–201.
3. A. Borsay, 'Returning Patients to the Community: Disability, Medicine and Economic Rationality before the Industrial Revolution', *Disability and Society* 13(5) (November 1998): 645–63.
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5. T. Wales, 'Poverty, Poor Relief and the Life-cycle: Some Evidence from Seventeenth-century Norfolk', and W. Newman Brown, 'The Receipt of Poor Relief and Family Situation: Aldenham, Hertfordshire, 1630–90', in R. M. Smith, ed., *Land, Kinship and Life-Cycle* (Cambridge: Cambridge University Press, 1984), pp. 351–404 and 405–22 respectively; P. King, 'The Parish State', paper delivered at the History of Poverty seminar, Oxford University, 6 May 1990; S. Macfarlane, 'Studies in Poverty and Poor Relief in London at the End of the Seventeenth Century' (unpublished DPhil thesis, Oxford, 1983), 198; P. Slack, *Poverty and Policy in Tudor and Stuart England* (London: Longman, 1988), 104–7 and 191–2; G. Walker, 'Crime, Gender and Social Order in Early Modern Cheshire' (PhD thesis, University of Liverpool, 1994), ch. 5; for discussions of the role of negotiation in social relations, see A. Strauss, *Negotiations: Varieties, Contexts, Processes, and Social Order* (San Francisco: Jossey-Bass, 1978); J. Fiske, *Understanding Popular Culture* (Boston: Unwin Hyman, 1989), 56–68; T. Hitchcock, P. Kind and P. Sharpe, *Chronicling Poverty: The Voices and Strategies of the English Poor, 1640–1840* (Basingstoke: Palgrave, 1997); L. Davison, T. Hitchcock, T. Keirn and R. Shoemaker, eds, *Stilling the Grumbling Hive* (Stroud: Alan Sutton, 1992); M. J. Braddick and J. Walter,

Negotiating Power in Early Modern Society (Cambidge: Cambridge University Press, 2001).

6. N. Z. Davis explores this approach in depth in her *Fiction in the Archives: Pardon Tales and their Tellers in Sixteenth-Century France* (Stanford, CA: California University Press, 1987), pp. 20–1. For another discussion of the strategic aspects of the petition, see J. C. Scott, *Domination and the Arts of Resistance* (New Haven, CT: Yale University Press, 1990), pp. 94–5.
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8. K.A.O., QM/SB, 1316.
9. C.R.O., QJB & QJF, 1593–1641.
10. Examples: W.R.O., A1/150/7, Trinity 1631; D.R.O., Q/S 1/3, fs. 77v (Hilary 1609/10); Q/S, 1/4, n.f. Easter 1616. The evidence from the Privy Council registers confirms the timing of the shift in how eligibility was determined. There is a clear change in the nature of the disabilities by which soldiers applied, and were recommended, for pensions. *A.P.C.*, XXII-XXXIX, *passim*; P.R.O., S.P. Dom. 11/174/83, 11/180/94.
11. T. Wales, 'Poverty, Poor Relief and the Life-cycle: Some Evidence from Seventeenth-century Norfolk', in R. M. Smith (ed.), *Land, Kinship and Life-Cycle* (Cambridge: Cambridge University Press, 1984), pp. 351–404; M. Pelling, 'Healing the Sick Poor: Social Policy and Disability in Norwich 1550–1640', *Medical History* 29 (1985): 115–37; M. Pelling, 'Illness among the Poor in an Early Modern English Town: The Norwich Census of 1570', *Continuity and Change* 3(2) (1988): 273–90; M. Pelling, 'Old Age, Poverty, and Disability in Early Modern Norwich', in M. Pelling and R.M. Smith, eds., *Life, Death and the Elderly* (London: Routledge, 1991), pp. 74–101, 77. See also W. Newman Brown, 'Receipt of Poor Relief and Family Situation: Aldenham, Hertfordshire, 1630–90', *Land, Kinship*, pp. 405–21, 411.
12. Conclusions based on detailed study of sessions materials from City of Oxford, Kent, Cheshire and Devon. Oxford City Library, 0.5.9; N.4.2; Kent Archive Office, Q/FM 1–11; Q/SO WI; Cheshire Record Office, QJB 1/4, fs. 89v, 90r; QJF, 28/4, f. 8 (Hilary 1598/99); QJF, 35/2, f.30 (Trinity 1606); Devon Record Office Q/S, 1/1, f. 298, Hilary 1599/1600; 1/2, n.f. Hilary 1603/4, n.f., Easter 1607.
13. The legal principle behind these practices was made explicit by two Staffordshire JPs. In a 1640 decision concerning a Poor Law matter they concluded that practices which ran counter to an Act of Parliament, even an Act passed as recently as 1601, are legal if these practices are begun and continued by the consent and agreement of the interested parties. Staffordshire Record Office, QS files, Easter 1640, cited in S.C. Newton, 'Staffordshire Quarter Sessions: Archives and Procedures in the Earlier Seventeenth-Century', *Essays in Staffordshire History*, ed. M. W. Greenslade (Collections for a History of Staffordshire, 4th ser., 6, 1970), p. 80.
14. *Acts and Ordinances*, Vol. I, pp. 938–40 (28 May 1647), 997–8 (10 August 1647), 1055 (24 December 1647); Vol. II, pp. 556–9 (30 September 1651); *S.R.*, 14 Car II c. 9.
15. Somerset Record Office, Q/SO5, fos. 34r-35v; CRO, QJB, 1/6, fols. 35r-36v.
16. Cheshire Record Office, QJB 2/6–2/7, 3/1–3/3; QJF 1593–1680; Devon Record Office, Q/S, 1/9–1/13; Q/S 128.

17. M. Fissell, 'Everyone Their Own Physician', ch. 2, in Fissell, *Patients, Power, and the Poor in Eighteenth-Century Bristol* (Cambridge: Cambridge University Press, 1991), pp. 16–36. For more, see V. Hutton, 'Humoralism', W. F. Bynum and R. Porter, eds, *Companion Encyclopedia of the History of Medicine*, vol. I (London: Routledge, 1993), pp. 281–91; A. Wear, 'Making Sense of Health and the Environment in Early Modern England', A. Wear, ed., *Medicine and Society* (Cambridge: Cambridge University Press, 1992), pp. 119–47; G. K. Paster, *The Body Embarrassed* (Ithaca, NY: Cornell University Press, 1993), 'Introduction'.
18. Cheshire Record Office, QJF, 84/1, f. 103 (Easter 1656).
19. Example: Cheshire Record Office, QJF, 79/2, f. 117 (Trinity 1651).
20. Although in 1628 Harvey revealed that blood circulated, and that more of it flowed through the heart than could be created as a result of digestion, people continued to believe for some time that the body's production of blood was related to consumption and other factors such as age.
21. The significance of age as a factor in pensionable disability is demonstrated in Devon. Between 1660 and 1692 83 of 240 (34.6 per cent) mention age as well as other causes of disability to work. Devon Record Office, Q/S, 1/9–1/13; Q/S, 128.
22. Cheshire Record Office, QJB, 3/1–3/3; QJF, 1660–1680; Devon Record Office, Q/S, 1/9–1/13, Q/S, 128.
23. Cheshire Record Office, QJF, 88/4, fs. 32 (Hilary 1660/1) and 62 (Hilary 1660/1).
24. G. L. Hudson, 'Ex-servicemen, War Widows and the English County Pension Scheme, 1593–1679' (DPhil thesis, Oxford University, 1995), chs. 3 and 5.
25. Example: Michael Oliver, *The Politics of Disablement* (London: Macmillan, 1990).
26. L. Gray, 'The Self-perception of Chronic Physical Incapacity among the Labouring Poor, Pauper Narratives and Territorial Hospitals in Early Modern Rural Germany' (PhD thesis, University College London, 2001); 'Petitioning for Survival: Medical Care and Welfare Provision in Early Modern German Hospitals', *Medicina & storia: rivista de storia della medicina e della santia* Anno 3, no. 6 (2003).
27. In Cheshire between 1647 and 1680, for example, petition evidence reveals that 19 were supported by family, friends and neighbours, and 14 worked in a variety of by-employments prior to applying for a county stipend. Cheshire Record Office, QJF, 1647–1680.
28. Cheshire Record Office, QJF 35/2, f. 8 (Trinity 1606).
29. Cheshire Record Office, QJF, 83/4, f. 120 (Hilary 1655/6); 79/3, f. 133 (Michaelmas 1651); 78/1, f. 19 (Easter 1650); 84/2, f. 246 (Trinity 1656).
30. For a detailed discussion of social agency and the disabled veteran, see G. L. Hudson, 'Ex-servicemen, War Widows', ch. 3. For more detail on the development of the county pension system, see G. L. Hudson, 'Disabled Veterans and the State in Early Modern England', in D. Gerber, ed., *Disabled Veterans in History* (Ann Arbor, MI: University of Michigan Press, 2000).
31. G. L. Hudson, 'Negotiating for Blood Money: War Widows and the Courts in Seventeenth-century England', in G. Walker and J. Kermode, eds, *Women, Crime and the Courts in Early Modern England* (London: University College London Press, 1995).
32. Hudson, 'Negotiating for Blood Money'. In addition, thousands of war widows and orphans were pensioned by the central government between the mid-1640s and 1660 via its Savoy and Ely House fund.
33. National Archives, WO 116/1.

34. National Archives, ADM 73/51–53.
35. National Archives, WO 116/1.
36. R. Cooter, 'The Disabled Body', in R. Cooter and J. Pickstone, *Companion to Medicine in the Twentieth Century* (London: Routledge, 2003; first pub. 2000), p. 370.
37. National Archives, ADM 73/51–53.
38. National Archives, WO 250/1, 6 October 1786.
39. J. Pringle, *Observations on the Diseases of the Army* (London, 1753). The inflammatory diseases were contrasted with the contagious diseases such as fever, smallpox and measles.
40. M. Fissell, 'The Disappearance of the Patient's Narrative and the Invention of Hospital Medicine', in R. French and A. Wear, *British Medicine in an Age of Reform* (London: Routledge, 1991).
41. Ole Peter Grell, 'War, Medicine and the Military Revolution', in P. Elmer, ed., *The Healing Arts: Health, Disease and Society in Europe, 1500–1800* (Manchester: Manchester University Press, 2004), pp. 257–83. See also L. Brockliss and C. Jones, *The Medical World of Early Modern France* (Oxford: Clarendon Press, 1997); G. Hudson, ed., *British Naval and Military Medicine, 1600–1800* (Amsterdam: Rodopi, 2007).
42. G. Gask, 'A Contribution to the History of the Care of the Sick and Wounded during Marlborough's March to the Danube in 1704, and at the Battle of Blenheim', in G. Gask, *Essays in the History of Medicine* (London: Butterworth, 1950), pp. 103–15; P. Mills, 'War, Medicine and the British Army in the Eighteenth Century: Reconstruction of a Hospital System', unpublished paper delivered at the Wellcome Centre for the History of Medicine at UCL, 30 November 1999, 3.
43. M. J. Vogel, 'The Transformation of the American Hospital', in N. Finzsch and R. Jutte, *Institutions of Confinement: Hospitals, Asylums and Prisons in Western Europe and North America, 1500–1950* (Cambridge: Cambridge University Press, 1996), pp. 39–54.
44. G. B. Risse, *Mending Bodies, Saving Souls: A History of Hospitals* (Oxford: Oxford University Press, 1999), p. 242.
45. H. Cook, 'Practical Medicine and the British Armed Forces after the "Glorious Revolution"', *Medical History* 34 (1990): 1–26, 16. See also P. Mathias, 'Swords and Ploughshares: The Armed Forces, Medicine and Public Health in the Late Eighteenth Century', in J. M. Winder, ed., *War and Economic Development: Essays in Memory of David Joslin* (Cambridge: Cambridge University Press, 1975).
46. See P. Mills, 'Privates on Parade: Soldiers, Medicine and the Treatment of Inguinal Hernias in Georgian England' in Hudson, *British Military and Naval Medicine*
47. Example: National Archives, ADM 67/122, 265 (28 January 1726/27).
48. Examples: National Archives, ADM 67/122, 261 (14 December 1726); 2/1133, 314 (11 April 1627).
49. National Archives, ADM 2/1133, 311 (December 1726).
50. National Archives, WO 250/458 and 459 (1703–49).
51. Examples: National Archives ADM 67/122, 261 (14 December 1726); 2/1133, 314 (11 April 1727).
52. National Archives, ADM 67/119, 70 (1August 1707).
53. The Proceedings of the Old Bailey Ref: t17450116–5; T. Hitchcock and J. Black, *Chelsea Settlement and Bastardy Examinations, 1733–1766* (London: London Record Society, 1999), vol. 33, p. 20.

54. National Archives, ADM ADM 73/51–53. Pensioners' families were excluded from the wards, and from the dining hall during meals. Instead, wives and children were left to beg for leftovers at the gates.
55. National Archives, ADM 67/119, 107 (16 April 1707)
56. National Archives, ADM 67/120, 45 (4 April 1711).
57. National Archives, ADM 67/121, 357 (1 June 1720).
58. National Archives, ADM 67/122, 142 (31 January 1723/4).
59. National Archives, ADM 67/119 and 120. Particularly interesting examples: ADM 67/120, 60 (29 August 1711); 2/1133, 102 (12 September 1711); ADM 80/69, 67 (20 July 1717).
60. National Archives, ADM 67/122, 200.
61. Slack, *Poverty and Policy*, pp. 5–6.
62. I. Archer, *Pursuit of Stability* (Cambridge: Cambridge University Press, 1991), pp. 98–9, 260.
63. *Selections from the Prison Notebooks of Antonio Gramsci*, ed. Q. Hoare and G. N. Smith (New York: International Publishers, 1971), esp. 'The Study of Philosophy'; E. P. Thompson, *Customs in Common* (London: Merlin Press, 1991), pp. 7, 10–11, 38, 44–6, 66, 74, 85, 300–2, 339; B. Sharp, 'Popular Protest in Seventeenth-Century England', B. Reay, ed., *Popular Culture in Seventeenth-Century England* (London: Routledge, 1988, 1st pub. 1985), pp. 271–308, 287–9, 303.

Chapter 9

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1. Works relevant to the history of Victorian psychiatry are now numerous. See Joseph Melling and Bill Forsythe, eds, *Insanity, Institutions, and Society, 1800–1914* (London: Routledge, 1999); Peter Bartlett and David Wright, eds, *Outside the Walls of the Asylum: The History of Care in Community 1750–2000* (London: Athlone Press, 1999).
2. See, among others, Mark S. Micale, *Approaching Hysteria: Disease and its Interpretations* (Princeton, NJ: Princeton University Press, 1995).
3. For a pioneering work on women's sense of their identity seen through asylum case entries, see Marjorie Levine-Clark, ' "Embarrassed Circumstances": Gender, Poverty, and Insanity in the West Riding of England in the Early Victorian Years', in Jonathan Andrews and Anne Digby, eds, *Sex and Seclusion, Class and Custody: Perspectives on Gender and Class in the History of British and Irish Psychiatry* (Amsterdam: Rodopi, 2004), pp. 123–48. Other essays in this volume present solidly based social history of gender and insanity.
4. The casebooks are now held in London Metropolitan Archive, H11/HLL.
5. For Conolly's life, see among others Andrew Scull, 'A Victorian Alienist: John Conolly, FRCP, DCL (1794–1866)', in W. F. Bynum et al., eds, *The Anatomy of Madness: Essays in the History of Psychiatry*, 3 vols. (London: Tavistock, 1985–8), 1, pp. 103–50; Akihito Suzuki, 'Politics and Ideology of Non-Restraint: The Case of the Hanwell Asylum', *Medical History* 39 (1995): 1–17.
6. An astute contemporary criticism of psychiatric etiology and statistics in the early nineteenth century is found in Isaac Ray, *Contributions to Mental Pathology*

(Boston, MA: Little, Brown, 1873), 'The Causes of Insanity' and 'The Statistics of Insanity', which had been published originally in 1863–4 and 1849 respectively. Barbara Duden, *The Women beneath the Skin: a Doctor's Patients in Eighteenth-Century Germany*, transl. Thomas Dunlap (Cambridge, MA: Harvard University Press, 1991). See also the discussion in Mark S. R. Jenner and Bertrand O. Taithe, 'The Historiographical Body', in Roger Cooter and John Pickstone, eds, *Companion to Medicine in the Twentieth Century* (London: Routledge, 2000), pp. 187–200.

7. Arthur Kleinman, *The Illness Narratives: Suffering, Healing and the Human Condition* (New York: Basic Books, 1988).
8. Sander Gilman, *Seeing the Insane* (New York: John Wiley & Sons, 1982).
9. Sander Gilman, *Difference and Pathology: Stereotypes of Sexuality, Race, and Madness* (Ithaca, NY: Cornell University Press, 1985).
10. Alexis de Tocqueville, *Memoir on Pauperism*, intro. Gertrude Himmelfarb (Chicago: Ivan R. Dee, 1997), p. 56.
11. See, for instance, John Charles Bucknill and Daniel H. Tuke, *A Manual of Psychological Medicine* (Philadelphia: Blanchard and Lea, 1858), pp. 240ff; J. E. D. Esquirol, *A Treatise on Insanity*, transl. E. K. Hunt (Philadelphia: Lea and Blanchard, 1845), pp. 45–54.
12. The prominence of the issues of poverty in the interviews at the Hanwell confirms recent emphasis on the fact that the machineries of the administration of the Poor Law and the Asylum Law had considerable overlaps. See, among others, Bartlett, *Poor Law of Lunacy*.
13. For an early psychiatric observation along such a line, see Richard Mead, *Medical Works of Richard Mead, M.D.* (London: C. Hitch et al., 1752), pp. 489–90.
14. Eric Hobsbawm, *The Age of Revolution: Europe 1789–1848* (London: Abacus, 1977), pp. 60–4.
15. H11/HLL/B20/1/354–355. See also the case of Ernest George Frederick Sievers. H11/HLL/B20/1/526–527.
16. H11/HLL/B20/1/42–3.
17. For the coexistence of different views in public health in this period, see John V. Pickstone, 'Dearth, Dirt and Fever Epidemics: Rewriting the History of British "Public Health"', in Terence Ranger and Paul Slack, eds, *Epidemics and Ideas: Essays on the Historical Perception of Pestilence* (Cambridge: Cambridge University Press, 1992), pp. 125–48; Christopher Hamlin, *Public Health and Social Justice in the Age of Chadwick: Britain, 1800–1854* (Cambridge: Cambridge University Press, 1998).
18. Friedrich Engels, *The Condition of the Working Class in England*, ed. and Foreword Victor Keirnan (Harmondsworth: Penguin, 1987). For occupational diseases, see Anthony S. Wohl, *Endangered Lives: Public Health in Victorian Britain* (London: Dent, 1987), pp. 257–84.
19. Edwin Chadwick, *Report on the Sanitary Condition of the Labouring Population of Great Britain* (1842), ed. and intro. M. W. Flinn (Edinburgh: Edinburgh University Press, 1965), pp. 190–204 and 335–49.
20. Gareth Steadman Jones, *Outcast London: A Study in the Relationship between Classes in Victorian Society* (Oxford: Clarendon Press, 1971), pp. 241–61.
21. Although psychiatrists routinely acknowledged the impact of overwork on labouring people's mental health, they were usually more concerned with their intemperance. See, for example, Andrew Combe, *Observations on Mental Derangement* (Boston: Marsh, Capen & Lyon, 1834), pp. 183–92.
22. H11/HLL/B20/1/520–1.

23. H11/HLL/B20/1/384–5.
24. Consequently, many took precautionary means of saving bank, membership in a friendly society, and so on. See Paul Johnson, *Saving and Spending: The Working-Class Economy in Britain 1870–1939* (Oxford: Oxford University Press, 1985).
25. H11/HLL/B20/1/550–551.
26. H11/HLL/B20/1/528–9.
27. H11/HLL/B20/1/380.
28. H11/HLL/B20/1/572–3.
29. H11/HLL/B20/1/346.
30. William Lanford's case reveals a quick downward spiral and the disintegration of a successful life, first prompted by an apparently minor event of the illness of his child, leading to severe depression, mental disturbances and outbursts of violence. H11/HLL/B20/1/594–5.
31. H11/HLL/B20/1/478–9.
32. H11/HLL/B20/1/560–61.
33. H11/HLL/B20/1/310–11.
34. H11/HLL/B20/1/438–9.
35. H11/HLL/B20/1/582.
36. H11/HLL/B20/1/510–11. See also the case of William Fowler, H11/HLL/B20/1/1, 1–2.
37. M. A. Crowther, *The Workhouse System 1834–1929: The History of an English Social Institution* (London: Methuen, 1983).
38. Crowther, *Workhouse System*
39. H11/HLL/B20/1/194–5.
40. H11/HLL/B20/1/384–5.
41. H11/HLL/B20/1/438–9.
42. H11/HLL/B20/1/334–5.
43. Ray, *Contributions to Mental Pathology*, pp. 32–3.
44. For the restructuring of consciousness and behavior of the working-class in the early nineteenth century, see E. P. Thompson, *The Making of the English Working Class* (Harmondsworth: Penguin, 1968).
45. Johnson, *Saving and Spending*, passim.
46. F. M. L. Thompson, *The Rise of Respectable Society: A Social History of Victorian England 1830–1900* (Cambridge, MA: Harvard University Press, 1988).
47. Anna Clark, *The Struggle for the Breeches: Gender and the Making of the British Working Class* (Berkeley, CA: University of California Press, 1995).
48. A similarly darker side of the breadwinner ideal could be found in instances of domestic violence. Anna Clerk has astutely observed that '[Among] those men who aspired to the breadwinner ideal, failure to succeed could trigger violence [against their wives].' Clark, *Struggle for the Breeches*, pp. 260–1.

Chapter 10

1. Barbara Taylor, *Eve and the New Jerusalem: Socialism and Feminism in the Nineteenth Century* (London: Virago, 1983), pp. 6–9; I am indebted to Richard Sha for sight of his work in progress on issues of sex, medicine and Romanticism.
2. M. L. Bush, *What is Love? Richard Carlile's Philosophy of Sex* (London: Verso, 1998).

3. Taylor, *Eve and the New Jerusalem*; Saskia Poldervaart, 'Theories about Sex and Sexuality in Utopian Socialism', in *Gay Men and the Sexual History of the Political Left*, ed. Gert Hekma, Harry Oosterhuis and James Steakley (New York: Haworth Press, 1995), pp. 41–67; J. Miriam Benn, *Predicaments of Love* (London: Pluto Press, 1992); 'A Student of Medicine', in [George Drysdale], *Physical, Sexual and Natural Religion* (London: Truelove, 1854), subsequent editions published as *The Elements of Social Science*.
4. Karen Hunt, *Equivocal Feminists: The Social Democratic Federation and the Woman Question, 1884–1911* (Cambridge: Cambridge University Press, 1996), pp. 86–94.
5. Serialized in *The Forerunner*, a monthly magazine produced by Charlotte Perkins Gilman, during 1915, but not separately published in book form until its rediscovery by 'second wave' feminism in 1979 (Pantheon Press in the US and The Women's Press in the UK); Matthew Beaumont, 'The New Woman in Nowhere: Feminism and Utopianism at the *Fin de Siècle*', in *The New Woman in Fiction and in Fact*, ed. Angelique Richardson and Chris Willis (London: Palgrave, 2001), pp. 212–23; Lucy Bland, *Banishing the Beast: English Feminism and Sex Morality* (London: Penguin, 1995), pp. 217–22; Frances Swiney, *The Awakening of Women or Woman's Part in Evolution* (London: William Reeves, 1905, third edition 1908), pp. 94, 118.
6. Sheila Rowbotham and Jeffrey Weeks, *Socialism and the New Life: The Personal and Sexual Politics of Edward Carpenter and Havelock Ellis* (London: Pluto Press, 1977).
7. Edward Carpenter, *England's Ideal and other Papers on Social Subjects* (London: George Allen & Unwin, 1887), pp. 2, 22.
8. Edward Carpenter, *Civilisation: Its Cause and Cure* (London: Swan Sonnenschein, 1889), pp. 1, 2, 15–16.
9. Roy Porter, 'Diseases of Civilization', in *Companion Encyclopedia of the History of Medicine*, ed. W. F. Bynum and Roy Porter (London: Routledge, 1993), pp. 586–600.
10. Carpenter, *Civilisation*, pp. 37–8, 41.
11. Carpenter, *Civilisation*, pp. 41, 43–4.
12. Edward Carpenter, *Love's Coming of Age: A Series of Papers on the Relations of the Sexes* (London: George Allen & Unwin, 1930, first pub. 1896), pp. 99, 122–3.
13. Carpenter, *Love's Coming of Age*, pp. 132, 157.
14. Richard von Krafft-Ebing, *Psychopathia Sexualis: With Especial Reference to the Antipathetic Sexual Instinct. A Medico-Forensic Study* (first pub. in German, Stuttgart: Erke, 1886; first British edition, London: F. A. Davis, 1892); Harry Oosterhuis, *Stepchildren of Nature: Krafft-Ebing, Psychiatry and the Making of Sexual Identity* (Chicago: University of Chicago Press, 2000); Edward M. Brecher, *The Sex Researchers* (London: André Deutsch, 1970), pp. 50–60.
15. Chris Nottingham, *The Pursuit of Serenity: Havelock Ellis and the New Politics* (Amsterdam: University of Amsterdam Press, 1999).
16. Havelock Ellis, *The Nationalisation of Health* (London: T. Fisher Unwin, 1892), pp. 6, 240, 17–18, 20–1.
17. Cited in Dennis Hardy, *Utopian England: Community Experiments, 1900–1945* (London: E. and F. N. Spon, 2000), p. 102.
18. Havelock Ellis, *Questions of Our Day* (London: John Lane/The Bodley Head, 1936), pp. 140–2, 142–3.
19. F. W. Stella Browne, 'Review: *Little Essays of Love and Virtue* By Havelock Ellis', *The New Generation* 1(5) (May 1922): 7–8.

20. F. W. Stella Browne, 'Women and Birth Control', in*Population and Birth-Control: A Symposium*, ed. Eden and Cedar Paul (New York: The Critic and Guide Company, 1917).
21. F. W. Stella Browne, 'Climb-down or Camouflage? Dr Marion Phillips Answered', *The New Generation* 3(10) (October 1924): 115; 'Critics and Champions at Westminster', *The New Generation* 5(6) (June-July 1926): 67. 'Women and Birth Control'; 'The Wastage of the Future', *Beauty and Health* (November 1916): 144–6.
22. F. W. Stella Browne, 'The Philosophy of the Free Spirit', *The New Generation* 3(2) (February 1924): 17; 'Women and Birth Control'.
23. Carpenter, *Civilisation*, 40–1; *Love's Coming of Age*, 110–29; Havelock Ellis, 'The Renovation of the Family', *More Essays of Love and Virtue* (London: Constable and Co., 1931), 22–76; Lesley A. Hall, ' "I have never met the normal woman": Stella Browne and the Politics of Womanhood', *Women's History Review* 6(2) (1997): 157–82, and "'Not a domestic utensil, but a woman and a citizen": Stella Browne on Women, Health and Society', in *Regenerating England: Science, Medicine and Culture in Interwar Britain*, ed. Chris Lawrence and Anna-K Mayer (Amsterdam: Rodopi, 2000), pp. 275–302; F. W. Stella Browne, 'Studies in Feminine Inversion', *Journal of Sexology and Psychoanalysis* (New York) 1 (1923): 51–8.
24. Edward Carpenter, *The Intermediate Sex: A Study of Some Transitional Types of Men and Women* (London: George Allen & Unwin, 1918, first pub. 1909), pp. 107, 116.
25. F. W. Stella Browne, 'A Few Straight Questions to the Eugenics Society', *The Freewoman* (1 August 1912): 217–18, and 'More Questions', *The Freewoman* (15 August 1912): 258; F. W. S. Browne, 'Review. *The Progress of Eugenics* by C. W. Saleeby, MD', *The Malthusian* 38 (July 1914): 51; F. W. Stella Browne, 'The Right to Abortion', in F. W. Stella Browne, A. M. Ludovici and Harry Roberts, *Abortion* (London: George Allen & Unwin, 1935), pp. 31, 40.
26. Eden Paul, 'Birth Control: Communist and Individualist Aspects', *Medical Critic and Guide* (New York) 25(6) (June 1922): 212–16.
27. Browne, 'The Right to Abortion', 41; Lesley A. Hall, 'Archives of the Pioneer Health Centre, Peckham, in the Wellcome Library', *Social History of Medicine* 14 (2001): 525–38, provides an overview of its history. See also G. Scott Williamson and H. Innes Pearse, *Biologists in Search of Material: An Interim Report on the Work of the Pioneer Health Centre, Peckham* (London: Faber and Faber, [1938]); Innes H. Pearse and Lucy H. Crocker, *The Peckham Experiment: A Study in the Living Structure of Society* (Edinburgh: Scottish Academic Press, 1985), and their other works. F. W. Stella Browne, 'Achieving Health', *The New Generation* 11(5) (May 1932): 58.
28. Miriaam Akhtar and Steve Humphries, *Far Out: The Dawning of New Age Britain* (Bristol: Sansom & Co., 1999), pp. 11–45; see also Hardy, *Utopian England*.
29. George Orwell, *The Road to Wigan Pier* (first pub. 1937) (London: Penguin, 1989), p. 161.
30. Lesley A. Hall, 'No Sex Please, We're Socialists: The British Labour Party Closes its Eyes and Thinks of the Electorate', in *Meetings & Alcôves: The Left and Sexuality in Europe and the United States since 1850*, ed. Jesse Battan, Thomas Bouchet and Tania Regin, *Territoires contemporains, cahiers de l'IHC* (Dijon: l'Institut d'histoire contemporain) 8 (2004): 65–78; see also Lesley A. Hall, 'Venereal Diseases in Britain from the Contagious Diseases Acts to the National Health Service', and David Evans, 'Sexually Transmitted Disease Policy in the English

National Health Service, 1945–2000: Continuity and Social Change', in *Sex, Sin and Suffering: Venereal Diseases in European Social Context since 1870*, ed. Roger Davidson and Lesley A. Hall (London: Routledge, 2001), pp. 120–36 and 237–52; and Lesley A. Hall, 'Birds, Bees and General Embarrassment: Sex Education in Britain from Social Purity to Section 28', in *Public or Private Education?: Lessons from History*, ed. Richard Aldrich (London: Woburn Press, 2004), pp. 98–115.

31. Nottingham, *The Pursuit of Serenity*; Rowbotham and Weeks, *Socialism and the New Life*; Lesley A. Hall, *Strong Red Rag: The Life and Times of Stella Browne, Feminist Socialist Sex Radical* (London: I. B. Tauris, forthcoming).
32. Lesley A. Hall, *Sex Gender and Social Change in Britain, 1880 to 2000* (Basingstoke: Palgrave, 2000), pp. 167–76.
33. Ellis, *The Task of Social Hygiene*, 192.

Chapter 11

I would like to thank Judy Greenway, Lesley Hall, Clare Midgley and the editors of this volume for their helpful comments, and Paco Romero for his assistance with some of the translations. I am grateful to the Wellcome Trust and the British Academy for funding the research on which this chapter is based.

1. José Prat, *A las mujeres*, first edition 1903 (Barcelona: Salud y Fuerza, 1912), p. 24. Prat (1867–1932) was a prolific contributor to the anarchist press, an editor of the journal *Natura* and an administrator at the Modern School in Barcelona.
2. For anarchism in Spain, see Temma Kaplan, *Anarchists of Andalusia, 1868–1903* (Princeton, NJ: Princeton University Press, 1977); George Richard Esenwein, *Anarchist Ideology and the Working-Class Movement in Spain, 1868–1898* (Berkeley, CA: University of California Press, 1989); José Álvarez Junco, *La ideología del anarquismo español, 1868–1910* (Madrid: Siglo Veintiuno, 1991); and Martha A. Ackelsberg, *Free Women of Spain: Anarchism and the Struggle for the Emancipation of Women* (Bloomington, IN: Indiana University Press, 1991).
3. The most comprehensive history of the women's movement in Spain is still Geraldine M. Scanlon, *La polémica feminista en la España contemporánea, 1868–1974* (Madrid: Akal, 1986).
4. Sharif Gemie, 'Anarchism and Feminism: A Historical Survey', *Women's History Review* 5 (1996): 422.
5. Ackelsberg, *Free Women of Spain,* pp. 17, 87–8.
6. Álvarez Junco, *La ideología del anarquismo español*, esp. p. 285.
7. For the role of science in anarchist thought see Álvarez Junco, *La ideología del anarquismo español*, ch. 3; Richard Cleminson, *Anarchism, Science and Sex: Eugenics in Eastern Spain, 1900–1937* (Bern: Peter Lang, 2000); and Álvaro Girón Sierra, *Evolucionismo y anarquismo en España, 1882–1914* (Madrid: Consejo de Investigaciones Científicas, 1996).
8. See Mary Nash, 'Un/Contested Indentities: Motherhood, Sex Reform and the Modernization of Gender Identity in Early Twentieth-Century Spain', in Victoria Lorée Enders and Pamela Beth Radcliffe, eds, *Constructing Spanish Womanhood: Female Identity in Modern Spain* (Albany, NY: SUNY Press, 1999), pp. 25–49; and Catherine Jagoe, 'Sexo y género en la medicina del siglo XIX', in Catherine Jagoe, Alda Blanco and Cristina Enríquez de Salamanca, eds, *La mujer en los discursos de género: textos y contextos en el siglo XIX* (Barcelona: Icaria, 1998),

pp. 305–67. This argument about medicine and science has also been made in relation to other countries. See Cynthia Eagle Russett, *Sexual Science: The Victorian Construction of Womanhood* (Cambridge, MA: Harvard University Press, 1989); and Anne Digby, 'Women's Biological Straitjacket', in Susan Mendus and Jane Rendall, eds, *Sexuality and Subordination: Interdisciplinary Studies of Gender in the Nineteenth Century* (London: Routledge, 1989), pp. 192–220.

9. See Lucy Bland, *Banishing the Beast: English Feminism and Sexual Morality, 1885–1914* (London: Penguin, 1995); and Katharina Rowold, ' "The Academic Woman": Minds, Bodies and Education in Britain and Germany, c. 1860-c. 1914' (PhD Thesis, University College London, 1997).
10. See Ramón Ruiz Amado, *La educación femenina* (Barcelona: Librería Religiosa, 1912); and Amado, *La educación moral*, first pub. 1908, 2nd rev. edition (Barcelona, Librería Religiosa, 1913).
11. Frances Lannon, 'Los cuerpos de las mujeres y el cuerpo político católico: autoridades e identidades en conflicto en España durante las décadas de 1920 y 1930', *Historia Social* 35 (1999): 71.
12. Frances Lannon, *Privilege, Persecution, and Prophecy: The Catholic Church in Spain, 1875–1975* (Oxford: Clarendon Press, 1987), p. 54; Lannon, 'Los cuerpos de las mujeres y el cuerpo político católico', 66.
13. See Rafael Rodríguez de Cepeda, 'Misión de la mujer cristiana en le hogar doméstico y su importancia para resolver la cuestión social', *Revista Católica de las Cuestiones Sociales* 8 (November 1902): 642; and Julio Alarcón y Meléndez, 'Un feminismo aceptable', *Razón y Fe* (April 1904): 455.
14. Álvarez Junco, *La ideología del anarquismo español*, p. 206.
15. Richard Cleminson, 'Beyond Tradition and "Modernity": The Cultural and Sexual Politics of Spanish Anarchism', in Helen Graham and Jo Labanyi, eds, *Spanish Cultural Studies: An Introduction. The Struggle for Modernity* (Oxford: Oxford University Press, 1995), p. 118.
16. Vicente Daza, 'Lo que debe ser el trabajo de las mujeres y de los niños', *La Revista Blanca*, 3 (1 July 1900): 30. This approach which conceived of women primarily as reproducers was exemplified by Ricardo Mella in Spain.
17. Mary Nash, 'Estudio preliminar,' in Nash, ed., *'Mujeres Libres': España, 1936–1939* (Barcelona: Tusquets, 1976), pp. 10–11.
18. See Prat, *A las mujeres*, pp. 24–6; Anselmo Lorenzo, 'La mujer (Para un libro en preparación)', *Tierra y Libertad* 4 (30 May 1907); Soledad Gustavo, *A las proletarias* (Buenos Aires: Biblioteca de la Questione Sociale, 1896), p. 10.
19. Soledad Gustavo, 'La mujer a través de las civilizaciones', *La Anarquía* 100 (11 August 1892).
20. Julio Camba, 'Sobre la emancipación de la mujer', *La Revista Blanca* 124 (15 August 1903): 101.
21. Cleminson, 'Beyond Tradition and "Modernity"', 116–17.
22. See Anon., 'La igualdad de la mujer', *La Bandera Social* 80 (2 October 1886); Camba, 'Sobre la emancipación de la mujer', 100; Lorenzo, 'La mujer' (9 May 1907).
23. Soledad Gustavo, 'La mujer: a mi joven amiga María Montseny', *El Productor* 183 (7 February 1890). Mañé mostly wrote under the pseudonym of Soledad Gustavo.
24. Gustavo, *A las proletarias*, p. 10.
25. Camba, 'Sobre la emancipación de la mujer', 100.
26. L. Manouvrier, 'Antropología de los sexos y aplicaciones sociales', *Natura* 12 (15 March 1904): 177. Manouvrier critiqued biological determinism and attacked

anthropological systems of human group inequality throughout his professional career. See Jennifer Michael Hecht, *The End of the Soul: Scientific Modernity, Atheism, and Anthropology in France* (New York: Columbia University Press, 2003), ch. 6. Stephen J. Gould has called him 'the nondeterminist black sheep of Broca's fold'. See Gould, *The Mismeasure of Man*, rev. edition (New York: W.W. Norton, 1996), p. 58.

27. José Prat talked about 'scientific lies of bad faith'. See Prat, *A las mujeres*, p. 15.
28. Theodor L. W. von Bischoff, *Das Studium der Medicin durch Frauen* (Munich: Th. Riedel, 1872).
29. Lorenzo, 'La mujer' (30 May 1907).
30. Martha Ackelsberg, '*Mujeres Libres*: Identity, Community, Sexuality, and Power', *Anarchist Studies* 9 (2000): 100.
31. Ackelsberg, *Free Women of Spain*, p. 25.
32. See Friedrich Engels, *Der Ursprung der Familie, des Privateigentums und des Staates*, first ed. 1884, in *Marx/Engels Gesamtausgabe* (Berlin: Dietz, 1990).
33. Prat, *A las mujeres*, p. 24.
34. See Teresa Claramunt, 'A la mujer', *Fraternidad*, 5 (6 January 1900); Claramunt, *La mujer: consideraciones generales sobre su estado ante las prerrogativas del hombre* (Mahón: El Porvenir del Obrero, 1905), p. 5. José Prat thought that the greater activity of the male and passivity of the female, visible in animal kingdom, led to women's initial subjection. See Prat, *A las mujeres*, p. 7.
35. Doctor Klotz-Forrest, 'La emancipación de la mujer', *Salud y Fuerza* 26 (1908): 357.
36. Claramunt, *La mujer*, pp. 3–4.
37. For instance, Manuel Devaldés, 'La individualidad femenina', *Salud y Fuerza* 9 (1912): 177–8; Nelly Roussel, 'Feminismo y maltusianismo', *Salud y Fuerza* 8 (1911): 65. For anarchist neo-Malthusianism, see Mary Nash, 'El neomaltusianismo anarquista y los conocimientos populares sobre el control de la natalidad en España', in Nash, ed., *Presencia y protagonismo: aspectos de la historia de la mujer* (Barcelona: Serbal, 1984), pp. 307–40.
38. Ackelsberg, '*Mujeres Libres*', 101. See Prat, *A las mujeres*; Camba, 'Sobre la emancipación de la mujer'; [Ana María Mozzoni], *A las muchachas que estudian*, first edition 1895 (Barcelona: José Garriga, 1903), pp. 7–8; Claramunt, *La mujer*, pp. 1, 19; Gustavo, 'La mujer: a mi joven amiga María Montseny' (7 March 1890).
39. Anónimo, 'La igualdad de la mujer'.
40. Claramunt, *La mujer*, p. 7.
41. Soledad Gustavo, 'Movimiento feminista', *La Idea Libre* 110 (5 June 1896). See also Aurora Vilanova, 'Movimiento feminista', *La Revista Blanca* 1 (1 July 1898): 23–4.
42. Teresa Mañé, 'El feminismo', *La Revista Blanca* 3 (30 July 1898): 67.
43. See also Richard Cleminson, 'Viewpoint: Anarchism and Feminism', *Women's History Review* 7 (1998): 135–8. For an anthology of later anarchist writings on homosexuality, see Richard Cleminson, ed., *Anarquismo y homosexualidad: Antología de artículos de la 'Revista Blanca', 'Generación Consciente', 'Estudios' e 'Iniciales', 1924–1935* (Madrid: Huerga y Fierro, 1995).
44. See, for example, Lorenzo, 'La mujer' (30 May 1907).
45. Prat, *A la mujer*, p. 15.
46. For the controversy over Hippocratic medicine, see Tomás Ramos, 'La polémica hipocrática en la medicina española del siglo XIX', *Archivo Iberoamericano de Historia de la Medicina* 6 (1954): 115–61. For an examination of the Hippocratic model of sexual difference in nineteenth-century Spain, see Jagoe, 'Sexo y género en la medicina del siglo XIX', 315ff.

47. Lorenzo, 'La mujer' (30 May 1907).
48. Schools in small villages in Spain were often co-educational by default, but in principle, there was a strong adherence to separate education for girls and boys. For Spanish anarchism and education, see Paul Avrich, *The Modern School Movement: Anarchism and Education in the United States* (Princeton, NJ: Princeton University Press, 1980), ch. 1, which discusses the rationalist school movement in Spain; and Álvarez Junco, *La ideología política del anarquismo español*, ch. 19.
49. Francisco Ferrer Guardia, *La escuela moderna: póstuma explicación y alcance de la enseñanza racionalista* (Barcelona, Imprenta Elzeviriana – Borrás, Mestres, y Compañía, 1912), pp. 54–5.
50. Claramunt, *La mujer*, pp. 7, 18.
51. Soledad Gustavo, 'Influencia social de la mujer', *La Idea Libre* 41 (9 February 1895).
52. Mañé, 'El feminismo', p. 68; Claramunt, 'A la mujer'.
53. Prat, *A las mujeres*, p. 15.
54. Dolores Aleu y Riera, *De la necesidad de encaminar por nueva senda la educación higiénico-moral de la mujer: Tesis doctoral* (Barcelona: La Academia, 1883), p. 27. For anarchists making reference to this passage, see Mañé, 'El feminismo', p. 68; Lorenzo, 'La mujer' (30 May 1907).
55. Gustavo, *A las proletarias*, p. 14.
56. Claramunt, 'A la mujer', Claramunt's emphasis.
57. Gustavo, 'Movimiento feminista'.
58. For Darwin's ideas on gender difference, see Evelleen Richards, 'Darwin and the Descent of Woman', in David Oldroyd and Ian Langham, eds, *The Wider Domain of Evolutionary Thought* (Dordrecht: Reidel, 1983), pp. 57–111.
59. For evolutionism and Spanish anarchism, see Girón Sierra, *Evolucionismo y anarquismo en España*; Girón Sierra, 'The Moral Economy of Nature: Darwinism and the Struggle for Life in Spanish Anarchism (1882–1914)', in Thomas F. Glick, Miguel Angel Puig-Samper and Rosaura Ruiz, eds, *The Reception of Darwinism in the Iberian World: Spain, Spanish America and Brazil* (Dordrecht: Kluwer, 2001), pp. 189–203; and the discussion in Álvarez Junco, *La ideología política del anarquismo español*, ch. 6.
60. By the turn of the century, Spanish anarchists' views on evolution were much indebted to Petr Kropotkin's *Mutual Aid*. Kropotkin embraced the Lamarckian concept of the inheritance of acquired traits. For the impact of Kropotkin's ideas on Spanish anarchism, see Girón Sierra, 'The Moral Economy of Nature'.
61. Camba, 'Sobre la emancipación de la mujer', p. 100. For a similar argument, see 'Prólogo: la mujer ante la ciencia', in Cristóbal Litrán, *La mujer en el cristianismo* (Barcelona: La Academia, 1892), pp. 11–12 by Odón de Buen, who cooperated with Ferrer Guardia in the Modern School.
62. See Álvaro Girón, 'La revolución come medicina: enfermedad mental y anarquismo en torno a 1898', in Ana I. Romero, Juan Casco, Filiberto Fuentenbro and Rafael Huertas, eds, *Cultura y Psiquiatría del 98 en España* (Madrid: Necodisne Ediciones, 1999), pp. 109–18. For anarchism and eugenics, see Cleminson, *Anarchism, Science and Sex*.
63. Soledad Gustavo, *La sociedad futura* (Madrid: Antonio Marzo, 1899), pp. 17, 24; see also Anselmo Lorenzo, *El proletariado militante: memorias de un internacional*, ed. Juan Gómez Casas (Madrid: Zero, 1974), p. 247.
64. Camba, 'Sobre la emancipación de la mujer', 102.
65. Gustavo, *La sociedad futura*, pp. 21–2; see also Lorenzo, 'La mujer' (30 May 1907).

66. Soledad Gustavo, 'Del amor', *La Revista Blanca* 59 (1 December 1900): 250.
67. Gustavo, *La sociedad futura*, p. 22.

Chapter 12

1. Mrs P. L. Oliver [Ethel Grace], 'Early Days of the Blood Transfusion Service', *Blood Transfusion Service Quarterly Circular (British Red Cross Society)* [*QC*], n.s. n. 6 (January 1951), 7–8.
2. The NHS was established in 1948; the National BTS was up and running by 1946.
3. Richard Titmuss, *The Gift Relationship: From Human Blood to Social Policy* (London: Allen & Unwin, 1970), reissued, with additional essays, ed. Ann Oakley and John Ashton (London: LSE Books, 1997), pp. 276, 292.
4. See my unpublished chapter, 'The Wealth of Nations', for the Keynsian influence on Geoffrey Keynes' blood donation ideas.
5. Geoffrey Keynes, 'Blood Donors', *BMJ* 2 (1924): 613–15, 613.
6. Walter Fletcher to Sir Arthur Stanley, 19 July 1923, National Archives/Public Records Office [NA/PRO], London, FD1 3245. Fletcher envisioned a *paid* service.
7. Stanley to Fletcher, 21 July 1923, NA/PRO FD1 3245.
8. See Keynes, *Blood Transfusion* (London: Henry Frowde/Hodder & Stoughton, 1922); N. S. R. Maluf, 'History of Blood Transfusion', *J. Hist. Med.* 9 (1954): 59–107.
9. Peter C. English, *Shock, Physiological Surgery, and George Washington Crile: Medical Innovation in the Progressive Era* (Westport, CT: Greenwood Press, 1980).
10. 'The Transfusion of Blood', *BMJ* 2 (1907): 1006.
11. Ernest Cowell, 'Wound Shock in Front Line Areas', in W. G. Macpherson, A. A. Bowlby, Cuthbert Wallace and Crisp English, eds, *Official History of the War - Medical Services – Surgery of the War*, 2 vols (HMSO, 1922), I: 58–78, 59.
12. L. Bruce Robertson, 'The Transfusion of Whole Blood: A Suggestion for its More Frequent Employment in War Surgery', *BMJ* 2 (1916): 38–40.
13. Keynes, *Blood Transfusion*, p. 20.
14. Pelis, 'Transfusion, with Teeth: Re-animation and the Re-introduction of Human Transfusion to British Medical Practice, 1810–1834', in Robert Bud, Bernard Finn and Helmuth Trischler, eds, *Manifesting Medicine: Bodies and Machines* (Amsterdam: Harwood, 1999), pp. 1–29.
15. William H. Schneider, 'The History of Research on Blood Group Genetics: Initial Discovery and Diffusion', *Hist. Phil. Life Sci.* 18 (1996): 277–303.
16. Louis K. Diamond, 'The Story of our Blood Groups', in Maxwell M. Wintrobe, ed., *Blood, Pure and Eloquent: A Story of Discovery, of People, and of Ideas* (New York: McGraw-Hill, 1980), pp. 691–717.
17. Schneider, 'Blood Transfusion between the Wars', *J. Hist. Med.* 58 (2003): 187–224.
18. See Brian Abel-Smith, *The Hospitals, 1800–1948: A Study in Social Administration in England and Wales* (London: Heinemann, 1964), esp. pp. 307–9.
19. 'House Committee Minutes, London Hospital, 1920–22', 27 February 1922, London Hospital Archives, LH/A/5/57, 396.
20. Transcript, 'Blood Transfusion', Paul Rotha 1941, NA/PRO INF6/518, J. No 11648 [M.I. 247, R825].
21. 'Camberwell Minutes', 3 December 1920, British Red Cross Archives [BRCA], Surrey and London, Acc 932/1. The Division reimbursed donors for expenses.

22. Anon., 'Percy Lane Oliver OBE', pamphlet produced by Tom Richards for The Oliver Memorial Fund, 1996; and Harold H. Gunson and Helen Dodsworth, 'Fifty Years of Blood Transfusion', *Transfusion Medicine* 6 (1996), suppl. 1, chapter 2, p. 5.
23. In its *Annual Reports*, the London County Branch commented: 'Camberwell was one of the best and keenest Divisions in the Branch, and Mr. Oliver was one of the most energetic Secretaries.' The British Red Cross Society, County of London Branch, *Report for 1914 to 1919, also Report for 1920 [-24*] (London: Harrison and Sons).
24. 'Minutes of the Committee of Management', 23 January 1919, 588, King's College Archives [KCA], London, KH/CM/18.
25. 'Minutes of the London County Council', City of London, London Metropolitan Archives [LMA], London, particularly 25 January 1916, 89; 7 March 1916, 225. On Sunday Cinema provisions, 18 April 1916, 370–1; 16 May 1916, 413; 18 July 1916, 757, 976–8. On the LCC, Ken Young and Patricia L. Garside, *Metropolitan London: Politics and Urban Change, 1837–1981* (London: Edward Arnold, 1982).
26. Executive Committee Meeting, 'Minutes Book, Camberwell Division' ['Camberwell Minutes'], 14 November 1919, BRCA, Acc 932/1.
27. Camberwell Minutes, 27 September 1920. This appears to have been but one of many clashes between Camberwell and the County Branch.
28. Camberwell Minutes, 27 June 1921.
29. Camberwell Minutes, 21 October 1921.
30. Camberwell Minutes, 5 May 1922. This was the next meeting after October 1921. Thereafter, the Executive Committee did not meet again until 5 October 1925.
31. 'Blood Transfusion', *The Red Cross* 11 (1924): 122.
32. British Red Cross Society, *Report of the Blood Transfusion Service for the year ended Dec. 31st, 1926* (London: Petley & Co., 1927), 13 ['BTS Annual Report, 1926'].
33. Ethel Oliver, 'Early Days', 7.
34. 'BTS Annual Report, 1926', 4. The Rover Scouts were founded in 1919 to extend scouting to boys over 14. R. S. S. Baden-Powell, *Rovering to Success* (London: 1922).
35. 'Blood Transfusion' (1924): 95. Throughout the Service's interwar existence, a significant proportion of donors were Rover Scouts.
36. 'BTS Annual Report, 1926', 4.
37. 'Blood Transfusion' (1924), 110. Statistics for the first few years vary slightly, according to source.
38. 'BTS Annual Report, 1926', 10–11.
39. Fascist donors appear to have had no control over the race of those who received their blood. 'Blood Donors Wanted: Appeal to Fascists by a Donor', *Blackshirt* 62 (29 June 1934): 3.
40. Canti established policies and techniques that Keynes would later approve. 'Minutes of the Committee' (16 January 1936), NA/PRO FD1 3246; and 'Ronald George Canti', *BMJ* 1 (1936): 137. Oliver cited Canti's authority alongside Keynes'. Oliver to Hayes, 25 February 1926, M.R. 26/6, St. Bartholomew's Hospital Archives [SBHA], London.
41. 'Minutes of the Committee of Management, King's College Hospital' (20 November 1924), 388, KCA, KH/CM/20.
42. 'BTS Annual Report, 1926', 7.

43. 'Report of the Divisional Secretary, 29 October 1925', inserted in the 'Camberwell Minutes', immediately after 5 October 1925, BRCA, Acc 932.
44. The Executive Committee silence meant there were no formal minutes during the period. An account of the early Transfusion Service and other Division work may be found in the first series of Oliver's 'Quarterly Circular', dating from 1924.
45. 'Camberwell Minutes' (5 October 1925), BRCA, Acc 932.
46. 'Report of the Divisional Secretary, 1925'.
47. 'Camberwell Minutes' (18 December 1925), BRCA, Acc 932.
48. When the Camberwell committee quit, the County Branch's Annual Report, which on similar occasions praised departing members enthusiastically, simply observed that Oliver had resigned. 'Annual Report, 1925', *London County Red Cross Reports* 5, BRCA. Thereafter, the Reports gave the new Camberwell Division perky annual praise for its work at the King's College Hospital canteen.
49. Oliver noted that Stanley himself said, 'there was no Branch of the Society of which he was so proud as Camberwell'. Oliver, 'Report of the Divisional Secretary, 1925'.
50. Interview with Francis Hanley, July 1998. Hanley, a Rover Scout who joined with the Service in 1926 and became actively involved in the organization, believed that Oliver had suffered a breakdown.
51. 'London Blood Transfusion Service, Notice', 18 January 1926, BRCA, Acc 562/3.
52. For Keynes' transcript, 'Geoffrey Keynes, Blood Transfusion', Royal College of Surgeons of England, Archives [RCSEA], London, MS A 514. Reproduced by kind permission of the President and Council of the Royal College of Surgeons of England.
53. I would like to thank Susan Lederer for information on American donors.
54. 'BTS Annual Report, 1926', 10–11.
55. Oliver to F. Walser, Gray's Inn Road instrument supplier, 29 October 1929, RCSEA, MS A 514.
56. 'Report of the BTS, 1926', 11–12.
57. 'Service Personalities, No. 3. Ethel Grace Oliver', *QC* (October 1951): 2–3, 2.
58. 'Minutes of the London County Council, Central Public Health Committee Report' (22 July 1930), LMA.
59. 'Minutes of the London County Council' (28 January 1930), 111, LMA.
60. *QC* (22 January 1939): 3.
61. Canti to Fletcher, 29 June 1931; Fletcher to Canti, 8 August 1931, NA/PRO FD1/3245. See 'Minutes, British Red Cross Society Blood Transfusion Committee' (9 May 1935), in RCSEA, MS A 514.
62. On the Voluntary Blood Donor Association, 'Minutes, General Meeting of Members of the VBDA', 21 April 1932, BRCA, Acc 562/65; on insurance from Eagle Star and British Dominions Insurance, 'Minutes of the British Red Cross Society Blood Transfusion Committee' (27 June 1932), 41. Personal papers of Francis Hanley.
63. 'Problems of Blood Transfusion', *BMJ* 2 (1936): 1035. This editorial background spanned an entire page: at once a remarkable tribute to a medical outsider, and an indication of just how much qualification the journal felt was necessary to include an article by such an outsider.
64. 'Second International Blood Transfusion Congress. General Remarks', *QC* 17 (October 1937): 4–5, 4. The emphasis is mine.
65. Gunson and Dodsworth, 'Fifty Years', 13. At this point, medicine was rapidly moving towards the use of cold, stored blood.

66. The Red Cross donor service continued after the war.
67. Titmuss, *Gift Relationship* (1970/1997), p. 271.
68. Roy Porter, *London: A Social History* (London: Hamish Hamilton, 1994).

Chapter 13

1. H. C. Erik Midelfort, *A History of Madness in Sixteenth-Century Germany* (Stanford, CA: Stanford University Press, 1999), pp. 183–6.
2. *Hamburgische Nachrichten*, 23 and 24 February 1853.
3. Ylva Greve, 'Richter und Sachverständige: Der Kompetenzstreit über die Beurteilung der Unzurechnungsfähigkeit im Strafproze des 19. Jahrhunderts', in Helmut Berding, Diethelm Klippel and Günther Lottes, eds, *Kriminalität und abweichendes Verhalten: Deutschland im 18. und 19. Jahrhundert* (Göttingen: Vandenhoeck & Ruprecht, 1999), 69–104, esp. 69, n.2.
4. Roger Smith, *Trial by Medicine: Insanity and Responsibility in Victorian Trials* (Edinburgh: Edinburgh University Press, 1981), p. 90.
5. Midelfort, *History of Madness*, 220–1; Johann Klefeker, *Sammlung der Hamburgischen Gesetze und Verfassungen* . . . 12 vols (Hamburg: J. C. Piscator, 1765–74), 5: pp. 446–7.
6. See Michael MacDonald, *Mystical Bedlam: Madness, Anxiety, and Healing in Seventeenth-Century England* (Cambridge: Cambridge University Press, 1981); Smith, *Trial by Medicine*; Midelfort, *History of Madness*; Esther Fischer-Homberger, *Medizin vor Gericht: Zur Sozialgeschichte der Gerichtsmedizin* (Darmstadt: Luchterhand Literaturverlag, 1988), pp. 124–64.
7. Charles E. Rosenberg, *The Trial of the Assassin Guiteau: Psychiatry and Law in the Gilded Age* (Chicago: University of Chicago Press, 1968), pp. 54–5, 63, 68–9.
8. Joel Peter Eigen, 'Delusion in the Courtroom: The Role of Partial Insanity in Early Forensic Testimony', *Medical History* 35 (1991): 25–49; Hannah Franziska Augstein, 'J. C. Pritchard's Concept of Moral Insanity – a Medical Theory of the Corruption of Human Nature', *Medical History* 40 (1996): 311.
9. Klefeker, *Sammlung*, 5: 447. The four cases discussed below are taken from Klefeker's account.
10. D. G. Jacobi, *Geschichte des Hamburger Niedergerichts* (Hamburg: Gustav Eduard Nolte, 1866), pp. 47–76; Klefeker, *Sammlung*, 5: pp. 261–568 on courts in Hamburg.
11. Johann Carl Daniel Curio, quoted in Percy Ernst Schramm, *Hamburg, ein Sonderfall in der Geschichte Deutschlands* (Hamburg: Christians, 1964), pp. 15–16.
12. Klefeker, *Sammlung*, 5: p. 448.
13. Klefeker, *Sammling*, 5: pp. 448–9.
14. Klefeker, *Sammling*, 5: pp. 450–1.
15. Klefeker, *Sammling*, 5: pp. 450–60.
16. Midelfort, *History of Madness*, pp. 187–93.
17. Klefeker, *Sammlung*, 5: p. 458; Samuel Stryk, *Disputatio juridica solennis de dementia et melancholia* (1683).
18. The report was filed on 3 September 1803, 'ad S.'; 'Fernere summarische Vernehmung des Arrestaten Johann Georg Rüsau' (20 August 1803), 'Lit. O.'; and Rüsau's 'Ueber den Gang meiner Bildung', all in StAHbg, Senat Cl. VII Lit. Me No. 8 Vol. 13.
19. Fernere summarische Vernehmung des Arrestaton Johann Georg Rüsau.

20. Johann Christian Reil, *Rhapsodien über die Anwendung der psychischen Curmethoden auf Geisteszerrüttung* (Halle: in der Curtschen Buchhandlung, 1803), pp. 306–60; quotation, 306–7.
21. 'Defensionales …', n.d., presented to the Lower Court (Niedergericht), StAHbg, Senat Cl. VII Lit. Me No. 8 Vol. 13.
22. The meeting took place on 28 December 1803, 'P.M.', in StAHbg, Senat Cl. VII Lit. Me No. 8 Vol. 13; the decision of the Niedergericht was pronounced on 10 February 1804, in ibid.; Schleiden filed an appeal to the Obergericht on 17 February 1804, 'Libellus appellatorius defensionalis in peinlichen Sachen …' in ibid.; Hermann Gustav Gernet, *Mittheilungen aus der älteren Medicinalgeschichte Hamburgs: Kulturhistorische Skizze auf urkundlichen und geschichtlichen Gründe* (Hamburg: Mauke, 1869), p. 354.
23. *Rüsaus Familienmord: Am Morgen des 15ten August 1803* (Altona: n.p., n.d.). In a similar vein: *Etwas von den schrecklichen Moord in der Gräu-Straat in Hamburg* (n.p.: n.p., 1803); J. O. Thie, *Wie kann ein Mensch so tief sinken, da er zum absichtlichen Mörder am seiner ganzen Familie wird?* (Hamburg: n.p., 1803); *Rüsau der Theologe und Kaufman! als siebenfacher Familienmörder vor dem Richterstuhl der Menschheit* (Hamburg: n.p., 1803). On deterrence, see 'Ueber Verstandszerrüttung, ihre Folgen und ihre Bestrafung', *Hamburg & Altona* 2(11) (1803): 140.
24. [Johann Carl Daniel Curio], *Rüsau's Leben und Hinrichtung in pragmatischer, moralischer und psychologischer Hinsicht* (Hamburg: Friedrich Hermann Bestler, 1804). For a positive review, see *Hamburg & Altona* 3(5) (1804): 217–19.
25. Curio, *Rüsau's Leben und Hinrichtung*, pp. 5, 33, 35, 39, 41, 45, 56–61.
26. Curio, *Rüsau's Leben und Hinrichtung*, pp. 78–9, 95.
27. *Noten zum Text, oder freymüthige Gedanken über die gerichtliche Vertheidigungsschrift des Inquisiten Rüsau* (n.p. [Hamburg]: n.p., 1804).
28. A. C. Wolters, *Ein Wort über Defensionen: Nebst einem, Bemerkungen über Rüsaus Criminalfall und drei gerichtliche Vertheidungen enthaltenden, Anhange* (Hamburg: Friedrich Perthes, 1805), pp. 50, 54.
29. Wolters, *Ein Wort über Defensionen*, pp. 59–62, 67. Johann Caspar Lavater, *Von der Physiognomik* (Leipzig: Bei Weidmanns Erben und Reich, 1772); Franz Joseph Gall,*Philosophisch-medicinsiche Untersuchungen über Natur und Kunst im kranken und gesunden Zustande des Menschen* (Vienna: Bei Rudolph Gräffer und Comp., 1791).
30. Wolters, *Ein Wort über Defensionen*, p. 72

Chapter 14

My thanks to Roberta Bivins for her helpful comments on various drafts of this chapter; to Jane Henderson for her encouragement; to Punam Zutshi for e-listening to my endless complaints about progress; and to the late Roy Porter for cajoling me into working on nerve medicine in Britain.

1. The quoted phrase is from Stefan Collini, *Public Moralists: Political Thought and Intellectual Life in Britain 1850–1930* (Oxford: Oxford University Press, 1991), p. 122.
2. On the prevalence of nervous breakdowns, see Janet Oppenheim, *'Shattered Nerves': Doctors, Patients, and Depression in Victorian England* (New York: Oxford University Press, 1991); George Frederick Drinka, *The Birth of Neurosis: Myth, Malady and the Victorians* (New York: Simon & Schuster, 1984) and the essays in

Marijke Gijswijt-Hofstra and Roy Porter, eds, *Cultures of Neurasthenia from Beard to the First World War* (Amsterdam: Rodopi, 2001).

3. John Stuart Mill, *Autobiography*, ed. John M. Robson (London: Penguin, 1989), pp. 112, 110.
4. John Robson, *The Improvement of Mankind: The Social and Political Thought of John Stuart Mill* (Toronto: University of Toronto Press, 1968), p. 22.
5. Mill, *Autobiography*, ed. Robson, p. 112.
6. See Philip Davis, *The Victorians* (Oxford: Oxford University Press, 2002), p. 173.
7. Mill, *Autobiography*, ed. Robson, pp. 112–13.
8. Mill, *Autobiography*, ed. Robson, pp. 114–15.
9. Mill, *Autobiography*, ed. Robson, p. 115.
10. Davis, *The Victorians*, p. 173.
11. Mill, *Autobiography*, ed. Robson, pp. 116–17.
12. See 'Introduction', in Mill, *Autobiography*, ed. Jack Stillinger (London: Oxford University Press, 1971), p. xiv.
13. See, for example, Mill's admission to Thomas Carlyle that he was 'indebted for all the most valuable of such insight as I have into the most important matters' to his occasional fits of dejection, into one of which he seemed to be falling at the time of writing this letter. See F. E. Mineka, ed., *Earlier Letters of John Stuart Mill* (Toronto: University of Toronto Press, 1963), p. 149.
14. See Alexander Bain, *John Stuart Mill: A Criticism, with Personal Recollections* (London: Longmans, Green, 1882), pp. 42–4. See also Janice Carlisle, *John Stuart Mill and the Writing of Character* (Atlanta, GA: University of Georgia Press, 1991), p. 95.
15. See Elie Halévy, *The Growth of Philosophic Radicalism*, trans. Mary Morris (London: Faber, 1972), pp. 282–4.
16. See Peter Gay, *Freud: A Life for Our Time* (New York: Norton, 1988), p. 36.
17. A. W. Levi, 'The "Mental Crisis" of John Stuart Mill', *Psychoanalytic Review*, 32 (1945): 86–101, at 97.
18. On the pros and cons of psychohistory, see Fred Weinstein, 'Psychohistory and the Crisis of the Social Sciences', *History and Theory* 34 (1995): 299–319; and H. Stuart Hughes, *History as Art and as Science: Twin Vistas on the Past* (New York: Harper & Row, 1964).
19. Bruce Mazlish, *James and John Stuart Mill: Father and Son in the Nineteenth Century* (London: Hutchinson, 1975), p. 175.
20. Mazlish, *James and John Stuart Mill*, p. 11.
21. See Rick Rylance, *Victorian Psychology and British Culture 1850–1880* (Oxford: Oxford University Press, 2000), pp. 147–202.
22. Quoted by L. S. Hearnshaw, *A Short History of British Psychology, 1840–1940* (London: Methuen, 1964), pp. 11–12.
23. Hearnshaw, *Short History*, p. 11; and Rylance, *Victorian Psychology*, pp. 174 and 177.
24. Roger Smith, 'The Background to Physiological Psychology in Natural Philosophy', *History of Science* 11 (1973): 75–123, at 95.
25. Bain, *John Stuart Mill*, p. 23.
26. Rylance, *Victorian Psychology*, p. 168.
27. Bain, *John Stuart Mill*, p. 38.
28. He did not, however, use the term 'neurasthenia', which was to become popular in late nineteenth-century medical circles to explain cases such as Mill's. On concepts of nervous disorder in British medicine and culture, see W. F. Bynum, 'The Nervous Patient in Eighteenth- and Nineteenth-Century Britain: The Psychiatric Origins of British Neurology', in W. F. Bynum, Roy

Porter and Michael Shepherd, eds, *The Anatomy of Madness: Essays in the History of Psychiatry*, 3 vols (London: Tavistock, 1985), 1: p. 102.

29. Bain, *John Stuart Mill*, p. 141.
30. See Oppenheim, *'Shattered Nerves'*, esp. pp. 155–7, for the breakdowns of celebrated figures, such as John Bright, Michael Faraday and Charles Kingsley. All of these were attributed at the time to overwork.
31. Bain also noted Mill's frugal diet and relative lack of sensuality. See Bain, *John Stuart Mill*, pp. 38, 64, 149.
32. 'I do not believe any man was ever the worse for work,' Mill had once remarked to Bain. See Bain, *John Stuart Mill*, pp. 38 and 77.
33. Levi, 'The "Mental Crisis" of John Stuart Mill', p. 94.
34. Mill's crisis occurred in 1826–7 but our description of it comes from his *Autobiography*, which he drafted in 1853–4; it was published only after his death in 1873 and incorporated many changes. See Jack Stillinger, 'Introduction: Mill's *Autobiography*: Imagination and the Growth of a Philosophic Mind', in J. S. Mill, *Autobiography*, ed. J Stillinger, pp. vii-xxv.
35. See John M. Robson, 'Introduction' to John Stuart Mill, *Autobiography*, ed. J. M. Robson, pp. 1–23, at pp. 5–6.
36. See Jack Stillinger, 'John Mill's Education: Fact, Fiction, and Myth', in Michael Laine, ed., *A Cultivated Mind* (Toronto: University of Toronto Press, 1991), pp. 19–43, at pp. 29, 23 and 31 respectively.
37. See Mill, *Autobiography*, ed. Robson, p. 25.
38. Mill, *Autobiography*, ed. Robson, p. 41.
39. Mill, *Autobiography*, ed. Robson, pp. 27–48, 45–6.
40. Mill, *Autobiography*, ed. Robson, pp. 44, 47–8.
41. Mill, *Autobiography*, ed. Robson, p. 54.
42. William L. Langer, 'The Next Assignment', *American Historical Review* 63 (1957–8): 283–304; and Erik Erikson, *Young Man Luther* (New York: Norton, 1958), p. 20.
43. The two biographies of Mill are Michael St John Packe, *The Life of John Stuart Mill* (London: Secker & Warburg, 1954); and Nicholas Capaldi, *John Stuart Mill: A Biography* (Cambridge: Cambridge University Press, 2004). The first complete biography of Immanuel Kant in a half-century was Manfred Kuehn, *Kant: A Biography* (Cambridge: Cambridge University Press, 2002). Hegel has had only one really full biography in English (Terry Pinkard, *Hegel: A Biography* [Cambridge: Cambridge University Press, 2000]), as has Wittgenstein (Ray Monk, *Ludwig Wittgenstein: The Duty of Genius* [London: Cape, 1990]). Friedrich Nietzsche is the obvious exception to this rule – for a recent biography that gives ample information on earlier ones, see Rüdiger Safranski, *Nietzsche: A Philosophical Biography*, transl. Shelley Frisch (London: Granta, 2002).
44. Capaldi, *John Stuart Mill*, p. 85.
45. Roy Porter, *A Social History of Madness: Stories of the Insane* (London: Weidenfeld and Nicolson, 1987), p. 1.

Chapter 15

For their interest and useful comments, I owe many thanks to Simon Schaffer, John Forrester, Roberta Bivins, Hubertus Jahn and Sarah Dry. A brief, preliminary account of Ella's story appeared in *Endeavour* 2 (2006): 65–70.

1. I reconstructed the case based principally on the September and October 1894 issues of the Hungarian newspapers *Pesti Napló* (hereafter PN), *Budapesti Hírlap*

(BH), *Debreczeni Ellenor* (DE), *Nyíregyházi Hírlap* (NH) and *Szabőlcsi Szabadsajtó* (SS); Dr Lajos Szilvek, *Hypnotismus lélektani, orvostudományi, történeti és törvényszéki szempontból* (Hypnotism from Psychological, Medical, Historical, and Forensic Perspectives) (Budapest: Athenaeum, 1900), pp. 192–209; and András Jósa, *Barangolás Németországba és visszaemlékezések* (Wanderings in Germany and Reminiscences) (Nyíregyháza, 1906).

2. See *SS*, 18 March 1894, *PN* and *BH*, 19 September 1894.
3. See, for instance, *DE*, 25 September 1894; *BH*, 26 September 1894. For Neukomm's experiments, see also *NH*, 21 July 1892, 20 October 1892, and *PN*, 11 October 1894.
4. See *The Times*, 22 September 1894. See also *PN*, 20, 21 and 22 September 1894.
5. Dr Vragassy, quoted in *The Times*, 22 September 1894 and *PN*, 21 September 1894.
6. Numerous newspapers reported on the case, including *The Times*, *Pester Lloyd*, *Neues Wiener Abenblatt*, *Neue Freie Presse*, *l'Eclair,* etc.
7. The decree was issued by the Ministry of Interior in December 1894. See Kornél Chyzer, *Az egészségügyre vontakozó törvények és rendeletek gyűjteménye. 1854–1894* (Collection of Laws and Decrees Concerning Health Care, 1854–1894) (Budapest, 1894), p. 752, emphasis added.
8. There were several legal attempts in Europe to ban public hypnotic séances and lay hypnotic practice; see Albert Moll, *Hypnotism. Including a Study of the Chief Points of Psycho-Therapeutics and Occultism*, transl. Arthur F. Hopkirk (London: Walter Scott Publishing Co., 1909), pp. 439–43. The 1894 Hungarian resolution was unique in prohibiting academic research in the field of hypnosis.
9. For general histories of hypnosis, see Alan Gauld, *A History of Hypnotism* (Cambridge: Cambridge University Press, 1992); Adam Crabtree, *From Mesmer to Freud. Magnetic Sleep and the Roots of Psychological Healing* (New Haven, CT: Yale University Press, 1993). For the role of hypnosis in the history of dynamic psychiatry, see Henri F. Ellenberger, *The Discovery of the Unconscious: The History and Evolution of Dynamic Psychiatry* (New York: Basic Books, 1970). For mesmerism in Victorian England, see Alison Winter, *Mesmerized. Powers of Mind in Victorian Britain* (Chicago: University of Chicago Press, 1998).
10. On Svengali, see Daniel Pick, *Svengali's Web. The Alien Enchanter in Modern Culture* (New Haven, CT and London: Yale University Press, 2000).
11. On the medical fascination with altered states and hypnosis at the turn of the century, see, among others, Henri Ellenberger, 'The Great Patients', in *Beyond the Unconscious. Essays of Henri F. Ellenberger in the History of Psychiatry*, ed. Mark S. Micale (Princeton, NJ: Princeton University Press, 1993), pp. 237–307; John Forrester and Lisa Appignanesi, *Freud's Women* (London: Weidenfeld & Nicolson, 1992); Sonu Shamdasani, 'Introduction' to Théodore Flournoy, *From India to the Planet Mars. A Case of Multiple Personality with Imaginary Languages* (Princeton, NJ: Princeton University Press, 1899/1994), pp. xi-li; Mikkel Borch-Jacobsen, *Remembering Anna O. A Century of Mystification,* transl. Kirby Olson et al. (New York and London: Routledge, 1996); Charles Bernheimer and Claire Kahane, eds, *Dora's Case* (London: Virago, 1985); Max Rosenbaum and Melvin Muroff, eds, *Anna O: Fourteen Contemporary Reinterpretation* (New York and London: Free Press, 1984).
12. On the Department of Mental Health and Pathology at the Budapest Medical Faculty, see Emese Lafferton, 'From Private Asylum to University Clinic: Hungarian Psychiatry, 1850–1908', in *Framing and Imagining Disease in Cultural*

History, ed. George S. Rousseau et al. (New York: Palgrave Macmillan, 2003), pp. 190–213.

13. Károly Schaffer, *A hypnotismus élettani, gyógytani és törvényszéki szempontból* (Hypnotism from Physiological, Therapeutic and Forensic Perspectives) (Budapest, 1895).
14. For a comprehensive discussion of this line of research, see Schaffer, *A hypnotismus*, pp. 17–36.
15. For the detailed analysis of Ilma's case, see Emese Lafferton, 'Hypnosis and Hysteria as Ongoing Processes of Negotiation. Ilma's Case from the Austro-Hungarian Monarchy', *History of Psychiatry* 2 (2002): 177–97; 3 (2002): 305–27.
16. See Jendrássik, 'A suggestióról' (On Suggestion), *Orvosi Hetilap* (Medical Weekly), 23 (1888): 746–9, 781–5, esp. 748; Jendrássik, 'Hypnoticus suggestio kísérletek' (Experiments with Hypnotic Suggestion), *Gyógyászat* (Medicine) (1887): 140–2.
17. Jendrássik, 'Hypnoticus suggestio', 140–2.
18. See Jendrássik, 'A hysteriás suggerálhatóságról' (On Hysterical Suggestibility), *Orvosi Hetilap* 42 (1892): 508–10; 43 (1892): 523–5; 44 (1892): 537–9; 45 (1892): 551–3, esp. 538.
19. See Jendrássik, 'A suggestióról', 746–9, 781–5; and 'Hypnoticus suggestio', 140–2.
20. See Jendrássik, 'Hypnoticus suggestio', 140–2; and 'Eszmecsere a hypnosis felett' (Discussion over Hypnosis), in *Orvosi Hetilap* 19 (1892): 228–9; 20 (1892): 254–60; and *Gyógyászat* 21 (1892): 250–3; 22 (1892): 263–5.
21. See *BH* and *PN*, 19 September 1894.
22. See *BH*, 19 and 20 September 1894.
23. See August Forel, *Hypnotism or Suggestion and Psychotherapy. A Study of the Psychological, Psycho-Physiological and Therapeutic Aspects of Hypnotism,* transl. H. W. Armit from the 5th German edition (New York and London: Rebman, 1906), pp. 320, 329. Also Schaffer, *A hypnotismus*, p. 30; and Moll, *Hypnotism*, p. 415.
24. See Schaffer, *A hypnotismus*, p. 73; Szilvek, *Hypnotismus*, pp. 239–40; Moll, *Hypnotism*, p. 415. For a discussion of the medico-legal debate between adherents of the Salpêtrière and the Nancy schools, see Ruth Harris, *Murders and Madness. Medicine, Law, and Society in the Fin de Siècle* (Oxford: Clarendon Press, 1989), pp. 171–93.
25. For a wonderful discussion of this in the French context, see Harris, *Murders and Madness*. See also Ragine Plas, 'Hysteria, Hypnosis and Moral Sense in French Nineteenth-century Forensic Psychiatry: The Eyraud-Bompard Case', *International Journal of Law and Psychiatry* 4 (1998): 397–407; and Ruth Harris, 'Melodrama, Hysteria and Feminine Crimes of Passion in the Fin de Siècle', *History Workshop* 25 (1988): 31–63. For more general works on legal psychiatry in the nineteenth century, see Charles E. Rosenberg, *The Trail of the Assassin Guiteau: Psychiatry and the Law in the American Gilded Age* (Chicago: The University of Chicago Press, 1968); Roger Smith, *Trial by Medicine: Insanity and Responsibility in Victorian Trials* (Edinburgh: Edinburgh University Press, 1981); Joel Peter Eigen, *Unconscious Crime: Mental Absence and Criminal Responsibility in Victorian London* (Baltimore, MD: Johns Hopkins University Press, 2003).
26. Liégeois, quoted in Moll, *Hypnotism*, p. 403. Forel similarly believed in the preventive power of such suggestions, Forel, *Hypnotism*, p. 331.
27. Moll, *Hypnotism*, pp. 402–3.
28. Schaffer, *A hypnotismus*, pp. 71–2. See also A. von Schrenck-Notzing, 'Zum Fall Czynski. Eine Entgegnung', *Zeitschrift für Hypnotismus* 3 (1894/5): 176–85.

29. Forel, *Hypnotism*, p. 332.
30. See Schaffer, *A hypnotismus*, p. 72; Moll, *Hypnotism*, pp. 406–7; Schrenck-Notzing, 'Zum Fall Czynski'.
31. Moll, *Hypnotism*, p. 406.
32. See Moll's examples in *Hypnotism*, pp. 403–4.
33. For such cases, see Moll, *Hypnotism*, pp. 404–5.
34. For the case of hypnotized abortion, see Forel, *Hypnotism*, p. 340. Harris makes a very similar point about the danger of hypnotized individuals, especially women, breaking contracts, see Harris, *Murders and Madness*, p. 170.
35. See Moll, *Hypnotism*, pp. 412–13, 415.
36. Moll, *Hypnotism*, p. 413.
37. For the Tiszaeszlár case, see Bernheim, *Suggestive Therapeutics,* pp. 167–78, esp. 167–9 and 176, and Bernheim, *De la suggestion et de ses applications à la thérapeutique*, 2nd edition (Paris, 1888), pp. 231–40.
38. Schaffer and Forel conducted similar experiments. See Bernheim, *De la suggestion*; and Schaffer, *A hypnotismus*, p. 75; Forel, *Hypnotism*, pp. 335–7; Moll, *Hypnotism*, pp. 428–31.
39. Moll, *Hypnotism,* p. 414.
40. Moll, *Hypnotism*, p. 407.
41. Forel, *Hypnotism*, p. 341.
42. Forel, *Hypnotism*.
43. Forel, *Hypnotism*, p. 342. See also A. von Schrenck-Notzing, *Über Suggestion und Erinnerungsfälschung im Berchthold-Process* (Lepzig: Barth Verlag, 1897).
44. Forel's term, see Forel, *Hypnotism*, p. 342.
45. At the 1889 International Congress of Psychiatrists in Paris dedicated to the experimental and therapeutic uses of hypnotism, the questions of the prohibition of open hypnotic sessions and the regulation of the practice of hypnosis by the authorities were widely discussed. See 'Beszámoló az 1889-es párizsi Nemzetközi Pszichiátriai Kongresszusról' (Report on the 1889 International Congress of Psychiatrists in Paris), *Gyógyászat* 36 (1889): 429–30.
46. See 'Eszmecsere', *Orvosi Hetilap*, pp. 254–60.
47. See Lafferton, 'Hypnosis and Hysteria'.
48. Rákosi in 'Eszmecsere', *Orvosi Hetilap,* pp. 228–9, 254–60.
49. Ödön Blum, 'Szabad-e hypnotizálni?' (Is Hypnosis Permissible?), *Gyógyászat* 31 (1889): 361–2.
50. See Jakab Salgó, 'A hypnotismus tudományos és gyógyértéke' (The Scientific and Therapeutic Value of Hypnotism), *Klinikai Füzetek* (Clinical Papers) 5 (1896): 1–17, esp. 10–11.
51. Salgó, 'A hypnotismus tudományos és gyógyértéke', 13, 17; and Blum, 'Szabad-e hypnotizálni?', 361–2.
52. See Gyula Donáth and Pál Ranschburg on spiritism in *Második Elmeorvosi Értekezlet Munkálatai* (Works of the Second National Congress of Psychiatrists), ed. László Epstein (Budapest: Pallas, 1903), pp. 222–9.
53. See Gyula Krúdy's article in *DE*, 29 September 1894; also *PN*, 21 and 31 October 1894, *NH,* 1 November 1894.
54. See, for instance, *BH*, 25 and 26 September 1894.
55. See *BH*, 27 September 1894.
56. Benedikt, quoted in *The Times*, 24 September 1894 and *BH*, 24 September 1894.
57. See *The Times*, 24 September 1894 and *BH*, 24 September 1894.
58. See *BH*, 20 and 25 September 1894.

59. *BH*, 20 September 1894. Foreign commentators followed this version until it was clarified in the press.
60. See Forel in *BH*, 29 September 1894; also *PN*, 28 and 29 September 1894, and Forel, *Hypnotism*, pp. 334–5.
61. A few days after the funeral, the court ordered exhumation and dissection of the whole body, which found the vascular and breathing systems intact. See András Jósa quoted in *BH*, 24 September 1894; also Albert von Schrenck-Notzing, *Kriminalpsychologische und psychopathologische Studien* (Leipzig: Verlag von Johann Ambrosius Barth, 1902), pp. 131–4. According to Forel, however, these findings did not offer any explanation for the death; see Forel, *Hypnotism*, pp. 334–5.
62. For Krafft-Ebing and Benedikt, see *The Times*, 24 September 1894, *PN*, 22 and 23 September 1894, *BH*, 23 and 24 September 1894.
63. For Schrenck-Notzing, see *Kriminalpsychologische*, p. 132. See also Schrenck-Notzing, 'La suggestion et l'hypnotisme dans leurs rapports avec la jurisprudence', in *Deuxième Congrès international de l'hypnotisme expérimental et thérapeutique: Paris 12–16 Août 1902,* ed. Edgar Bérillon and Paul Farez (Paris: Revue de l'hypnotisme, 1902), pp. 121–31, esp. 125. For Moravcsik, see *BH*, 20 September 1894.
64. See the French doctor Felix Regnault's opinion and the interview with doctors at the Salpêtrière in *l'Eclair*, quoted in 'Un cas de mort dans l'état d'hypnotisme', *Revue de l'hypnotisme expérimental et thérapeutique*, (1894–95): 122–3. For similar French expert opinion, see *PN,* 6 October 1894; and Bernheim, quoted in *PN*, 28 September 1894.
65. See Forel, quoted in *BH*, 29 September 1894 and *PN*, 28 September 1894.
66. See Schrenck-Notzing, *Kriminalpsychologische*, p. 132; also see 'Sur un cas d'hypnotisme mortel', *Revue de l'hypnotisme* (1894–5): 347.
67. *Lancet*, 22 and 29 September 1894, 701 and 749–50; *British Medical Journal*, 29 September 1894, 719.
68. See eminent Hungarian legal experts' opinion in *BH*, 21 September 1894.
69. See the 1895 court decision and explanation published in *PN*, 4 November 1895. The explanation explicitly refers to the Council's different opinion.
70. See Jósa in *BH* and *PN*, 24 September 1894. See also Jósa, *Barangolás*; and Jósa, *Eloadás a hypnosisról* (Lecture on Hypnosis) (Nyíregyháza, 1916); Szilvek, *Hypnotismus*, pp. 192–209.
71. See Jósa's report published in *BH* and *PN*, 24 September 1894. Last quote is from *PN*, 25 October 1894.
72. See *BH*, 19 September 1894.
73. In an interview published in October, Neukomm – previously so proud of his experiments – explicitly stated that the main goal *in most* séances was to treat Ella's headaches and other symptoms therapeutically. *BH*, 4 October 1894.
74. See court decision published in *PN*, 4 November 1895.
75. See *PN*, 4 November 1895. The Royal prosecutor appealed against this decision; see *PN*, 3 November 1895. Also *NH*, 7 November 1895.
76. *PN*, 25 September 1894.
77. See *PN* and *BH*, 25 September 1894.
78. *PN* and *BH*, 25 September 1894.
79. See *PN,* 22 September 1894 and *BH*, 23 September 1894.
80. See *BH*, 26 September 1894.
81. Jósa was an amateur archaeologist who founded archaeological and ethnographic research in the region as well as the local county museum which is named after him.

82. See Jósa, *Eloadás* and *Barangolás*; and Szilvek, *Hypnotismus*.
83. See Jósa, *Előadás*, p. 5; and *Barangolás*, p. 66.

Chapter 16

1. Roy Porter, *Flesh in the Age of Reason* (London: Allen Lane, 2003), p. 15.
2. For illustrations of the raging controversy, see the letters pages of the *British Medical Journal*, 17, 24 and 31 January and 7 February 1914.
3. See Isabel Hutton, *Memories of a Doctor in War and Peace* (London: Heinemann, 1960), pp. 128, 241–6 and the discussion by Pines, in Jeremy Holmes, ed., *Textbook of Psychotherapy in Psychiatric Practice* (Edinburgh: Churchill Livingstone 1991), ch. 2.
4. Clara Gallini, *La Sonnambula meravigliosa. Magnetismo e ipnotismo nell'ottocento italiano* (Milan: Feltrinelli, 1983); for mesmerism in Victorian Britain, see Alison Winter, *Mesmerized: Powers of Mind in Victorian Britain* (Chicago: University of Chicago Press, 1998).
5. Dr Harry Roberts, writing in *John O'London's*, 15 May 1920, p. 152, quoted in G. K. Chesterton, 'Our Notebook', *Illustrated London News*, 29 May 1920, p. 910.
6. Chesterton 'Our Notebook'; cf. Dean Rapp, 'The Reception of Freud by the British Press: General Interest and Literary Magazines, 1920–1925', *Journal of the History of the Behavioural Sciences* 24 (1988): 191–201, at 196.
7. Hallam Tennyson, *Alfred Lord Tennyson: A Memoir by his Son* (London: Macmillan, 1905), p. 265.
8. Quoted in Rapp 'Reception', 191.
9. On this connection, see Frank Sulloway, *Freud: Biologist of the Mind: Beyond the Psychoanalytic Legend* (London: Harvard University Press, 1979); L. B. Ritvo, *Darwin's Influence on Freud: A Tale of Two Sciences* (New Haven, CT: Yale University Press, 1990).
10. As Ernest Jones put it, the Freudian blow to man's narcissism disturbed 'belief that at the centre of his personality there resides something, whether called his ego or his soul, that is informed about all that goes on within him, and has a full knowledge of his motives and interests; that furthermore he possess an instrument, "will-power", which exercises command and control over the rest of his personality'. Ernest Jones,*Sigmund Freud: Life and Work*, 3 vols (London: Hogarth Press, 1953–7), 2: p. 253.
11. Sigmund Freud, 'Some Elementary Lessons in Psycho-analysis' [1938], in James Strachey et al., eds, *Standard Edition of the Complete Works of Sigmund Freud* (London: Hogarth Press and the Institute of Psychoanalysis, 1953–74) [SE], 23: p. 282.
12. For examples, see John Forrrester, *Dispatches from the Freud Wars* (Cambridge, MA: Harvard University Press, 1997), ch. 6; on Jung's anti-Semitism, see Sander Gilman, *Freud, Race and Gender* (Princeton, NJ: Princeton University Press, 1993), p. 31. On the dangerous psychological powers of the Jews, as perceived in various forms of anti-Semitism, Daniel Pick, *Svengali's Web: The Alien Enchanter in Modern Culture* (London: Yale University Press, 2000).
13. Anon., 'Two Dream Books' [review of Brill's English Translation of the third edition of *The Interpretation of Dreams* and Reginald Hine's *Dreams and the Way of Dreams*], *The Athenaeum: Journal of Literature, Science, the Fine Arts, Music, and the Dream*, 4460 (13 April 1913): 424. In 1941, the professor was pointedly referred to as 'Herr Freud' in a leader in *The Times* ('Dreams and Dreamers', *The Times,* 22 March 1941, p. 15).

14. W. H. R. Rivers, 'Freud's Psychology of the Unconscious', *Lancet* 1 (1917): 912–14; cf. R. H. Hingley, *Psycho-Analysis* (London: Methuen, 1921), p. 21. The ultimate 'rock of offence', obstructing the development of psychoanalysis in England, according to this study, was Freud's sexual theory.
15. *The Times*, 31 December 1925, p. 11; cf. Vincent Brome, *Ernest Jones: Freud's Alter Ego* (London: Caliban Books, 1982) p. 109. In another leader, on 25 April 1933 (p. 15), entitled 'The Distinction of Dreams', *The Times* bemoaned the fact that the liberty to dream without fear or reproach has been made impossible by virtue of the 'intimidating treatises' of Freud and Jung.
16. Perry Anderson, 'Components of the National Culture' [*New Left Review*, 1968], reprinted in Alexander Cockburn and Robin Blackburn, eds, *Student Power* (Harmondsworth: Penguin, 1969).
17. Robert Hinshelwood, 'Psychoanalysis in Britain: Points of Cultural Access, 1893–1918', *International Journal of Psycho-Analysis* 76 (1995): 135–51; Laura Cameron and John Forrester, ' "A nice type of the English scientist": Tansley and Freud', in Daniel Pick and Lyndal Roper, eds, *Dreams and History* (London: Brunner-Routledge, 2003) ch. 11; S. Raitt, 'Early British Psychoanalysis and the Medico-Psychological Clinic', *History Workshop Journal* 58 (2004): 63–85.
18. Letters page, *Church Times*, 5 February 1926. This cutting can be found in the rich newspaper archives held at the British Psychoanalytical Society. See, for instance, the various reviews and letters on psychoanalysis, 1925–30, filed as G12/BC/F05. File G12/BC FO3 contains pertinent material from 1903–25.
19. D. H. Lawrence, *Psychoanalysis and the Unconscious* (London: Martin Secker, 1923), pp. 14–16. On Lawrence's relationship with Freud, see Peter Collier and Judy Davies, eds, *Modernism and the European Unconscious* (Cambridge: Polity Press, 1990), ch. 3.
20. 9 December 1939, in Virginia Woolf, *The Diary of Virginia Woolf* [1936–41], (London: Penguin Books, 1985), vol. 5, p. 250.
21. Jones, *Sigmund Freud*, 1: pp. 62–3.
22. J. S. Mill [1873], A*utobiography* (London: Routledge, 1989), pp. 134–5.
23. J. S. Mill [1863], *Utilitarianism, Liberty, Representative Government* (London: J. M. Dent, 1964), p. 37. Mill invited the reader to 'take into consideration, no longer the person who has a confirmed will to do right, but him in whom that virtuous will is still feeble, conquerable by temptation, and not to be fully relied on; by what means can it be strengthened? How can the will to be virtuous, where it does not exist in sufficient force, be implanted or awakened? Only by making the person *desire* virtue – by making him think of it in a pleasurable light, or of its absence in a painful one'; cf. Alexander Bain, *The Emotions and the Will* (London: John W. Parker and Son, 1859) for similar arguments about the role of punishment and reward in shaping the will.
24. Preface to William Carpenter, *Principles of Mental Physiology*, 4th edition (London: Henry S. King, 1876), pp. xxii-xxiii. Carpenter conceded a great deal to the determinist and the materialist, but he was intent on retaining free will; this could be likened, he suggested, to a rider, while the automatic activity of the body approximated to the rider's horse. To a degree, the rider could be expected to keep charge, but now and then 'some unusual excitement calls forth the essential independence of the equine nature' (p. 24). Carpenter noted, however, that a horse might well find its own way home, even when the rider was wrapped in a profound reverie and entirely ceased to guide the animal (p. 24).

25. Evidence about the 'trends' of birth, marriage and death, or about the rates of suicide and murder, also exacerbated doubts about the free decision-making powers of individuals; see Ian Hacking, 'Nineteenth Century Cracks in the Concept of Determinism', *Journal of the History of Ideas* 43 (1983): 455–75, at 470. The emergence of a psychiatric language based on the notion of irresistible 'instincts' is brilliantly analysed by Michel Foucault in his 1974–5 lecture series at the Collège de France, *Abnormal. Lectures at the Collège de France* (New York: Picador, 2003).
26. See Ian Hacking, *Rewriting the Soul: Multiple Personality and the Sciences of Memory* (Princeton, NJ: Princeton University Press, 1995), p. 458.
27. Charles Taylor, *Sources of the Self: The Making of Modern Identity* (Cambridge, MA: Harvard University Press, 1989), p. 36.
28. Stefan Collini, *Liberalism and Sociology: L. T. Hobhouse and Political Argument in England, 1880–1914* (Cambridge: Cambridge University Press, 1979).
29. Brian O'Shaughnessy, *The Will: A Dual Aspect Theory*, 2 vols. (Cambridge: Cambridge University Press, 1980), 1: p. xxii .
30. O'Shaughnessy *The Will*, 1: p. xxiv.
31. For further discussion of Nietzsche's account of diseased will, see G. Moore, *Nietzsche, Biology and Metaphor* (Cambridge: Cambridge University Press, 2002), pp. 126–7. For illuminating general surveys of the shifting conceptual terrain of 'body and will' sketched in the present essay, see B. Haley, *The Healthy Body and Victorian Culture* (Cambridge, MA: Harvard Univerity Press, 1985); and J. R. Reed, *Victorian Will* (Athens, OH, 1989).
32. Friedrich Nietzsche, *Writings from the Late Notebooks* (Cambridge: Cambridge University Press, 2003), p. 207.
33. Quoted in Anson Rabinbach, *The Human Motor: Energy, Fatigue, and the Rise of Modernity* (New York: Basic Books, 1990), p. 162.
34. Ribot's books included *Heredity: A Psychological Study of Its Phenomena, Laws, Causes, and Consequences* (1875), *The Diseases of Personality* (1887) and *The Psychology of the Emotions* (1897).
35. Quoted in C. E. Forth, 'Moral Contagion and the Will: The Crisis of Masculinity in *Fin-de-siècle* France', in Alison Bashford and Claire Hooker, eds,*Contagion: Historical and Cultural Studies* (London: Routledge, 2001), pp. 61–75, at p. 61.
36. 'The term, coined from the Greek, came into clinical use during the 1840s. By 1858, it appeared in some medical dictionaries as 'absence of will, type of insanity in which this symptom is dominant'; G. E. Berrios, and M. Gili, 'Abulia and impulsiveness revisited: a conceptual history', *Acta Psychiatrica Scandinavica* 92 (1995): 161–7, at 164. Ribot referred to the abulias, as did various other late nineteenth-century writers, some of whom enthusiastically adopted the term. Max Nordau referred to 'abulia' in his scaremongering socio-biological text of the 1890s, *Degeneration*. Freud also occasionally adopted the term, for instance in his psychobiographical exploration of Leonardo da Vinci; Sigmund Freud, 'Leonardo da Vinci and a Memory of his Childhood' [1910], SE, 11: p. 131.
37. The translations are from Hacking, *Rewriting the Soul*, pp. 206–7. See also Theodule Ribot, *Diseases of Memory: An Essay in the Positive Psychology* (London, 1912), pp. 38–9, 107.
38. Quoted in Jaap van Ginneken, *Crowds, Psychology and Politics, 1871–1899* (Cambridge: Cambridge University Press, 1992), p. 25. For further examples of late nineteenth-century discussions of what Ellenberger calls 'the polypsychic structure of the mind', see Henri Ellenberger, *The Discovery of the Unconscious:*

The History and Evolution of Dynamic Psychiatry (New York, Basic Books, 1970), p. 167 and *passim*.

39. 'Mafia gang' was a metaphor used by the psychoanalyst Herbert Rosenfeld; for a discussion of Rosenfeld and other related ideas of the 'pathological organization' in psychoanalysis, see John Steiner, *Psychic Retreats. Pathological Organisations in Psychotic, Neurotic, and Borderline Patients* (London: Routledge, 1992).

Chapter 17

1. From a large offering, see Roy Porter, *The Creation of the Modern World: The Untold Story of the British Enlightenment* (New York and London: W. W. Norton, 2000), ch. 6; Roy Porter, *The Greatest Benefit to Mankind: A Medical History of Humanity* (New York: W. W. Norton, 1997), ch. 10; Marie Mulvey Roberts and Roy Porter, eds, *Literature and Medicine during the Eighteenth Century* (London and New York: Routledge, 1993); Roy Porter, 'Bedlam and Parnassus: Mad People's Writing in Georgian England', in George Levine, ed., *One Culture: Essays in Science and Literature* (Madison, WI: University of Wisconsin Press, 1987), pp. 258–84; and Roy Porter, '"The Hunger of Imagination": Approaching Samuel Johnson's Melancholy', in W. F. Bynum, Roy Porter and Michael Shepherd, eds, *The Anatomy of Madness*, 3 vols. (London: Tavistock, 1985), 1: pp. 63–88.
2. Snow, *The Two Cultures and a Second Look* (Cambridge: Cambridge University Press, 1964).
3. Snow, *The Two Cultures*, with an Introduction by Stefan Collini (Cambridge: Cambridge University Press, 1993), pp. vii-lxxiii.
4. The Wordsworth quote is from his poem 'The Tables Turned' (1797–98), and Coleridge's statement comes from *The Definition of Poetry* included in his 1818 *Lectures and Notes*.
5. Thus, one scholar has recently asserted that at this historical juncture the sciences and the humanities became 'divergent cultures of discovery and creation'. After the arrival of Romanticism, another quips 'the dialectic was lost' between the two endeavours, and literature in particular 'becomes an enemy of science and medicine'. See respectively Donard R. Maxwell, *Science or Literature? The Divergent Culture of Discovery and Creation* (New York: Peter Lang, 2000), ch. 1; and Michael Neve, 'Medicine and Literature', in W. F. Bynum and Roy Porter, eds, *Companion Encyclopedia of the History of Medicine*, 2 vols. (London and New York: Routledge, 1993), 2: pp. 1526–7.
6. The list includes Gottfried Benn, André Breton, Bertolt Brecht, Louis-Ferdinand Céline, Anton Chekov, Alphonse Daudet, Arthur Conan Doyle, Max Ernst, Edmond de Goncourt, William James, Karl Jaspers, Robert Musil, Arthur Schnitzler and Gertrude Stein.
7. Limon, *The Place of Fiction in the Time of Science: A Disciplinary History of American Writing* (Cambridge and New York: Cambridge University, 1990), ch. 1; Gillian Beer, *Darwin's Plots: Evolutionary Narrative in Darwin, George Eliot, and Nineteenth-Century Fiction*, 2nd edition (Cambridge and New York: Cambridge University Press, 2000).
8. See also Richer's *L'Art et la médecine* (Paris: Gaultier, Magnier, et Cie, 1901); and *Anatomie artistique*, 2 vols., avec 110 planches (Paris: E. Plon, Nourrit et Cie, 1890).
9. The text of the play may be found in André de Lorde, *Théâtre d'epouvante* (Paris: Charpentier et Fasquelle, 1909), pp. 1–81.

10. Aragon and Breton, 'Le cinquantenaire de l'hystérie (1878–1928)', in Maurice Nadeau, ed., *Histoire du surréalisme: Documents surréalistes* (Paris: Seuil, 1948), p. 125.
11. My thanks to Roberta Bivins for bringing this last likeness to my attention.
12. Kockerbeck, *Ernst Haeckels 'Kunstformen der Natur' und ihr Einfluss auf die deutsche bildende Kunst der Jahrhundertwende* (Frankfurt am Main: Peter Lang, 1986), esp. pp. 43–71.
13. Richard Noll, *The Jung Cult: Origins of a Charismatic Movement* (Princeton, NJ: Princeton University Press, 1994), pp. 47–54.
14. Including, according to Daniel Gasman, those drawn to Nazi *Volkisch* ideology. See Gasman's *Haeckel's Monism and the Birth of Fascist Ideology* (New York: P. Lang, 1998).
15. William R. Everdell, *The First Moderns: Profiles in the Origins of Twentieth-Century Thought* (Chicago and London: University of Chicago Press, 1997), pp. 104, 353.
16. Beaunis, *Théâtre composite* (Cannes: Imprimerie Guiglion, 1911), which reprints nine plays by Beaunis.
17. Kraepelin, *Werden – Sein – Vergehen: Gedichte von Emil Kraepelin* (Munich: J. F. Lehmanns. 1928).
18. Meynert, *Gedichte* (Vienna: W. Braumüller, 1905).
19. Just as novels become more 'psychological'. See Leon Edel, *The Psychological Novel, 1900–1950* (Philadelphia: Lippincott, 1955).
20. Another *fin-de-siècle* medical initiative that was deeply informed by literary precedents is sexology, especially in France and Austria, as Vernon Rosario and Harry Oosterhuis have recently emphasized. See Rosario, 'Inversion's Histories/History's Inversions: Novelizing Fin-de-siècle Homosexuality', in Rosario, ed., *Science and Homosexualities* (New York and London: Routledge, 1997), pp. 89–107; and Harry Oosterhuis, *Stepchildren of Nature: Krafft-Ebing, Psychiatry, and the Making of Sexual Identity* (Chicago: University of Chicago Press, 2000), pp. 26, 49–50, 175–9, 231–7.
21. Michel Serres, *Hermes: Literature, Science, Philosophy* (Baltimore, MD: Johns Hopkins University Press, 1982).
22. Carroy, *Les personnalités doubles et multiples. Entre science et fiction* (Paris: Presses Universitaires de France, 1993).
23. Peter Gay, *Schnitzler's Century: The Making of Middle-Class Culture, 1815–1914* (New York: W. W. Norton, 2002), ch. 1.
24. Jan Fontijn, *Trots Verbrijzeld: Het leven van Frederik van Eeden vanof 1901* (Amsterdam: Querido, 1996); Jan Fontijn, *Tweespalt: Het leven van Frederik van Eeden tot 1901* (Amsterdam: Querido, 1999).
25. Van Eeden, *The Bride of Dreams*, transl. Mellie von Auw (New York and London: M. Kennerley, 1913); Van Eeden, *Gedichten: Een bloemlezing* (Amsterdam: Wereld biblioteck, 1949); and Van Eeden, *Studies*, 5 vols. (Amsterdam: W. Versluys, 1894–190[?]), which includes essays on Dante, Hals, Rembrandt, Thoreau, Ruskin and Van Gogh. The predominance among these examples of doctors in dynamic psychiatry might suggest that this type of psychology, with its overt humanistic content, was especially congenial to cross-cultural contact. Conversely, we might expect scientists such as Wilhelm Wundt, with his programme of laboratory experimental psychology, and Ivan Pavlov, with his embrace of physiological reflexology, to have shunned the ideas, methods and insights of the humanities. But the artistic activities cited above of Charcot, Beaunis, Binet, Meynert, Kraepelin and Ramón y Cajal, who all worked in anatomical, physiological or histological traditions, calls this generalization into

question. It is well to recall that the most widely read literary work authored by a psychologist in the twentieth century was B. F. Skinner's *Walden Two* (1948).

26. Carroy, 'Playing with Signatures: The Young Charles Richet', in Mark S. Micale, ed., *The Mind of Modernism: Medicine, Psychology, and the Cultural Arts in Europe and America* (Stanford, CA: Stanford University Press, 2004), ch. 7.
27. This same era represents as well the highpoint of Western positivist science, which typically characterized the sciences, with their dispassionate methods and objectivist rhetoric, as a masculine force. However if the artistic/cultural influences on *fin-de-siècle* science were as extensive as I am maintaining, and the arts were correspondingly cast as more feminine enterprises, then this line of analysis also suggests a reassessment of the gendered identity of science as a Modernist cultural practice and body of knowledge.
28. Janik, *Wittgenstein's Vienna Revisited* (New Brunswick, NJ: Transaction Books, 2001), pp. 189–90.
29. Hughes, *Consciousness and Society: The Reorientation of European Social Thought, 1890–1930* (New York: Knopf, 1958), ch. 2.
30. Roy Porter, ' "The Whole Secret of Health": Mind, Body, and Medicine in *Tristram Shandy*', in John Christie and Sally Shuttleworth, eds, *Nature Transfigured: Science and Literature, 1700–1900* (Manchester: Manchester University Press, 1989), ch. 3; Roy Porter, 'Against the Spleen', in Valerie Grosvenor Myer, ed., *Laurence Sterne: Riddles of Mysteries* (Totowa, NJ: Barnes & Noble, 1984), ch. 6; and Roy Porter, *Flesh in the Age of Reason* (New York: W. W. Norton, 2003), ch. 17.

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